

Metropolis Health. (METROPOLIS)

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BLOOD TESTS • DIAGNOSTICS • WELLNESS

Metropolis Healthcare Limited

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To,
Head, Listing Compliance
Department
BSE Limited
Phiroze Jeejeebhoy Towers
Dalal Street, Mumbai - 400001.

Scrip Code: 542650 To,
Head, Listing Compliance Department
National Stock Exchange of India
Limited
Exchange Plaza, Plot No. C/1. G Block,
Bandra -Kurla Complex, Bandra (East),
Mumbai- 400051.

Scrip Symbol: METROPOLIS

Sub: Intimation of Transcript of Earnings Call for Q4/FY 22-23

Ref: Intimation under Regulation 30 of SEBI (Listing Obligations and Disclosure Requirements) Regulations, 2015

Dear Sir/ Madam,

This is in continuation to our letter bearing Reference No. MHL/Sec&Legal/2023-24/08 dated May 11, 2023. Please find enclosed herewith the transcript of Q4/FY 22-23 Conference Call (i.e. Post Earnings/Quarterly Call), as organized by Axis Capital Limited on Wednesday, May 17, 2023 at 9:00 a.m. (IST) wherein the financial results of the Company for the quarter ended March 31, 2023, were discussed.

The same has also been uploaded on the Company's website which can be accessed at www.metropolisindia.com

We request you to kindly take the above information on record.

Thanking you.

Yours faithfully

For Metropolis Healthcare Limited

Rakesh Agarwal
Chief Financial Officer

Q4 FY '23 Earnings Conference Call "
May 17, 2023

Disclaimer: E&OE - This transcript is edited for factual errors. In case of discrepancy, the audio recordings uploaded on the stock exchange on 17th May 2023 will prevail.

MANAGEMENT : MS. AMEERA SHAH – MANAGING DIRECTOR –
METROPOLIS HEALTHCARE LIMITED
MR. SURENDRAN CHEMMENKOTIL – CHIEF
EXECUTIVE OFFICER – METROPOLIS HEALTHCARE
LIMITED
MR. RAKESH AGARWAL – CHIEF FINANCIAL OFFICER
– METROPOLIS HEALTHCARE LIMITED

MODERATOR : MR. PRAKASH AGARWAL – AXIS CAPITAL LIMITED

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Moderator: Ladies and gentlemen, welcome to the Q4 FY 2023 Earnings Conference Call of Metropolis Healthcare Limited, hosted by Axis Capital Limited. This conference call may contain forward-looking statements about the company, which are based on the beliefs, opinions and expectations of the company as on the date of this call. These statements do not guarantee the future performance of the company, and it may involve risks and uncertainties that are difficult to predict.

As a reminder, all participant lines will be in the listen-only mode and there will be an opportunity for you to ask questions after the presentation concludes. Should you need assistance during the conference call, please signal an operator by pressing star, then zero on your touchtone phone. Please note that this conference is being recorded.

I now hand the conference over to Mr. Prakash Agarwal from Axis Capital Limited. Thank you, and over to you, sir.

Prakash Agarwal: Thanks, Ryan, and good morning, everyone. On behalf of Axis Capital, I, Prakash Agarwal, welcome you all for the Q4 Fiscal '23 Earnings Call for Metropolis Healthcare Limited. Today, from the management, we have Ms. Ameera Shah, Managing Director, Mr. Surendran Chemmenkotil, CEO, and Mr. Rakesh Agarwal, Chief Financial Officer.

With this, I hand over the call to Ms. Ameera for her opening remarks. Thank you.

Ameera Shah: Good morning, everyone, and thank you for joining us for the Q4 FY '23 earnings call. As mentioned, I'm joined by Suren and Rakesh and our IR advisors, SGA. We've uploaded our updated investor presentation and press release on the stock exchanges and the website, and I hope you have the opportunity to go through the same.

FY '23 has been a financially challenging year for the diagnostics industry. While we witnessed the entry of many new competitors leading to more B2B discounts and decentralized testing, on the B2C side, the industry witnessed certain changes in consumer trends like the increased adoption of digital technology. At the same time, there was a mismatch in the profit and loss in the first quarter, but COVID revenues fell off steeply, while manpower cost from previous years has increased due to COVID. The new industry trends in the last 18 to 24 months have weakened the unorganized players with decline in volume growth as consumers are shifting to bigger brands and better customer experience.

Further, in the absence of a regulatory environment, consumers are assuming the 2,000 labs who are approved for COVID PCR tests are the more advanced labs compared to the rest of the unorganized sector of 300,000 labs and migrating to these 2,000 due to brand. This will provide a long runway of growth for the organized sector.

Let me update you on the competitive landscape in the industry. Investors have been hearing a lot about competitive intensity in the last year. And while it is certainly true that the entry of many new competitors has brought a different dynamic to the table, the ground reality is that the Metropolis Healthcare Limited

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competitive intensity is more nuanced than general and high in some pockets and not high in some. The industry has been witnessing pricing pressure, particularly in the B2B semi-specialized segment. Where the intermediate margins or customer discounts are expanding, the price is not changing for the customer. And for Metropolis, this segment comprises a single digit of our overall revenue. This is the easier entry point into the industry as the most scientific approach takes great expertise and time to build trust.

We are not facing heat in pricing on the B2C side as customers come to us when they are unwell for critical areas, and pricing is truly not the major factor in their decision making or in the specialized segment in B2B, where, again, trust

and quality drive prescription. This business, the value wellness and the highly discounted B2B was never our focus and, therefore, does not dent Metropolis growth prospects. Even in our core markets like Bombay and Pune, there was a fear of competitive intensity impacting Metropolis growth.

We have not seen this to be an obstacle in our growth in FY '23. In fact, our growth in Mumbai and Pune has been extremely strong and outperformed all peers. Equipped with a strong scientific brand and opportunity to build density of centres in these markets, along with strong prescription generation from doctor and B2C activity, we believe a double-digit growth will continue in our key markets.

As more illness volumes increased and as we have already now seen non-COVID normalized, the organized players will be the biggest beneficiary and will grow at a faster pace vis-à-vis the players which are focusing on discounts and low pricing first rather than science and customers. At Metropolis, we are successful in building this trust amongst the doctor fraternity and our consumers, which will help us with more sticky business and repeat testing.

Specifically, FY '23 was a challenging year for Metropolis but also a critical year for our transitioning to Metropolis 3.0. Some of these challenges were external challenges like the financial impact of a steep drop of COVID test -- PCR testing, higher time for normalization of non-COVID testing, many new competitors entering during COVID, which increased market intensity and the digital adoption by consumers, which itself was new in health care and require the industry to build new capability.

Some were internal challenges like the change of the CEO, the vast rumour surrounding Metropolis and the promoters' intent to raise investments, the unexpected income tax surge faced by us and several large health care players and an unethical competitor using the Hitech brand for nine months and trying to fool consumers despite a legal trademark owned by Metropolis. While some of these affected the momentum of business in FY '23, the long-term opportunity continues to be robust, and I'm happy to share that despite these challenges, Metropolis successfully reported 18% non-COVID revenue growth, which is the highest amongst leading players in the industry. Even excluding COVID, excluding the government contracts and excluding the acquisition, the growth was 15% similar to pre-COVID.

We continue to keep a laser-sharp focus on execution, and we strengthened the quality of our business last year. We retained and expanded our talent pool, grew our network significantly

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and built material new capabilities for the future. We have also just concluded our Metropolis 3.0, 3-year strategic planning, and we'll be happy to share details in the next quarter's investor call. However, we would like to touch briefly upon a few of the points.

We will be looking at a very aggressive network expansion in the next three years. We will focus on penetrating deeper in our core geographies by expanding our reach into Tier 2 and Tier 3 towns, and these are locations where Metropolis enjoys brand recognition. We're also widening our geography by entering relatively new markets in North and East and have been able to successfully grow in these markets as well.

In this last year, we have opened five new labs in the North in Bareilly, Varca, Ambala, Noida and Faridabad and have grown faster than competition in the specialized segment. Our growth in North India was at 30% in Q4 FY '23 due to improved brand recognition amongst doctors and hospitals and more D2C activities, taking our brand to the consumer for wellness. We continue to commit our energies and resources to our strategy of opening 90 labs and 1,800-plus centres by 2025 across the country. Of which, we have so far created 30

labs and 1,057 centres in the last 2 years.

The second thing we'll be focusing on will be wellness testing. We've created the right set of

wellness packages based on data analytics and science and have been educating consumers via our D2C engagement model for wellness packages. We believe that wellness will be a strong pillar of growth for Metropolis going forward, and we don't use price as a USP like many peers in the industry but the brand promise of better content that you could use to improve your health, the trust that your results are accurate and the ease to engage with Metropolis with the click of a button. Our average realization for these premium packages is INR2,000 -plus, and these tests are margin -accretive to Metropolis. We -- in Metropolis 3.0, we will continue to focus on our wellness business and take our current contribution of 15% to closer to 20% in the future.

The third area we will focus on is our specialized test menu as we know that Metropolis has the widest range of test menu on our umbrella, and we are continuously adding more and more. This last year, we added 83 new tests in FY '23. And in this next year, we will continue to engage with the scientific community, educate them on the better and alternative tests available with Metropolis and add one test every three days.

We are also adding people in our sales force to strengthen and accelerate our doctor engagement activities in our new geographies to gain doctor's trust for higher volumes, especially for specialized test. We have increased our doctor outreach program by conducting CMEs, which is continuing medical education programs, and approach new specialized doctors in our core geography and new territories as well.

We have done approximately 200 CMEs in the FY '23 and have approached more than 10,000 new doctors through one -on-one connect and 20,000 doctors digitally through Met robots , especially the doctors practicing in the field of oncology, gastroenterology and neurosurgeons. These are all the specialized doctors in the country. We're happy to inform you that these doctors have been continuously engaging with our teams to exchange thoughts for better outcome. In

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the next year, we plan to conduct more than 500 CMEs and continue to expand our reach to many doctors on a continuous basis.

The fourth thing that we'll focus on will be to continue to strengthen our IT infrastructure. As we know, we have been driving significant growth via digital channels, and we have adapted from being only scientifically focused pre -COVID and driving doctor -led B2C business to now also being a brand -led business driving D2C, which is direct -to-consumer testing. We have built our new technology stack with new and improved app, redesigned website and a new channel via WhatsApp messenger that will help us serve all our customers better and drive more efficiencies in the business.

Pilots of the new technology have been rolled out and will be completed in quarter 1 of this year across India. This will significantly improve customer experience. Although our NPS is already high, we believe that this can go even further and improve our visibility and data analytics in the business.

If we look at -- the last opportunity will be the inorganic opportunity and a complementary business. Apart from the organic growth, we'll continue to explore bolt -on acquisitions to grow via the organic route . We believe sooner or later, the industry has to consolidate with few large players controlling the majority of the market share on a Pan -India basis, with some regional competition in specified pockets. We continuously look out for opportunities, which will be a strategic fit for Metropolis in terms of culture, DNA, sticky consumer play, but at the right track. We're also exploring opportunities in the basic radiology space, which

is a strategic cross -sell

for Metropolis due to its wide reach to its network across the country and brand equity. Basic radiology usually is things like ECG, x -ray, sonography and not MRI and CT.

We are extremely happy and proud to inform that Metropolis is the first diagnostics company certified as a great place to work. In the industry of a short pool of talent, we have been able to retain and add more talent to our organization.

I'd like to just quickly comment on our ESG initiative. And most of our initiatives will be focused on energy conservation, water and waste management, strengthening the diversity and inclusion program and improving the diversity and new hire ratio, launch of an exclusive program on women's leadership. Foster -- we foster diverse and accountable governance by partnering with external consultants to initiate with the consulting firm the process of ISO, continue to maintain high ethical standards, protecting and securing data infrastructure and identity and, from a governance perspective, complete patient privacy and electronic health records. We have enabled the program to set targets for each of these and track our progress against these consistently.

With this, I'd like to hand over to Suren for our update on the year gone by along with the execution plans going forward. He has nicely settled into Metropolis and is bringing in stronger execution rigor, and I'm confident of the success of our plan with the new momentum that I'm

seeing.
Over to you, Suren.
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Surendran Chemmenkotil: Thank you, Ameera, and good morning, everyone. So on business trend, while our reported numbers show a decline, the underlying story is very promising. Our core business growth, that is revenue excluding COVID testing, Hitech and the PPP contract, which is not our core focus area, our revenues for quarter 4 grew by 15% year-on-year, which is at pre pandemic levels. The same 15% growth was delivered for the whole year as well.

The PPP contract impact was due to government decision to insource a business, which is earlier managed by Metropolis. All tender business will come to an end, depending upon the year it was contracted for. This has impacted quarter 4 numbers. However, the positive news is that the core business revenue is growing faster than the market and the peers, driven by volume growth of 13% and RPP increase of 2% for full year '23.

The quality of growth is strong, as driven by B2C revenue growth, which has been 17% for quarter 4 '23 and 20% for full year '23. We have been able to deliver 18% B2C growth in Mumbai and 23% B2C growth in Pune, highlighting our strength in our core geographies despite competition from existing players and entry of new players. The growth in this market is a mix of volume, RPP and product mix.

At the end of March, we have taken a price increase of approximately 4% on about 900 low-volume tests, which should give a net benefit of -- on overall revenues of approximately 1% for full year '23/'24. And also, we will evaluate a price increase on other cash in the course of the year.

For margins, our operating EBITDA margins are comparable to pre-COVID in the range of 37% while the reported margin is about 25.5%. The cap is on account of new investments in network expansion, which has diluted the margin by about 120 bps.

Despite of this aggressive lab expansion, we have delivered the highest margins amongst similar industry players. This has been a combination of operational efficiency, realization benefits and product mix, giving -- driving specialized test and premium wellness. We believe with normalized volumes and higher productivity from new business -- new investments in the network, we will be able to deliver better margins in the future. In prescriptions from doctors for illness and specialized B2B segment, where accuracy and trust

are more critical, we do not envisage much pricing pressure.

We believe technology is important for the evolving business needs, and during COVID, while we saw digital platforms play a super leading role in all industries, including health care, this trend has not continued as significantly post-COVID. Now the use of technology is more inclined for servicing the consumer and enhancing the overall experience less as a customer acquisition tool. While many wellness and affording tech-savvy consumers prefer digital booking and home visit, many middle-class customers on the illness that prefer walking to a centre for the diagnostics requirement or getting a home visit via phone. Hence, we have been deploying equal focus on the digital platform and our network expansion strategy.

A Metropolis customer can access services as comfortably through both options. While home visits continue to grow, this is a normalized growth and not a hockey-stick growth. While most

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peers do home visits through franchisee centres, at Metropolis, part of it gets accounted directly and part of it is scattered through our retail franchisee centres. Both get accounted as a part of the B2C revenue, but it is difficult to differentiate home visit versus walk-in at the retail franchisee centre level.

Moving on to update on Hitech. As mentioned, Hitech was a strategic fit under the Metropolis umbrella to expand a middle-income brand in Tamil Nadu. As mentioned, poaching by unethical player in the South, which have impacted revenue growth of -- for about nine months. However, we defended our trademark and won and stabilized our operations and brand and still grew positively. We are confident on Hitech business growing. Going forward, our strategy will be add another 50 centres in the next one year, aiming at faster industry growth in full year '24, focus on margin expansion with increased efficiency and productivity to operating leverage playing out.

Margins in Hitech for full year '23 stood at 27%, which we believe will increase going forward. Addition of test menu to attain better product mix and productivity per centre, deployment of

talent especially from the scientific engagement perspective to cross-sell more specialized tests, which is currently offered by Metropolis, also by creating a digital journey for customers for acquisition and servicing of customers.

Coming to the strategy for Metropolis going forward. We will have a clear focus on expanding the network aggressively. We aim to expand our lab network in strong states of Karnataka, Gujarat and Maharashtra to provide complete testing services in Tier 2, Tier 3 markets and strengthen our leadership position at the state level.

Additionally, we plan to enter strategic locations to penetrate newer markets, especially in the states of Assam, Bihar, West Bengal, Madhya Pradesh, Uttar Pradesh, Orissa, Jharkhand, Telangana and Andhra Pradesh. We plan to open 30 new labs, around 800 centres in full year '24 as compared to 14 labs and 540 centres last year.

While the largest network will surely drive faster volumes and revenue, we would also want to increase the visibility of Metropolis brand across the country and improve revenue productivity per centre. We have deployed a lot of data analytics in the strategy planning of labs and collection centres and a separate distribution and project team, which will be directly reporting to me, for opening a lab until they reach maturity with a stipulated time frame. With this focus, our goal is to start lab and centres on time and improve success rates on opening new centres. With stronger execution rigor, we believe the business will be on track to deliver better volumes and growth. Not only our focus will be to expand our network but also, as I mentioned, focus on enhancing the productivity of each centre

and, especially the young network through our D2C engagement model.

We have been working on -- working to strengthen our digital and technology capabilities over the last one year. We have been introducing various B2C engagement programs via our web, mobile app and social media platforms. Through these engagements, we ensure customer acquisition and educating the customer for the right set of tests, packages or symptom to take an

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appropriate test. We have recently enabled our system to engage customers via WhatsApp chat box. It's a virtual, which will help to solve most of the queries via WhatsApp chat and cross-sell services. The feature is a function like test booking, centre location optimization, reports with historical trends, suggested test, et cetera, and also be able to track the life cycle of a member, enabling us to have focused engagement.

We believe in our industry, whether for ordering medicine or booking diagnostic tests, these are transaction purchases where some consumers log into the apps only when they need to order something or to book a test. We do not see a much browsing time on the app by how much engagement over content. And this is seen by the low time spent by the consumers on these apps. So while Metropolis is completely omnichannel, we believe this will be used more for servicing our customers and for customer life cycle management. Online customer acquisition will happen primarily via the website, and majority of customer acquisition will be soon off-line.

On the back end, we have been piloted our new sales and service CRM and registration tools which will enable the customer experience. In full year '24, we will roll out significant lab automation and process automation for better productivity and a separate module for our B2B customers for ease of doing business. We are continuously working on increasing our revenues through digital media and alongside making our back-end system stronger for better process and control for enhancing productivity.

Now on the wellness. We have also been -- we have seen volumes picking up from the wellness segment and are hopeful of this trend to continue. Additionally, the spread of health care services and insurance has made the diagnostic industry the critical segment of the industry. The collaboration of various corporates and MNCs, diagnostic centres for the health care check-up for their staff and families will further boost the volume, especially for the large, organized players on a Pan-India basis.

For Metropolis, our wellness segment has grown exceptionally -- shown an exceptional growth of 45% last year, with revenue contribution moving to 15% of the total pie. We believe the opportunity here continues, and we aim to take this number close to 20%.

On the D2C engagements, we have done about 800 number of health check-up camps in the last year and would be accelerating these efforts to engage more direct-to-consumer for targeting customers of repeat testing. These health check-up scans are basically done for large societies or to capture a catchment area within a specified geography. These camps typically add more branding and customer outreach for Metropolis, which will efficiently help us in consumer acquisition and retention. This will act as a funnel for customer acquisition and cross-sell and upsell for chronic as well as wellness testing.

Now to sum it up, we have witnessed a consistent high-teen to mid-teen growth of non-COVID

revenues for the last four quarters, not only by continuing to be very respected doctors across the country but also by introducing technology and consumer connect that makes life easier for consumer. In quarter 4, we have seen 15% growth in our core diagnostics business, excluding Hitech and PPP contracts. We believe Metropolis is rightly placed to attain higher market share

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across geographies and should be able to outperform the industry growth on revenue front with better margins to follow.

With that, I will hand over to Rakesh Agarwal, CFO of Metropolis Healthcare.

Rakesh Agarwal: Yes. Thanks, Suren, and good morning, everyone. I will take you through a few key highlights for the quarter and year -ended full year '23, following which, we will open the floor for Q&A session.

Let me begin with the operational highlights. First, our B2C revenue has grown at 17% for Q4 financial year '23 and 20% for full year financial '23, indicating our strong doctor and consumer connect across markets. Our B2C business for our focus cities like Mumbai and Pune has grown by 18% and 23%, respectively. This is again witnessing our strong brand presence in our core geographies. The growth in focus cities has been a mix of volume and value growth, volume growth indicating increase in market share and the value growth indicating better product mix and sustainable prices.

Revenue share of the B2C business for focus cities stood at 60% in 2023, which sticks to our target of 65% contribution from B2C business in focus cities in the short to midterm. Revenue contribution from B2C stood at 50% of the total revenue as compared to 47% in financial year '22 and 42% in financial '20. Our B2C includes revenue from home visit and walk-in at our own and franchisee centres. It does not include B2B collections done by franchisee centres revenue from ARC. That is exclusive to the franchisee centres.

Our specialized and premium wellness segments are the fastest-growing segment. Revenue contribution from specialized test, excluding COVID and allied test, stood at 33% for quarter 4 financial year '23, indicating a growth of 16% on a Y-o-Y basis. Contribution from revenue from wellness increased from 11% in Q4 financial year '22 to 15% in Q4 financial '23, with a growth of 43% on a Y-o-Y basis. And the segment for full year '23 grew by 45%.

We have not only seen growth in our core geographies but also on our noncore geographies. Our revenue in noncore market like North has grown by 30% for Q4 financial year '23 on a Y-o-Y basis. On this, 25% has been account of volume growth and 5% on account of realized benefit. Of the above core growth of 15%, 14% has been driven by volume growth. And 1% has been on account of the realization benefit.

Let us now come to the Q4 and full year financial year key financial highlights. Total revenue for Q4 financial year '23 stood at INR283 crores and for financial year '23 stood at INR1,148 crores. Non-COVID revenue for financial year '23 stood at INR1,104 crores, at a growth of 18% for financial year '23 on Y-o-Y basis. Non-COVID revenue for Q4 financial year '23 grew by 9% for Q4 financial year '23 and stood at INR276 crores.

EBITDA before CSR and ESOP is at INR72.5 crores in Q4 financial year '23 as compared to INR82.2 crores in Q4 financial '22. EBITDA before CSR and ESOP for financial year stood at INR304.4 crores. EBITDA margin before CSR and ESOP stood at 25.7% for Q4 financial year '23 and 26.5% for financial year '23.

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Profit after tax stood at INR33.5 crores in quarter 4 financial year '23. PAT margin has been impacted on account of higher interest costs due to acquisition of Hitech Diagnostic and higher depreciation on account of investment done to fuel the future engine growth. Since we have paid our majority of the debt and balance INR79 crores is planned to be paid in H1 of this year, we expect increase in PAT margin going forward.

PAT for financial year '23 stood at INR143.4 crores as compared to INR214.7 crores, down by 33%. However, we had onetime exception gain in financial year '23 -- '22 of INR16 crores. If you exclude that one-off gain from the base year,

PAT will be down by 26%. We envisage PAT margin to grow going forward on the back of low interest cost on account of debt repayment by H1 of this financial year.

Coming to our working capital ratios, our debtor days as on March '23 stood at 30 days, and there is a slight improvement as compared to March '22. Overall, working capital stood at 14

days as of March '23, which is a similar level as compared to March '22. Our OFC to EBITDA is stronger at 102% for financial year '23. Cash and cash equivalents stood at INR106 crores as of March '23. Gross debt stood at INR79 crores as of March '23. We plan to repay this debt by H1 financial year '24.

This is all from our side. We now leave the floor open for Q&A. Thank you.

Moderator: Our first question comes from Tanmay Gandhi with Investec.

Tanmay Gandhi: So my first question is on the PPP revenue. So it accounted for almost 5% to 6% of the revenue in 4Q FY '22, right? So why is this not a core segment for us to focus on, given that it's margin - accretive, realization - accretive? And why were we unable to renew the contract given we have strong presence in specialized testing?

Ameera Shah: Thanks for the question, Tanmay. I think PPP has not been a focus area for Metropolis because, frankly, this contract was a one -off contract, which focused on specialized tests. All the other PPP contracts in India only focus on routine test, and it's very commoditized. And the quality of service is expected, and therefore, the price connection we don't feel it matches the kind of product that Metropolis provides. So therefore, it is not a core focus for us.

The NACO contract was a one -off contract, which focused on specialized test, which is why we had bid for it, and we won it for the initial three years, which then got extended for another two years because they were very happy with our work. At the end of five years, the government decided to do this testing on their own. So actually, they have not done a new tender at all, and they have not given it to anybody else either. They decided to insource it.

So tender business, as we know, will come and have a contractual period. And therefore, our focus has always been more on the core business, which is about sort of a continuing revenue, which is not based on a contracted business in short period of year.

Tanmay Gandhi: Understood. And secondly, on the realization front. So if you look at our wellness, the realization, so it has declined by 8% to 9% year -on-year. And even if you look at last three, four Metropolis Healthcare Limited

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quarters, the trend, it has been declining. So anything to read into it? And how do we expect realization growth going forward?

Ameera Shah: See, I think Metropolis has always been in the premium wellness space. And if you just step back and look at that, our earlier realizations were close to 2,800. They are currently close to 2,200. So it's really not in the mass space at all where most mass health check -ups are in 500, 400, 600. So we are very much still in the premium wellness space. We're just based on different markets.

So for example, in our urban city or in a city like Bombay, you may -- we still get much higher realization or Pune, our other market. But in obviously, smaller markets, your realizations would be lesser. So as the geography mix is changing slightly and as we are trying to create even more inclusivity into our packages, our range is really between 2,000 to 3,000, and we'll expect to maintain within that.

Moderator: Our next question comes from Shyam Srinivasan with Goldman Sachs.

Shyam Srinivasan : Just the first one is on volume growth. I think core -- like what you're calling out, core business volume growth of 15%. Can you help us disaggregate it into like same -store plus network? I'm just trying to see what's the impact of the new network additions that we have done o

ver the last

couple of years. How -- what is the kind of volumes it's contributing versus the -- our pre-existing stores?

Rakesh Agarwal: So you are right that our core volume growth is around 14% for the full year and 15% for quarter ending Q4. And the total revenue contributed by the new network, which is coming after April '22 is around 4%, and this is increasing as the network is maturing. So whatever came after April '22, the contribution is 4% as of now on the total revenue. And the volume contribution is again in a more or less similar range, and we expect this to grow as we mature in the future.

Shyam Srinivasan: So if I then break it down, 10% has come from old stores before April 2022, and 4% has come from stores after 2022. Is that correct?

Rakesh Agarwal: Yes.

Shyam Srinivasan: So in terms of the 10% growth on the mature stores, if I can use the term, so what is driving growth there for us specifically? Is it penetration increasing at these stores? Or if you can help us understand the dynamics there, please?

Rakesh Agarwal: So yes, definitely, one is obviously the patient coming in per store is increasing for us, and we are seeing that continuously. We are going -- as rightly mentioned earlier also that the D2C, which is -- we are increasing in our core areas, and we are doing a lot of health camps , which is giving us a lot of visibility. So we are continuously seeing the patient per centre going up in our own centre as well as franchisee centre , and that is the major area which is increasing our volume and growth in the current centres .

Shyam Srinivasan: Just last couple of questions. I'll keep it short. Mr. Surendran talked about price increases, 4% price hike in March. So what's the motivation for taking price increases? And when you talk
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about evaluating price increase for the rest of the portfolio, so what gives us the comfort? I think Ms. Ameera also talked about competition. So just trying to weigh in the competitive dynamics versus the price hike, what we are contemplating.

Rakesh Agarwal: So this price hike of 4% is done basically on a very selected test of 900. So we have not changed the overall portfolio. This 900 test are basically a specialized segment test where there is a low volume, and we feel that the competition pressure is much lower in this 800, 900 test. And that is the reason why we have taken a bit of a moderating price increase in this test, and as said by Surendran, this whole increase of price will give us around 1% of the growth in the coming year, which will be full year '23/'24 and we will contemplate as we go forward in '23/'24 that whether we have further opportunities of price increase because we have seen our competitor actually increasing price more aggressively, but we have been bit muted in this. But we'll see if the opportunity arise, and we'll get this opportunity '23/'24.

Ameera Shah: Just to add to that, we believe that the brand premium is strong and people would accept price increases. But I think the key question to ask at this point is the focus on volume or is the focus on RPP realization. So while we believe that there is an acceptance of price increase, we are choosing at this point to grow the volume without really impacting realization through price and really impacting realizations more through product mix as we keep moving up the ladder and as we keep trying to push more and more specialized test. So I think that's more of our strategy. But we believe like we said that the brand premium can sustain the price increase if we choose to go in that direction.

Shyam Srinivasan: Very helpful. Just the last data point, maybe you can get it off-line as well, but just a split of volume into RT-PCR. I know it's getting smaller PPP contract, core volume when you call it, which is non-COVID, non-PPP, non-Hitech, so if y

ou can get the split, even later is fine, but just a request.

Surendran Chemmenkotil: Yes, yes, we will be get back to you.

Moderator: Our next question comes from Rahul Agarwal with Incred Capital.

Rahul Agarwal: First question to Ameera, essentially on the market behaviour and the sentiment. So in a 3-part question, have you seen entry of new players, specifically in the last 12 months? Because I have seen none.

Second is in terms of employee attrition behaviour. Do you see that has normalized for national brands like you? Because it was -- it seemed like it was higher when there were a lot of entry of new digital labs.

And third is on the -- since normal life is back and the normal infections are back, obviously, that helps non-COVID volumes, do you see next three years better than the last three years specifically on a non-COVID business? That's the first question.

Ameera Shah: Thank you. Thanks for the question. For sure, I'll start with the last one. I believe the next three years will be definitely superior to the last three years as far as non-COVID goes.

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Obviously, the two years of COVID really brought down the non-COVID business. And obviously, this last year of a lot of consumer trends and disruption happening with competitive entry as well as -- it's just the trends changing, and more than anything, non-COVID not coming back to its normalcy was, like I said, a challenging year. So I believe the next three years for the industry for the organized sector will be, I think, quite strong for non-COVID. And I certainly think Metropolis will be ahead of the pack just from a perspective of the opportunity for us as well as the execution plan that we have.

Your second question. As far as new people come in, in the last 12 months, I would say after the announcements that were made of a bunch of pharma companies coming in, hospital companies, etcetera, I think some of them are still in preparation mode. A couple of them have not launched yet. One of them has not launched yet, and I think that launch will happen this year of a pharma company. But I think otherwise, everybody has already launched and is already operating in the market. So we don't -- we are not expecting at this point any new entrants to come in who have not already come in.

And thirdly, your question around attrition. And just one point that even when these new players have come in, like we said, we have not really seen any of the new players doing anything in a very

different way. It's definitely more of sort of the similar and pursuing two parts of the business. One is the value wellness business, which is around INR500, INR600 patient as well as the B2B discounted business, which is the semi-specialized, where again, the revenue per test tend to be much lower, not only because these tests tend to be less critical clinically but also because of the high discounting. So we are not expecting to see anything very different from that from any new players coming in.

On the last question of attrition, the attrition we saw during the COVID year was more on the scientific side because everybody who wanted to start COVID testing was looking for trained people in the molecular biology department. So we certainly saw technical team attrition at that time. The last year, the attrition we saw was actually not because of the digital players, was more because of the players from hospitals or pharma companies entering and who wanted salespeople in their off-line work, right?

Actually, if you see the people who started as digital players have actually all moved to off-line as well because they've understood that the demand online, which actually spiked during COVID for COVID testing as well as for wellness, has definitely come down, and that demand is now not sustaining that growth of revenues anymore. And therefore,

all of them are attempting to

again go after the B2B discounted semi-specialized business off-line. And therefore, their cost base is increasing quite dramatically. And without obviously -- or the lack of sort of the large test menu or all of that, the challenges will be obviously different. But I think we're definitely seeing the competitive intensity not be as hectic as it was last year.

Rahul Agarwal: Perfect. And second question is on numbers. Just specifically on PPP, the INR76 crores in fiscal '22 has come down to INR67 crores. What should we expect for fiscal '24? Because I'm not really sure what was a macro annual revenue.

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And secondly, on the margins, does that hold up at current levels, 25.5% going forward? Or do you expect more dilution because of your investment into infra and IT?

Ameera Shah: Sure. So on the PPP, we -- like we said, the PPP contract ended in February of '23. And therefore, it has not reflected completely in this Q4 either. It only reflected for one month in Q4. And the two months in Q4, there was no revenue from PPP. Obviously, the business will not continue into FY '23/'24, and that was about INR67 crores for the whole year in FY '22/'23.

As far as the margin goes, we are definitely a spirational that we would continue to protect the current margin. As you know, we are continuing on the expansion, lab expansion. We are adding 30 labs for this year along with 600 to 800 centres. And therefore, we want to continue to focus on expanding our market, our market share and our revenue growth, and we believe that this investment in margin, while it will continue to dilute instead of operational EBITDA at 27%, we have reported EBITDA of 25.5%. This 1.5%, we'll continue to invest back in the business because we know that at the point we stop investing in new labs, we can very easily move back to an operational EBITDA level. So I think that will continue to be our plan over the next couple of years.

Rahul Agarwal: Just a clarification, the INR67 crores was entirely in NACO, is it?

Ameera Shah: Yes, that's right.

Moderator: Our next question comes from Anuj Saleja with ICICI Prudential Life.

Anuj Saleja: Yes. So just one question on the lease liabilities front. If you can just explain -- so I understand that labs have -- more labs have come in, but lease liabilities are up from INR120 crores broadly to about INR186 crores year-on-year. So could you please explain what all is included in these lease liabilities?

Rakesh Agarwal: Yes. So basically, the lease liabilities consist of two, three things. One, obviously, we keep moving from the rental piece of the equipment, which we purchased, so there is a two modelling equipment. One is the purchase model. One is the rental model. The moment we go with the rental model, there is one lease component, which is already included in that. So last year, we have moved a lot of contracts of our -- from outright purchase model to lease model because we had seen a lot of benefit out of it. And you can see overall, our gross margin has increased. So that is one area where the more we shift from a purchase model to a lease model in the -- from the equipment point of view, we will see this lease liability going up.

Obviously, second is that our -- the moment we opened a lot of A-PSC, there is a rent component, which is one of the major component coming into that. So as we increase our centre and we again have an aggressive plan this year, so the moment we increase our -- this centre so that rent component will come as a lease liability.

So these are the two major components. But I will say that this equipment -- change of equipment contracts from outright purchase to a lease model has made some difference in this amount.

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Anuj Saleja: Okay. So

that's one. Just one last question on the borrowing fees. So borrowings have come down significantly, which is a really big positive from INR259 crores to broadly about INR78 crores, INR80 crores now. So would it be right to say that you've already paid up the loan that you've taken for Hitech?

Rakesh Agarwal: Yes. So there is only INR79 crores left out, which we are planning to pay in H1 this year. And that will also help us to reduce our interest costs, which may reflect and extending our PAT margin as we ll.

Moderator: Our next question comes from Prakash Agarwal with Axis Capital Limited.

Prakash Agarwal: Just one question on the guidance you gave on double -digit growth. So with the expansion that you're talking about, how do you think the double -digit would shape up? Would it be early teens, mid-teens or high teens or it could cross even that?

Ameera Shah: Look, I mean, I think at this point, the point I mentioned about double -digit growth was actually specifically for Bombay and Pune. Look, I think on the core business side, I think there's a lot that we are doing. There are many, many levers in place as far as growth of business. One is obviously the geographic expansion. The infrastructure we talked about building, also potential realization and product mix improvement, wellness. So there are a lot of levers at play which are all under execution as we speak. And we are hopeful to sort of surprise ourselves this year as to where the core business can reach.

So I just want to be a little bit careful around giving too much guidance. But I think we are very hopeful, like I said, about certainly this year and the next couple of years on the core business just keeping aside the PPP side. But otherwise, I think we are very excited and hopeful about this year and the next couple of years.

Prakash Agarwal: Okay. Great. You also mentioned about your focus is on volumes given the expansion and also spoke about that you're investing 1.5% in the expansion of new centres , etcetera. So do you think with the current competitive landscape, achieving this 25% plus/minus is still achievable? Would that be correct to model it?

Ameera Shah: Like we said, look, the current margin is a reflection of three, four things. One, obviously, the competitive intensity has already been playing out for the last 18 to 24 months, and that's already captured in the current margin. We are seeing obviously some segments of the business margin is not growing as fast. Some segments of the business margin is growing faster. So it all depends upon which segment, which channel, which product to focus on, and that's going to determine your ultimate margin, but it's also a reflection of some amount of efficiencies and productivity in the system. So as I mentioned, we are hopeful that we can continue to protect the current margin and while we continue to expand on it.

Prakash Agarwal: Lovely. And lastly, on your comment on bolt -on acquisitions. So these are largely North and East. And have the valuations come off versus what you would be seeing in the last two, three years?

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Ameera Shah: See, frankly, in the last two, three years, we really did not attempt to do any bolt -on acquisitions because the bolt -on acquisition size would be obviously quite small. And the reality is the level of effort and integration required to do any acquisition big or small is the same. And we never really attempted to do bolt -on in the last two, three years. I think we were too busy with COVID and the craziness in the industry.

But I think going in this next three years, certainly, we would like to do that. That could be entry points into new markets. So some of those could be even in West and South, in markets in which we do not have currently sort of strong leadership. It could be in a small -- it could be in a city in Andhr

a Pradesh, Telangana. It could be in North and East. It could be Madhya Pradesh. So it would be across the country but basically looking at it from a city level and not looking at it from a state level.

Moderator: Our next question comes from Sayantan Maji with Credit Suisse.

Sayantan Maji: So my first question is on EBITDA margin. I read in your comments -- in your presentation that it will be back to pre -COVID levels once the network expansion that you're doing, they fully mature. So for example, if the network expansion happens in FY '25 and they take, say, the last net incentives that are added in FY '25, if they are maturing in FY '26, '27. So by then, over longer term, you expect the margins back to 27%, 28% that we were doing pre -COVID?

Ameera Shah: So pre-COVID, our margins were approximately between 26.5% to 27%. Yes, our expectation is that once the lab expansion matures and gives us sort of normalized EBITDA, we do hope that we can come back to those places.

Sayantana Maji: Okay. Great. And you mentioned about adding 800 collection centres in FY '24. So we have added 1,000 since we undertook the network expansion. So the remaining 800, most of it is coming in FY '24 while, say, 50 labs, which are remaining out of the 90 labs, 30 are coming in FY '24 and 20 in FY '25. Is that correct?

Rakesh Agarwal: Yes, absolutely, roughly. Actually, 30 labs out of the 90 already come in and 30 more labs planned for this year. So then that we will leave with about 30 more labs to come up from the plan of 90-30.

And on the collection centres, 1,070 collections have already come up, and roughly 800 will more come in this year. So we may be almost completing that aspiration in this financial year itself.

Sayantana Maji: Okay. Got it. And next question is on Hi tech. So in Hitech, the revenues have sort of declined from INR100 crores, if I'm talking only non-COVID from INR100 crores to INR88 crores. So is it due to the restructuring that we're doing, the normalization of lab service network? So is it like a temporary thing and we expect the growth to come back to what the company -- overall company is reporting in the next year?

Rakesh Agarwal: Yes, so Hitech I think we have already explained that there were some disturbance last year, which had happened because of the -- some unethical poaching the brand, et cetera. So I think that affected the momentum for us in Hitech for almost nine months. This is now back. If you
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look at the overall revenue, INR100 crores also included the rub-off, which basically you are not excluding that. So we have moved -- INR83 crores, INR84 crores of revenue had moved to INR84 crores -- INR88 crores, which is not a big number. But yes, we are expecting that this number will in line with or maybe more than what we're expecting in Metropolis this year.

Sayantana Maji: Okay. Got it. And my final question is on other income. So on other income, that has reported a steep decline this quarter. So we used to report INR3 crores, INR4 crores, INR6 crores in the last quarter. This is INR1.2 crores. So what is the reason for this? Is there any one-off impact on this?

Rakesh Agarwal: So see, other income is accumulation of a lot of factors. So one is obviously what is the cash you have in hand and what is the interest income you are earning. So if you look at currently because we are more focused on repaying our loan because that's at a higher interest rate. So that is the focus, and the cash in hand is quite small. So the interest income generated from that cash in hand is lower.

And if you look at quarter-on-quarter, basically last year, we had other income INR1.5 crores, which has come down to INR1.2 crores, which is not a significant drop. But yes, from an overall year also if you look at INR13.4 crores of other income in financial year '22 has actually gone

up to INR15.2 crores in financial year '23. So we'll check back the numbers because you are giving a different number. So see why these numbers are showing a decline. Yes, in quarter 3 financial year '23, if you look at INR6.8 crores, there was one-off on gain of some forex exchange and something. So that I think is coming into that. So just we'll explain that. Otherwise, it's in line with what we are looking at every quarter.

Sayantana Maji: Yes. So that's what actually was my next question because in the presentation in Q4 FY '22, it shows INR1.5 crores, but here when we reported, that was 5.7, so there was some dissonance over there. Anyway, so this -- the large part of the decline is mainly due to interest income is what you're saying?

Rakesh Agarwal: Yes.

Moderator: Our next question comes from Girish Shetty with Banyan Tree Advisors.

Girish Shetty: Congrats on a good set of results. I just wanted to know on the breakup like you said -- and said that the PPP revenue will go to 0 from INR67 crores, right? And if I see your core business, which is growing at around 15%, PPP will go to 0, and the COVID business will become negligible. So how do you look at growth for the next year? Is it as a number -- overall base for this year like INR1,148 crores pre-COVID impact or overall growth will be around 5% for FY '24?

Ameera Shah: No, I think that would be definitely more subdued than what our expectation is. So obviously, we are looking at the pieces in three parts. We are looking at, one, COVID, obviously, as we know, is only going down. Even this year, there has been a reasonable contribution from COVID, which obviously, as WHO has declared COVID is over and PCR testing is certainly not happening in any significant numbers. So COVID will certainly go down.

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Yes, the NACO PPP will be 0. And now it is going to be really up to the core business to be able to continue to grow through all the levers or to be able to deliver sort of strong numbers. And -- but we certainly don't think that the overall number is going to be as low as 5%.

Girish Shetty: Okay. Now then if I see the number of test, if I compare the non-COVID period 2019, where you used to do in 17 million tests and now we are at 24.5 million. So if I look at the 4-year number, it comes to around 9.5% growth over a 4-year period. So this also includes Hitech. How do we see this? Is this a normal number? Or is the core business still not normal? How do we see this volume growth over a 4-year period and comparing like-to-like as in non-COVID to non-COVID?

Ameera Shah: As you see, we have done about 11% revenue CAGR in this 4-year period, and the volume growth, like you said, is about 9.5%. The challenge with looking at a CAGR, when you are looking in the crisis in the middle for 2.5 years, it's a bit challenging because the reality is that in those two years, the non-COVID obviously crashed. So therefore, it would be a -- it would not be a like-to-like to actually look at a CAGR period for non-COVID alone, but the numbers are the numbers. The whole industry CAGR, obviously, for these four years is very low for the simple reason that there was a crisis for 2.5 years.

So I'm not really sure how to normalize that number. But I think if we look at the next three years of the business for non-COVID, I certainly believe it will be stronger than the last four years, assuming that there is no crisis from an epidemic perspective.

Girish Shetty: Okay. Sure. And just on the core business breakup. So if I just break it up into premium wellness and the other part. So premium wellness has done very well, like 45% growth. And the other part, if I see it's around 12% growth. So fair to say the core business, excluding premium wellness, we can consider 12% to 13% growth as a fair number, and premium wellness is expected to grow at

a faster rate going forward. Would that be a fair assumption?

Ameera Shah: See, some of these things are not totally separate. I tell you why. And it's difficult to look at them completely separately because you're also part of premium wellness' bundling of packages. So for example, you have consumers or patients who will walk in, maybe requesting two or three tests and then you will present to them offerings that see they feel they have an advantage to do a larger package since they are getting blood, and they collect blood. It's being collected anyway. And they will choose to move to a wellness package.

So these things are not so separate. Actually, the one feeds into the other in many ways. So while we are calling it premium wellness, it's also including bundling of sort of packages and upselling sort of existing patients or consumers who walk in and then who may choose to move to a larger package. So we have to see these sort of together and not so separate because there is an overlap between the 2.

Girish Shetty: Sure, ma'am. That is helpful. Just last bookkeeping question. So other expenses in FY '23 -- FY '22, we used to do like INR250 crores to INR300 crores, which is like 25% of overall sales, which went up sharply in FY '22 and now in FY '23 as well. So if I break those other expenses out of this, the main cost component is rent, legal and professional and the postage and courier

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expenses. So what can we classify here as a onetime expense? Or will this be a similar number like 30% of sales going forward as well like we did this year?

Rakesh Agarwal: So other expense obviously has moved up. Mainly, one is obviously because that inflation is there, and that increases the -- all the expenses which we incur, so that obviously is one part of it. And obviously, if you look at the other expense, one component which is increasing was variable rent because we are expanding our network in a big way, and we have to pay a variable rent to be -- of our franchisee partner.

So one component of this other expenses is increasing a bit, other than the other items is the variable rent because if you look at in the last three years, we have expanded our collection centres in a big way, and that is the plan going forward also.

But to your question of whether this will remain at a 30% revenue or it will go down to 25%, which was pre-COVID, so I think this will be a factor of revenue generation and how the revenue move up, so my sense is that somewhere it will remain between 25% to 30%, but it's not expected to go further up.

Girish Shetty: And legal expenses was around INR80 crores this year as well. It used to be in the INR30 crores, INR40 crores range before.

Surendran Chemmenkotil: So just give us some time. I think we'll take this point, and we'll come back to you on this.

Moderator: Our next question comes from Aditya with Securities Investment Management Company.

Aditya: I just had one question. So you mentioned in your opening remarks that we are looking to foray into basic radiologic tests. So earlier when we used to talk to diagnostic companies, they weren't keen on radiology business as it was a capital-intensive business. So I just wanted to understand what has led to this change in your strategy?

Surendran Chemmenkotil: So basically, like Ameera mentioned, we are talking about the basic radiology stuff, right? We're talking about ECG, x-ray and sonography, right? And definitely, ECG doesn't require too much of a capex and it's easy to expand and across the country. That's the first thing that we are trying to take it forward.

And then, of course, we have some assets around x-ray, which we will not been -- I mean, relatively underutilized, and we try to maximize the usage of this and also use it to package along with the wellness packages that we have. And also whatever -- I

mean, additional key markets

like Mumbai, Pune, Bangalore, Chennai, et cetera, we'll just try to take this forward into some more centres. And sonography, of course, its got its own some amount of limitations, so we will cautiously but carefully go into some of the big markets that we have. So that's the plan around radiology as such. We are just taking some steps as we start this year, and we'll try to expand it as we go forward.

Ameera Shah: Just to add, you have to remember that Metropolis has approximately almost 380 to 400 centres, which are actually owned and operated by us. What we -- and the retail segment call patient service centres. These spaces are utilized purely for blood collection and sample collection at Metropolis Healthcare Limited

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this point. And therefore, we do have an ability to also add radiology into these without adding infrastructural cost in any significant way because we already have the real estate with us. And it's about adding one more service in centres that already exist. So therefore, we believe that this could be an opportunity with the brand trust that the consumers do have for Metropolis. Offering a holistic service of basic radiology plus pathology will give value to the consumer and patients so they don't have to go to two different locations.

Moderator: Ladies and gentlemen, due to time constraints, that was the last question for the day. I now hand the conference over to Ms. Ameera Shah for closing comments.

Ameera Shah: Thank you to all the people for joining us today in this conference call. As mentioned, I think while we have all seen lots of noise and a bit of chaos in the industry in the last 18 to 24 months, we do believe that the dust is settling. COVID has come back -- has sort of -- has left us, and non-COVID, which is our regular business, illness has come back to normalcy. We've also had a chance to put behind some of the challenges of the last year, and we start afresh with Sun on board as well and a very strong team.

We have a very aggressive plan for the next two to three years on where we would like to go. And we believe this business is all about good management, execution rigor, operational excellence and really being able to fine-tune all the things and be able to make each team stronger. And I think we're very much on track to be able to do that. And we are quite -- like I said, quite hopeful and excited about what we believe the industry will bring.

COVID was a great catalyst for many new people to come in, but it was also a great catalyst for the industry to become stronger from an organized sector perspective because for the first time, consumers started recognizing pathology in a significant way in their lives and started recognizing branded pathology as something which they made a decision on. So we do believe this is going to be a strong inflection point for the industry going forward. And we're very hopeful that as a very strong brand, which is trusted by doctors and consumers, we can go on to capture a significantly higher market share of it.

In our next call, the next Board meeting, we would actually go into a lot of detail around the market and the details and the nuances of the market and help sort of all participants get an even a closer sense on how we plan to navigate this in detail. So very much looking forward to that conversation. And thank you, everybody, for joining us today.

We continue to focus very strongly on our core business growth, which has been very high, even in a challenging year, probably industry leading. And we feel quite hopeful that, that will continue to be the case for this year and the next couple of years as well.

Moderator: Thank you. On behalf of Axis Capital Limited, that concludes this conference. Thank you for joining us, and you may now disconnect your lines.

Call: Aug 2023

BLOOD TESTS • DIAGNOSTICS • WELLNESS

Metropolis Healthcare Limited

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Ref: MHL/Sec&Legal/202 3-24/45 Date: August 09, 2023

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Sub: Intimation of Transcript of Earnings Call for Q 1/FY24

Ref: Intimation under Regulation 30 of SEBI (Listing Obligations and Disclosure Requirements) Regulations, 2015

Dear Sir/ Madam,

This is in continuation to our letter bearing Reference No. MHL/Sec&Legal/202 3-24/37 dated July 29, 2023. Please find enclosed herewith the transcript of Q 1/FY24 Conference Call, as organized by Nuvama Wealth Management held on Thursday, August 03, 2023 at 09:00 a.m. (IST) wherein the financial results of the Company for the quarter ended June 30, 2023, were discussed.

The same has also been uploaded on the Company's website and can be accessed at

<https://www.metropolisindia.com/investors>

We request you to kindly take the above information on record.

Thanking you.

Yours faithfully

For Metropolis Healthcare Limited

Neha Shah
Interim Company Secretary &
Compliance Officer
Membership No. A51465

Enclosed: a/a

"Metropolis Healthcare Limited
Q1 FY '24 Earnings Conference Call "
August 03, 2023

Disclaimer: E&OE - This transcript is edited for factual errors. In case of discrepancy, the audio recordings uploaded on the stock exchange on 3rd August 2023 will prevail.

MANAGEMENT : MS. AMEERA SHAH – MANAGING DIRECTOR –
METROPOLIS HEALTHCARE LIMITED
MR. SURENDRAN CHEMMENKOTIL – CHIEF
EXECUTIVE OFFICER – METROPOLIS HEALTHCARE
LIMITED
MR. RAKESH AGARWAL – CHIEF FINANCIAL OFFICER
– METROPOLIS HEALTHCARE LIMITED

MODERATOR : MS. AASHITA JAIN – NUVAMA WEALTH
MANAGEMENT

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Moderator: Ladies and gentlemen, good day, and welcome to Metropolis Healthcare Q1 FY '24 Earnings Conference Call hosted by Nuvama Wealth Management.

This conference call may contain forward -looking statements about the company, which are based on the beliefs, opinions and expectations of the company as on date of this call. These statements are not the guarantees of future performance and involve risks and uncertainties that are difficult to predict. As a reminder, all participant lines will be in the listen -only mode. And there will be an opportunity for you to ask questions after the presentation concludes. Should you need assistance during the conference call, please signal an operator by pressing star then zero on your touchtone phone. Please note that this conference is being recorded.

I now hand the conference over to Ms. Aashita Jain from Nuvama Wealth Management. Thank you, and over to you, Ms. Jain.

Aashita Jain: Thank you, Neeerav, and good morning, everyone. On behalf of Nuvama Group, I, Aashita Jain for Metropolis Q1 FY '24 Earnings Call. With us, we have Metropolis senior management team represented by Ms. Ameera Shah, Managing Director; Mr. Surendran Chemmenkotil, CEO; and Mr. Rakesh Agarwal, Chief Financial Officer.

With this, I hand over the call to Ms. Ameera for opening remarks. Thank you.

Ameera Shah: Good morning, everyone, and thank you for joining us on the Q1 FY '24 Earnings Call. Today, I'm joined, as mentioned, by Surendran and Rakesh and SGA, our IR advisors. We've uploaded our updated results documents on the exchanges and the company's website, and I hope everyone had a chance to go through the same.

In the last few interactions, we have guided our near -term objectives. Q1 FY '24 has largely played on these objectives. Let me give you a bit of an overview.

We achieved a 12% Y -o-Y increase in core business revenue through a combination of increased test volumes, revenue per test and better product mix. Over the past 5 quarters, our core business revenues have consistently experienced double -digit growth, and we're optimistic of improving the same.

Our overall revenue decreased by 1%, primarily on account of a large B2C contract imposed by the government impacting the total revenue. Excluding this, our revenue for the core business, B2C, B2B grew by 12% as I mentioned earlier. Going forward, we are confident of scaling up revenue for the coming quarters, which will have a positive impact on margins with high operating leverage play in our business.

Looking at the details, we saw that B2C revenue had a 13% year -on-year increase, whereas even mature markets like Bombay witnessed a higher growth of over 15% with volume growth of 10% and RPT growth, which is revenue per patient growth, of 5%. I would also like to add that we continue to see a competitive environment in the B2B segment although the intensity has reduced. In spite of the challenging environment, our revenue for B2B grew by 9% with much faster growth rate coming from the specialized segment which tends to be stickier as the need

It's important to note that the growth we have seen is for the quarter which is historically the slowest quarter in the financial year for Metropolis as seasonality is different in every region of India. Every year, Q1 is the lowest quarter for us. Therefore, embracing a positive outlook for FY '24, we remain confident of revenue buildup through the year with the second quarter and the fourth quarter expected to scale even higher, leading to an expansion in market. Over the last few quarters, we have received many queries about industry opportunity and the addressable market for Metropolis. Let me take this opportunity to provide some insight and hope that th

is helps you appreciate our long -term strategy interaction. In the fiscal year '23, the diagnostic industry's estimated size is 1 lakh crores . Within this, the pathology sector is approximately 58,000 crores.

This figure can be further broken down into three segments. The OPD, which is the out side patient departm ent market, where it is outside the hospital has a share of INR 33,000 crores; the inside hospital industry is INR 20,000 crores; and the B2G, the business, the Government business is about INR 5,000 crores. Out of this, we have identified specific areas w hich we believe have substantial room for growth but also require expertise in testing and are therefore characterized by a sticky and a loyal customer base.

Accordingly, the two segments that we are targeting are one that is an organized market between the OPD services, which is the out of hospital services that is already an organized market, which is estimated to be about INR 11,000 crores. We are seeing this category increasing faster after COVID as doctors and consumers are moving from unorganized lab s to organized labs. So, while the organized sector is INR 11,000 crores today, it is not capped yet because there is a constant movement from unorganized to organized increase in this category.

Within this market, we are specifically focused on prescript ions coming from specialist doctors as they are the ones who care more about the quality of results and therefore, are willing to pay a premium for the services. The outsourced market from IPD, which is the hospitals which are doing testing inside the hosp ital but still need to outsource specialized test lab players like us is approximately valued at INR 4,000 crores.

Increasing growth in this category depends on taking our brands to more hospitals and delivering on our commitment of accurate reports, wide r test menu, doctor recommendation and reports delivered on time. Combining these two segments, the current addressable market is at about INR 15,000 crores. And in the next 2 to 3 years, by '26, this is expected to be growing at about 12% CAGR, to reach INR 21,000 crores.

What sets this addressable market different from the rest of diagnostics is that this is what we consider as stickier business, characterized by sustained volumes, loyal customers with a preference for quality services, a better margin p rofile, low churn rates and therefore, offering a long runway of growth, both from a revenue and a profitability perspective.

Within this addressable market, our focus lies on catering to the top tier of the pyramid targeting customers that prioritize qua lity and reliability above all ends. This is the organic opportunity of Metropolis Healthcare Limited

the addressable market in addition to the growth that can be taken on account of a leadership position in innovation, test menu network and science -based benefit.

The Metropolis brand has earned the reputation for being highly reliable and trusted, both amongst consumers and medical practitioners. This trust factor plays a crucial role in fostering long-term relationships with referring doctors, ultimately contributing to our continued success. With these advantages firmly in place, we are optimistic about our ability to outpace industry growth in the years to come.

Let me now take you through Metropolis 3.0, our 3 -year strategic plan based on this addressable market. Our focus revolves around 3 pillars. The first one is to strengthen our core business, and the goal here is to grow faster than the industry by focusing on quality and sticky business that sustains long term to become an efficient testing pathology player driven by automat ion and medical expertise, to stay a pioneer in introducing new tests and innovation and testing and to provide access to patients across the country up to all Tier 3 and Tier 4 towns.

In the next 3 years

, we will be in more than 700 cities compared to th e 370 cities that we are in today. With 270 labs and 6,000 -plus collection centres , our goal will be to increase our network across the country with increasing productivity per centre . Our target will be to have 20% to

25% revenue coming from wellness testing and 40% to 45% coming from specialty testing. Simultaneously, we would be focusing on margin expansion with margins stabilizing at Pre-Covid levels and ROCE to increase proportionately. While we maintain our leadership position in West and South, we are also aiming to be in the top 3 players in North and East.

The second part of our strategy on the Metropolis 3.0 strategy relates to expanding adjacency. As part of our strategic plan, we are embarking on a journey of exploring adjacencies in health services closely linked to our core business such as basic radiology, like ECG, X-Ray and Sonography as one of those possible services. By doing so, we aim to unlock new avenues for growth and diversification.

Through this expansion into complementary segments and services, we aim at leveraging our existing expertise to cater to a broader spectrum of health care needs. This approach allows us to not only deepen our relationship with current customers and doctor fraternity, but also attract new stakeholders into our ecosystem. These adjacencies will aid our position of being a reliable partner in the industry -- a health care partner in the industry.

Whichever adjacencies we pick up, they will be capex light and will help us grow our organic business of pathology. At this point of time, we won't have any details to share, but we believe in the coming quarters, we'll come back with a far more detailed plan and action.

The third part of the strategy as part of Metropolis 3.0 relates to the bolt-on acquisition. We recognize the potential of inorganic growth as a means to strengthen and complement our existing capabilities. As part of our strategic plan, we will consider smaller bolt-on acquisitions that align with our value position. These acquisitions will serve as strategic entry points into new geographies, enabling us to expand our reach, enhance our service offering and leverage with Metropolis Healthcare Limited

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the local brand expertise that you get through acquisition and consolidate our position as a leader in the health care industry. We plan to do about 5 to 6 bolt-on acquisitions in the next 3 years. Given the ample opportunities for growth in the diagnostics industry coupled with the robust brand strength of Metropolis, we're poised for a very exciting journey ahead. Additionally, with the Suren coming on board, bringing fresh perspective and leadership infused with a wealth of experience and strong execution rigor to drive efficiency, positive innovation and optimize our operational abilities, all of this will lead us to greater heights.

Suren, if you could come in and talk more about the operational priorities, please.

Surendran Chemmenkotil : All right. Thank you, Ameera. Let me speak about our strategic priorities and avenues of growth for the year. From an operations point of view, 3 big priorities guide the organization forward.

That's network expansion, focus on specialty and strengthen our wellness portfolio. Our journey of network expansion is well underway with a focus on establishing new laboratories and asset-light collection centres. During quarter 1, we successfully added 6 new labs in about 120 centres. Most of the new lab additions scale from North and East and the collection centres across geographies.

Our target is to open an additional 25 labs and around 700 centres within the remaining 9 months of the fiscal year. We have started 70 new towns in the last quarter, and now we are present in almost 370 towns across the country. Our target will be to expand this and take it to

500 towns

by the end of this year. This expansion is supported by a robust data science approach, like I mentioned in our last call. I must mention here that not only we are focusing on opening new network, but we are also making a concerted effort to enhance the productivity of all our existing centres across markets.

Over the next couple of quarters, we are actively considering the reclassification and structuring of our focus, Seeding and Other Cities categories. And I feel that after 7 years, this requires a review now. We are considering inclusion of 4 to 5 additional cities into our focus cities portfolio, and these cities will see higher investment outlay and better infrastructure support that is required to build the true potential of these locations.

And some of the other cities will similarly move into Seeding category as we continue to expand the Other Cities category. These potential additions are based on crucial factors such as current revenue performance, market share in those markets, future growth prospects, B2C focus and the feasibility of scaling operation through our existing network as well as planned expansions. Now to further strengthen our specialty business, we have built a strong and dedicated team of sales force who will single-mindedly focus on key specialty portfolios across the country. We have trained them well and equipped with all necessary work tools. We're actually working on expanding our presence and collaboration with specialized doctors across the country. By engaging with the Renowned Medical Professionals, we have been able to offer them tailored scientific propositions and foster a collaborative approach for the most suitable diagnostic parameters for their patients, which strengthen the doctor's preference for the Metropolis brand and enriches our value proposition in the long term.

We continue to believe that Specialized business of Metropolis is very quality -focused business and with high degree of stickiness. This is further aided by an enhanced test menu and the introduction of new tests.

Now allow me to provide an update on our revenue, volume and RPT growth. At a group level, our revenues were at minus 1% year -on-year in quarter 1 basis, all inclusive, as Ameera mentioned. It is important for me to mention here that in the last quarter, the same number was at minus 8%. And we see this getting better as we go forward from here.

Just to recap, we did not have the volumes of COVID and entire PPT in this quarter. We have achieved a 12% year -on-year growth in our core business revenue, excluding these 2. This growth has been driven by a combination of 9% increase in volume and 3% growth in RPT and change in product mix.

While we did a marginal price increase in specialized test in end of April, adding about 1% benefit, we also made certain price corrections in about 25 routine tests in Delhi NCR geography to align with our strategy there. In every local market in India, we adopt different strategies based on our market share and the level of competition in specific regions. In certain micro markets, we may implement volume -driven strategies to gain market share. This balanced approach incorporating more volume -driven and RPT focused strategies has contributed to our growth and market leadership.

As we move forward, we remain committed to delivering growth with volumes being the primary driving force. Our pricing decisions are driven by careful analysis and aim to align with competition intensity from both organized and unorganized markets.

Let me also provide an update on Hitech business. Hitech has been successfully integrated, and we have identified multiple synergies and cost efficiencies across the system. We also have been able to cross -sell, upsell and merge the lab operation, which will further enhance revenue growth

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and profitability going forward. Our revenue growth from Hitech stood at a double digit in the last quarter, with margins slightly higher than company level EBITDA.

Our strategy for Hitech business this year will be to add another 50 touch points across Chennai and rest of Tamil Nadu, targeted campaigns for increasing Wellness segment revenues, and step up cross -selling Radiology to the existing and new customers, focus on B2C campaigns and digital marketing to increase the brand strength, and talent acquisition to increase consumer connect with expanding doctor network.

I also want to take this opportunity to give a brief overview of our international business.

Currently, our international business revenue accounts to about 7% of the group revenue. Our strategy for international expansions are based on the premise of that the markets we are present are relatively untapped, presenting us with opportunity to establish a strong presence.

In these virgin markets, we have noticed lack of organized players, creating a favourable environment for our growth. Our approach to international expansion is focused on organic growth, complemented by a mix of B2B and B2C strategies. We are strategically utilizing

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specialized testing capabilities to maximize the utilization of our assets in India, ensuring a higher return ratio with minimal investments.

International business is driven by a dedicated team reporting directly to me. Going forward, we remain committed to nurturing our international business in an organic way, generating higher growth and attractive returns in the future without any major capital commitment. With that, I hand over the call to Rakesh.

Rakesh Agarwal: Thank you, Suren. Most of our financial and operational highlights have been shared in the presentation uploaded. We hope everybody has an opportunity to go through the same.

Let me provide specific insight into our productivity and scaling metrics for the labs opened in the last 15 months. For lab opening financial year '23, revenue stood at INR 20 crores. During Q1 FY '24, the revenue for these labs amounted to INR 7.38 crores. Lab established in quarter 1 financial year '23 witnessed growth of 270% in quarter 1 financial year '24 compared to the previous year. Our strong execution rigour, operational efficiencies and focused marketing and brand building efforts have significantly contributed to sales of revenue from these new labs. We are optimistic about the potential of these nascent plans to mature over time and generate even higher revenues in the future. We believe this launch will continue to achieve significant growth and contribute significantly to our overall revenue going forward.

Speaking of the productivity of these labs, we have been able to achieve 5% EBITDA margin for this lab from negative 5% in year 1. By scaling up the revenue, we will have high operating

leverage, which should deliver higher margin going forward. We believe that in 24 to 36 months of maturity of this lab, we will be able to clock company level EBITDA margin for all these newly opened labs.

Now I'd like to give an update on the financial and operational KPIs. Our revenue for quarter 1 financial year '24 was down by 1% on account of in-sourcing of large B2C contract and high base of COVID revenues in quarter 1, financial year 2023. We are optimistic of high revenue growth for quarter 2 onward backed by high utilization and productivity of new network.

Our EBITDA margin before CSR and ESO P stood at 24.2%, dilution of 1.1% on account of network expansion for Q1 financial '24. EBITDA before network expansion stood at 25.3% for Q1 financial year '24. Our EBITDA has been impacted largely on account of network expansion to a tune of 1.1% and 0.7% due to loss of revenue on account of in-sourcing of B2C contract, which is a negative operating leverage play.

We'd like to highlight that we have taken stringent cost control measures to keep the cost structure under check, despite higher cost of people, aggressive lab and network expansion, increasing marketing initiatives and implementation of digital initiatives in our system, our cost just grew by only 1% Y-o-Y for quarter 1 financial '24. We are optimistic of volume and revenue bouncing back in quarter 2, and should have a positive operating leverage with uptick in margin. Our Premium Wellness business grew by 27% Y-o-Y for Q1 financial year '24 with average realization in the range of approximately INR 2,200. Overall contribution of Wellness stood at

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15% for Q1 financial year '24 as compared to 13% for Q1 financial year '23. Moving to the balance sheet, our gross debt as on 30 June stood at Rs 67 crores. We plan to be debt free by the end of this year.

That's all from my side. With this, I open the floor for Q&A. Thank you.

Moderator: Thank you very much. We will now begin the question-and-answer session. The first question is from the line of Rahul Agarwal from In Cred Capital.

Rahul Agarwal: First question essentially was Mumbai growing faster within the overall B2C. Question on that essentially, as we hear from your peers about health tech platforms cutting discounts, was Mumbai a function of that, the growth? And your thoughts on business competition, is this sustainable going forward as well? That's the first question.

Ameera Shah: Thanks, Rahul. Look, I think if you see for the last 4 to 5 quarters, Bombay has been actually growing at a similar pace. So, what we always maintained was that if you have built a good quality business in pathology like Metropolis had, the digital competition would not have impacted you much because the customers who value their brand did not really value price and we're not the ones who are going to move to competitors for price benefit. And I think we have seen that play out very clearly in Metropolis where we have not seen an impact from a B2C perspective. In fact, we've only been growing even in mature markets like Bombay.

So, we see this trend to continue. And as digital guys have cut back on their discounting, it only means that the noise in the industry will start settling. I don't think those discounts have aided them in building very large businesses from the numbers that we know, and I think there is a recognition coming in that price is not going to be the driver in pathology.

Rahul Agarwal: So, you think going forward, even this should continue, right? It's good news, right? I mean I understand Metropolis has not lost much of the business, but going forward, if this happens. I think whatever we have lost, maybe it's like not very material to the company, it still gets back to us, right? At least from an industry standpoint, that's positive, right?

Ameera Shah: See, I think whether people choose to discount nor frankly is not in anyone's control, right? So of course, until the time that they don't -- they choose not to discount, I think it's healthy for everybody including the industry. My only point was that I think our growth continue to lead the way.

Rahul Agarwal: Got it. Secondly on International, 2 questions there. Firstly, is it similar to Indian profitability, 24%, 25%? And second is any plans here to grow faster? Because I think Surendran also has decent amount of experience in similar markets. We also hear some Indian diagnostic player trying to enter Africa sometime this year. Your thoughts, please?

Ameera Shah: So, I think we have been doing well on the international side. The business model is actually very similar to what it is in India. And the reason why we are in these markets is because we actually sweat our assets from India. For example, in these locations, we do about 60%,

70% of

the testing required locally, and almost 30% to 40% of the samples come back to our global reference lab in Bombay for specialized testing.

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So, we believe it is very strategically connected to the Indian business. And the biggest challenge in some of these markets is lack of talent and scientific expertise, and we are able to bring it from our core DNA into these markets. So, I think we'll keep seeing International do well for Metropolis. The margin profile, as mentioned in the speech as well is actually -- for this quarter is slightly around a similar profile as the group profile. So, the international business is on a good track as well.

At this point, the only concern, obviously, with the international business is the currency devaluation from some countries and that's the only reason why we are not pushing much, much harder. And the opportunity is there, and I think we'll continue to invest small amounts of capital from the profits that are being generated in these markets and keep driving organic growth.

Rahul Agarwal: It should reflect like 10%, 12% of similar growth what you do in India, right?

Ameera Shah: Growth is actually a little bit higher in the international market because the base of some of the labs is also smaller.

Rahul Agarwal: Okay. Got it. And lastly, just your outlook for the full year. Like for like sales grew 12% for the first quarter. I understand reported basis, top line will be lower, but just your thoughts on full year outlook, please?

Ameera Shah: So, as we mentioned, I think we feel quite confident about the core business growth for the full year. Quarter 1 is usually our slowest quarter. We certainly expect Q2 and Q4 to be better quarters for us. And as we know and as volumes increase, it certainly impacts our profit margins as well.

Rahul Agarwal: Any number you want to put?

Ameera Shah: Well, not at this time.

Moderator: Next question is from the line of Shyam Srinivasan from Goldman Sachs.

Shyam Srinivasan: Just the core revenues of 13%. I think you've given volume growth of 9% and RPT of 4%. I want to just deconstruct the volume a little bit further down, like SSG plus -- you've been expanding as well. So, I just want to understand how are your mature stores, what's the kind of volume growth? Maybe as a corollary, you can also tell us how Mumbai, the 15% that we have grown, how is that deconstructed please?

Rakesh Agarwal: Yes. So basically, if you look at the construct, 12% is in overall growth for us, and as I said in my speech also that in quarter 1, whatever labs we started in the last 15 months, for those labs, we have got around -- INR7 crores of revenue -- sorry, INR7.38 crores of revenue this quarter, which is 3% of the overall revenue. So overall, if you look at 9% is coming from our old base and 3% is coming from the newly opened labs.

Shyam Srinivasan: Got it. So, Rakesh, can we do it like the SSG way? Well, in the volume growth is where I'm more trying to get a sense of how much of the incremental volume? Is it similar like 75% coming Metropolis Healthcare Limited

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from old store volume and 35 -- sorry, 25% coming from new stores? If that is one way to deconstruct?

Rakesh Agarwal: No, volume, I don't think 25% will be contributed in the new store. We will just check and come back to you on that. So overall construct is 9% plus 3%, 9% coming from the volume and 3% coming from the pricing. Let us come back specifically on what is the volume growth coming out of the 9% from the new labs.

Shyam Srinivasan: That's helpful. Just second question, again, just on the network expansion, we had about 120 basis points, if I recollect rate of dilution on the margins in fiscal '23 because of the network expansion. We've seen it slightly moderate. 110, I think is what the pre

sensation calls out. So,

any sense on where we would likely see this number starting to even get lower? And then is there a corresponding uplift that the overall corporate margins we will see, or will this -- will we use this to do our continuing investments? So that's my second question.

Rakesh Agarwal: Yes. So basically, what we intend is that we intend to open as mentioned by Surendran that we can open around 20, 25 labs this year. And obviously, that will have a dilution of it, but there will be a moderation of the labs opened already in the last 2 years. So, what we feel is that this number will remain in the range of 1.1% to 1.3%. It will keep on moving 0.1% here and there as we go along. But the next 2 years, as we have expansion plans, we see this percentage dilution coming in the EBITDA margin question for sure.

Shyam Srinivasan: And the last question, Ameera, I think you called out the INR 15,000 crores as the kind of addressable time for us. A little surprised. If I add all the listed players, fiscal '23 revenue, just the pathology bit, it's INR 6,000 crores and ballpark. So, have you -- had these players already achieved 40% of that share? Do you think -- I just thought the number was higher. So that's where my confusion is?

Ameera Shah: No, thanks for asking the question. It's a fair question. See firstly, not all the listed players and

unlisted players are targeting the same segment. So, we have to remember that it's not like listed players are all following one strategy and unlisted are following another strategy.

So, each company is choosing to target different segments of the diagnostics industry. And what we are attempting to do in this speech is actually be more clear about the segment that Metropolis is going after. As we have always mentioned, our goal has been to go to specialists and get them to recommend patients to come because the patients who go to specialists tend to have more critical illnesses. And the people who have more critical illnesses, for the doctor and for the patient, the quality of the report matters more.

So therefore, the segment we are going after is about 15,000 crores. And our goal will be to keep measuring what percentage of market share we have in that segment. Other players are choosing other segments. So, some people are focusing on B2G, which is the INR 5,000 crores segment. There is -- some listed players may choose to go there. Some listed players are focusing -- not focusing on specialist-driven pathology. They are focusing on GP-led pathology or they're focusing on B2B, which is a different part of the segment.

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So, I think just the clarity important is that not all listed players have the same strategies and goals, and therefore, it's important to see each in its own strategy.

Shyam Srinivasan: And last question, again, Rakesh, just data point, Hitech, what is the revenue and maybe volume there?

Rakesh Agarwal: Yes. So, Hitech, as mentioned by Surendran, Hitech is growing a bit higher than the -- our core revenue for the group. So that is one data point. And from a volume and RPT point of view, this is more or less similar to us, 8% is the volume growth and 4% is RPT growth for the Hitech.

Moderator: Next question is from the line of Rishi Mody from Marcellus Investments.

Rishi Mody: Yes. So, my first question is on Hitech, right. So, we have seen a bit of labs and centre closures out there in the last 1 year or so. So, on Hitech, right, we've seen lab closures and centre closures.

So just wanted to understand why were these centres closed? What was wrong with them? What efforts were taken to term them when they were not profitable? And secondly, is this exercise now complete? And the third question on Hitech was the 50-center expansion that has been announced, that you all are targeting, in what form is it is

it going to come? Is it going to come in the COCO form or in the FOFO form?

Ameera Shah: Sure. So just one clarification first. We have not closed centres for Hitech, either collection centres or labs. What we have done is merge them with Metropolis labs in certain markets. So, the Hitech that we acquired had three parts. One is the Chennai business; one is the rest of Tamil Nadu business, and one is the Bangalore business.

The Bangalore business and the rest of Tamil Nadu business, we have merged the brands. And therefore, wherever there were synergy opportunities, we have merged lab operations and merged some collection centres. In Chennai, as we know, we have continued to maintain both the brands separately. And therefore, we have not yet merged operations or merged collection centres or labs. So, I hope that this is clarity, we have not shut any lab for Hitech without a synergy reason.

The second question about adding approximately 50 -- 45 to 50 centres for Hitech, the idea is basically this will be in Chennai and Tamil Nadu. These will be a combination of COCO and FOFO. So, there'll be some franchise centres and some company-owned company-operated centres across these markets. And those centres that we've already opened last year, I think we opened 11 last year, if I'm not mistaken -- 12 last year, I think most of them are doing well. There are maybe a few 2 or 3 from what I know that have not performed as well as we had hoped for, but the rest have done fairly well.

Rishi Mody: Okay. And like -- sorry, did you mention the mix of how much of that 45, 50 will be a FOFO, COCO mix or like, is there any plan proportion? Or is it just how the opportunity presents itself?

Surendran Chemmenkotil: See, we just not prepared any specific mix between COCO and the ones under the franchisees, it depends upon the respective markets. So, it's largely today out of 50, I mean wherever we get an opportunity to do our own, we'll do that, bigger markets, etcetera. And in the rest of Tamil Nadu, we may go through the APSC route. So, we'll take a combination of both without having

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a fix in our mind about how much should be the PSCs and how much should be the franchisee's PSCs.

Rishi Mody: Second, Ameera, you mentioned that you all are planning to do some 5 to 6 bolt-on acquisitions in the next 3 years. Firstly, are there good quality assets available in the market at reasonable valuations, which you can acquire in the next 3 years? Secondly, how big will these businesses

be in terms of top line? And thirdly, what is the budget that we have planned for these acquisitions? And finally, have we -- are we investing in a team which will continuously help us acquire such bolt-on acquisitions year-on-year? Or is it all ways -- is it going to be a more opportunistic way of doing M&A?

Ameera Shah: So firstly, in terms of the assets that are available, it is difficult to find, not easy, which is why the numbers we have quoted are 5 to 6 and not large numbers. It's very important to us that we buy assets that we believe are assets that we run clean and can be scaled up versus where you actually have to clean up huge number of skeletons in the closet and lose revenue and profit as a part of it. So, I think we'll continue to be selective and careful, but at this point, we do believe that, that is possible. Of course, if anything changes, we'll come back and update you.

We don't have a number at this point of time on valuation. But obviously, our goal will be to try to do acquisitions that add fair and reasonable valuations as possible, which are not at very high numbers. What I've often seen in our market, it is only the large clean acquisitions, which tend to be very premiumly priced because there are such few of them in India.

At the smaller level, it will be a little bit -- we believe

it will be a little bit more fair in terms of valuation and not as distorted and we should be able to get this done. We don't have any budget in mind at this point that we can sort of quote because it will depend on each opportunity that comes forward.

Rishi Mody: And what size are we targeting these bolt-on acquisitions, in terms of the revenue size?

Ameera Shah: Look, I mean, they can be anything from INR5 crores a year to INR20 crores, INR30 crores a year, right? So difficult to tell at this point. But these will be all sort of usually individual pathologist-run labs which have done well for themselves from a brand equity perspective in the city. And I'm not really sure either of their success on plan or their scaling and therefore, are looking for a partner like us that we can work together.

You will be -- because Metropolis has already done about 23, 24 bolt-on acquisitions in the past many years, we have the internal DNA on knowing on how to integrate them and how to get value and scale them. So, I'm not too concerned around building the expertise for integration.

Rishi Mody: My third question, like my next questions are more -- okay, just on the strategy front, right. So, the commentary that you've given on the call seems that you all are now targeting the specialized and the wellness mix. So, I remember a few quarters back when you had first announced the Metropolis 3.0 plan, you've said that we're going to build a B2C consumer led brand and we'll have organic footfall and reduce doctor dependency.

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Now is that plan still the same? Because if it's going to be footfall-led, there will be a good chunk of routine mix that will also come through? So, are we not targeting that segment now or are we -- like if you could just give some clarity on that aspect?

Ameera Shah: No, thanks for the question. I'm happy to provide clarity. See, Metropolis 3.0 is a combination of multiple strategies. It is not a single strategy. Metropolis has always been a company which has gone to specialist doctors as well as top quality GPs and gotten recommendations for patients to walk into our centres from them, and that will continue. That is not going to change. But the second thing that we have added in Metropolis 3.0, which Metropolis did not do earlier, which is to try to go directly to the consumer and to acquire the consumer for chronic and for wellness testing directly.

See, for chronic testing, if you're a diabetic, you're a heart patient, you're a vitamin B deficient, you're anemic, etcetera, these are decisions you as the patient can make independently and for wellness, you can make independently. So, we go to the consumer directly to acquire these consumers for chronic and wellness.

But for anything more complicated than that in your life and you have an illness, you will depend on your doctor to make decisions for you. And therefore, we have to go to the doctor for getting more complicated illnesses and consumers to come to us. So, the strategy is twofold, to go to the consumer for routine and chronic and to go to the doctor for more critical cases of patients.

I hope that clarifies things.

Rishi Mody: Yes, that's good for me. Just 2 bookkeeping questions. So, I see your core business mix has grown by 11% while -- your focused areas have grown by 11%, while your Seeding has grown by 5%. But the chart on the left, on Slide 17, we see that the proportion of Seeding has increased compared to focused. So, I just wanted to know is there a typing error out there or is there some methodology change in calculation?

Surendran Chemmenkotil : No, So, Seeding has been growing lesser for us, for sure. As you can see that the 11% growth is coming in the Core market, 5% in Seeding and other markets because we are opening up new labs, so that is seeing a 22% growth.

Rishi Mody: So -- but

your Seeding mix has gone from 21% to 23%. So just trying to understand whether there's some methodology change or it's just a typing error?

Surendran Chemmenkotil : So that is basically to do with the NACO because NACO was coming in the other space and that basically was gone. So, we'll check back on this because your point is valid. That 21% is now moving to 23%. So let me just check back on this and come back to you, but it may have a NACO impact. But the point well taken and let us clarify it further.

Rishi Mody: And secondly, Ameera, for the Seeding Cities, right, you've seen that focus has been growing ahead of Seeding for the last few years. So just trying to understand like what efforts are we taking to build up the Seeding to grow faster than Focused. Because ideally, if you're in a mature markets versus upcoming markets, the upcoming markets growth should ideally be higher. So just trying to understand what investments are we doing on the people and the infra side in the Seeding Cities too, build that growth faster than the Focused areas.

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Ameera Shah: I think first, you've got to understand what we are building in each, right? So, if you see the focus markets are our B2C markets, primarily B2C markets, right, where we have continued to focus on B2C and we've been able to, through the doctor route and the D2C route, we've been able to generate a fair B2C percentage -- growth percentage. The Seeding Cities are usually ones where we have had more B2B, primarily B2B.

Now those markets, as we know, in the last many years have faced a fair amount of competition on the B2B side. And while the competition did not impact our customers directly too much on the head, our tail customers, which were the smaller customers, which were not as quality conscious. And we have some amount of those in the tail, but not a large amount, those started to churn a little bit.

So as competition intensity came in and as we started to see price and discounting sort of increasing, we saw the tail end of our customers starting to have a little churn, which has brought down the overall growth of B2B, specifically in the semi specialized segment, which happens to be in the Seeding Cities. So, what are -- we've actually created a new plan for B2B in the Seeding Cities, which is now in execution, and which has already shown us -- as Suren mentioned in his speech, we have seen a 9% increase in B2B in the quarter 1 of this year, which means that the plan seems to be working and is on better track.

My estimate is that this should look better for the rest of the year result. Suren, you want to add anything on that?

Surendran Chemmenkotil : No, I just wanted to reiterate that this definition of Focused, Seedings on Other Cities have been slightly older version now. That's why I mentioned that we are reviewing the Cities. Maybe next time when we talk about it, we'll have additional kind of Cities in the Focused Cities where we're going to put more emphasis on the B2C and some fresh investments in terms of stores, etcetera. And also, the Seeding Cities will also go for a revision. So, we are just working on these categories to further strengthen it, right?

Rishi Mody: Understood. And your B2B strategy...

Moderator: Sorry to interrupt you, Rishi.

Rakesh Agarwal: Sorry. I just clarify that there is a typo. Actually 23% and 18% will be interest change. So, there is a contribution of 18% from Seeding and 23% for others. Just to get it right.

Moderator: Next question is from then Anish Devra from Nomura.

Anish Devra : Firstly, I just wanted to understand whether there is an increasing trend of insourcing happening on the Government side. Like do you see that the Government has a tendency to -- rather than outsourcing to itself, and whether the profitability of the Government business is higher or lower than

the normal core business. Just wanted your colour here?

Ameera Shah: See, B2G is an area as you know that we don't focus on much, and we have been very, very selective about what we pick up. Generally, we are not huge fans of the B2B business because

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we find it's not in line with our sort of way of doing business in terms of the quality parameters, in terms of the compliances, in terms of all of that.

So, I think this is an area that we don't expect to focus on, frankly. And we find that the profitability of any business, whether B2C, B2B, B2G, all depends on your own underlying governance value quality that you apply to it. And frankly, in an industry like India where there is no minimum standard or regulatory context, it is up to each player to decide what kind of level of quality they apply to their own business.

So frankly, there are no 2 businesses you can find, unfortunately, which are comparable from an apple -to-apple basis and quality basis. So, each company will have their own practices. And really the reputation built on the back of doctors either recommending your brand or not? And

that's how you know whether a company has got good quality business value.

Anish Devra : And secondly, ma'am, on the seasonal weakness that the West region experiences in the first quarter of the fiscal, and the fact that it differs across regions in India. So, can you just give some broad -- high-level reasons as to why there is this difference in the seasonality across the regions in India? Like I think, the Northern region does not tend to be seasonally weaker in 1Q as compared to the Western region? So, any colour there, why -- and odd levels...

Ameera Shah: See. There are two, three reasons that we are aware of unlike the others, but one thing which we certainly knows impact seasons is festivals. And festivals as we know in India are celebrated at different times in different states. So, festivals play a hugely important role. And each festival has elevated a certain amount.

For example, you will see Diwali is very large in North India as well as in Gujarat. And -- but you'll see, for example, something like Ganesh Chaturthi is very large in West, right, and Onam in Kerala, etcetera, etcetera. So, depending on festivals, you find a big impact. And the reason around that is because people's mindset is not so concerned around health care, people had to travel a lot, they're spending time with family.

The second thing that impacts is holidays. So, depending on where school holidays and children holidays are, people end up planning their travel. And doctors also, as human beings at the end of the day, would like to travel and take holidays. So usually, we find that when doctors are traveling, that's when we find that the business comes down. And in West India, the school holidays are usually in the first quarter, which is then doctor end up traveling and that changes things.

Third, obviously, thing that impacts this is weather. Weather is different in different parts of the country. The rains in Bombay, for example, may in some years create dengue, in some years it may not, depending on the kind of rain. In winters or where there is pollution in some markets will cause more illnesses in those quarters. So, these are the three broadly, I would say, that impact the seasonality across the country.

Moderator: Next question is from Bino from Elara Capital.

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Bino : Just a question on the lab addition. So, you had this target of adding 90 labs and 1,800 pick-up points. So, if I do the math from the time of original target setting, it adds up to around 215 labs by the end of FY '25 and maybe around 4,400 PUPs. Is that math right? And third, does it change with your new strategy of expanding newer geographic pers

ons, etcetera, etcetera?

Surendran Chemmenkotil : Yes, I think we have not changed those numbers, right? In the 90:30 project that we started, we talked about adding 90 labs in 30 months, right? So that remains same and year on course. And same as the service centres we talked about it, right? So, by 2025, right, we will get 1,800 more service centres, that we've started off with and we are on course to deliver that.

Yes. So, the 6,000 centres, if that's what the question? That is mentioned by Ameera, I think that goes beyond the 2025 time period, we've talked about it.

Moderator: Next question is from the line of Kapil Agarwal from Tara Capital.

Kapil Agarwal: I just see in your notes that there is an income tax notice under Section 147, 148, which I believe is income escaping assessment. So, can you please elaborate a bit on this part?

Rakesh Agarwal: Yes. So basically, you understand the whole -- certain process after search. This is a very normal procedure that after search, the income tax authorities sends 147, 148 to reopen the assessments done previously. They can open up to 10 years, that's the limit. And this is just a procedural thing which has happened with us. Without doing anything specific, they have opened the cases, and we'll comply with what is required and will provide all the details. So, this is just a procedural thing which has happened. And whatever development will happen in this, we will keep coming back and sharing that case.

Kapil Agarwal: Any amount that is being mentioned in the notice, or we have -- we need to carry a provision maybe in the current quarter?

Rakesh Agarwal: No, no, there is nothing. It is -- again, I'm retrieving that this is a procedural thing where we opened the previous assessment. Any search happens, it happens in 100% cases. So, there is nothing specific, and this is a procedural thing where they're opening the assessment of the previous years.

Kapil Agarwal: Okay. And lastly, what is the time limit for replying to this notice?

Rakesh Agarwal: Time limit for?

Kapil Agarwal: Time limit for replying to the notice?

Rakesh Agarwal: No. So basically, when they open, then we just -- there is nothing we have to do about it. So, we said, okay and now they will have -- reopened the assessment and they will have some specific queries and questions on those assessments, and then we will go and reply to them. So, this is just a notice that they are opening our cases, and they will scrutinize the earlier assessment done with all numbers. And if there is anything which they feel they want to get information on, then

So, this is not nothing to do with us. They are just informing us that they are reopening the assessment and they will come back on any query questions regarding any for reassessment.

Moderator: Next question is from Aniket Kulkarni from B MSPL Capital.

Aniket Kulkarni: I had a question regarding to the overall industry perspective. So, I just wanted to get a sense on how is the diagnostic industry currently covered in insurance schemes. And if -- and how will the insurance coverage and future escalations evolve in this industry as in -- it can help us to solve the issue of under penetration and take market share from the unorganized sector? So, if you can give some colour on this.

Ameera Shah: So currently, if you see the connection between insurance and diagnostics happens mostly when people are buying insurance at a pre policy level, or in some cases, at a post policy level. At this point, diagnostics is only covered by insurance when you are hospitalized, not when you are outside hospitalization. There are some insurance policies, which are covering OPD care, but it is a very small, small number and not have taken any significance of scale.

The direction we do believe the industry

will move in, in years to come will be where insurance

policies do cover out of hospital, what is called as OPD care. And this will land up increasing the volume of testing quite significantly. And like you rightly said, will move more unorganized to organized.

Having said that, we do not have a timeline on when this is expected to happen. But if you look at other countries and the direction they've gone, this is a no-brainer. And this will not only increase the volume of patients but will also increase the volume of tests per sample because that's what we've seen happening in most of the countries.

Moderator: Next question is from Rushabh Shah from KR Choksey.

Rushabh Shah: I have a couple of questions. So, in the introduction, you said you -- your target is to get 25% of the revenues on the Wellness segment. So, don't you think, the Wellness segment is kind of an expensive test, and where few category of people in India are able to afford that test? So, what are the views on that?

Surendran Chemmenkotil : Sorry, I didn't get that clear.

Rushabh Shah: I was asking you all, you said in the introduction of 25% of the revenues will come from the Wellness test going forward. But don't you think Wellness test is kind of an expensive test, which only few category of people are able to do it in India. So, what are your views on that? Don't you think that's an over estimation of tests if you want to do?

Surendran Chemmenkotil : This is a plan for the next -- see if you look at it on a year-on-year basis, we have enhanced our Wellness portfolio from where it was last year versus now at this point of time. What you're able to see is that once we're able to get a customer doing their full health check, then we're able to follow up with a -- at a periodic level and get them back doing the tests with us again.

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And also, some of the customers walking up to our centres for the normal illness related test, we're able to upsell them the entire Wellness package. So, I mean, it's basically a combination of both where we are able to move up the revenue from Wellness. So, I mean, earlier this used to happen in the few cities. Now, we're able to see that even the Tier 2, Tier 3 cities. We're also able to do some of these movement through the -- even the B2B partners as well.

So overall, the portfolio is growing and the customer also getting increasingly aware about getting the regular health check done. So that's the reason we have -- believing that this portfolio will go up to 25%. And with more and more centres getting opened, you get an opportunity for the consumers to come, walk in and we get an opportunity to interact with them, and we'll be able to grow this particular portfolio. So that's where the confidence is coming from.

Rushabh Shah: Okay. My next question is what steps are you taking to expand Metropolis in the North side where the competition is very strong. How will you penetrate that India?

Surendran Chemmenkotil : So, I think in my earlier speech also, I mentioned, and if you look at it, the new labs, the large portion of the new labs that we have opened, and we're opening up is in the North and East where we wanted to further expand our footprint. And of course, that doesn't mean that in the South and West, we really don't add -- we will increase the density in South and West and where we will increase the density as well as North and East is concerned.

So, we are picking up to start with strategic markets and then we are going and opening up the labs and then supplemented by adding more collection tenders and B2B clients, etcetera. So, we have -- North definitely is an opportunity market for us, and we have our plans ready for North in the coming quarters.

Moderator: Due to time constraints, we'll take the last question from the line of Saurabh Kapadia from Sundaram Mutual

Fund.

Saurabh Kapadia: Sir, if you can talk about what is the percentage of revenue coming in Wellness from the focus cities? And how has the growth been in this in the Wellness segment?

Ameera Shah: Majority of the Wellness revenues, remember when we are calling it Wellness, actually, the product is -- could be Wellness or it could be Bundling. So, this could be a patient walking in for 3, 4 tests for illness and has been saying that, look, I would prefer a larger variety of tests because I'm giving my blood. So, it could be a Bundled package, or it could be Wellness in particular. So, it could be either of them. So, this mostly we are finding is coming from the focus cities. But also, we are now beginning to see an increase from Wellness coming from smaller markets.

Saurabh Kapadia: Okay. Has the discount gone up in the Wellness segment for you?

Ameera Shah: Not really. I think it's about the same. I think depending on -- see, we are focused -- if we are in the core -- focus cities, the price for package is the same. But as we are entering into small markets -- smaller markets, the prices are obviously lower. Now it doesn't mean that the discount has increased, but the product itself is different. The product is lesser tests and therefore, lesser price.

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Because we find that the entry point for smaller markets maybe a little bit different than they would be for metro market. So that's why overall, we may find that the RPT for Wellness is marginally gone down, but it's not because of increased discounting, it's more because of product mix.

Moderator: Thank you very much. I now hand the conference over to the management for closing comments.

Ameera Shah: Thank you, everybody, for joining us and discussing with us today. As we mentioned, we feel very positive about this year. As we have shown that we have had a volume-based growth and we've been firm to be able to generate those kind of volumes even in the lowest quarter of the year, we firmly believe that quarter 2 and quarter 4 will continue to be the strongest quarter for the year from the revenue and the profitability perspective.

And we sort of stand by our comments and our commitment to get to sort of a similar margin profile as we were at last year, over the course of the year. Having said that, there will be seasonality, and therefore, every quarter will not be exactly the same.

We believe that overall, the next 3-year journey at Metropolis is going to be a really exciting one with not only our Core strengthening. We are very focused and clear on where we need to go in terms of the current business. And the opportunity we really see for Metropolis is expanding to a large number of cities with distribution all across the country, and continuing to build, as we said, strong B2C-led growth. But we also see opportunity in the adjacencies in health services and like we said in the bolt-on acquisitions.

So, I think as a management team, we are very focused and clear on where we need to go. And we are just at this point, just maintaining the execution rigour to achieve operational excellence as well as the numbers that we have committed to ourselves as well as to our shareholders.

So, I look forward to engaging with all of you on this journey in the next quarter, and of course, in the middle with those of you who we'll end up chatting with. Thanks so much.

Moderator: Thank you very much. On behalf of Nuvama Wealth Management, that concludes this conference. Thank you for joining us. You may now disconnect your lines. Thank you.

Call: Nov 2023

BLOOD TESTS • DIAGNOSTICS • WELLNESS

Metropolis Healthcare Limited

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Global Reference Laboratory: 4th Floor, Commercial Building -1A, Kohinoor Mall, Vidyavihar (W), Mumbai - 400 070.

Ref: MHL/Sec&Legal/ 2023-24/70 Date: November 10, 2023

To,
Head, Listing Compliance Department
BSE Limited
Phiroze Jeejeebhoy Towers
Dalal Street, Mumbai - 400 001.

Scrip Code: 542650
Head , Listing Compliance Department
National Stock Exchange of India Limited
Exchange Plaza, Plot No. C/1. G Block,
Bandra -Kurla Complex, Bandra (East),
Mumbai - 400051.

Scrip Symbol: METROPOLIS

Sub: Intimation of Transcript of Earnings Call for Q 2 and H1 FY24

Ref: Intimation under Regulation 30 of SEBI (Listing Obligations and Disclosure Requirements) Regulations, 2015

Dear Sir/ Madam,

This is in continuation to our letter bearing Reference No. MHL/Sec&Legal/202 3-24/62 dated October 30 , 2023. Please find enclosed herewith the transcript of Q 2 and H1 FY24 Conference Call, as organized by Nuvama Wealth Management held on Monday , November 06, 2023 at 09:00 a.m. (IST) wherein the financial results of the Company for the quarter and half year ended September 30, 2023 were discussed.

The same has also been uploaded on the Company's website and can be accessed at

<http://www.metropolisindia.com/investors>

We request you to kindly take the above information on record .

Thanking you ,

Yours faithfully ,

For Metropolis Healthcare Limited

Neha Shah
Interim Company Secretary &
Compliance Officer
Membership No. A51465

Encl osed: a/a

"Metropolis Healthcare Limited
Q2 FY '24 Earnings Conference Call "
November 06, 2023

Disclaimer: E&OE - This transcript is edited for factual errors. In case of discrepancy, the audio recordings uploaded on the stock exchange on 6th November 2023 will prevail.

MANAGEMENT : MS. AMEERA SHAH – MANAGING DIRECTOR –
METROPOLIS HEALTHCARE LIMITED
MR. SURENDRAN CHEMMENKOTIL – CHIEF
EXECUTIVE OFFICER – METROPOLIS HEALTHCARE
LIMITED
MR. RAKESH AGARWAL – CHIEF FINANCIAL OFFICER
– METROPOLIS HEALTHCARE LIMITED

HOST: MS. AASHITA JAIN – NUVAMA INSTITUTIONAL
EQUITIES

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Moderator: Ladies and gentlemen, good day, and welcome to the Q2 FY '24 Earnings Conference Call of Metropolis Healthcare Limited hosted by Nuvama Institutional Equities. This conference call may contain forward -looking statements about the company, which are based on the beliefs, opinions and expectations of the company as on the date of this call. These statements do not guarantee the future performance of the company, and it may involve risks and uncertainties that are difficult to predict.

As a reminder, all participant lines will be in the listen -only mode and there will be an opportunity for you to ask questions after the presentation concludes. Should you need assistance during the conference call, please signal an operator by pressing star, then zero on your touch -tone phone. Please note that this conference is being recorded.

I now hand the conference over to Ms. Aashita Jain from Nuvama Institutional Equities. Thank you, and over to you, Ms. Jain.

Aashita Jain : Thank you, Michelle , and good morning, everyone. On behalf of Nuvama Institutional Equities, I, Aashita Jain, welcome you all for Metropolis Q2 and H1 FY '24 Earnings Call. With us, we have Metropolis' senior management team, represented by Ms. Ameera Shah, Managing Director; Mr. Surendran Chemmenkotil, CEO; and Mr. Rakesh Agarwal, Chief Financial Officer. Thank you. And over to you Ameera for your opening remarks.

Ameera Shah: Good morning to everyone, and thank you for joining us on the Q2 FY '24 Earnings Call . Today, I'm joined by our CEO, Surendran, Rakesh Agarwal , the CFO; and SGA, our IR advisors. We've uploaded our updated results documents and the exchanges and the company's website, and I hope everyone has had an opportunity to go through the same.

We are happy to report a 13.4% year -on-year revenue growth in our core revenue for Q2 FY '24, mainly contributed by 10.4% patient growth and around 3% by product mix. And very marginally from price increase. However, the performance in Q2 was slightly affected due to the low rainfall in Maharashtra in August, leading to a decrease in dengue and other infectious diseases in the month of September. We absorbed an increase in infectious diseases case during the end of September and October, which has resulted in some of the September revenues carrying over from Q2 to Q3.

We have seen a steady increase in our volume over the last 6 quarters and are optimistic about volume growth going forward. Happy to share that our volume growth currently is trending higher than the pre -COVID level of 9%. Timing of price increases in our industry are company -specific and Metropolis would consider one early next year.

Also, we -- deliberately chose not to increase prices currently if we want to concentrate more on volume growth while the price level can be leveraged at the appropriate time. Let me begin with some industry insights. In the past two years, we've seen the emergence of new players in the e -pharmacy and hospital sectors venturing into diagnostic services. Based on their aggressive pricing strategies, with decent service, they have impacted companies who have built their

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business, the incumbents I'm talking about on competitive pricing, more than players focused on quality service.

We have observed a consistent decline in competitive intensity over the past two quarters. The new competitors are under margin pressure, forcing them to reduce discounts, reduced marketing budgets and increased prices. Moreover, they are also not finding much traction outside the metro town and metro towns are not showing much growth opportunity, especially since the wellness segment post -COVID has settled down since these companies based their entire business model on wellness.

On the contrary,

our ecosystem at Metropolis has demonstrated a marketable stability with significant reduction in attrition rates for both franchise partners and employee talent pool in the current fiscal. The competition that we are primarily experiencing is on the B2B semi-specialized care coming through the B2B channel, accounting for less than 10% of our revenue. Within this segment, there is the head and the tail of the business.

Considering the 80-20 principal, Metropolis concentrates about -- more on the head, which is 80% of the business, which are sticky customers who rely on our quality of testing and services rather than pricing and discounts. We're also happy to report that we have been able to increase our B2B revenues by 12% on a Y-o-Y basis without much dilution entry regardless of this competitive intensity.

Speaking of our strategy going forward. As communicated previously, we remain committed to our three pillars of growth under the Metropolis 3.0 strategy. As we discussed, one of the prime strategies was strengthening our core which is about fortifying our core business and expanding into new markets.

To achieve this, we've been working on a large network expansion plan, which allows us to increase penetration in our core markets and setting up infrastructure in new cities and towns with high growth potential. We're also focused on driving premium value products by offering curated customized packages for accurate diagnosis to our customers. We have also added more than 75 new specialty tests in the first half of this year.

Our second pillar is to expand into adjacencies. As we have mentioned, we are expanding into asset-light basic radiology like ECG, x-ray, phonography and allied services. We have allocated resources for providing these basic radiology services within our existing infrastructure. And therefore, by leveraging our loyal end customer, our extensive customer base, we aim to provide these complementary ancillary services so that it can increase our revenue per patient.

The third pillar has been digital transformation, and we're happy to share that we have launched our new registration platform along with our sales and service CRM, which has now been rolled out across the country. This move will give us an edge in improving customer relationships, enhancing customer services data centralization and better utilization of data pool benefiting the end user.

While we've come a long way in our website, we are still working on some of the other digital teams like the app to improve the experience and enhance it to the next level. But however, our Metropolis Healthcare Limited

WhatsApp messenger service has been very successful, allowing us to enhance our customer service and operational efficiencies, providing a seamless experience to our customers.

Lastly, the fourth pillar will be the bolt-on acquisitions to strengthen and complement our existing capabilities, either in new geographies or in existing geographies but with use cases. Despite the limited talent pool available in the industry, we have made substantial additions and retention in our talent base across all levels. As a result, we have assembled a committed team of senior executives.

And recently, we have also added Mr. Mohan Menon, as Chief Marketing Officer, he carries a rich experience over 20 years, which is last in at Max Hospital. This collective expertise and leadership ability will drive the company forward, creating efficient systems and processes and ensuring that we can continue to deliver high-quality services and our commitment to excellence. We're building an institution, which is not so much dependent on individuals, but more processes and systems. We have also demarcated a clear understanding on roles and responsibilities across the organization, including between myself and Surendran. While I focus more

on long-term

strategy, governance, acquisitions, alliances and culture, Suren and team will overlook operational excellence, productivity, process systems, digital agenda and control.

We would now like to -- update you a little bit on Hitech, which we acquired in 2021. At that

time, we had done this acquisition for three, four primary reasons. Number one, Hitech was the number two brand in the Chennai market with a sticky B2C customer base, they had almost 70%, 75% B2C contribution. But had low overlap in customer base with our brand Mr.

Metropolis. As both companies target a different segments with different prices.

Second of all, the promoter had been looking for an exit. And, if any, irrational player, would have purchased the company of Hitech, it could have spoiled the market to irrational pricing, which would have been all source impacted all the other players including us in the industry, including our pricing and our margin.

Number three, the company we acquired was on the basis of pre-COVID EBITDA numbers and revenue and not on inflated unsustainable COVID revenue margin. And we believe that, therefore, we were buying a sustainable vessel.

And four, in the past, Hitech had not done meaningful expansion for multiple years or pricing increase, and they did not have management expertise in -- besides the promoters. We're not investing much into the business either. And because they have not been investing much, the pre-COVID growth for Hitech was muted at 8% per year with stable margins.

We knew the potential of Hitech had a much larger runway for growth. So in the last two years now, I would like to talk about our performance in Hitech. We're happy to state that we believe this has been a successful acquisition for us, despite the multiple, unexpected faced, by the first year post acquisition from an unethical competitor, who tried to infringe on our brand and poach our team. Since then, we have added more than 35 centers, increasing the total count of centers now to 80 centers across Chennai. We introduced new models of growth such as B2B testing, curated wellness, and home testing, all of which was a very, very small part of Hitech before.

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We have integrated Hitech into the parent Metropolis and capitalize on synergies such as procurement efficiency, logistics, manpower management and marketing. And fourth, following the last year's resolution of brand infringement issue, we harnessed Hitech's regional brand trends and have grown our revenue growth of 17% in H1 of this year on a Y-o-Y basis, volume growth of 14% with an expanded EBITDA margin. The way forward for Hitech will be to continue to have see a substantial opportunities by expanding our specialty business as currently most of the business is routine and semi-specialized via B2C.

Second, expanding our network by adding additional 50 centers by the next year end. Thirdly, strengthening our corporate B2B business to maximize the capacities available at our labs.

Fourth, expanding your network and brand presence not only in Chennai but across Tamil Nadu through a large franchise network. Fourth, leveraging local synergies between Metropolis and Hitech to expand margins. And lastly, Hitech will continue to be growing positively from a revenue and a margin growth perspective going forward.

On a separate note, we received a whistleblower complaint from an employee in Metropolis, regarding some accounting adjustments in the debtors accounts for FY '22 -FY'23 and the current financial year. Upon receipt of the complaint, the matter was referred to an expert third-party auditor as per the whistleblower policy. The auditor submitted their report to the Audit Committee on 4, November, 2023.

As per the report, the Audit Committee noted that three employees were found and have indulged in passing accounting entries

es, which involved an amount of INR9.28 crores, wherein

INR7.18 crores have been adjusted between debit and credit entries in debtor account, and entries of INR2.1 crores were passed in the TDS recoverable account. The entries of INR7.18 crores passed in the debtor and creditor accounts, largely nullified each other, and therefore, the entry do not have any material impact on the accounts of the company.

Having said that, to be conservative and careful a one-time provision for doubtful entry of INR3.34 crores has been provided in Q2 FY '24 to ensure any full estimated impact for the report has taken. Audits have been done across the Group level post this complaint to find if there is any such issue in any other location, and we can confirm the issue have been found in only one region of India done by the selective three employees.

This incident was clearly a deviation from the standard accounting policies and controls and processing defined by the company and our culture of no shortcuts. The company has worked very hard to build strong process and systems and we were disappointed with the employee actions to breach these policies. The irregularity that it did does not involve any misappropriation of funds and therefore, does not require any amount to be recovered from the employees.

The motivation for these employees to go against the processes of the company was to show better individual performance and meet their performance targets. The company has taken appropriate action against the errant employees, including separation with immediate effect when necessary.

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We would like to emphasize that while we did encounter this irregularity our robust whistleblowing culture within the organization ensure that this issue came to life from within. Subsequently, we swiftly implemented all the required corrective measures to ensure the right practices. We will continue to further strengthen controls and audit and uphold our culture of integrity and good governance.

With this, I would like to hand over to Surendran for our update on the quarter gone by, along with the execution plans going forward. Suren, over to you.

Surendran C.: Thank you, Ameera, and good morning, everyone. The last three interactions we have guided are near-term objectives. Quarter 2, '24 as largely focused on those objectives. Let me give you an overview. We achieved 13.4 percentage Y-o-Y increase in core business revenue through a combination of increased patient volume growth, which is 10.4 percentage and the revenue per patient of 3 percentage growth.

Our cash volume stood -- growth stood at 8.4 percentage in quarter 2. Over the last five quarters, our core business revenue have consistently experienced double-digit growth, and we are optimistic of the same to continue. Higher volume growth across the territories in the key indicator of increasing market share of Metropolis.

We would like to highlight that historically, the second half of the year has been better than the first half for Metropolis. This year also, we anticipate a similar trend continuing and are confident of even better performance in H2 in terms of both revenue and growth -- revenue, growth and the margin.

Based on our internal estimates, we believe we can achieve a growth pattern in H2 similar to that of the previous year with the revenue growth in mid-teens and enhanced margins on account of better operating leverage.

Talking about B2C performance. Our B2C revenues were 16% year-on-year increase with patient volume growing at 14 percentage and 2 percentage on account of product mix change RPT. Our overall contribution from B2C segment has improved from 47 percentage one year before to 52 percentage when we ended quarter 2.

Mumbai market witness season higher growth of 23% in B2C revenue. Despite new competition in the market, our operation

al performance in Mumbai market has been exceptional, which has helped us to grow our market share. We continue to add more touch points in Mumbai and bring out more efficiency from existing centers.

We believe there is enough potential to grow touch points in Mumbai going deeper and closer to the patient. In spite of competitive intensity in certain process of the B2B segment, our B2B revenues increased by 12 percentage year-on-year, with patient volume growing at 8 percentage and RPT growth of 4 percentage. We have seen the competition intensity coming down over the last few quarters and are optimistic of higher growth going forward.

We have implemented multiple strategies to increase our share of B2B business, including B2B engagement via digital channels, centralized help desk to build a strong support system for B2B Metropolis Healthcare Limited

clients and appointing key relationship managers for our key B2B accounts. This clear balancing of portfolios between B2C and B2B is helping us to achieve the desired growth level.

Talking about the Specialized test segment, our revenues from specialized testing grew by 12 percentage year-on-year basis with a volume growth more than 9 percentage. Our B2C specialized testing revenues grew by 18% and our B2B specialized revenue grew by 10 percentage on a year-on-year basis.

For our specialty business, we have established a dedicated sales team, which we talked about in the past calls as well, trained them exclusively to focused specialty portfolios across the nation. We are continuously seeing volume uptick from our Wellness segment and are hopeful of this trend continuing.

Our Premium Wellness segment has been the fastest-growing segment for the company. We have witnessed a 27 percentage year-on-year growth in the quarter 2 with volumes growing by 21 percentage over the same period last year. Contribution from Premium Wellness has increased from 13 percentage when we ended last year to 14 percentage in quarter 2, '24. On geographical split, our revenue from North has grown by 15 percentage on year-on-year basis in quarter 2, outpacing the competition growth.

I would just like to give a glimpse of the network expansion. Under the 90/30 projects, we have successfully added 12 labs during the first half of the fiscal year '24. We plan to incorporate 30 labs during the year and 30 labs during year '25 as well. Our strategic focus in these new newly opened labs, mainly in newer geographies is to drive our B2B business in the specialty segment

and expand into B2C market by aggressively establishing a network of centers around this newly opened labs to serve as leader.

Since April '22, we have added 790 new centers, and we plan to incorporate 800 centers during this year and 800 centers in the coming year as well. We are now present in 488 towns as compared to 307 towns at the end of last year, '23. We plan to reach around 600 towns by the end of this year. Labs opened after April '22 have contributed 4 percentage of revenues and 4 percentage of volumes in quarter 2, '24.

As these labs are growing as much -- at a much faster pace, we expect them to contribute much more in the coming years. We are seeing continuous improvement in the margin for the labs and over the period, and we believe these labs can achieve company-level margin in about 24 months. We are equally focused on increasing the productivity of our existing lab and center network, which has shown an impressive during this quarter.

In summary, I would like to highlight that maintaining a healthy equilibrium between our B2C and B2B strategy, along with accelerating are key growth drivers, such as expanding our network, emphasizing on the speciality and wellness segment and implementing technology transition is propelling us swiftly towards our growth objectives.

Now I hand over to Rakesh to take you through the financial highlights. Over to you Rakesh.

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Rakesh Agarwal: Yes. Thank you, Suren. Good morning, everybody. Let me share some of the key financial performance for the quarter. Our reported revenue for quarter 2 financial year '24 has grown by 3% on a year-on-year basis. Reported growth for Q2 was largely impacted on account of in-sourcing of large B2G contracts in quarter 4 financial year last year.

Reported EBITDA margin stood at 24.3% as compared to 27% in Q2 last year. Compared to last year, the margin impacted due to more intensity in lab expansion, negative operating leverage because of the loss of PPP contracts and a onetime impact on account of provision for doubtful debt taken. These three elements impact the margin by around 2.5%.

Onetime impact in provision for doubtful debt on account of whistleblower complaint, which has been spoken by Ameer earlier, has been taken with impacted EBITDA by 1.1%. This is a one-time impact, and we have taken the full estimated impact and there is no further impact expected out of it. Excluding this one-time exception, our reported margin stood at 25.4%.

Reported PAT margin stood at 11.6% as compared to 13.5% in Q2 last year. Compared to last year, the PAT margin is mainly impacted because of the lower EBITDA margin.

Moving on to the balance sheet. Gross debt stood at INR 44 crores, and cash and cash equivalents stood at INR 108 crores as on 30, September 2023. As earlier confirmed, we plan to repay the balance debt by the end of this financial year. OFC-to-EBITDA for September '23 stood at 93%. Working capital stood at 18 days, marginally higher than the last year of 14 days. The Board of Directors have announced an interim dividend of INR4 per equity share.

That's all from my side. With this, I will open the floor for questions-and-answers. Thank you very much.

Moderator: Thank you very much, sir. We will now begin the question-and-answer session. The first question is from the line of Rahul Agarwal from In Cred Capital. Please go ahead.

Rahul Agarwal: Ameer, just one question. Firstly, a bit longer, pardon me for that. But whenever, we are doing channel checks across India, we obviously don't see further entry of new digital labs or online labs, this is the trend the last 15 months, which is good news. But we see regional local labs that started during COVID still trying to be competitive on pricing and taking a way tid-bits of market share.

Also I realize patients at large the affordable category mass market, will limited customer understanding of quality and service standards and customers and patients live in ignorance and happy with low pricing and just a name of the lab on the report, no NABL, etcetera, doesn't really matter to them. As a national lab like yours, trying to go into Tier 1, Tier 2 markets, I think patient, education and repeat of this business is the biggest challenge. I just wanted to hear your thoughts on this?

Ameer Shah: Thanks, Rahul. So a couple of things to address it. Look, there's always been competition in this industry. And I think while investors have noticed bigger players coming in the last two years, three years, but frankly, that has been the case for the last 15 years in our industry. Small players have always been there and they have continued to try to take market share locally. But it has never stopped at least our growth in the last 15 years to be able to continue. So I don't believe

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that, that is some significant change today that will, in any way, impede our growth going

forward.

To your point on consumers not recognizing quality mass consumers and only looking at price, largely when it comes to wellness, when it comes to common tests, you're right. Many consumers are not educated on understanding

the difference between a good quality lab and not so good quality, which is why 85% of the industry continues to remain in the unorganized sector, right? But what happens is that when they go to a doctor, especially for critical illness when they have a heart issue, they have a brain issue, they have a stroke or they have a neurological problem. Then the decision about which lab to go to is not really in the hands of the consumer. It is in the hands of the doctor because the doctor realizes the difference in the quality and requires the right quality report to make the right treatment decision because if a doctor gets the wrong report, they may land up removing an organ or doing something, which was actually not required at all. And therefore, the doctor will insist on a good quality lab.

Now the kind of business that Metropolis is focusing on has been always maintained, has been largely illness and have been largely more specialized. So we have always focused on going to the top quality doctors across the country in the Specialized segment. And what happens over time is that as your brand builds to doctor's recommendation for Metropolis.

Over time, that trust moves to consumers and consumers start recognizing that actually this brand is truly differentiated from others, and then they start coming to you directly. So the process, therefore, is a time-taking one, and one which is difficult for any new competitor to come in and suddenly build that kind of trust either with doctors or with consumers.

And which is why you see that pricing has been the main tool used by most of these guys to come in. But while it may give them some short-term benefit in terms of price and some volume accumulation, the reality is the market in which you can only build through price is limited. And if you want to get into a profitable business that is large and gets into scale, there is only one route which is through building trust with doctors and consumers.

Rahul Agarwal: I absolutely, agree. But the point being that most of us are trying to figure out whether the growth for National Labs is actually back to mid-teens. And hence, my sense is that these regional local labs are still doing dirty games but -- anyway, I get your point, I'll move forward. Second question on focus cities -- sorry, go ahead.

Ameera Shah: Yes. Just as Surendran said, H1 for Metropolis is always not the -- it's usually a smaller contribution to the overall year's revenue. And H2 is always a larger contribution to the revenues, and we expect nothing different this year. We do expect H2 to contribute more significantly to the overall year's revenues. And therefore, our target of mid-teen growth like it was pre-COVID remains the same.

Rahul Agarwal: Perfect. Get it. And on focus cities, there was some discussion happened on last quarter about redefining markets and some management focus has to be redefined in new cities. Any update here, please?

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Ameera Shah: So as you know, we are expanding a lot into some smaller markets. In fact, majority of the labs, we are starting are like -- 30 labs we are starting this year, I would say, majority of them are markets where we don't have a lab today, right? In fact, probably close to a very high number. So we -- what we are doing is really trying to recategorize and we didn't want to do this in the middle of the year, which is why we'll probably announce it at the end of the but we will recategorize the focus seating and other cities list at the end of this year. Based on the changes and the trends that we are seeing and the kind of investments that we are making. So we will come out with a new list by the end of the year. So that tracking is easy for us as well as for the investor.

Rahul Agarwal: Got it. And lastly, just sneaking in one smaller one on creditors. I see last two years, every six

months, the creditors as a percent of sales is declining. Is there some change with how we're posting stuff from vendors?

Rakesh Agarwal: Yes. So I will take this. Basically, there are a bit of a credit decline because we have made some prearrangements with some of the vendors in terms of discounting and the payment terms. So in -- particularly in the material piece, we have done some maneuvering while we are getting a bit higher discounts from the vendor in view of giving them an advance or a quicker payment. So I think that is the reason why you are seeing a bit of drop in the in the creditors balances, but that is correspondingly helping us to increase the discount from these vendors.

Rahul Agarwal: So gross margin goes up and creditor goes down and ROCE goes up. That's how we look at it.

Rakesh Agarwal: Yes. So gross margin, you will see largely in last two years, you have seen gross margin either remaining stable or increasing. So I think that's the reason why in spite of everything happening around us and dollar appreciating, we are still able to maintain the kind of gross margin we are

having.

Rahul Agarwal: Perfect. Thank you so much and have a good festival season. Happy Diwali to all of you. Thank you.

Rakesh Agarwal: Thank you.

Moderator: Thank you. We'll take the next question from the line of Jainil Shah from JM Financial. Please go ahead.

Jainil Shah: My first question is on the EBITDA margin, where do we see our sustainable EBITDA margin going forward?

Ameera Shah: So I mean, we had always indicated that our EBITDA margins this year will be similar to last year. We have to remember that we are having a dilution, as we mentioned, of about 1.2% because of the expanded lab expansion that we did not have in pre-COVID periods. So if that was kept aside, then we would be closer to 26.5% to 27%. We believe that a sustainable margin going forward is going to be -- without this expanded lab expansion, we'll be between 26% and 27%.

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And our growth of expansion, this lab expansion will continue till FY '25, post which the lab expansion will then slow down and our collection center expansion will expand. So we are, therefore, quite hopeful that post FY '25, we will see that expansion in margins happening at each year.

Jainil Shah: Sure. So is it fair to assume that over the next three years, we'll see -- if you take a slightly longer term view, three years to four years, we'll see mid-teens revenue growth along with healthy operating leverage play out because then our lab expansion would be done and then that would also come into play. And how would the addition of basic radiology services add to this margin? Or will it increase or dilute the margins?

Ameera Shah: So I think you're right, that's exactly the direction we are targeting. And for the basic radiology services, the goal, as we mentioned, is it going to be continue to be a small part of our revenue. It is not going to become a very large part of our revenue. It is more to complete the service for the consumer when they walk into our center. And therefore, the hope is that we can add on some of the ancillary radiology revenue to the existing patient who walks in, and therefore, that could expand the RPP for that particular patient.

It's completely asset-light. It's not capex heavy at all. And therefore, we believe it strategically makes sense in adding them in our existing infrastructure. That would be -- but considering it will be a small part of the business. I don't see it meaningfully impacting EBITDA or revenue in any significant way, but there -- which hopefully it will impact the RPT.

Jainil Shah: And how much of this mid-teens growth would be from realization?

Ameera Shah: So as we mentioned, about 10.4% is coming from volume. And about 3% -- currently about 3% is coming from product

mix, which is realization. Hardly about a -- very marginal amount is coming from price increase because we have not really done a proper price increase this year, which like we said we are targeting maybe for early next year. So out of the mid-teen growth, if you see pre-COVID, we were doing about 9% from volume and about 6% from RPT. Our goal is probably 3%, 4% from RPT and the balance through volume.

Moderator: Thank you. The next question is from the line of Ranvir Singh from Nuvama. Please go ahead.

Ranvir Singh: Sir, two things I wanted clarity. One is that it seems that quarterly number has been restated. So just a book keeping because earlier the reported number in last previous corresponding quarter was INR 300 crores. Now this is showing INR 277 crores. So is there -- has been a restatement or readjustment here on revenue item?

Rakesh Agarwal: We will look into it. But I think what you are referring is that there is one revenue which we are talking about, a core revenue and one is the reported revenue. So I think when you are saying INR 300 crores, you are looking at a reported revenue, and there is a INR 277 crores, which you are looking at a core revenue. So these are the two elements of it. If you look at our investor presentation, Slide number seven, I think you will particularly get the answer of that.

Ranvir Singh: No, I'm not referring to presentation. I'm referring to the reported -- your audited reported number, which is INR 300 crores. Last year, it was INR 277.0911 crore. I think last year, in what

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model I see that last year, this was INR 300.3493 crore. So there has been some INR 20 crores -- roughly INR 23 crores difference. And even in Q1 also, reported number was INR 277 crores in last time -- sorry, this time it's a INR 300 crores, yes. So -- it seems that there has been either number had been readjusted somewhere or...

Rakesh Agarwal: So I don't think this is the case, but I -- in the audacity of time, we will note down this and we'll

come back to you after this call.

Ranvir Singh: Okay. Because otherwise, excluding that PPP...

Rakesh Agarwal: I think maybe you are -- you are not including the NACO. Last year, there was a NACO PPP contract of INR 21 crores. May be because of that, you are looking at a different number last year because last year, we have given a core revenue, which was including the NACO revenue. And if you look at this year, when we are talking about is without this PPP contract revenue and we are trying to do apple-to-apple comparisons.

So just Mr. Singh we'll pass this question a bit, but we'll clarify you for sure. There should not be any kind of difference in that.

Ameera Shah: But Rakesh, we have not done any readjustments of statements that .

Rakesh Agarwal: No, nothing. I think we have to clarify this, and I just want to not to just spend time here, but we will clarify for sure up after -- just immediately after this call.

Ranvir Singh: Sure, sir. And secondly, on PPP side, is there any chances we are re -getting that PPP contract or -- do we see any such opportunity going forward? Or are these opportunities gone?

Ameera Shah: See, the government business was never our interest, frankly. This particular contract we did for the last five ye ars, more because it was specialized tests, it was profitable and practices were in line with us. And therefore, we went ahead and did the contract. But otherwise, government business, we are not frankly focusing on. And this particular contract has been e nforced by the government. They are doing the testing themselves.

So it has not gone to any other player. But the government has wanted to do it themselves and be a provider. So therefore, now the chances they bought the machines they have set up the staff and the facilities, the chances of this getting outsourced

again are extremely low.

Ranvir Singh: Okay, fine. And margin was -- how was the margin there in PPP contract that was better than consolidated our normal margin or has been lower there?

Rakesh Ag arwal: Yes. So the margin in the PPP contract was approximately 43% for us, and that business contributed to 7% for us. So therefore, once the business is not there, we are having a 1.1% contract in the margin because of the sale. And Mr. Singh, I think yo u are referring to quarter two number, which was INR 277 crores in our financial statement, quarter one number. So let us reconcile it, but I think you have INR 277 crores reference is coming from quarter 1 number rather than in our last year numbers. So a nyway, we'll confirm you that.

Ranvir Singh: Sure, sir. Thank you. That's all from my side. All the best.

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Moderator: Thank you. We'll take the next question from the line of Shyam Srinivasan from Goldman Sachs. Please go ahead.

Shyam Srinivasan: Just in li ght of the B2G contract going away. And just a request, maybe we can do this after your call as well. Just to see what the overall volume was over like a three years, four years, five - years period, either you want to remove B2G historically and -- or keep it in there?

And just for us to see how the volume growth is over a longer period? I mean, I think you said 9% was what your historical volume growth was. So I just wanted to see maybe we take out B2G out and see it historically or included -- so the idea is just to see what is the like -for-like because when we report core volumes now, we are unable to see what is the B2G volume element. So that's just a request. But the additional question I have is just on test growth has been -- on core volumes, test gro wth has been slower than patient growth. So anything we need to keep in mind here?

Ameera Shah: So Shyam, we've always mentioned that don't focus on our test growth, please focus on our volume growth. The reason is because each company has their own way of accounting tests. So for example, in Metropolis, one wellness package is accounted as one and not as, let's say, even if it has 30 tests in it, right? So therefore, the -- while other companies may account for it differently, so therefore, we have always mentioned, please focus on patient growth because that is more unique and fair to compare versus test growth may not be appropriate to compare.

Shyam Srinivasan: Got it. And the request on volume growth, if you could have...

Ameera Shah: We'll come back on that post.

Shyam Srinivasan: Yes. Perfect. Just a second question on Hitech. I think a very commendable job of turning around. You mentioned the difficulties in the first year. I don't know whether you have put the subsidiary financials or now it's merged , so we may not get the subsidiary financial, but what is the fiscal '23 number for where revenues was? And also maybe one at -- since you've talked about the 17% growth and margins being better, if you could quantify some of these numbers?

Ameera Shah: So I mean I think the 17% growth, I think, is quite clear. The margins are better than what we acquired than that. We've seen a few hundred basis points expansion. And the idea will be to obviously continue to grow this business well at a higher margin. And too many more details will become a competitive problem for us. So I think we'll probably stick to these numbers as of now. But we can chat offline. If there's any other specific information with it.

Shyam Srinivasan: Got it. And my -- just a last question on the overall guidance. Your ability to grow faster than National Networks. I think also I think one of your slides you've shown you're growing faster in the north. So just maybe talk about your regional strategy in terms of noncore markets, if I can use that term maybe your seeding markets and your other market

s. What's the strategy out there?

How can we outpace -- continue to outpace competition there?

Ameera Shah: Suren, you want to make that?

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Surendran C.: Yes, let me take this. So I think we have mentioned in the past also the strategy of expansion is go deeper in a strong market and go wider in the noncore market. So what we're doing exactly the same thing for let's say, come to the North or the eastern part of the East. We are picking up the strategic locations, and we are putting up the lab there.

And we continue to do that, and we start with the B2B client acquisition first and then we get into some level of volumes, and then we put up the lab and then we put up the center, right? So that's what we are doing, and we are seeing success around that. So we'll continue to do that. We go -- we pick up the strategic locations in both these geographies, which is North and East, and then we will keep adding the centers there.

And if you have seen that I just mentioned that the number of towns that we operate has gone up from 307 to 488. So this is -- most of these new towns have actually come up in the north and the east, right? And the -- west and the south, we keep going deeper. For example, Mumbai, we believe that we can have more centers in Mumbai to geographically become stronger. And similarly, let's say, in Chennai at Kerala. So that's largely been the strategy that we're adopting. Moderator: We'll take the next question from the line of Rishi Mody from Marcellus Investment Managers.

Rishi Mody: Yes. So a couple of quick bookkeeping ones. Firstly, your gross margin has expanded I think you all gave a comment on that. Just could you repeat how was that 2.5% increase gross margin how much of it is structural versus how much is a function of some seasonality element?

Rakesh Agarwal: So I will let you know that this is basically because of the contract which we had on PPP because the majority -- the cost coming in that contract was of material. And once that has gone away, majority 80% of the benefit which you are seeing in the gross margin coming because of that. And 20% of the margin is basically on the price negotiation, which I discussed earlier also and some part of operational efficiency. So whatever gain you have seen, 80% of the gain basically is because of the PPP contract going off, which had a higher material cost in comparison to other material cost.

Rishi Mody: Okay. Got it. Okay. Secondly, in order to understand from Suren -- so we've been seeing that our seeding cities revenue growth has been lagging our focus cities, which is largely our mature markets, the focus cities. Just wanted to understand what -- could you help me understand the disconnect here? The -- ideally our seeding city should run ahead of our focus cities? So if you could just give me an understanding why we have been lagging for the last couple of years out there?

Surendran C.: Yes. I think Ameera mentioned this, we're just going to recalibrate the old definitions so on, the focus city, seeding cities and other cities. So we have actually kept our focus mainly on the other cities and new markets, right? So maybe -- I mean, we are not only focused on getting the new labs and centers and productivity from the seeding cities. So at the end of the day, you have to prioritize what you want to do.

So we have just got an expansion on the -- some other cities. If you see the other cities and the growth is substantial, right? So we have -- going to recalibrate the definitions of the seeding

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cities and the focus cities in the next -- when we get into the next year. But otherwise, this is the reason why in the seeding cities we are not finding significant growth.

Ameera Shah: If I can just add something to that. If you look

at the seeding cities, primarily in markets like

Delhi, Calcutta, some other markets that are there in the seeding cities, the kind of growth that has been built was B2B and coming primarily from semi-specialized tests. Now I may mention in the competition, not only in the last year but the last many years has been primarily on the B2B semi-specialized space. So therefore, even though you may not see volumes often going down in these markets, but sometimes because of increased discounting, the revenue per patient in the seed markets may have slightly decreased.

Versus, if you look at the focus cities. The focus cities have been primarily B2C for us. That's how we categorize the focus cities, right? And these markets were all brand-led very B2C -

focused. And we've actually gone much deeper and expanded our networks in B2C in these markets, which are Bombay, Pune, Chennai, Bangalore, Surat. And what we've done is we've gone more aggressive on the B2C side, which frankly has not been so impacted in the last few years despite all the competition. So therefore, you are generally seeing a much faster growth in focus and a slightly slower growth in the seeding cities.

As we are now making some B2C headway in seeding cities, we are seeing our B2C contribution in the seeding cities actually increasing. So earlier where it was much, much lower. It has probably now come closer to 25%, 30% of contribution of B2C in the seeding cities, so maybe a little bit higher, which is giving us good opportunities to actually expand this area more and not have only focus on B2B.

Rishi Mody: Okay. So just then the structural one, right? Are we changing our approach to new geographies because you say, right, that B2B semi-specialities, how we enter a new market, and that's always going to be a commoditized market. Then how do we intend to geographically expand if we are all in already struggling on this front?

Ameera Shah: See, like you said, the B2B semi-specialized again has a head and a tail. It's not that the entire category is commoditized. We have a lot of B2B semi-specialized business today, which is very profitable for us. So the issue is about whether you get into a market and like we said, 80% of our business traditionally comes from the head. So even that 80% of that business is stable and sticky. But if 20%, which is coming from your tail is churning, you are going to see slower growth, right?

So the strategy now going into most of the market is focusing more and more on the head, focusing more and more on in the semi-specialized segment and focusing more and more on specialized test. So this is naturally more sticky because obviously, not everybody can do it and because the quality of the result is that much more important and critical for the doctor.

So I think, the strategy going into whether the existing seeding markets or into new markets will be to focus on the head of semi-specialized to reduce the tail and second, to focus more on specialized.

Moderator: We'll take the next question from the line of Aashita Jain: from Nuvama Institutional Equities.

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Aashita Jain: So just two questions from my side. First is on the Mumbai market and the focus cities market.

So you mentioned one comment that you're still going deeper in the Mumbai market and opening more of the touch points. So how do you think that the Mumbai and other focus cities can grow for us in the next two years to four years?

Rakesh Agarwal: Can you just repeat the question? I didn't get the question, Aashita.

Aashita Jain: Sir, you mentioned in your comments that you're still going deeper in the Mumbai market, and there's a lot of headroom in your -- even in the core regions of Mumbai, Pune and other focus cities. My question is, how do you think the Mumbai and other focus cities can grow for us in the next two years

to four years?

Rakesh Agarwal: Yes. So exactly what we have done in Mumbai is what we would like to replicate in other cities as well. So we are going deeper in the other cities as well. For example, Chennai, Bangalore, right, on some of these big cities also, we are going deeper. So -- and also the focus on the specialty in all these markets. So what we've done in Mumbai is exactly base -- I don't we would like to do this in the other big focus cities as well.

Aashita Jain: So is it fair to assume that these markets will continue to grow, say in high teens, higher than your overall revenue growth?

Rakesh Agarwal: Yes. That's the fair expectation, right.

Aashita Jain: Okay. And my second question is on the acquisition front. So just your thoughts on the kind of geographies, the size, the target company -- and has the valuation come down compared to the COVID time?

Ameera Shah: The valuations during COVID times were based on COVID revenue and EBITDA, which frankly, if anybody did would have been unsustainable if you bought it on the back of that, right?

So clearly, today, with no COVID and revenue and EBITDA, there's a far better chance of it being a sustainable business.

So having said that, a large part of the unorganized market is still not completely compliant with best practices, etcetera. And therefore, it reduces the funnel of possible acquisitions you want to do, otherwise, you landed up in helping with these skeletons with closets. And many of the larger deals that are available, again, I would say the quality of these always available is not great. So here we -- we are very selective about what we buy.

And even if that means paying a slight premium for a good quality business, we would rather do that than buying something cheap, which is a poor quality, and we know 30%, 40%, 50% of the revenue will have to go because either they don't follow good practices or things that we are not in line with.

So I think we'll continue to be selective, but we definitely are building a funnel of potential acquisition targets. Like we said, we are looking at things which are strategically going to add to us either in new geographies or in new skill set. So I think once there is some more -- something to announce, we'll be happy to share more. But at this point, it's more general.

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Moderator: Ladies and gentlemen, this will be the last question for today, which is from the line of Kaustav

Bubna: from BMSPL Capital.

Kaustav Bubna: So basically, I had two questions. One was on the possible government regulations that could face this industry, I basically wanted to understand what could be -- I know it's a little bit of a futuristic question, but what could be the possible government regulations that could impact. This industry had -- more importantly, how could this benefit players like Metropolis, Dr. Lal, etcetera, the organized player, how could it benefit us?

Because if a majority of this industry is unorganized, I would assume that certain regulations could actually work in advantage for organized players. So could you explain that a little bit? That's the first question. And the second question is basically, what is the company's philosophy on debt going ahead? Do we -- I mean, because there will be acquisition opportunities ahead going into the future and growth opportunities going future, too.

But also, we have the potential to be a negative working capital business with more B2C contribution and generate cash. So I wanted to understand, do we have some goal and aspiration to continue being net cash I mean to be net cash and continue being net cash over the years. So those were the two questions.

Ameera Shah: Sure. So regard of regulation, you're absolutely right I mean this regulation was to come in, I think it would be very advantageous to the larger players because

today, we are operating in a completely uneven platform, as an industry, where there are some of us who are following global quality and some who may or may not do the test at all, right?

So it's a uneven platform and therefore the conversations lands up -- going to things like trying other things, which frankly, in health care tend to be less relevant. So if regulation were to come in it would -- could be very beneficial to organizations like us who are already following all the compliances and practices.

And actually, it would create a burden on smaller companies to now come up to a certain quality, which means that we would possibly see a reduction in the number of players or at least an increased cost in their operations, which would not then allow them to spoil the market with very, very poor price. So we have been obviously talking to the government for long time to say, look, please put some minimum standards in our industry, it's very much required. Government has shown a lot of intent and excitement to want to do it. But obviously, we have no control on any of the time lines or when and how that would happen.

I think on your second question around debt and cash flow. So actually, starting this year, we are moving to a bi-annual dividend policy, unlike the last year where we're doing once-a-year dividend. And therefore, we have declared something right now, as Rakesh said, and we would probably declare a dividend at the end of the year as well.

So we're looking to see how, one, we can continue to have a very stable, predictable dividend payout, which we've had for the last many years at about 35% to 40%. And we'll continue to that. We would then like to use our cash to obviously do these bolt-on acquisitions wherever we feel that they can strategically benefit Metropolis going forward.

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See, generally, we would like to be a net cash organization where we are very efficient as far as our working capital, and that's the direction that we are wanting to head to. And we continue with net cash but having said that, if there are acquisitions that do come up that require us to take a conservative level of debt in a market that we know is stable.

We are not completely closed to the idea of taking debt as we have seen with Hitech as well. But we are not the organization to use debt as an expansion strategy on a regular basis, neither would we do it in any significant multiple of EBITDA. So it would be a conservative approach for us. But generally, we would like to be in a net cash situation.

Moderator: We'll take one more question from the line of Tanmay Gandhi from Investec.

Tanmay Gandhi: Yes. Sorry, I got disconnected for a while. So it's probably, this is repeated. So firstly, are we qualifying what is the impact of dengue in this quarter?

Ameera Shah: Dengue for us has not been a very strong thing because dengue was mostly seen in the north of India, it tends to be a lot around Delhi -NCR. We don't see as much dengue -- at least we haven't this year or the last few years in the west of India. So it's very -- it's not a very significant number.

Tanmay Gandhi: Understood. And just a bookkeeping question. So if we look at our NACO contribution, right? So if we look at what we have disclosed for the entire year in 4Q FY '23. And if we come up so -- and again, we have disclosed for each quarter as well. Based on that annual number minus all the remaining three quarters, so the ideally the base should be INR 17 crores, but this quarter, we have been paying INR 21 crores. So can you help us reconciling that? And is INR 21 crores is the correct number to look at?

Rakesh Agarwal: Yes, INR 21 crores is the correct number to look at because there were two contracts which we had. Majorly the PPP contract, which we are talking about, which has ended last year Feb. And there was another

small contract which was there, which was MCGM in Bombay. So these are major contract or one was the major one. One was a very small contract, two contracts taken together, the impact is INR 21 crores.

Tanmay Gandhi: Okay. So INR 17 crores plus INR 4 crores is correct number to look at. Got it. That's all from my end.

Moderator: The next question is from the line of Harith Ahamed from Avendus Spark.

Harith Ahamed: I'm looking at the segmental breakup that you provided B2C, B2B and the other segment. What exactly is this other segment? Is it B2G, 7% of revenues for the quarter?

Rakesh Agarwal: Yes. So this will include your clinical trials, which we have around 2% to 3%. There is a corporate business of 2%, 3%, and there is some government business in. So this all including together, is your -- others.

Harith Ahamed: Okay. And I'm also looking at the revenue per patient growth for the core business for the quarter at 3%. So is it correct to assume that this 3% growth is almost entirely coming from the higher share of premium wellness business. Just trying to understand if there is an element of price

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increase here. Your competitors had talked about taking price increases in the last few months and that has reflected in their earning potential growth...

Rakesh Agarwal: Yes. So basically, the increase in RPP has come through product mix, rightly, as you said, from a higher volume of wellness revenue and, to some extent, specialty growth. And the price increase is very, very marginal. As we mentioned earlier also, we had a very small marginal increase in price, which is contributing to around 0.7% very, insignificant percentage for the RPP.

Moderator: As there was the last question, I would now like to hand the conference over to the management for closing comments. Over to you.

Ameera Shah: Thank you, everybody, for joining us today and hearing all the experiences over the last quarter, and for all the questions. I just wanted to say a happy Diwali to all of you. We obviously feel very positive about the direction that Metropolis is heading in. Not only do we see a very robust substantial volume growth and RPT growth, but we do believe that H2 will be a better half for the year for Metropolis from a revenue and a margin perspective.

Overall, we feel that the hump of COVID is done with. And while there was -- while it continues to be hump competitive intensity. But regardless of that, we believe that we are on our own track and our own journey and can continue to meet our targets regardless of the market.

We have built a strong leadership team led by Suren and bunch of CXOs, and we feel very confident around the robustness of the processes, system and the kind of momentum and rigor that we are seeing happening in the market and Metropolis being seen as an aggressive player. And the expansion of network that we are doing across all the different towns basically will obviously continue to give us a lot of rewards in the years to come as we open a lot of markets with labs as well as the collection centers.

As we mentioned as FY '25 -- while we continue to add all these collection centers now, that will become even more aggressive post FY '25 as the lab expansion part slows down. But -- we look forward to continuing to engage with all of you outside of these reviews, meetings as well. And again, happy Diwali to all of you, and best wishes to all of your families. Thank you so much.

Moderator: Thank you, members of the management. On behalf of Nuvama Institutional Equities, that concludes this conference. We thank you for joining us, and you may now disconnect your lines. Thank you.

Call: Feb 2024

BLOOD TESTS • DIAGNOSTICS • WELLNESS

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Ref: MHL/Sec&Legal/2023 -24/92 Date: February 09, 202 4

To,
The Manager - Listing
BSE Limited
Scrip Code: 542650 To,
The Manager - Listing
National Stock Exchange of India Ltd
Scrip Symbol: METROPOLIS

Sub: Intimation of Transcript of Earnings Call for Q3 FY24.

Ref: Regulation 30 read with Schedule III of SEBI (Listing Obligations and Disclosure Requirements)
Regulations, 2015 .

Dear Sir/ Madam,

This is in continuation to our letter bearing reference no. MHL/Sec&Legal/2023 -24/82 dated January 29, 202 4.
Please find enclosed herewith the transcript of Q 3 FY24 Conference Call, as organized by YES Securities (India) Ltd. held on Monday, February 05, 202 4 at 09:00 a.m. (IST) wherein the financial results of the Company for the quarter and nine months ended December 31, 202 3 were discussed.

The same has also been uploaded on the Company's website and can be accessed at :

<http://www.metropolisindia.com/investors>

We request you to kindly take the above information on record.

Thanking you,
Yours faithfully

For Metropolis Healthcare Limited

Kamlesh Kulkarni
Head – Legal & Secretarial
Membership No.: A27554

Encl. : A/a
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“Metropolis Healthcare Limited
Q3 and 9 Months FY'24 Earnings Conference Call ”
February 05, 202 4

Disclaimer: E&OE - This transcript is edited for factual errors. In case of discrepancy, the
audio recordings uploaded on the stock exchange on 5th February 2024 will prevail.

MANAGEMENT : MS. AMEERA SHAH – MANAGING DIRECTOR –
METROPOLIS HEALTHCARE LIMITED
MR. SURENDRAN CHEMMENKOTIL – CHIEF
EXECUTIVE OFFICER – METROPOLIS HEALTHCARE
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MR. RAKESH AGARWAL – CHIEF FINANCIAL OFFICER
– METROPOLIS HEALTHCARE LIMITED
SGA, INVESTOR RELATIONS ADVISORS – METROPOLIS
HEALTHCARE LIMITED

MODERATOR : MR. BHAVESH GANDHI -- YES SECURITIES

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Moderator: Ladies and gentlemen, good day, and welcome to the Q3 and 9 Months FY'24 Earnings Conference Call of Metropolis Healthcare Limited hosted by YES Securities. This conference call may contain forward-looking statements about the company, which are based on beliefs, opinion and expectation of the company as on date of this call. These statements do not guarantee the future performance of the company, and it may involve risks and uncertainties that are difficult to predict.

As a reminder, all participant lines will be in the listen-only mode and there will be an opportunity for you to ask questions after the presentation concludes. Should you need assistance during the conference call, please signal an operator by pressing star, then zero on your touch-tone phone. Please note that this conference is being recorded.

I now hand the conference over to Mr. Bhavesh Gandhi from YES Securities. Thank you, and over to you, sir.

Bhavesh Gandhi: Thank you, Rakesh, and good morning, everyone. On behalf of YES Securities Limited, I, Bhavesh Gandhi, welcome you all for Metropolis Q3 FY'24 Earnings Call. With us today, we have the Metropolis senior management team, represented by Ms. Ameera Shah, Managing Director, Mr. Surendran Chemmenkotil, CEO, and Mr. Rakesh Agarwal, Chief Financial Officer.

I will now hand over the call to Ameera ma'am for her opening remarks. Over to you.

Ameera Shah: Good morning, everyone, and thank you for joining us on the Q3 FY'24 Earnings Call. Today, I'm joined by Suren, the CEO, Rakesh, CFO and SG A, our IR Advisors. We've uploaded our updated results documents on the stock exchanges and the company's website, and I hope everyone's had an opportunity to go through the same. We are happy to report a 12.3% year-on-year revenue growth in our core revenue for Q3 FY'24, of which 9% has been contributed by patient growth volume and the balance 3% is on account of product mix change.

The performance in Q3 was marginally impacted on account of heavy rainfall and floods in Chennai and adjacent areas in December '23, leading to a loss of revenues of approximately 7 to 8 days from the Chennai region, which is one of the focus cities for Metropolis. We estimate growth would have been closer to 13.5% without this disruption. We have been witnessing a consistent increase in our volumes for our core business over the last 7 quarters and are optimistic about the growth trajectory going forward.

Let me begin with some broad industry insights and specific trends for the quarter gone by.

Diagnostics as an industry is poised to grow at about 8% to 10% CAGR by FY'27. And we had laid out our addressable market, which is poised to grow at about 10% to 11% CAGR. And being a pioneer in the industry, we are confident of outperforming the market growth. Outpacing industry growth will be primarily driven by number one, geographical expansion that is deeper into retail existing markets and wider penetration into new markets.

Second, capturing large market share in the wellness segment amongst consumers who are healthy. Number three, increased specialized testing segment by building more credibility with Metropolis Healthcare Limited

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the best specialized doctors in the country and number four, better execution in terms of distribution, service quality, testing support quality and strength of test menu.

Speaking of the quarter gone by, on ground situations and relative competitive intensity.

Historically, the third quarter typically experiences a slowdown in the healthcare sector due to festivities and holiday season. Looking at past trends in hospitals, pharmaceuticals and diagnostic companies, Q3 usually reflects a decrease in activity and this quarter follows the same

pattern across the healthcare industry.
The decline in OPD hosp

ital visits directly influences diagnostic volumes in the B2C segment.

And additionally, the slower IPD business outsourced by hospitals to larger chains like us affects the B2B segment. Despite a softer quarter for the overall healthcare industry, our volumes at Metropolis have increased faster than the market on a year-on-year basis, indicating growth and increase of our market share across segments.

On the competition front, we have been hearing of expansion of labs by new age and new healthcare competitors in terms of geographical reach. But it's important to emphasize that these players seem to be primarily focusing on low-priced B2B business via this network expansion. This particular segment is characterized as being non-sticky with poor unit economics.

We had previously highlighted competitive intensity prevailing in the semi-specialized B2B business, which contributes only a single-digit revenue to our overall revenues. However, over the last quarters in this segment also, we have observed positive trends in the revenue per patient, indicating an improvement in our B2B pricing with increasing volumes.

In Q3 FY'24, our overall B2B business grew by 7% with a volume increase of 5% and an RPP growth of 2%. And for the 9 months ending FY'24, RPP growth also stood at 2%, reflecting a positive shift in the environment. This indicates that the competitive intensity in the semi-specialized segment of B2B is diminishing, emphasizing a preference for quality over price or discount.

Also, the growth rates of app downloads and usage for emerging health tech players have been slowing down, suggesting challenges in acquiring new customers in the B2C segment. It appears that aggressive pricing strategies and convenience have been the primary competitive edge of these firms, which was valuable to build the wellness segment during COVID but in the post COVID era where illness testing is back in focus, the dynamics have changed.

As Metropolis undergoes transformation in digital technology and customer convenience, we are well positioned to capture higher volumes through online channels. Additionally, our aggressive plans for lab and center expansion further strengthen our position in the big brick and mortar route.

I would like to share some color on the recent news articles and onetime provision done for receivables from the Aam Aadmi Mohalla Clinic project, which we had received a work order for in February '23. Let me highlight the following.

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The company has followed all due procedures and guidelines as per the contract, and all samples that were handed over to us have been duly tested and reported. Billings for testing has been strictly done based on the sample received, test performed and reported. Interacting with patients, collecting their samples, collecting their data and registering in the system was the primary responsibility of Mohalla clinics as per the contract.

Our role was only to pick up the sample after collection done by Mohalla Clinic and then to do the testing and generate and submit reports back to the Mohalla clinics. Due to multiple payment delays. This was not a viable contract for us, and we have sent multiple notices for recovery of payments.

The company also sent a notice on December 19, '23 to the government authorities for discontinuation of the contract. Although we would like to exit this contract, we may have to continue to provide services if the government insists. Considering the current circumstances, we believe the best practice would be to provide 100% of the receivables of the Mohalla Clinic project from February '23 to December '23, as we have limited influence on the future direction of this project.

This onetime provisioning has an overall impact of 0.8% on EBITDA for Q3 FY'24. The company is fully cooperating with any relevant authori

ties and providing all information

proactively as they investigate the matter. We filed a statement with SEBI on the above on January 15, 2024, and you can access that document for any more information.

Going forward, with the brand strength of Metropolis, the trusted partners for doctors and consumers, coupled with aggressive expansion plans and foray into adjacencies with opportunities for inorganic growth. We are optimistic of outperforming in the industry growth in terms of revenue growth and profitability.

We feel optimistic about core revenue growth and margin expansion in Q4, as usually, it's the best quarter of the year for the industry. Also, Q4 will be the first quarter where reported revenue and EBITDA growth will be similar to core revenue growth and EBITDA growth because our NACO contract had gotten completed in the last year by February 2023.

And therefore, the base year will now only have 1 month of NACO revenue in Q4 compared to

the first 3 quarters of FY'23, '24, where NACO revenue was a continuous part of it, making reported revenue and profits look subdued even though the core business is growing very well. With this, I'd like to hand over to Suren for our update on the quarter gone by, along with our execution plans going forward. Over to you, Suren.

Surendran Chemmenkotil: Thank you, Ameera. Let me give you the specific highlights of this gone quarter. In quarter 3 '24, our core revenue demonstrated a robust year-on-year growth of 12.3% driven by a patient volume increase of approximately 9%. The remaining 3% of the growth can be attributed to changes in product mix.

Notably, our test volume expanded by around 11% compared to the same period last year, consistently surpassing industry growth figures over several quarters. Please note, Metropolis Metropolis Healthcare Limited

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counts 1 package or panel as 1 test. And therefore, our individual test volumes would be much higher.

We would like to highlight that we have taken a price increase on our B2C segment on a pan-India basis, effective January '24, the impact of which will be seen in this quarter and also full year '25, with increased revenues and margins. We estimate the benefit of this to be approximately 3% of the revenue.

We're also planning to take a price increase in our B2B segment, specifically for the specialized and super specialized tests in quarter 1 FY25, which will further boost revenues and margin profile. Specifically focusing on our B2C segment, we achieved a substantial year-on-year growth of approximately 15% in quarter 3. This impressive performance is underlined by a patient volume growth of 13%, a testament to the strong brand recognition, we enjoy among both the medical community and consumers alike.

This takes our B2C contribution to 53% of total revenues, gradually working towards our target of 60% to 65% contribution. Having started this journey, from 35% B2C contribution a few years ago, we have made a great stride and I'm confident of reaching our 60% to 65% contribution target in few years.

In quarter 3 '24, revenues from Mumbai market experienced a notable growth of 18%, surpassing the company's average growth rate. This highlights our enhanced market share in Mumbai region. Even in the face of the entry of new players and the perceived intense competition in the Mumbai market, we have achieved substantial growth and expansion of our market presence in this region without using any tactics that would dilute the brand or the pricing metrics.

Our emphasis on the premium wellness portfolio, featuring carefully curated packages tailored to different genders, age groups and wellness profiles has yielded positive results. Through a combination of upselling initiatives and targeted strategies to enhance the wellness portfolio, we achieved a substantial 15% year-on-year revenue growth in the premium wellness

segment for

the quarter 3. This growth is slightly subdued compared to the previous quarters due to festivals and consumer focus being on other kinds of consumption. We believe quarter 4 to be showing a higher growth for wellness like the first 2 quarters of this year.

In quarter 3, our revenues from specialized testing experienced a noteworthy 13% year-on-year growth, accompanied by a robust 11% increase in test volumes. Over the years, we have consistently witnessed a steady rise in volumes for specialized testing. This can be attributed to the introduction of new tests and technologies, coupled with growing trust among doctors and specialists in Metropolis for our capabilities in specialty testing. This trust has positioned us as a preferred choice for diagnostic partners in specialty testing within the industry. We hope quarter 4 to be the quarter where specialized revenues to grow even faster as a result of all the inputs and rigor put into this area in the last 6 months.

Speaking of network expansion, we remain steadfast in our commitment to expansion with plans to add 30 labs in the current financial year, of which we've already added 17 labs so far and additional 13 labs in the next financial year. Complementing these lab additions, we are also

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expanding our network of collection centers, creating a robust system, which will feed these labs to generate higher volumes and thereby adding to the overall revenue growth going forward.

While the ongoing network expansion may have temporary impact on our EBITDA margins, we view this expansion as a significant opportunity for the future. It serves as a strategic move to reinforce our presence in core geographies and explore untapped markets with limited diagnostic players and facilities. We anticipate that the combination of enhanced productivity from the new network, increased utilization of labs and higher operating leverage, coupled with organic growth, will contribute to an improvement in margins from the current levels in the future.

Since April 2022, we have successfully added 31 labs with 21 labs strategically located in new geographies, significantly enhancing our national reach. We take absolute pride in announcing that Metropolis has crossed 4,000 collection centers marked in this quarter. We have added 1,000 collection centers since April 2022, with plans to add another 1,000 over the next 12 to 18 months.

A noteworthy achievement is our expanded presence from 307 towns in the beginning of the year to 595 towns within the last 9 months. We aim to further extend our reach to a total of 700 towns by the end of this financial year, solidifying our footprint across diverse regions. Labs opened after April '22 have contributed 3% of revenues in this quarter. Given the growth rate of these recently established labs, we anticipate a more substantial contribution from them in the upcoming years. Furthermore, the labs initiated in the current financial year are expected to further augment revenues and volumes in the future.

In parallel, our strategic focus extends beyond expanding the network. We have also been dedicated to enhancing the productivity of existing centers. The results in the past 9 months have been positive, showcasing the untapped potential for growth. We believe that this emphasis on improving the overall productivity of existing centers will significantly contribute to revenue and margin growth in the times ahead.

We've also been vocal about our technological upgradation and transformation over the last few quarters, I would like to share a few highlights on the same. The current quarter gone by, we have rolled out the second-generation point-of-sale platform, R&I 2.0, that is a registration invoicing platform developed on a highly agile Mendix platform. Salesforce CRM with fully customized -- for customi

zable CPQ to manage dynamic pricing engine and community cloud

for clients for better life cycle management of partner communities. Rolled out Salesforce service CRM at all, is touch points to capture customer interaction effectively.

First ever, Salesforce customer data platform in Indian healthcare landscape, enabling 360-degree view of its consumers and automated CLM through the Salesforce Marketing Cloud. All these initiatives have led to capabilities building for future like 360-degree customer view, personalization for consumers, efficient patient service, relationship management, efficient registration, billing and pricing, improved financial management and increased reported and analytics -- report and analytics for the future references. Half of the projects we have worked on are -- now lies in across the company and quarter 4, we'll see a few more go-live improving Metropolis Healthcare Limited

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ease of doing business with Metropolis. We'll continue on this transformation journey next year, too.

In summary, I would like to highlight that with accelerating our key growth drivers such as expanding our network, emphasizing the specialty and wellness segments and implementing technical transformation is propelling us swiftly towards our growth objectives. With this, now I hand over the call to Rakesh to take you through the financial highlights and the numbers.

Rakesh Agarwal: Yes, thank you, Suren. Let me share some of the key financial performance for the quarter. Our reported revenue for Q3 financial year '24 has grown by 2% on a Y-on-Y basis. Reported growth for Q3 was largely impacted on account of in-sourcing of large B2C contracts in quarter 4 financial year '23. Core revenue as we know is much higher. Our core business revenue grew by 12% for Q3 '24, with patient volume growth of 9% and balance 3% on account of product mix. Our core revenue for 9 months financial year '24 grew by 13% year-on-year.

Reported EBITDA margin stood at 22.5% as compared to 25.5% in Q3 financial year '23 while we all expected some margin compression due to intensity in lab expansion and negative operating leverage because of loss of PPP contract, the additional dilution has some due to onetime impact on account of provision for doubtful debt taken on account of Mohalla Clinic contract. These 3 elements impacted the margin by around 3%. Onetime impact in provision for doubtful debt on account of Mohalla Clinic contract has been taken which impacted EBITDA by 0.8%. We have provided 100% use for Mohalla Clinic from Feb to December '23.

Excluding this onetime exception, our reported margins stood at 23.3%. Q3, as we know, is always about 2% lower than Q2. So this is not unnatural considering the seasonality of the business. Reported PAT margin stood at 9.4% as compared to 12.6% in Q3 financial '23. Compared to last year, the PAT margin is marginally impacted because of the lower EBITDA margin due to the above mentioned reason and also because of capitalization of IT transformation project, which got completed in Q3 '23.

Moving on the balance sheet. Gross debt stood at INR 12.7 crores and cash and cash equivalents stood at INR 89 crores as on December 31, 2023. We are in line to close the financial year with 0 debt. O2C to EBITDA for December '23 stood at 94%. Working capital stood at 17 days as on December 31, '23 compared to 14 days in March '23. Between this period, debtor has gone up by 1 day, inventory has improved by 4 days and credit has gone down by 7 days. We are hopeful

that by March end numbers will look better than last year. That's all from my side. With this, I open the floor for question and answer. Thank you.

Moderator: Thank you very much. We will now begin the question -and-answer session . The first question is from the line of Rahul Agarwal from Incred Equities. Please go ahead.

Rahul Agarwal:

Two questions. Firstly, on the price hike. You said B2C national price hike, just wanted to understand that the entire Metropolis for B2C works on a single price point nationally? That's one. And second is, could you give a bit more color on when we talk about price hike, does it mean on the overall menu? Or is it still talking about only certain tests? That's my first question.

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Surendran Chemmenkotil: All right. Thanks, Rahul, and let me answer this. See, the price hike across the country, we were not at the same price point. We were at different price points even for the same test. So -- but the increment that we have taken across the country is largely the same across price point, maybe a little bit of plus/minus on some of the markets, which we thought we would do it differently. But otherwise, largely on the similar lines, and the second question was about? Can you just repeat the second question, Rahul? I mean, the price hike already answered.

Rahul Agarwal: Yes. What I was asking was is it like across the entire test menu or is it very specific?

Surendran Chemmenkotil: Largely across the board. I mean we would have made some exclusions based on some competitiveness in the marketplace, but it's largely -- large part of the portfolio we have done a price revision.

Rahul Agarwal: And you said 3% impact of the same, which basically means it enhances RPP by 3%. Is that understanding correct?

Surendran Chemmenkotil: Roughly, yes. Yes.

Rahul Agarwal: Okay. Perfect. Secondly, on the growth rates, this question both to you and Ameera, we've been discussing this on the call every quarter. Just wanted to get a sense again. When I look at a longer -term picture, it still looks like we are still lower than double digits for the industry, it's not only Metropolis.

My sense is growth rates pre -COVID versus post COVID has actually declined versus our understanding was we were expecting healthcare actually to go up post COVID. Any analysis on this obviously, you have mentioned that 10% to 11% is where Metropolis TAM should grow and that is what you will achieve which you have in the industry. But just some more sense on the underlying demand trend will be helpful.

Ameera Shah: So as we mentioned, Rahul, that the industry growth is about 8% to 10% overall. And our addressable market, we believe the growth is about 10% to 11% because we are focusing more on the Head of the market, as we had described a few quarters ago. Metropolis has been consistently outpacing the market growth.

And currently, as you see, our 9 -month number is at 13%. I think your question around is the growth lower than pre COVID, et cetera. I think the growth is similar. It's not very different. If you see -- I mean at least for Metropolis, I mean I can't comment on other peers.

But at least for Metropolis, if you see in '22, '23, I think our growth was approximately 15%.

And by the end of this year, we should be not -- maybe not at 15%, but closer to 15% again as well. So I think we are not going to be very different from where we were in '22, '23. And if you look at our CAGR from FY'17 to FY'19, which was the pre COVID period, Metropolis grew at approximately 16% CAGR in that period, but that was also including the government contracts that we had signed. So overall, I would say that the growth is not significantly different. It's very similar. But obviously, it depends on each company's execution, rigor and the ability to embrace the post COVID era and making whatever decisions need to be made to be able to utilize that opportunity.

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Surendran Chemmenkotil: Yes, if I can just add 1 more point, Rahul. Ameera mentioned this in her speech that the -- on the denominator, the NACO contract were sitting till January last year. So in this

quarter, you

will see that our overall growth will be largely equal to the core revenue growth and we'll also get an upside from the price increase that we have done, which has so far has gone very smoothly.

So I think these 2 elements will see a much different picture by the end of the quarter.

Rahul Agarwal: So essentially , if we talk about next 3 years, we should focus on largely the band of 11% to 13% top line. Is that fair value -wise?

Ameera Shah: Yes, I think see it's difficult for me to tell you a projection for the model. But I think, like we said by the end of this year, we should be closing around 14%, 15% for this financial year. After

that, I'll leave it to you to see where you want to put it in your model. But I think we feel reasonably comfortable with these numbers.

Rahul Agarwal: Get it. And 2 smaller clarifications for the provisions, I think the provision for the quarter for the Aam Aadmi Mohalla Clinic was about INR 4 crores, and it was about INR 3 crores odd last quarter. So overall, INR 7.5 crores. When you said 0.8% of impact, I couldn't really figure out because INR 4 crores is 1.5% of EBITDA. Is that correct?

Rakesh Agarwal: Rahul, Rakesh here. The total Aam Aadmi Mohalla Clinic provision we have taken is INR 2.2 crores and that becomes a 0.8% for the quarter. And what you are referring to last quarter, INR 3.4 crores, which we have taken was basically on the whistleblower case. That was a whistleblower complaint which came and we clarified last quarter as well that on the basis of that complaint we have investigated and then found that there was some discrepancy in taking these -- in having the accounting process, there was nothing financial irregularity, there was an accounting process because of which we have taken a onetime provision of INR 3.4 crores last quarter. This quarter, it is INR 2.2 crores on account of Aam Aadmi Mohalla Clinic and we have provided 100% of it.

Ameera Shah: The total revenue for the Aam Aadmi Mohalla Clinic project from February to December was INR 3.68 crores, of which we have received approximately INR 1.3 crores and the balance, we have just taken a very conservative stand of providing for the 100% of the due receivables because very difficult for us, obviously, to know which direction this is going in.

Rahul Agarwal: The numbers are pretty small, but the press release talked about the notes to account in the results said INR 4 crores provision. So I don't know what -- where I'm going wrong. But anyway, I'll take it off-line. And just 1 more clarification on B2C and B2B slides, others is about 12%. What does this others depict? Is this international plus what else?

Rakesh Agarwal: There are 3, 4 accounts one is international, one is institutional, HLMs and clinical trial.

Rahul Agarwal: Okay guys I'll come back in the queue. Thank you so much for answering my questions. All the best.

Moderator: Thank you. Next question is from the line of Shyam Srinivasan from Goldman Sachs. Please go ahead.

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Shyam Srinivasan: Just the first one, just delving into the 3% contribution from revenue for labs started after, I think, April 2022. So just want to just expand a little bit more if you could also think about the -- or disaggregate as the volume growth, the 9% core business patient volume. Is there a way to look at it from a same-store versus new store in the format that you mentioned the revenues? And just want to understand, like Ameera, I think, at the start said, you're growing faster than some of our peers from a volume perspective. So is the building blocks coming from same-store where you're doing better or new store addition is helping as well?

Surendran Chemmenkotil: Srinivasan, I think we will come back on the specific maths on same-store growth and the new stores. But I can only tell you that the same-store

growth are in double-digit numbers at this

point of time, okay? But the exact numbers on split between the 2, we have not put it together, we'll come back to you.

Shyam Srinivasan: Sorry, double digit for the same-store would mean we should have more than 9%, right? Am I missing something?

Surendran Chemmenkotil: Revenue growth year-on-year basis for the same store.

Shyam Srinivasan: You're saying revenue growth. Revenue growth is double digit, you say.

Surendran Chemmenkotil: Yes.

Shyam Srinivasan: Yes, yes, I was more referring to volume growth. So that's where I was coming from.

Surendran Chemmenkotil: Yes volume, that's what I said, the volume split, we'll have to come back to you. I think we will closely look at this.

Shyam Srinivasan: Understood. Understood. Just second question is on the premium wellness, right? I think it's now 14%, 15% of sales, growing well. So again, if you could -- again, I think volumes have been also pretty reasonable, right, 11% volume growth. So what's working for us here? This used to be a fairly competitive space in the past? Is it just the health tech going away? Anything on the competitive dynamics will be helpful.

Surendran Chemmenkotil: Well, I think we continue to do what we do, like we said, like our wellness revenues are largely driven by through our centers and through the digital, right? We continue to put focus on these 2 engines. And in fact, in the last quarter -- end of last quarter, we revamped all our wellness packages and created a much diverse portfolio and specific to gender, specific to illness, et cetera. So I mean, now we have seen -- and also we have revised the prices of all our wellness packages as well at the end of the last quarter. So we are clearly seeing much better traction with all the revamped portfolios from the beginning of this quarter and we are looking forward to see the wellness revenues growing at the same pace that we have done in the first and second quarter or maybe better. Does that answer your question?

Shyam Srinivasan: Yes, yes, sure. That's helpful, yes. And just my last question is on the margin trajectory. So by fiscal '25, is there any outlook in terms of the dilution coming from the investments that you're doing on either, say, if -- and if you could give us some outlook around how margins should

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kind of trend up now. Q4, I think you mentioned briefly, but I'm more looking at it from a fiscal '25 perspective.

Rakesh Agarwal: So Srinivasan, one thing for sure is that whatever price increase we have taken as we mentioned 3%, that should come down to a large extent in the bottom line because we'll have very little cost towards it. And definitely, the quarter 4 being a good quarter for us in the overall scheme of things so the revenue should also look up from a quarter 3 point of view. So overall, I think we'll see a good jump in the margin in quarter 4, and it should be more or less in line with what we have done in pre COVID era. So that's how I just -- on a nutshell we can give it. And next year, obviously, we will work out the strategy and maybe we'll talk about it more when we discuss the next quarter results.

Ameera Shah: Just to talk about a little bit more forward-looking on FY'25 margins. See, there are 3 things which are impacting the margins currently, right? One thing is obviously that like the lab expansion and that dilution will continue till FY'25 because like we said, we are adding labs this year, and we are also going to add 30 labs next year. So that dilution will continue even in FY'24, '25 post FY'25, we expect that dilution to reduce. The second thing, obviously, is obviously the onetime things which are hitting the bottom line today in the last quarter and this quarter. Obviously, hopefully, does not play out in the future. The third thing is obviously, what

never

benefit we get from operating leverage, which comes from 2 areas.

One is as our B2C contribution increases and as our specialty contribution increases and wellness contribution increases. If these 3 areas keep increasing as we are expecting it to then we should see some operating leverage getting created at the EBITDA level, which will certainly help the company.

And obviously, like Rakesh mentioned, the price increase. So I think Q4 it would be fair to expect the pricing benefit to come into the bottom line. And then obviously from -- that will continue next year. And then as we keep growing next year, we should hopefully continue to see some expansion. But I think overall, like Rakesh mentioned, the pre COVID margin, which was usually between 26% to 27%, we think would be a fair number, that would be okay till FY'25.

Shyam Srinivasan: Understood. Thank you and all the best.

Moderator: Thank you. Next question is from the line of Abdulkader Puranwala from ICICI Securities. Please go ahead.

Abdulkader Puranwala: Just 2 questions from my end. So firstly, on this Aam Aadmi Mohalla Clinic so what was the balance contractual amount or the obligation what we have for the next few quarters coming up?

Ameera Shah: Currently, like we said, we have given a notice to the government, but if the government insists on us continuing to provide services till they can find a replacement, then we would obviously continue to support them in public interest. And till that time, our plan is to keep providing for 100% of whatever revenues we book per quarter within the quarter itself. So from quarter 4 also onwards, if there is whatever revenues we do will be provided for within the quarter itself.

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Rakesh Agarwal: I think revenue will be in the range of around INR 75 lakhs to INR 1 crores in the quarter it used to be...

Abdulkader Puranwala: Okay. Okay, and I mean, what is the contractual duration? Is it 1 year or more than that?

Ameera Shah: The contractual duration is longer, but like we said that the idea is that as soon as they would be in a position to replace us with another service provider.

Abdulkader Puranwala: Okay, sure. Got it. And secondly, on this network expansion so if I look at your PPT where you mentioned the 9-month impact and on the labs that you have added in the last say 24 to 36 months. Last quarter, if you see the presentation says that it was at par with what the company level is this quarter, again, there has been a sizable dip. So just wanted to understand in terms of the new labs, what you are adding how soon you think this can reach to where your company level margins are, think of it like 3 years, 4 years? Or the gestation can be a little higher than that?

Rakesh Agarwal: So basically, what happens is that it depends upon the geography in which we open the lab. So we have seen that in the geography, which are core geography, we open lab and generally in 2.5 years, 2 to 2.5 years, we are back to the normal company margin but our intent is also to expand in the new geography where we are not very competitive. So there, it takes around 3 to 4 years to come back to the normal EBITDA margin level. So overall, I think from an average point of

view, it should be 3 years to expect that we should come, 3, 3.5 years we should come back to the normal margins. That is what we are seeing.

And from a contracts point of view, last year, we had not expanded our labs to that extent. We have only opened fewer labs. And this year, we have -- intensity is a bit higher. So therefore, you are seeing a 0.4% of dilution from last year to this year if we look at the network expansion and dilution of margins. So last year, it was 0.8%. And this year, with the 17, 18 labs coming in the first 9 months, we are seeing this dilution going to 1.2% if that answers your question.

Abd

ulkader Puranwala: Okay. Thank you and wish you all the best.

Moderator: Thank you. Next question is from the line of Prashant Nair from Ambit Capital. Please go ahead.

Prashant Nair: Just 1 question on your lab addition target. So once you're done with the current target up to fiscal '25, how should we think about this on an ongoing basis? I mean, do -- would you be adding labs in set of steps? Or would there be a consistent increase due in fiscal '25 that we have to look at?

Surendran Chemmenkotil: All right. So we have already mentioned that we are on a course of adding 30 labs this year, of which 17 has already been done and the remaining is work in progress in this quarter. And we also projected a similar number for the next year based on the opportunity markets and the new markets that we want to get in. I think that planning is already -- also happening. And post that, I think the lab additions will slow down and may not be at the levels of 30 labs, 20 labs in a year. But on need-based and as and when we get an opportunity to put a lab in an existing market to improve the penetration or a very good new market, we will, I mean, keep exploring those but

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the numbers will be very less, will not be to the tune of 20 or 30 labs per annum, but maybe purely on need based and much smaller in number.

Ameera Shah: So just to add to that, in FY-- if you look at FY'17, '18, '19, '20, we added approximately 4 to 5 labs a year, just in comparison to the 30 labs that we're adding now. So the idea post FY'25, as Suren said, will be to slow down the lab expansion closer to where we were earlier, but then to really ramp up the collection center expansion to feed the labs which obviously is not capital intensive, will be done through a combination of franchise and our own center route, mostly through franchise. And therefore, post FY'25, we expect the collection center ramp up to be significantly high.

Prashant Nair: Great. And just 1 more question. This is on your tax rate, I mean where should we assume it settling down this year and then on an ongoing basis?

Ameera Shah: Sorry, we couldn't hear you. Could you repeat that?

Prashant Nair: Yes, just a question on your effective tax rate, where should it settle down this year and in future?

Rakesh Agarwal: I'm not very clear, but you are asking for a tax rate. I think tax rate, we are not seeing any difference happening as of now. This budget also the tax rate remain same. And going forward, we are not having any prediction of when it is changing really drastically either positive side or negative side. So as of now, the next 3 years have been built up more or less in the same range what we are doing right now.

Prashant Nair: Okay. thanks a lot.

Moderator: The next question is from the line of Rahul Agarwal from IncRED Equities.

Rahul Agarwal: One question I had on the North and Eastern markets. South and West, obviously, are the bulk of our business. Any thoughts in terms of the network expansion happening right now over the last 9 months and ahead, how are we looking at growth rates here? It looks like these markets are not really picking up for us. Is that right understanding? And what's the way ahead here?

Surendran Chemmenkotil: No in fact, we have done -- we are putting a lot of efforts in the North and East markets. I mean, I think we also talked about last time picking up UP, MP, Assam and AP/TS or new markets for expansion. And we are on our course. And the labs that we are putting up in North and East are doing reasonably well as per the plans that we have for these labs in the first year, second year, etcetera. So our efforts and our focus on North and East continues, and we are in line with our plans and projections, Rahul.

Rahul Agarwal: Sir, in terms of the current network, how are we -- what's the mix like be

tween, let's say, South

and West and North and East in terms of labs or collection centers either way? How is that set up? And should that change in favor of North and East going forward?

Surendran Chemmenkotil: I can tell you of the new network coming in, new centers and the new labs coming in, more than let's say, almost half of them -- a little more than half of them are going towards North and East markets. And on the centers, maybe 60% of the centers are coming up in North and East new --

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because we have seen that we have increased the number of towns where we operate from 307 to 595 and a good chunk of this has come from the north and east markets, right? So there is definitely a slightly disproportionate focus on increasing the footprint in North and East. Of the total labs, how many of them are in North and east or south and west, I think I have to get to do that maths and come back to you.

Rahul A garwal: Sure. I understand. Secondly, any updates on Hitech or the international business? Any point you would like to talk about and highlight?

Surendran Chemmenkotil: We talked about Hitech last quarter as well. Hitech is largely on course and in line with the company levels of revenue. On a YTD basis, Hitech revenue growth is at about 13%, 14% and the margins are higher than the company margins and on course. So I think it's -- I mean, I told you last time also, we have completed the integrations and in the last quarter -- or 100% integration in terms of IT, people, resourcing labs, everything has been done.

Now Hitech on its growth journey now as we speak. And international business, again, is doing well. And we are doing -- we are getting -- seeing a much higher levels of growth than what you -- and on a company level. And hence, we are also now putting our focus on expanding some of the centers and getting a little bit more on the B2B business on the international market. So both continue to be our focus areas and doing fine as we speak.

Ameera Shah: Q3 obviously had some impact in Hitech in -- yes, because of Chennai floods as it did for Metropolis as well. So that growth would have been a little bit higher in Q3 if obviously not for that division, otherwise, things are on track.

Surendran Chemmenkotil: Yes.

Rahul Agarwal: Got it. And the plan of this lab and center addition, does that include the international expansion? Or is it only domestic?

Surendran Chemmenkotil: Yes. This year, we have been really focused on expanding the labs and centers in the international market. Now looking at the response that we are getting from the international market, we will have a few at least center expansion, definitely on the plan for the next year.

Labs, we are exploring multiple options whether we do the deduct model or we do through a franchisee model. So we are exploring multiple options and so that's what the status for the plans for the next year in international.

Rahul Agarwal: Got it. And just 1 last question from my side. In terms of the revenue mix between what you get from metros and cities which are smaller and towns which are smaller, obviously, we all know Mumbai -Chennai is very large contribution to Metropolis top lines. Incrementally, Tier 2, Tier 3, Tier 4, are these markets behaving in a manner where you see whatever growth we're talking about, the base is smaller there.

Obviously, the percentage numbers would look higher. But incrementally, the reception to bigger brands, doctor influence there in smaller towns, are they playing in our favour? The realization might drop because we're expanding there, but just your sense on how will Tier 2, Tier 3, Tier 4 play out for the entire industry and Metropolis will be really helpful.

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Surendran Chemmenkotil: I'll tell you what we have shared

with you all in the investor deck, I think for the quarter 3, of the 12% split, I can tell you, 8% has come from the focused cities, which you know that in the Tier 1 cities for us. 7% growth has happened in the seeding cities, which is largely the Tier 2 type of cities. And 28% has come from the new markets that we have got into, right? So I think that's largely on the Tier 3 and -- a little bit Tier 2, Tier 3 and even smaller cities. So the response from the Tier 3 and smaller cities are really good for us. That encourages us to take the journey forward.

Ameera Shah: Also just as an entry point, usually when you enter a Tier 2 or a Tier 3 market, you enter usually with a franchise center, you build up a small base, a collection center. And then, of course, when you open your own greenfield lab there, initially, while you may end up picking up more of semi-specialized testing and B2B through the B2B channel the final goal, obviously, is, like you said, to get to the top specialists of the city, to the top hospitals of the city and be able to get more specialized work.

So there should not be a dilution of RPP if this plan plays out the way we want it to. And we are definitely seeing that happening. So in core markets of South and West, this obviously happens faster in challenger markets of North and East, this takes a little bit more time, but the goal and the destination is the same.

Rahul Agarwal: Got it. Perfect. Thank you so much for answering my questions. Best wishes.

Moderator: Thank you. Next question is from the line of Bhavesh Gandhi from YES Securities. Please go ahead.

Bhavesh Gandhi: Just 2 questions. First on wellness. So if I look at 9-month growth, it's around 22% for premium wellness and the quarter growth is around 15%. So any color on is the growth kind of slower for

this quarter? And then how should we look at Q4 and the next year from a premium wellness perspective?

Surendran Chemmenkotil: Yes. Bhavesh I will try to answer this question. So like I mentioned, quarter 3 normally is not a wellness quarter because it's all festivities, et cetera. People have not focused on the wellness.

But having said that, I think a lot of our team was very busy on transitioning into the new registration and the billing system.

Like I mentioned, because at the end of the day the wellness has been largely driven by our touch points, and we were doing this transition into the new registration system. So it has consumed a lot of energy and they were just trying to stabilize this without any difficulties or any disruption in the consumer services. So has a little bit of less of time being spent on that.

And also, I mentioned that in December, we have gone through a full-fledged ramp-up or revamp of the wellness packages. So when we do a full change of packages, it goes through a lot of training, a lot of understanding of the new packages, why we have done that and also answering to the consumers about the changes, etcetera. So it took a little bit of time for us to establish the new revamped wellness packages.

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And largely because of these 2, 3 reasons, you are seeing a little bit of slowdown in the quarter 3, but I think we are back in quarter 4. I mean I think YTD, we are doing 22%, 23% if I'm right, and I think first 2 quarters were better. So in this quarter, we are seeing the similar levels of growth that -- what you have seen in quarter 1 in quarter 2. And with the price corrections, we are doing a little more than that. So I think wellness is back and better.

Bhavesh Gandhi: Okay. Thanks for that. My second question is on in your opening commentary, you have mentioned about upcoming price hikes in specialized and super specialized tests in the Q1 FY25 quarter. So what is the potential quantum here? And what makes you confident the price hikes will be digested

in this segment of the test menu?

Surendran Chemmenkotil: So, I mean, B2C, we have already done, including the specialized and semi specialized everything, B2C, we have already done from 1st of January, right? What I mentioned in the comment is like B2B specialized and say specialized will be done in the next quarter. So that could be maybe another 1% is the company revenue.

If we do all that, we want to do and we really don't find any big challenge in implementing it because it has been done through a lot of planning, a lot of discussion with the B2B clients and considering all the sentiments and all the feedback from the partners as well. So I think we should be able to sail through seamlessly once we implement this. So we should be able to do this latest by the early next quarter.

Bhavesh Gandhi: Okay. Thank you. That's it from my side.

Surendran Chemmenkotil: Thanks.

Moderator: Thank you. Next question is from the line of Aashita Jain from Nuvama. Please go ahead.

Aashita Jain: Am I audible?

Surendran Chemmenkotil: Yes.

Aashita Jain: Yes. Just 1 quick question, like 1 clarification I wanted on this PDD that you've created. So what was the amount for this quarter? And what is the amount for the 9 months? If you can clarify.

Surendran Chemmenkotil: You're talking about the onetime PDD that we have created or the total PDD?

Aashita Jain: So what is the total PDD that we have created for this quarter?

Rakesh Agarwal: So yes, overall PDD, which we have created is approximately INR 4 crores. And out of that, INR 2.2 crores is on onetime impact of Aam Aadmi and INR 1.8 crores is generally the ECL provisioning as per the provisioning policy we take every quarter. So that's something which is a normal in the normal course of business. So it's a normal course of business, something we have taken. And INR 2.2 crores is taken on onetime Aam Aadmi Mohalla Clinic provision.

Aashita Jain: Understood. That's helps. Thank you. That's all.

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Moderator: Thank you. Ladies and gentlemen, that was the last question of today. I now hand the conference over to Ms. Ameera Shah for closing comments.

Ameera Shah: Thank you all for joining us today on our Q3 quarterly call. As mentioned, I hope we've been able to clarify all the questions and any doubts that were there. We feel extremely positive about where the industry is headed and especially about where Metropolis' progress is tracking. We continue to obviously focus on our execution rigor, our technology transformation, our geographical expansion, good leading management team and really making sure that we bring that additional ring to sort of the execution that we are doing. As we are moving into smaller markets, as we are moving into -- and deeper into our existing cities, the focus remains the same,

which is to increase our B2C contribution to only make sure that we are looking at getting business from the top doctors across the country who really value quality and our kind of reporting and personalized service that we give them.

But at the same time, for all customers to be able to really continue to strengthen the customer journey whether they are a partner, franchise partner, whether they are a B2B customer or a B2C customer, and all the different inputs that go into making life easy for customers to work with Metropolis.

And we believe that with that goal in mind and with this very strong expertise and science that we have at the back end, that is a winning combination in the industry that will allow us to continue to outperform as we have been doing. Look forward to chatting with all of you at the end of Q4, and have a wonderful year. Yes, thank you.

Moderator: Thank you. On behalf of YES Securities, that concludes this conference. Thank you for joining us, and you may now disconnect your lines.

Call: May 2024

BLOOD TESTS • DIAGNOSTICS • WELLNESS

Metropolis Healthcare Limited

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Ref: MHL/Sec&Legal/2024 -25/18 Date: May 28, 2024

To,

BSE Limited

Scrip Code: 542650

National Stock Exchange of India Ltd

Scrip Symbol: METROPOLIS

Dear Sir/ Madam,

Sub: Earnings call transcript for Q4 FY24 .

Pursuant to Regulation 30 of the Securities and Exchange Board of India (Listing Obligations and Disclosure Requirements) Regulations, 2015 , please find enclosed herewith the transcript of Q 4 FY24 earnings conference call held on May 22, 2024 . The transcript is also available on the Company's website i.e., <http://www.metropolisindia.com/investors>

We request you to kindly take the above information on record.

Thanking you,

Yours faithfully,

For Metropolis Healthcare Limited

Kamlesh C Kulkarni

Head – Legal & Secretarial

Encl: as above

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“Metropolis Healthcare Limited Q4 FY '24 Earnings
Conference Call”

May 22, 2024

Disclaimer: E&OE - This transcript is edited for factual errors. In case of discrepancy, the audio recordings uploaded on the stock exchange on 22nd May 2024 will prevail.

MANAGEMENT : MS. AMEERA SHAH – MANAGING DIRECTOR ,
METROPOLIS HEALTHCARE LIMITED
MR. SURENDRAN CHEMMENKOTIL – CHIEF
EXECUTIVE OFFICER – METROPOLIS HEALTHCARE
LIMITED
MR. RAKESH AGARWAL – CHIEF FINANCIAL OFFICER ,
METROPOLIS HEALTHCARE LIMITED
MODERATOR : MR. AMEY CHALKE – JM FINANCIAL LIMITED

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Moderator : Ladies and gentlemen , good day, and welcome to the Q 4 and FY24 Earnings Conference Call of Metropolis Healthcare Limited hosted by JM Financial .

This conference call may contain forward -looking statements about the Company , which are based on the beliefs, opinion s and expectation s of the Company as on date of this call. These statements do not guarantee the future performance of the Company , and it may involve risks and uncertainties that are difficult to predict.

As a reminder, all participant lines will be in the listen -only mode and there will be an opportunity for you to ask questions after the presentation concludes. Should you need assistance during the conference call, please signal an operator by pressing star, then zero on your touchtone phone. Please note that this conference is being recorded.

I now hand the conference over to Mr. Amey Chalke from JM Financial . Thank you, and over to you, sir.

Amey Chalke : Thank you , and good morning everyone on behalf of JM Financial. I am Amey Chalke . Welcome you all for Metropolis 4Q FY '24 Earnings Call .

With us today, we have the Metropolis Senior Management Team represented by Ameera Shah, Managing Director, Mr. Surendran Chemmenkotil – CEO, and Mr. Rakesh Agarwal, CFO.

I will now hand over the call to Ameera ma'am for her opening remarks. Over to you , ma'am .

Ameera Shah: Good morning everyone and thank you for joining us on the Q4 FY '24 Earnings Call . Today I am joined by our CEO, Surendran ; Rakesh Agarwal, CFO ; and SG A, our IR advisor. We have uploaded our updated results document on the stock exchan ge and website and I hope everyone had an opportunity to go through the same.

Talking about the industry trends, the diagnostic sector is expected to grow at about 10% CAGR over the next five years. The organized segment of the industry is estimated to grow at a faster pace marked by a transition from unorganized entities to larger organized players.

Between 2020 and 2022, the industry also witnessed an influx of new entrants attracted by the perceived high margins and return profiles. However, most of them have realized that while entering the market is easy, scaling up profitable revenue poses a significant challenge for new entrants. And while aggressive pricing can play a role in creating the wellness market, it has not proven to be a silver bullet in the illness thing.

HealthTech players have the hope of acquiring customers via wellness and then converting them to illness customers. However, the conversion rates of these have been low single digits and therefore , these players have also now increa sed prices of wellness packages.

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Even globally, companies like Theraanos , which were focused on wellness, and 23andMe, focused on wellness DNA testing, have been struggling as very few businesses in the pure wellness space build credibility amongst doctors or make profits.

On the contrary, Metropolis and similar established players have benefited from an increase in post-COVID wellness trends. And having strong brand recognition and developing an omnichannel plan have been able to increase the share in the wellness segment.

We have also seen hospital chains enter diagnostic segments in regional pockets where they believe their consumer brand is relatively stronger. While the perception is that they already have lab testing infrastructure a nd doctor connect and therefore , it's an easy pivot into diagnostics. The reality is that in healthcare, each area has expertise length and doctor's perception of expertise doesn't translate easily from hospitals to pathology experts.

Also, hospitals face some inherent structural challenges. Like the difficulty in attracting prescriptions from specialized doctors ou

tside the hospital due to fear of patient loss to the doctors inside the hospital. Also, another challenge they face is on account of differential pricing inside and outside the hospital.

With these continuing challenges, hospital entrants have largely focused on B2B business for routine and semi-specialized tests by using aggressive types of strategies which tends to be focused on the tail end of B2B customers who care more about pricing over other areas. While competition persists in this space, there has been a recent trend towards reduced competition intensity and more rational pricing strategies.

Interestingly, the emergence of organized players in the industry has prompted a noticeable shift in customer attitudes towards organized diagnostic players. As a result, standalone labs operating in the unorganized sector have experienced a decline in market share. This shift highlights the growing preference among customers for the reliability, efficiency and use of technology offered by the organized entity.

Building a specialized diagnostic business is all about your expertise behind the scenes. It's not only about offering the largest test menu but having the institutional knowledge for testing in each therapeutic area that produces a better quality report compared to others.

At Metropolis, we are focused on the fastest growing therapeutic areas such as transplants, neurology, nephrology, gastroenterology, oncology, et cetera. Additionally, to be future ready, we are investing deeply to build world-class testing in areas like genomics and molecular diagnostics which is the future of the industry.

Looking at these trends and opportunities in the industry, the opportunity for Metropolis is to continue to outpace the industry and peers which will happen via organic and inorganic growth. For accelerated organic growth, we will do the following:

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1. We will focus on being the pioneer in new test advancement and amplify our engagement with specialty doctors to grow our specialty volumes.
2. We will also expand our collection center networks in new cities which will be largely tier two and tier three markets of India.
3. We will heighten our service standard and digital marketing initiatives to improve the productivity of existing centers.
4. We will expand our B2B presence, not only in India, but also to new markets outside India, using our global reference lab in Bombay as a specialty hub for testing. This would be an asset-light model, which would be moving samples from outside India to Bombay for testing.

When we look at inorganic growth, we will assess M&A opportunities through three lenses:

1. We will acquire firms where we can get cutting-edge capabilities and skills within the technical testing sphere and sell through our large distribution network.
2. We will acquire ethical and strong local B2C players in markets where we don't have a strong consumer plan in order to establish a foothold to grow from. Many of these local firms lack prudence in capital allocation and scaling operations to the next level. We intend to evaluate such targets nationwide and do multiple bolt-on acquisitions that would create accelerated growth. This would largely be in the regions of northern east, but also could be in specific cities in south and west, as our industry is hyper-local and not regional in nature.
3. We could look at acquiring good quality brick-and-mortar firms, which are subscal es, and making inadequate losses, and use our management skills and advanced testing capabilities and strong brand recognition to scale these businesses and turn them around.

For mergers and acquisitions, we adhere to predefined internal parameters encompassing IRRs, growth prospects, EBITDA, PBT margins, along with return on ca

pital and return on equity to assess possible acquisitions.

Thankfully, there has been a modulation in valuation expectations compared to the COVID and we will be open to using options of internal cash accruals, external debt, and our own stock to complete deals. We would be financially conservative in allocating capital or raising debt.

Currently, we see a fair number of opportunities to buy assets, which meet our strategic and financial checklist, and to capitalize on this opportunity of consolidation, the Company plans to alter its dividend payout from historic 30% to 35% of PAT to 15% to 20% of PAT for the next couple of years.

Cash will be retained by the Company for accelerated growth opportunities in terms of selected M&A, which we believe will fuel growth in the future. Once these growth initiatives are successfully executed, we will restore the dividend payout to historical levels, if not higher.

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Should there be a case of non-deployment of these funds for M&A, we will prudently return the cash with a suitable option.

Over the past two decades, we have successfully completed 23 acquisitions along with strategic thought processes that have generated high IRRs for the Company. While most were done at very reasonable valuations, it will be integration and the ability to drive a high organic growth from them that truly stands out.

Since the integration of our most recent acquisition Hitech, core revenues have shown significant growth. And with synergies and operational efficiencies, we have been able to enhance EBITDA margins also by 4%. Going forward, we are confident in our ability to do deals at fair valuation and integrate them well and execute plans to generate positive IRR for Metropolis.

Over the past many years, my focus has been on professionalizing the Company and to accomplish this. I have been augmenting our talent pool from within the healthcare domain and from outside. In pursuit of this objective, I brought Surendra on board approximately 18 months ago to spearhead our business execution efforts.

With his extensive background in running scaled consumer-facing and distribution operations, Surendra has assumed full responsibility for driving the implementation of Metropolis strategy. Over the past 15 months under his guidance, Metropolis has grown faster than industry and peers and significantly broadened its struggle to present, setting the base for future growth.

With Surendra and the CXO team now formally established and demonstrating a strong track record of execution results, we believe it is the opportune moment to delineate and separate governance from operations and run the firm as a promoter-led but professionally managed Company. Promoters in India who have successfully done this before have generated large value for shareholders and I believe this is the right direction to go for Metropolis as well.

Accordingly, I will be transitioning into the role of Executive Chairperson and Whole-Time Director. In this capacity, my responsibilities have encompassed driving the strategy of the business and monitoring it, strengthening governance, strategizing capital allocation including driving the M&A activities, acquiring talent and fostering the culture at Metropolis.

Meanwhile, Surendra – our CEO will lead the execution of all our operational initiatives by reporting to me. Surendra and I have worked together in this construct for the past 15 months and we complement each other well. Together with our strong board, we will prioritize governance, strategy and sustainability initiatives aiming for even greater change.

Let me also take the opportunity to thank our Founder and Chairman, Dr. Sushil Shah, who will now transition into the role of Chairman of Metropolis. Under Dr. Shah's

leadership, Metropolis

has emerged as a trusted pathology brand, earning recognition from both medical professionals and consumers solidifying its position as an industry leader. Dr. Shah's visionary leadership, commitment and dedication have been instrumental in shaping the Company's success. In his

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new capacity as Chairman Emeritus and Director on the board, Dr. Shah will continue to provide invaluable guidance to the board and team, drawing upon his extensive medical experience and expertise to offer mentorship. We express our heartfelt gratitude to Dr. Shah for his years of service and support in propelling Metropolis to greater heights.

With this, I will hand over the call to Surendra to take you through the quarter and the year gone by and give some flavor of our strategies for FY '25 and beyond. Thank you, and over to you, Surendra.

Surendra Chemmenkoti: Thanks, Ameera, and good morning, everyone.

Let me take you through the Business Highlights for this quarter gone by and along with the strategies going forward:

For Quarter 4 '24, we are happy to report a 11% year-on-year growth on reported revenues and a 15% year-on-year growth in our core revenues with corresponding volume growth of almost 7%. And on 7% on account of product mix change and realization benefits.

We have delivered industry-leading volume growth over the last nine quarters and are optimistic about the growth trajectory going forward. Our B2C revenue has grown faster at 20% year-on-year basis, and Quarter 4 with the volume growth of 7% and a 13% increase in RPP, attributable to the recent price increase implemented in January and shifts in product mix. We are pleased to report that the price hike has been effectively absorbed, and despite this increase, our volumes are maintained at a growth rate of 7% plus.

Alongside our revenues from Mumbai market have grown by 20%, underscoring our increased market share and brand pull in our core geographies. Our specialized and wellness testing grew by 17% and 22% respectively on year-on-year basis in line with our strategy of expanding our specialized and wellness business.

Despite some competitive intensity on B2B side, we have been able to grow our B2B revenues by 11% for Quarter 4 with corresponding volume growth of 7%. So discounting in the B2B segment has reduced as compared to full year '23, and B2B volumes have grown positive recovery. More and more players are preferring trusted labs with experience on the track of better service and quality of diagnosis.

Let me shed some light on the network expansion and outcome of the same :

Over the past two years, we have dedicated efforts to extend our presence across India. In the last 12 months, we have added 24 new labs with 7 labs added in the last quarter.

Our plan for the current financial year entails the addition of another 25 more labs in strategic locations to target underserved markets, thereby aiming for accelerated revenue growth and market share expansion.

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In tandem with lab expansion, we are focused on expanding our collection center network and distribution channels. Full year '24, we have added more than 550 centers, including 150 -odd centers in the last quarter alone. Our footprint has expanded from 307 towns in April '23 to 600 towns by the time we exited last year.

With all these efforts, we have been able to grow our revenues from other cities by 34% for the Quarter 4 and 28% in full year '24, respectively. Growth from the other cities largely on the back of growth and revenues from tier 2 and tier 3 cities. Our new labs are also showing robust growth over the last two, three quarters with revenue contribution of 4% for the full year '24.

In addition to p

rioritizing network expansion, volume and revenue growth, we have placed equal emphasis on efficiency, productivity and margin improvement. We are pleased to announce that our reported EBITDA margin stood at 25.5% for the Quarter 4 '24. Prior to factoring in CSR and ESO P, this is 26.5% and if we add back the dilution cost in the short term by the new lab, our EBITDA stood at 27.4% for the same period.

We maintain an optimistic outlook regarding the sustenance of Quarter 4 margins in full year '25. Upon the completion of our accelerated lab and network expansion by the end of '25, we are anticipating that the enhanced revenue stemming from both existing and new labs will further enhance our margin profile post full year '25.

Speaking of our organic strategy, we have been focusing on basically four or five big things. Infra expansion into new markets to target accelerated growth with lab, network and expansion of distribution channels.

We have been focusing on technology transformation, both on the front end, consumer facing for convenience to customer experience we can improvise and also to strengthen our back end digital infrastructure to improve our service level .

With increased focus on technology and digital marketing , we have been able to acquire new customers via the digital route and revenue contribution led by digital channels has shown robust growth in full year '24.

We have also revamped our TruHealth wellness packages to suit right for consumer needs with focus on bundle testing packages, curated for specific target audience and also upselling to existing customers.

We have also taken multiple initiatives to enhance our service levels for B2B customers. We have set up dedicated relationship managers and special programs for our B2B clients along with centralized helpline desk, B2B partner portal for strong support system, etc. As a result, we have seen an increase in B2B volumes with rationalization in discounts over the last two to three quarters.

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Lastly, as highlighted by Ameera earlier, we have strengthened our leadership bench, simplified the organization structure and have fortified our organization's capacity for efficient planning, execution and leadership continuity.

The wealth of experience and diverse backgrounds that these senior professionals bring from various industries enriches our collective knowledge, knowledge base and enhance our ability to navigate complex challenges and seize the opportunities. Their contribution extends beyond formulation of strategies and play a pivotal role in executing the roadmap ahead of us.

Going forward with the brand strength of Metropolis, talent pool, trusted partners for doctors and consumers coupled with aggressive expansion plans , foray into adjacencies with opportunities of inorganic growth , we are optimistic of outnumbering in the industry growth in terms of revenue growth and profitability.

With this, I will hand it over to Rakesh for the Financial Update.

Rakesh Agarwal: Thank you, Suren, and good morning everybody.

Let me share some of the key Financial Performance for the Quarter :

Reported revenue for Q4 FY24 stood at 313 crore, a growth of 11% Y-on-Y. Our core revenue excluding revenue for COVID and COVID allied, and PPP contract grew by 15% Y-on-Y for Q4 FY24. Our core revenue for FY24 grew by 13.3% Y-on-Y with 9% volume growth and approximately 4% on account of price increase and product mix change.

Reported EBITDA for the quarter stood at Rs. 79.7 crore as compared to 69.2 crore, a growth of 15%. Reported EBITDA margin for Q4 FY24 stood at 25.5%. EBITDA margin adjusted for ESOP, CSR and New Lab margin dilution stood at 27.4% for Q4 FY24. EBITDA for the full year stood at Rs. 284.6 crore with EBITDA margin of 23.9%. One-off expenses of Rs. 6.8 crore has been booked

on account of whistle blower and Aam Aadmi Mohalla Clinic provision for Financial Year '24 .

PAT for the quarter stood at Rs. 36.1 crore with margin at 11.6%. PAT for full year stood at Rs. 128.1 crore with 10.8% margin.

Moving on to balance sheet, we are happy to share that we have paid all our debt in the last quarter and have zero debt as on 31st March 2024. We have a net cash surplus of Rs. 117 crore as on 31st March 2024.

Our working capital days have reduced from 14 days on March '23 to 7 days on March '24. Our OFC to EBITDA has improved by 3% and stood at 105% indicating a higher cash conversion cycle.

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The Company has aligned the accounting year of its 4 overseas subsidiaries with the Indian accounting year with effect from 1st April 2024. Because of this change, the accounting year for the purpose of consolidation will henceforth be 31st March . For the transition year that is financial year '23-'24, the accounting impact of this change is an increase in revenue of Rs. 18.27 crore and PAT by Rs. 0.4 crore.

That's all from my side. With this, I open the floor for Q&A. Thank you.

Moderator: Thank you very much. We will now begin the question -and-answer session. The first question is from the line of Anshul Agarwal from Emkay. Please go ahead.

Anshul Agarwal: My first question is on the quantification of any sums that we would be keeping aside for this M&A activity in the next two years?

Ameera Shah: So, currently we have , I think, approximately 120 crore cash on our books, if I am not mistaken, Rakesh. And we will obviously be using this in internal accruals plus raising debt or using stock of equity.

It is difficult to obviously quantify an amount as of today because there are a bunch of things being evaluated, but my sense is that if we ever do look at raising debt, we would not look at it beyond 2 to maximum 2.5 x our EBITDA and if we were to use obviously stock of currency, then that would potentially play out well as well if the stock price is valuable for us.

So, I don't have an exact number for you at this point because there are many things in the pipeline, but we will obviously look at each deal from an IRR and an ROCE, ROE perspective.

Anshul Agarwal: And while you have outlined the strategy behind selecting these M&A targets, would there be any specific region that we would be planning to target?

Ameera Shah: So, as I mentioned, we have three strategies for the M&A and one of them is our geographical expansion strategy, right, which would be primarily in, we would look at obviously North and East, which are the markets where we would like to expand towards sort of a B2C route. That's one of the opportunities that we are sort of looking at, one of the lens of M&A.

But these, like I said, could also be in South and West of India because there are still markets in South and West, certain cities in which we may not have a strong B2C brand and entering those markets may also be useful. So, I would say primarily North and East, but also South and West for the geographical expansion strategy.

The other two strategies, which are about technical capability, which obviously could be anywhere in the location of the country. And the third one, which could be a regional player or any other kind of player, could be in any part of the country because we are looking at taking businesses, which may not be necessarily providing the kind of profit on their own, either

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because they are sub-scale or need better management and picking those up and actually sort of being able to turn them around under our management skills and scaling possibilities.

Anshul Agarwal: My second question is on any indicative broad margin guidance for FY '26 post our netw

ork

expansion plans?

Ameera Shah: See, I think we are comfortable with our Q4 margin . We believe that we will be able to sustain that into the next year and hopefully try to expand it as the volumes grow and we are able to execute some synergy. So, I think 25.5 % to 26% margin, give or take, I think , is sustainable.

Anshul Agarwal: This would be reported as margin, right?

Ameera Shah: That's right.

Moderator: Thank you. The next question is from the line of Amey Chalke from JM Financial Limited . Please go ahead.

Amey Chalke: Ma'am, there was a period when nationalized diagnostic chains were being impacted with the proliferation of e-commerce players who were cutting in the prices. Now , we are seeing the trend reverse where most of the nationalized chains or larger chains who are having good physical presence are outgrowing the market. Is this only linked to pricing in your assessment or anything else which is also driving this trend?

Ameera Shah: See, I think when health tech firms came in, it was during COVID or some of them which had started just before COVID, and it was a black phone event that really gave rise to digital usage, which obviously helped with many industries before . We saw it in EdTech . We saw it in food tech. We saw it in many industries . And we saw it in health tech as well. Mostly because a lot of the unorganized centers were closed because they couldn't operate due to lack of employees, et cetera and it gave rise to more of this home testing, home collection, especially for COVID. Now in some industries, obviously, that trend is consistently sustained, but in our industry, once COVID sort of came down, people actually prefer to go into brick -and-mortar labs and actually give their samples. While the home testing, home collection has grown compared to pre -COVID, it is still a minority share compared to more people walking into brick -and-mortar .

So, pricing was never really the key. Even that time, the health tech firm used convenience as a way to really drive their business. What they used pricing for was for wellness , expanding of the wellness market, because in the wellness market, there is no need and you are actually doing a push product to create a need of preventive healthcare.

And when they used pricing to create that kind of awareness, as I mentioned, it helped the incumbents a lot, because it actually created a new market that didn't exist before, which was a preventive care, which was earlier a very small market. So, it has actually been beneficial to the Metropolis Healthcare Limited

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incumbents as well, but that preventive care market while it's growing fast, is not growing at the pace it was growing in COVID anymore.

So, I think for the health tech firms, really now the question is about where should they really play, what is going to be really their unique curve, what they bring to the table. And I think many of them are figuring out what that means for them.

From our perspective, pricing has never been the most important thing in illness in healthcare. Because at the end of the day, you fall sick once or twice a year. And the more sick you are, the more critical your illness, you are really not looking at who is giving you something Rs. 50 or Rs. 100 cheaper . You are looking at who's best in labs to get the best diagnosis, usually recommended by your doctor, so you can get treated quickly.

Amey Chalke: And the second question I have is on the expansion side, we have been saying that we are adding around 20, 30 labs over one year. Why not 40 or 50 or why not 10? What is it that decides this number? Generally, for businesses, there is a capital limitation to expand or management bandwidth, or maybe a lack of opportunity, or some of them are not ready to take the financial hit. What is it in our case that decides this

factor organically? How much labs we will be adding going ahead? How do we decide?

Ameera Shah: I will give you my input and Suren can come in as well. Look, in my opinion, the only reason there is no science behind this number 30 is that earlier we were, till about FY '19-'20, we were only adding about four, five labs a year. You have to remember the lab is only a factory which does the testing. The demand generation really happens when you go to the customers, you build collection centers, and you are able to build demand in those collection centers.

The hard part is not in building a lab. Could we set up 100 labs in a year? The answer is yes. The question is, can you generate demand in all those locations? Because for an average lab, you need at least 20, 30 centers and B2B customers to be able to fill that lab. So really, it's about getting the right quality of talent on the front end, training them, and to be able to go and scientifically sell to doctors and generate that demand and set up those collection centers, unlike other industries which have ready distribution, whether it's pharma or modern trade. In our industry, we have to set up each distribution point. So, it's not like a product that we just push through an existing distribution. So really, that's where the time goes in getting the right talent , training, and setting up the distribution and generating demand. Suren, anything you want to add to that?

Surendran Chemmenkotil: No, I think you have covered it all, Am eera. I think , if at all I can add one more thing , setting up the lab doesn't start from the day you start working on the infrastructure of the lab, but it actually starts almost a year before. We have to enter into a city . We need to familiarize with the Metropolis reports, and we need to get the channels appointed. And then we do start getting some level of customers and revenue from that market . And once the revenue reaches a particular level, that's the time that we start looking at putting up a lab there.

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So, I think the whole process takes a little time consuming , and it's based on analytics . It's based on a lot of hard efforts. And hence, it may be prudent for us to do it more judiciously and at the right places, and at the right pace.

So, like Am eera said, the previous two years, you know, put together, we have set up 30 labs.

And last year alone, we set up 24 , 25 labs. So , we definitely speeded up the process and this year again, we are going to put up another 25 labs.

So, the required speed has almost been brought in and once we get this as our norms and even if there is an opportunity to get into any other market in the coming years, we will look at it. But otherwise, we are pretty happy with the current pace and also we have the ability to turn around the new labs into profitable ones in about two year's time .

Moderator: Thank you. The next question is from the line of Kunal Randeria from Axis Capital . Please go ahead.

Kunal Randeria: So Am eera, just one of the points you made that a sick patient definitely looks at quality and not really the price. So you know, while it completely I am with you on that, but on the wellness front, do you think pricing is a lot more important factor than it is in sickness and perhaps some of the online players would have an advantage over a traditional players ?

Ameera Shah: Sure. See, on the wellness side, you can remember there is no one kind of customer . There are different kinds of customers, right? If you look at a customer segmentation, you have a more health conscious customer or a more brand conscious customer who is not educated and aware. Usually they will prefer if they are doing a health check up once three years, you know, and they are going to spend Rs. 2000, they are not thinking, oh, let me go and spend o

nly Rs. 1200 with

somebody new who have not experienced and take a chance on my health check. Because you can understand why are you doing a health check is to find something early that you are able to then treat so that it doesn't become a much bigger issue.

So, there is a large amount of trust that is required, whether it is an illness or whether it is in wellness, but in wellness, because the doctor is not involved, consumers can be a very variety.

So the health conscious educated customer will still choose a brand which they are trusting, or slightly less aware customer may say that, look, let me go on price because everything same test, same result . That might be a perception. And those people, those customers who have a wrong perception, but all the results are the same, may go for a lower price and choose somebody who is sort of giving a more discounted price , right?

But finally, the question is that if it's being done at the discounted price, is it a structural cost advantage that health tech players have ? And the answer is no . Actually , there is nothing differently being done that makes it cheaper . Only probably the levels of quality control and quality will be different between the top incumbents and maybe some of the health tech players.

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Now , there is no structural cost advantage in actually doing it the way they are doing it and therefore , it is about burning cash in the short term to acquire customers , and then hoping that those customers will come to you again and again , or hoping that those customers will convert to illness , and that's how you will recover your money. So , I think it's more of a difference in business model than I think it is really a structural cost advantage that they have.

Kunal Randeria: Just taking forward from there , does your B2B business include some business from the online players? I meant , does your B2B business include the business from online players and they may generate it on the website but your lab would be analyzing the sample?

Surendran Chemmenkotil: Yes, so the B2B business includes some of the online players and aggregators , but that contribution is very , very low for us , you know . It's largely through the B2B labs and hospitals that we generate revenue on B2B segment.

Kunal Randeria: And just one more question , if I can. So , just taking forward the discussion on lab expansion.

So, I think in the previous quarter you had shared that you have added somewhere I think 50 labs in the last three years and these two act as a dilution to your EBITDA margins . It's around 100, 120 bps right. So , the fact that you are going to add more labs in future , does your sort of guidance take these expansion plans for the next two to three years?

Surendran Chemmenkotil: Yes, so like I mentioned in the last year , the 24 labs that we added had an EBITDA dilution of roughly one percentage , right ? And the next one year also we will add another 25 labs and we will have an equal amount of dilution of a bit of one percentage , but the guidance that we have said 24 , 25.5 to 26 percentage is already considering that lab addition related dilution. So , that's how it's going to be . Maybe after full year '25 and beyond that and I think our lab addition may not be at this pace , and hence we will be able to see a little more expansion of the margins beyond the full year '25.

Kunal Randeria: But any very indicative number you can give that beyond '25 how many you plan to add and what part of the country you are targeting?

Surendran Chemmenkotil: We will be adding about maybe six , seven , eight labs beyond '25, and we will go to specific markets that we want to penetrate further , and this number could be definitely well under 10. So, that's our current estimate.

Kunal Randeria: And so just one more , if I can. Ameera,

Mumbai is the biggest market . Yet your revenue has been growing in double digits. Just want to understand what has been driving it. Are you adding more centers, or is it more package steps? How is it, and what should we expect going forward?

Surendran Chemmenkotil: Let me answer this for you. Basically, three things. One is our distribution expansion in Mumbai continues , right? And we are expanding enough number of the service centers and collection centers in Mumbai. And we still see there is a good opportunity going forward, and we will also get into the peripheries of Mumbai. And we believe that we could rather set up a collection

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center for every 2.5, 3 kilometers in Mumbai. So, we really clearly see an opportunity to further expand in Mumbai, and that will continue.

And the second one is, of course, we are a specialty business. Mumbai has got all the top of the hospitals in the country. And hence, our engagement with the doctors in the top hospitals is getting amplified. And we are trying to expand our business on the B2B segment also here.

And also, the third thing is our existing centers. We are not allowing the productivity of the existing centers to drop despite the increase in the number of centers that we are adding. So, we have a very, very clear and dedicated focus on expanding the productivity of the existing centers.

So, a combination of these three things is what is actually driving our volumes in Mumbai.

Kunal Randeria: So, can you remind us how many centers we have in Mumbai and how many we plan to add in the next two to three years?

Surendran Chemmenkotil: Mumbai, we have about 430 centers we have in Mumbai. And we want to take this number to 500 plus in the coming year or so.

Moderator: Thank you. The next question is from the line of Shyam Srinivasan from Goldman Sachs. Please go ahead.

Shyam Srinivasan: Just the first one is, Ameera, on the opening remarks, you talked about the two kinds of competition. I am just more interested in the hospital -based competition. So, if you could elaborate, I think you made two points. But just to elaborate, one was on B2B, and the other one was the dissonance between pricing of hospital labs versus inside and outside. So, if you could just clarify on those two points or elaborate on that , please ?

Ameera Shah: So, if you see today as a customer if you walk into a top sort of 20 , 30 hospital, you will find the prices of pathology of tests inside these hospitals is a significantly higher you can say 50 to 100% higher compared to let's say Metropolis or other incumbent prices outside. So , there is a much higher pricing when you actually go to the hospital and do these tests.

So, when hospitals go outside to consumers, some cases they are choosing to have a lower pricing when they go to consumers outside , but that as you can understand creates a dissonance for customers and for consumers , right ? Because there is no logic why or there is limited logic why the pricing is so different inside and outside a hospital and that actually limits the hospital's ability to actually control the pricing outside because they already have a very high pricing inside the hospital. So , that becomes one structural problem and one fundamental issue for hospitals. The second issue for hospitals that I mentioned was that you have to remember that hospital business is about bringing patients to a hospital and then doing everything under the aegis of the hospitals , right ?

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But so there are doctors which are sitting outside of this as the gynecologist sitting outside the hospital and that gynecologist refers that patient to the hospital. The worry will always be that listen , will the patient then go to a gynecologist inside

the hospital ? And therefore , you know , there is sometimes there tends to be a concern for doctors outside the hospital to refer those patients to the hospital or to the hospital collection center. So, these are the two structural issues that hospitals may face and in the B2C side of the business and therefore , I was saying that many of them have focused much more on the B2B side of the business.

And either they are going to small hospitals , nursing homes where they are doing 5 lakhs business , 10 lakhs business of pathology per month and trying to do management contracts for these small hospitals which we find in our experience tends to be a very , very poor margin and poor receivables or therefore we don't do that business much at all.

Or it tends to be on the B2B side you know where you are going to the customers which are on the tail end because if hospitals go to the other top hospitals and say you give me your business , there is a competition there . So, that doesn't happen. So , they have to go to the labs which are the unorganized sector to say that look , why don't you outsource a thyroid or a vitamin D , like a more common test to me , and that tends to be more price sensitive. So , these are some of the sort of challenges that come up in the B2C and the B2B side for any players.

Shyam Srinivasan: Ameera, just harping on this again , so when in Apollo Diagnostics , which is the chain outside the hospital , puts up like a 500 crore revenue number for a fiscal in a 12 -month basis , you would imagine this is also probably cannibalizing their own inpatient s, I am just trying to understand , or it's largely led on B2B. That's the color I am unable to understand.

Ameera Shah: So, I would not like to comment on any individual player. But what I can share with you is that a lot of the players, some of the business comes from their own hospitals. Some of their business comes from other ventures that they have in healthcare potentially. There are people who are doing multiple things in healthcare. So , it comes from captive, you know, their own sort of service facilities. Some of it may come from an online digital. And a large amount of it will come from B2B and these HLM contracts.

So, when you set up these collection centers, for example, Metropolis reports separate B2C revenues and B2B. But most players, what they do is they don't . They report generalized revenue. And therefore, whether a patient is walking into a center, a collection center, or whether that collection center is picking up a sample from B2B is not necessarily fully clear. So , therefore, there is no data that you may get. But from the ground, we are aware that a lot of this tends to be B2B.

Shyam Srinivasan: Just the second question is on the outlook for fiscal '25. While we talked about margin guidance trying to sustain at four th quarter levels, what's our outlook on revenue for fiscal '25? And how we could likely split this between volume and price or ASP?

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Surendran Chemmenkotil: See, we have delivered 13.3% year -on-year growth during the last year. We are really looking forward to taking this further beyond this number and definitely looking forward for mid -teens kind of a growth in the year, full year '25. And the volumes, I mean, the split could be maybe, again, the volume is 8% to 9%, and five to six percentage may be coming from the realization. That could be the split.

Shyam Srinivasan: This includes the 3% increase we took in Jan. And there is also a mix change towards, I am just trying to see where the 5% comes from, 5% or 6%.

Surendran Chemmenkotil: Yes, it's a combination of both the mix change as well as the price increase that we have taken.

Shyam Srinivasan: My last question, just bookkeeping one. When I look at Slide #17, last column, we have the 15 -

month data. We also have fiscal '24, few columns before, which is the one which doesn't include 15 months. So, when we talk about flat margins, sorry, I am unable to understand. It should be actually 25.5, right , when we look at guidance for fiscal '25?

Rakesh Agarwal: Yes, so we are saying 25.5 is the actual margin. And when we add up just these four entities for quarter four, for additional three months, marginally the reported EBITDA goes up from 25.5 to 25.6 because this entity has just reported a bit higher margin in Jan to March. So , the guidance is on 25.5% only.

Moderator: Thank you. The next question is from the line of Prakash Kapadia from Spark PMS. Please go ahead.

Prakash Kapadia: I have two questions. You know, if I look at the debtor days, they are still at around 30 days. So , how much can they reduce in the coming year? And secondly, if I look at the premium wellness segment, it's steadily growing for us and is now almost 15% of sales. So , what is driving this? Is it self -awareness? Is it doctor advocacy? And if you could comment on the ARPU in this segment, is it top 5, 10 ci ties, or is it beyond that? And some color will be helpful.

Rakesh Agarwal: So, I will take the debtor thing and then hand over to Suren for the next question. So , debtor we have been maintaining and reducing substantially in last three, four years, you see, we were at around 46, 47 days, and now it is coming down to 30 days. Definitely, it's also a combination of your cash and credit business.

So, now going forward this year, we are planning to reduce it at least by 20%. So, we will be aiming to come down to 24 to 25 days of debtors as per the targets we have internally taken, and then keep improving it year-on-year basis. So, that is definitely there is a lot of focus is there and 31 days last year has come down to 30 days and we should move around 25 days next year. I am just handing over to Suren to answer the next question.

Surendran Chemmenkotil: See, on the wellness expansion, you know, basically, there are two ways of looking at it. One is definitely the external reasons, which is one, the consumer awareness, the increasing consumer awareness, wanting to get his health checkups done on time, etc.

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And also, a lot of awareness has been created by the health tech players in the past by huge advertisements and the price, etc. And hence, now more and more people are coming to the banner of getting the wellness checked on a regular basis that we are getting the advantage of that both.

And internally, if you ask me, there are two or three things that we definitely do. One is the upsell at our centers. You know, when a patient walks into a center even with an illness, our ability to upsell a full panel, or maybe a wellness package at every center that we operate, it's getting increasingly better. So, that's one place where it definitely we are able to increase the wellness volumes.

And second is our digital initiatives, you know, our website, our app, etc, where we and all the way we reach out in the social media. So, that's definitely able to get some traction on the wellness.

Third thing is our own customer base. We have now a good marketing CRM available with us, our ability to reach back to the customers on a life cycle management, reminding them on time about their next wellness checkup and on the basis of certain illness patterns, advising them about the best packages and the best panels. So, all this is helping us. There is a combination of all, both these external, internal activities, actions that we put together is taking the wellness revenues, which has grown 22% last quarter and the last year. And we see that this can be further expanded in the days to come.

Prakash Kapadia: And is it a top city centric? If you could give some

insights into what has been the realization or

the ARPU per patient in this segment for us and what are we looking at?

Surendran Chemmenkotil: So, definitely the big city, the top four, five cities, the wellness growth is much better. If you ask me, Mumbai, Chennai, Bangalore, etc., Pune, etc., the wellness growth is relatively higher, but definitely the next set of cities also picking up. As we said for the reasons I mentioned it to you, the wellness growth is picking up in there. The base is relatively smaller in some of the cities, but the growth rates are much better, right?

So, overall, the wellness awareness is definitely picking up tier one, tier two cities, and we are able to see the traction. And the average revenue on the wellness package is about 2,400, and it's 2,400. We are able to see a Rs. 200, Rs. 300 of growth over the last two quarters before versus now because of the realignment and the restructuring of the packages that we have done in the month of November, December.

Moderator: Thank you. The next question is from the line of Aashita Jain from Nuvama Institutional Equities. Please go ahead.

Aashita Jain: I have one question on the volume growth. I think this quarter we reported 7% patient growth versus high single digits that we usually report in last three, four quarters. Just wanted to understand, could there be an impact of price increase taken in B2C? Are you seeing any impact

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in those markets? Or this is just a seasonal phenomenon that should phase out as we expand going forward?

Surendran Chemmenkotil: Yes, that's right. We have seen in our non-core markets, we have seen slight softness on the volumes after the price increases, immediately after the price increases have been done, maybe the first quarter. And then we are also now seeing that it's getting stabilized, and we are seeing this is coming back to the normalcy. And also, we have done some few corrections in some of the markets. We found that the impact was relatively higher.

But overall, there is some softness in some of the non-core markets as a result of the pricing. Like any industry, if you have seen that whenever there is a price increase happen, there is a little bit of heat on the volumes, but then it stabilizes maybe in one quarter or two quarter. And we clearly seen that maybe by the end of quarter two, the volumes will come back to our earlier levels for sure.

Moderator: Thank you. The next question is from the line of Tushar Manudhane from Motilal Oswal

Financial Services. Please go ahead.

Tushar Manudhane: Just on this core business slide, as I see the number of patients visit and the number of tests has increased at a similar rate in FY 24. And effectively, let's say test per patient is largely stable at two. But at the same time, there is a good increase in B2C or let's say the preventive healthcare space where typically the number of tests taken by patient is higher. Some disconnect here or am I missing in terms of understanding this?

Surendran Chemmenkotil: I am not sure whether I got the question right, but the revenue per patients on the B2C has gone up higher because ...

Tushar Manudhane: No, not revenue per patient. I meant test per patient.

Surendran Chemmenkotil: Test per patient, you know, I think it's seven percentage only, right? 7.8% is the test per patient what they have grown up .

Tushar Manudhane: Test per patient is almost 2 for FY '24 as well as FY '23. And both number of tests as well as number of patient visits have increased at a similar rate of 8.8% for core business.

Ameera Shah: Yes, let me comment on this. See, one of the things is that when we declare our test volumes, and for example, we declare a wellness profile as one test, not as if even if it has 10 tests inside it, we declare it

as one test. So , as there is a movement of some patients upselling and bundling the packages, even though the number of tests are increasing, you are not necessarily seeing that reflecting in the test per patient.

Now going forward into FY '25, we are planning to change the way we are presenting that. And we are trying to recalibrate the numbers where we are able to give the real test volume, including Metropolis Healthcare Limited

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the breakup within the wellness packages. So , just bear with us and hopefully in a quarter or so , we should be able to align that for you.

Tushar Manudhane: That's really helpful. And just secondly, maybe in FY '24, some amount of price increase was actually with respect to reduction in the discount per se. So , is there any further scope to reduce the discount? And if you could elaborate only in terms of pricing separately on routine tests on premium wellness and specialized tests, that would be helpful .

Ameera Shah: Yes, I don't think discount is going to change much . It didn't discount , it didn't change much from last year to here. There was a slight decrease in the discount. And I think what we were referring to is that basically we have seen a marginal decrease , which means that the competitive intensity has sort of stabilized , right , because the discount is not going up , but it's actually stable and marginally sort of coming down. So , there is not too much of a difference that really is going to attribute any benefit in terms of price.

As far as the price increase on the different segments , you know , the way the price increases are done is obviously done across the spectrum , but we look at sort of competition prices local and national. We also look at our cost base and that's how we do it. I don't think we have a breakup of the price increase across the segments , but largely it would be pretty similar across the segments.

I don't think you will find too much of a difference , but probably you will find a higher price increase for routine because you get B2C through that and some increase on specialized. We will probably find a lesser price increase on semi -specialized right now because that area tends to come a little bit more from B2B which tends to be a little bit more price sensitive .

Tushar Manudhane: And just to connect this on the final aspect in a sense , so effectively the cost per test also then , does it remain more or less stable irrespective of whether it is routine or specialized or premium wellness, and so effectively that converts to better profitability?

Ameera Shah: No, I mean , your cost per test is different for every test , right ? So, for example , your gross margins on routine tend to be higher , but your gross margins on specialized tend to be lower , even though you may find a different result on a net margin because your material cost is only one cost . Your servicing cost , your production cost is also additional.

So, generally we find that the specialty segment tends to be a fairly profitable net margin segment , and that's one of the reasons why Metropolis strategy is focused on B2C because that channel tends to give you better profitability , doesn't matter which test comes to that channel . And specialty as a segment , again doesn't matter which channel it comes to , but specialty as a segment we find tends to give us a better profitability . And obviously , there is a stronger moat for the business because very few people are able to actually compete in this segment and provide the kind of quality of results that we do.

Moderator: Thank you . The next question is from the line of Vinod from Ela ra Capital . Please go ahead.

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Vinod: Just a clarification on an earlier question regarding guidance . So when you say mid -teen growth ,

last year

on the base we had some non-core businesses because of which the reported growth was lower, but the core growth like you said was 13%, 13.3%. So, when we look at FY '25, are all the known core businesses out of the base in FY '24, so will the reported growth be in the mid-teen range, or will there be some adjustments still to be made?

Surendran Chemmenkotil: Yes, you are right. In fact, from this quarter onwards, the reported revenue is equal to core revenue is equal to the group revenue. So, there is only, you know, everything is same for this year. What guidance we provided is for the reported revenues.

Ameera Shah: And what Suren mentioned is that the growth which was 13.3% from a core basis last year can go up to mid-teen. So, the guidance would be 13% to 15% would be sort of the guidance that we would provide for FY '25.

Moderator: Thank you. As that was the last question for the day, I now hand the conference over to the management for closing comments. Over to you.

Ameera Shah: Thank you everybody for joining us today and being part of us with in this quarter and in this journey. And it's been a very interesting and exciting year for Metropolis. We have done lots of changes from the technology side, lots of rollouts, including obviously the price change, including some leadership and management changes. And I think despite all these changes, which obviously sometimes cause some flux within the business, we have managed to really demonstrate a great set of results, a strong volume growth, ability to be able to take price increases and ability to be able to really navigate our expansion despite all of the changes. And I think we are very excited about FY '25 as well. We feel confident of our team. We feel confident of our aggression on the ground. And we have really put in a lot of the ingredients this last year on the revenue side, as well as on the cost side, to be able to really demonstrate a good set of results next year.

Our focus is also going to move to not only the financials, but also really focusing on the sustainability of the business, the governance of the business, and really ensuring our processes and systems only get tighter as we move along. So, very excited about next year. And we believe that the industry has, like we said, settled down, and it gives us the right platform to continue to be able to execute really well and be able to create more value for all the shareholders. Thank you so much. Suren, is there anything you would like to add?

Surendran Chemmenkotil: No, that's good enough, Ameera. And thank you, everyone.

Moderator: Thank you. On behalf of JM Financial Limited, that concludes this conference. Thank you for joining us, and you may now disconnect your lines. Thank you.

Call: Aug 2024

BLOOD TESTS • DIAGNOSTICS • WELLNESS

Metropolis Healthcare Limited

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Ref: MHL/Sec&Legal/2024 -25/41 Date: August 17, 2024

To,

BSE Limited

Scrip Code: 542650

National Stock Exchange of India Ltd

Scrip Symbol: METROPOLIS

Dear Sir/ Madam,

Sub: Earnings call transcript for Q1 FY25.

Pursuant to Regulation 30 of the SEBI (Listing Obligations and Disclosure Requirements) Regulations, 2015 , please find enclosed herewith the transcript of Q 1 FY25 earnings conference call held on August 12, 2024 . The transcript is also available on the Company's website i.e., www.metropolisindia.com

We request you to kindly take the above information on record.

Thanking you,

Yours faithfully,

For Metropolis Healthcare Limited

Kamlesh C Kulkarni

Head – Legal & Secretarial

Encl: A/a

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“Metropolis Healthcare Limited Q1 FY 25 Earnings
Conference Call”

August 12, 2024

E&OE - This transcript is edited for factual errors. In case of discrepancy, the audio recordings uploaded on the stock exchange on 12th August 2024 will prevail

MANAGEMENT : MS. AMEERA SHAH – CHAIRPERSON AND EXECUTIVE
DIRECTOR , METROPOLIS HEALTHCARE LIMITED
MR. SURENDRAN CHEMMENKOTIL – CHIEF
EXECUTIVE OFFICER – METROPOLIS HEALTHCARE
LIMITED
MR. RAKESH AGARWAL – CHIEF FINANCIAL OFFICER ,
METROPOLIS HEALTHCARE LIMITED
MODERATOR : MR. SUMIT GUPTA – CENTRUM BROKING LIMITED

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Moderator: Ladies and gentlemen , good day, and welcome to the Metropolis Healthcare Q 1 and FY '25
Conference Call hosted by Centrum Broking Limited .

As a reminder, all participant s' line will be in the listen -only mode and there will be an
opportunity for you to ask questions after the presentation concludes. Should you need assistance
during the conference call, please signal an operator by pressing star, then zero on your touchtone
phone.

I now hand the conference over to Mr. Sumit Gupta from Centrum Broking Limited . Thank you,
and over to you, sir.

Sumit Gupta: Thank you. Good morning , and a very warm welcome to all the participants on Metropolis 1Q
FY '25 Earnings Call hosted by Centrum Broking Limited.

With us today, we have the Senior Management Team represented by Ms. Ameera Shah –
Chairperson and Executive Director, Mr. Surendran Chemmenkotil – CEO, and Mr. Rakesh
Agarwal – CFO.

I will now hand over the call to Ameera ma'am for her "Opening Remarks ." Over to you, ma'am.

Ameera Shah: Thank you , and good morning , everyone. Thanks everyone for joining us for this Q1 FY '25
Earnings Call today.

I am joined by Suren and Rakesh and our IR advisors from SGA. We have uploaded our updated
Results on the Stock Exchanges and the Company 's website, and I hope everyone has had an
opportunity to go through them.

I want to start by thanking each of you for your continued trust and support. And it 's your belief
in our vision that drives us to push our boundaries and achieve new milestones. Let me begin
with giving you an overview of the industry scenario.

The diagnostics industry is set for significant growth driven by rising health care awareness, the
prevalence of lifestyle -related and critical diseases and the expansion of tertiary care across the
country, especially in non -metro towns.

Currently we are observing increased consolidation within the industry. The standalone
unorganized players are struggling to grow in terms of volume and the health tech players are
facing a stagnation in terms of wellness growth. Both groups lack the pricing power of branded
players in the illness segment who offer superior quality testing and tech -enabled solutions to
enhance the consumer experience.

The wellness -oriented health tech players don 't enjoy the same level of confidence from doctors,
making it difficult for them to enter the illness space in any meaningful way.

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Moreover, consumer preferences for superior testing experiences are driving the industry to
adopt higher standards such as a more specialized talent pool, advanced infrastructure, and large
investments in technology. These elements are now integral to stand apart in the market and
scale profitably.

A year ago, the industry was characterized by aggressive pricing and intense competition.
However, the current landscape is shifting towards a more stable pricing environment.

Consumers are now prioritizing higher quality services over low prices. There is also a clear
focus on developing niche capabilities in genomics and molecular diagnostics, which is in the
illness space , expected to drive future growth.

The industry is seeing an increase in M&A opportunities with more reasonable valuations
compared to COVID times, but high quality and ethical players will always demand a premium
because they are so few. New players have realized that while entry barriers are low and scale
can be built via the tail end of the market, the sustainability of the business is challenging, and
profitability will remain elusive by building it this way.

At Metropolis, our focus remains on scientific innovation that differentiates us from the others .
Secondly , geographical expansion provides access and driving growth through strategic
initiatives

ves.

I have always maintained that science is at the core of Metropolis. Our specialty segment is a key differentiator, collaborating with tertiary care specialists and providing them comprehensive, super-specialty diagnostics for treating complex diseases. There is an increase in demand in this segment, and it is more margin accretive due to its higher value and the specialized nature of tests which command premium pricing. To this effect, we are strengthening our Centers of Excellence for specialty testing by expanding our test menu and accelerating our efforts in molecular genomics.

We have also created IMA B, which is the Internal Medical Advisory Board, to provide expert guidance on clinical and medical matters. Their insights and guidance will drive innovation and maintain our reputation for clinical excellence.

In line with the Government's focus on upskilling and workforce development, we have recently launched MILE S, which is Metropolis Institute of Laboratory Education and Skilling, in collaboration with reputed universities to provide continuous education and training for doctors and technologists.

We are actively expanding our market share in both core and emerging geographies focusing on specialized tests, and bundled wellness and illness packages. Currently, the top 8 Tier-1 cities in India contribute about 60% of our revenue, and we are leaders in four of these eight towns, while our peers are leaders in only one such city each.

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Metropolis' strategy has been about deep penetration in large cities through our own and retail franchise centers, which has led to premium pricing and word-of-mouth marketing. When we look at the top 20 towns in India, they contribute 75% to our domestic revenue. We believe the growth opportunity in these 20 markets remains robust, providing us a long runway for growth and continued expansion.

Given our strong operational lab infrastructure in these markets, our key strategy is to keep opening new collection centers and increase productivity at all these centers through brand trust and excellent service. This will remain our focus this year.

Meanwhile, we have built 79 labs between FY '21 and FY '24, primarily in Tier-3 and 4 markets. By '25, we will have achieved our goal of establishing 90 labs. While these markets are growing rapidly for us, they still represent a small portion of our overall business.

However, the significant growth opportunity that lies ahead of us is generating revenue from these 630 Tier-3 and 4 markets where we are already present but have not focused on extensively and therefore they contribute to small percentage of the revenue.

Since we have already established the necessary infrastructure in these markets through our labs and collection centers, we believe that by intensifying our sales and product efforts, we can turn these regions into a strong growth engine for the future.

Our goal over the next 18 months is to additionally expand our network from 650 towns to 1,000 towns and at the same time focus on driving more sample load through our existing centers in these markets. We plan to offer a full range of tests and work with both B2C and B2B channels through this franchise ARC model in these 650 going to 1,000 markets.

While we extended our network in geographic reach, we also remain dedicated to enhancing the growth and profitability by existing partners and the network. Our aim this year is to create a differentiated brand and empower our services with AI and digitization for a seamless customer experience for both B2C and B2B customers. This will make it easier for customers to access services, schedule appointments, make payments and receive results digitally. These enhancements also facilitate quick resolutions to queries and concerns, overall improving customer satisfaction.

satisfaction.

As we mentioned last quarter, exploring bolt-on and strategic acquisitions remains a key focus, allowing us to enter new markets and acquire new skills. I am personally dedicating a considerable amount of my time in identifying and pursuing acquisitions that I believe will align with our strategy going forward. Any acquisitions will be done after due consideration of strategic benefits and return ratios over mid to long term.

Additionally, we have focused our time and money on building our IT infrastructure to optimize operations, streamline processes, reduce manual interventions and overall improve productivity. Our partnerships with renowned industry players ensure access to the latest technologies and

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best practices, enabling us to implement operational automation and efficiency improvements.

Since we are investing significant capital in infrastructure and technology, we have established a CAPEX Committee to oversee these investments, ensuring efficient capital deployment.

We are deeply committed to enhancing compliance and solidifying our governance framework. Prioritizing impactful ESG initiatives and social responsibilities is also central to our long-term strategy. One of the key focus areas for me t his year will be to strengthen our governance to ensure that we operate with the utmost integrity, transparency, accountability and no surprises. Speaking of the quarter gone by, w e have delivered strong financial and operational performance for Q1 FY '25 with revenue and margin growth as was expected in Q1 in line with our overall anticipation for the year.

In closing, our strategy for this year focuses on expanding organically in our core markets of pathology, exploring new growth opportunities and alli ed services, and maintaining our commitment to scientific excellence and sustainability.

With these strategic initiatives, we are confident in our ability to deliver exceptional value to our shareholders, customers, and partners. I personally want to than k our team, investors, and board for their commitment to Metropolis. Your contributions are what help us build a brighter future for Metropolis and the communities we serve.

With that, I will ask Suren to now take you through our "Operational Performance " for the Quarter and our "Strategies " for FY '25.

Surendran Chemmenkotil: Thank you , Ameera, and good morning everyone on the call today morning.

We have had another superb quarter and begun the year on a strong footing. Our Quarter 1 revenue grew by 13.1% year-over-year, well within our guided range of 13 %-15% growth for the year. We need to read this along with the fact that Quarter 1 is historically a relatively weaker quarter for the West and South of India for the industry.

EBITD A and PA T for Quarter 1 , '25 grew by 21.2% and 31.3% respectively. This margin increase is attributed to operating leverage and operational efficiencies. We have maintained better OpEx control with costs increasing by only 10% while revenues grew by 13%. In the coming quarters, we anticipate to build on this further better.

Moving on to the operational KPIs :

Our patient volumes for Quarter 1 grew by 7% year-over-year, with a 6% increase in revenue per patient. Test volumes grew by 10% year-over-year, with a 3% increase in revenue per test. We have consistently seen high single -digit patient volume growth over the last 9 quarters and remain confident in our volume trajectory going forward as well.

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In this quarter, patient volume growth was partially impact ed by a price increase in our B2C segment effective January '24. As I told you all earlier, typically after a price increase, there is a temporary dip in volume for one or two quarters. However, we have observed a month -on-month upti

ck with volumes of June growing by 7.5% and we expect this positive trend to continue.

In Quarter 1 , our revenue from TruHealth wellness and illness package bundling grew by 28% year-on-year. Our approach to selling packages focuses on empowering consumers with health informati on along with providing commercial benefits all backed by an excellent tech -enabled service.

This strategy has enabled us to upsell to existing customers, resulting in higher revenue per patient and more tests per patient. Scientifically curated packages and targeted marketing allow us to maintain a laser focus on digital channels, enhancing our margins. With continued emphasis on this approach and further automation, we believe TruHealth 's contribution will increase with each coming quarter.

Through our network expansion strategy, we have increased our presence to over 650 towns by Quarter 1 end, up from 300 towns in Financial Year '2 3. We have successfully identified and entered markets with significant potential that face a supply -side challenge for quality diagnostic services.

While we have entered these markets through our franchisee centers and this expansion has boosted our revenue from Tier-3 cities, it is still early days and we believe this part of the business has a long runway for growth. Revenue from Tier-3 cities has grown by 18 % year-on-year, and now contributed to more than 33% of total revenues.

We are committed to our expansion strategy of adding 25 labs and 500 centers this financial year with a focus on Tier-3 cities. This involves b uilding a lab infrastructure and establishing network spokes to serve these labs as feeders, which will drive long -term revenue growth. While building labs adds to costs initially, once the feeders are established and productive, they will significantly increase our revenues in the future.

Our B2C revenue growth has been very robust, increasing by 18.4% in Quarter 1 . The B2C segment now contributes to 54% of total revenue, up from 51% in Quarter 1 , '24, keeping us on track to reach our target of 60% B2C revenue. Our B2C revenues from TruHealth and Specialty grew by 31% and 22% respectively. Our B2C revenue for the Mumbai market has increased by

18%. The overall B2C growth of 18.4% indicates we are expanding across various markets. We have also seen faster than average revenue growth in cities like Bangalore, Pune, Surat, indicating increased consumer mindshare for quality diagnostic testing. Based on our internal costing analytics, we understand that our B2C segment yields higher margins in TruHealth and Specialty areas, which is why we continue to focus more on these segments.

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Additionally, our strategic initiatives on B2C channels have enabled us to achieve robust, same-store growth from our own centers in established markets. We aim to replicate the same in our partner network as well, ensuring their growth and profitability.

Our recently launched partner engagement program, Metlink, will be instrumental in this regard. By providing structured support and benefits to our partners, we have enhanced our network's reach and efficiency. This program will help us strengthen relationships with our existing partners and retain them, thereby ensuring growth and success.

Our B2B segment has also performed exceptionally well with revenues growing by 12.4% in Quarter 1, '25. Over the last four quarters, we have increased our market share in the B2B business while maintaining our discount structure, ensuring profitable growth. B2B revenue contributions stood at 37% for Quarter 1.

We have implemented several initiatives including engagement through the partner portal, a centralized help desk and assigning dedicated key relationship managers to enhance service levels and drive B2B growth.

We have consciously decided to focus

on our institutional business coming from Government

tenders, low margin corporate accounts and aggregators. As a result, the remaining 9% of our business, excluding B2C and B2B has grown at a slower pace than the Company average, impacting overall revenue growth for short term. However, we believe that this is a well thought out plan and there is sufficient potential in our B2C and B2B segments to meet and exceed our revenue growth aspirations in the near future.

Lastly, regarding our broader numbers for Full Year '25, we hope to achieve revenue growth in the range of 13% to 15% for the year as guided at the beginning of the year. Keeping in mind that coming quarters are historically stronger, particularly Quarter 2 and Quarter 4, this growth will be driven by a high single-digit volume increase and favorable change in product mix, which will help enhance the revenue per patient.

In terms of operating margins, while Q1 has been a strong start, we expect margins to expand further during the rest of the year due to operational leverage and seasonal factors within the business. We will continue to focus on margin enhancing segments like TruHealth and Specialty through our B2C channels.

As we continue building new labs at a rapid pace this year, we anticipate roughly a 1% EBITDA dilution due to the OpEx losses associated with these new labs. This dilution is already reflected in our current P&L and is expected to continue at a lesser number until full Year '26, after which it should cease and contribute positively to our margins for the long term.

On the closing, I would like to take a moment to acknowledge the hard work and dedication of our partners, the trust of the clinical fraternity and the efforts of our team. Together we are building a stronger and more resilient organization.

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With this, I will now hand over to Rakesh for the financial updates.

Rakesh Agarwal: Thanks, Suren. Let me share some of the key "Financial Performance" for the Quarter:

Revenue for Q1 Financial Year '25 stood at Rs. 313.4 crore, a growth of 13.1% Y-on-Y with 7% patient volume growth and 6% on account of price increase and product mix change.

Our test volume growth stood at 10% for Q1 Financial Year '25. We count profiles as one test in MHL and the underlying test count will be higher. Revenue per test has increased by 3% on a Y-on-Y basis.

Our B2C revenue stood at Rs. 169 crore as compared to Rs. 143 crore in Quarter 1 Financial Year '24, an increase of 18.4%. Our B2C revenue for TruHealth grew at 31% and for Specialty at 22% on a Y-on-Y basis.

Our B2B revenue grew by 12.4% for Q1 Financial Year '25 compared to same period last year with B2B TruHealth and Specialty growing by 24% and 11% respectively. Revenue contribution of TruHealth segment to total revenue has moved up to 17% from 14% indicating a growth of 28% Y-on-Y.

Reported EBITDA for the quarter stood at Rs. 78.2 crore as compared to 64.5 crore, a growth of 21.2%. Reported EBITDA margins for Q1 Financial Year '25 stood at 25%, a growth of 170

basis point year-over-year.

PAT for the quarter stood at Rs. 38.1 crore with margin of 12.2% , up from 10.5% for Q1

Financial Year '24, which is an increase of 170 basis point year-on-year basis.

Moving on to the balance sheet , we have a net cash surplus of Rs. 137 crore as on 30th June 2024.

That's all from my side. With this , I open the floor for Q&A. Thank you.

Moderator: Thank you very much. We will now begin the question and answer session. First question is from the line of Amey from JM Financial. Please go ahead.

Amey : The first question I have is on the other expenses which have gone down sequentially. So, in opening remarks , we mentioned that we are optimizing cost etc. , which has led to margin improvement as well. So, should we

assume these other expenses as a structural change or is it led by some seasonality or something like that?

Rakesh Agarwal: So, I will take this. So , this is a combination of both. Obviously , the seasonality will help us to ramp up the revenue and that will come in a bit of a margin. And then at the same time , we are also looking at some structural changes like a bit more of automation, process improvement , and

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efficiency coming through productivity increase. So, all this combination will also help us and at the same time , the seasonality will help us. So, this is a combination of both.

Amey : So, going ahead , considering we are expanding as well , so how should we look at on the margin front? If you can guide us for full year?

Rakesh Agarwal: So, we have already guided that we will be in between 25% to 26%. So, we have closed our Quarter 1 at 25%, and we believe that we should go upside to 26% as we go further. So, we will be somewhere landing at between 25% to 26% during the year for the full year.

Amey : And on the top -line guidance of 13 % to 15% for the full year, I believe we also said that high single digit would be coming in from the volume and rest of it would be maybe the mix and the price hike. So, is it possible to guide us on the price hike trend for next year? It would be similar nature like we saw last year.

Surendran Chemmenkotil: So, see, at this point of time , there is no discussion going on the price hike because too early for us to think about it because we just only over with two quarters with the earlier price hike . And our focus is definitely to increase volumes, improve productivity out of the stores from the franchisees, expand into the 650 plus markets etc . So, that's what we are focus going to be when it comes to revenue growth.

If any price increase at this stage or early next year is not under discussion at this point of time .

We will take a call at the appropriate time depending upon what 's the market conditions as well you know . So, volume growth definitely i s one of our big focus areas and you will see that rolling out as you go towards the rest of the year.

Ameera Shah: Just want to add something to that . I just want to add something to that that, see, you remember that the 13% is a breakup of three things. One thing is the 7% volume growth. The second is the price input and the third is actually the product mix which is changing the average revenue per patient . So, the price does not impact the product mix part of it .

So, what we will certainly see going into next year and out of this 13% , only approximately 2%-2.5% has been effectively added by price . So, the rest of it is actually coming because of sort of work on the ground in terms of volume s and in terms of changing the product mix .

So, when we are thinking about next year, we will obviously not have any guidance at this point of time in terms of revenue etc. But the way we can think about is these three levers and even if tomorrow we don 't land up doing price or we land up doing price, we are thinking about price as an annual or once in 18 months kind of a scenario .

So, we don 't have a timeline fixed yet at this point , but we certainly , obviously , the volume and the increase in ARPP , which is the average revenue per patient , will certainly happen regardless whether we do the price increase or not.

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Amey : Just one follow -up is that if the price hike is not taken, let 's say, assume next year, then the ask for the ARPP is significantly higher and are we confident of achieving that next year?

Ameera Shah: See, ARPP is factored in by multiple things, right? It is about one, educating doctors about the more advanced diagnostics that are available and therefore recommending

more specialty

diagnostics for patients . That's one.

The second is how much we are able to drive more of our TruHealth , which is the bundling and

the wellness, right? So, as patients walk in, there are patients who want to then buy bigger packages and what is our ability to do that conversion?

Now on both sides, as you can see, as we have shared with you in the presentation, both these areas are growing faster for us than the overall revenue for the business. And that's what our aim is to continue to do, is to drive specialty and TruHealth at a faster pace. And therefore, if we are able to continue to do that, automatically the ARPP will keep increasing. So, I think we are confident in where we stand on the execution front. And like we said, it's not that we are not planning to do a price increase, we just have not decided on the timeline, whether it will be 12 months, 18 months. So, I think we will just have to wait for that.

Moderator: Thank you very much. The next question is from the line of Kaustav Bubna from the BMSPL Capital. Please go ahead.

Kaustav Bubna: I wanted to understand what's stopping us from growing 25% on a year-on-year basis. Why are we guiding for mid-teen growth? What is it in the market and the factors that are stopping us from growing faster? Could you explain?

Ameera Shah: So, the industry, the way the industry is structured is, you have a head and a tail. The head is really where you have your best quality doctors, best quality hospitals, and top labs, which are your customers. And then you obviously have a tail, which is made up of a lot of unorganized players, which tend to be sort of a lower standard, right?

Now, the customer base that we are sort of going after is really more in the head because in the tail we find that the quality of work, because there is no minimum standards in India, the quality of work and therefore the kind of discounting and the kind of service requirement, it makes your business actually not so profitable if you follow good quality minimum standards.

So, if you try to grow much, much faster, you have to then be willing to play in all parts of the business, in all parts of the industry which may then actually have a negative impact from a margin perspective because you land up providing over servicing, unit economics of customers which are not very profitable. It might impact receivables days and fairly among other KPIs as well.

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And we have to remember also that the market is at this point still quite fragmented and unorganized in nature and while we have always said it is an easy market to enter from a competition perspective, it is not an easy market to scale profitably.

And therefore, I think these are some of the thoughts when we are thinking about where we should really play, where we should focus our energies, because it is easy to build scale and grow at 25%, but you are not going to necessarily get the commensurate profit that actually makes this a business worth building or a business which is sticky enough and where you are building it on the back of brand and science rather than building it on the back of pure discount.

So, I think our model is more unique based on science and service and then we would like to build it in a way that is sustainable and profitable.

Surendran Chemmenkotil: Kaustav, we can just add one more statistic to that. In Quarter 1, if I look at the industry average growth of all the operators who have published the results, you know, it stands at 12.6% and we are at 13.1%. So, our endeavor is always to grow higher than the industry average and we are putting our efforts to be there.

Kaustav Bubna: Just one more question. You know, a few years ago, we did an acquisition to expand our presence in the south. So, want to understand going

ahead, obviously, this market could consolidate going ahead, and we could be the ones consolidating it. So, wanted to understand where would we like to expand inorganically? Now where are the opportunities you are seeing, like in which states, in which regions in India?

Ameera Shah: So, we have about two or three different strategies now when it comes to acquisitions. One is what we have traditionally done, which is the geographical expansion, right? And Hitech also, which we did in October '21, was not so much about expanding into South as we were already the leaders in Chennai, and we were a significant player in Bangalore. And by acquiring Hitech, it accomplished two or three different things for us. It helped us solidify our leadership in Chennai.

And what we have found in healthcare is when you are a dominant player in a market, there are specific benefits on word-of-mouth, premium pricing, profitability, all of those benefits come through. And therefore, rather than being in 20 different places in markets, it's always better to actually be leaders in fewer markets rather than just being a very, very diversified option. So, keeping this in mind, we would like to continue the strategic thought around geographical expansion. So, that's one.

The second thought we also have is that, you know, there will be sort of technical or medical skills you may want to acquire in terms of products, in terms of new services and new tests that can actually help us grow for the future. So, there might be acquisitions also potentially linked

to bringing in new service offerings into the business.
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And the third kind of acquisition which could be looked at could be that we have seen a lot of entry over the last 15 years of players who have come into the market. And there are many what we call as sort of zombie firms in the industry today, which are knowing that they are never going to make money because they are either sub-scale or structurally there are issues, but a larger player can potentially turn these around with better management.

So, we are looking at these three different buckets for strategic acquisitions. And we will obviously, whenever we are applying our lens of acquisition, we look at not only the strategic imperative, we look at obviously the return ratios, the IRRs of these projects over a period of time, and we obviously look at the quality of the business.

Moderator: Thank you very much. The next question will be from the line of Pranav Chawla from Antique Broking. Please go ahead.

Pranav Chawla: My first question would be regarding; we have seen a slowdown in the growth in our focus cities. So, would you like to give any color on the reason for this slowdown, is this competition, is this saturation or a high base or anything of that sort?

Surendran Chemmenkotil: The focus cities, you know, it's in this range only over the last couple of quarters, two, three quarters, we are in the 8% to 10%, 11% range on the growth, because these are the big market, the top markets, and already keeping with the reasonably good high base.

Our efforts to drive these markets for further growth is like, just I told during the speech is about increasing the same-store growth further and also bringing in the franchises to add on to the overall growth story and also focusing on the TruHealth and the Specialty segments, right?

So, you know, this focus cities in the coming days, you will see a little more upside on the growth perspective, but if you look at it, two, three quarters is in the similar range, and we want to do better on the focus cities.

Pranav Chawla: And on the M&A front, any particular geographies that we are looking for? Because some of our players are, I think, are in South as of now,

South and East. So, what would be our preferred markets given that we already have a strong presence in the South and West?

Ameera Shah: See, our belief, our model is a little bit different from our peers' models. I think some of our peers look at sort of regional expansion as a goal. We always have looked at this business as a city-wise focus. And the reason for that is like the statistic that we mentioned in the speech that out of the Top 8 Tier-1 markets in India, we are leaders in four.

Now that cannot happen unless you have a city specific mindset. If you have a regional mindset, then that will never happen, right? Because in these Tier-1 markets, you have to be able to build very strong B2C. And the ability that we have is to be a built it in four markets where we are the leader, right? So, while we will continue looking at North and East, obviously, because we are challengers and not leaders in those markets, and therefore M&A will be helpful.

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But there are many cities even in the West and South in which today we are not leaders. And therefore, if tomorrow we did get a good acquisition that allowed us to have that platform to build dominance and leadership in a market in the South and West, we would not refrain from it. Because it's very different to go into a market and like through a distributor and build a B2B business, but it's very different to actually go into a market and build a B2C business. And therefore, we will be open to all markets, but obviously the preference is North and East.

Pranav Chawla: Last question from my end. Given that we have changed our presentation, can you provide us a split between North, South, East, and West for the quarter?

Ameera Shah: I think that's already in the presentation. Right, Suren?

Surendran Chemmenkotil: Contribution? No, it's an overall contribution. So, we will look at it and provide you. We will have noted down this.

Pranav Chawla: And as well as value on volume split for our tests.

Surendran Chemmenkotil: All right.

Pranav Chawla: Routine, specialized and semi-specialized.

Surendran Chemmenkotil: Right. We will do that.

Moderator: Thank you very much. The next question will be from the line of Sumit Gupta from Centrum Broking Limited. Please go ahead.

Sumit Gupta: I want to understand all the market dynamics in the Western India part of it. And how is the pricing scenario going there? And how much is like, what is the competitive intensity that you see in the Western India market, particularly Mumbai region?

Surendran Chemmenkotil: See, the Western India, particularly Mumbai, the competition intensity is not any lesser or more

than the past, right? I mean, we continue to see a similar amount of rigor from all the competitions.

But of course, we have not seen too much competition from the pricing point of view over the last two, three, four quarters, right, purely on the basis of productivity, efficiency, product mix et cetera is what is playing in this market. So, you have seen that we have done 18% growth in Mumbai and our Pune is another city in the west region is doing very well. Surat is doing very well.

So, we are on our trajectory with respect to growth on all the western part of the country and also Maharashtra is doing very well for us. So, any specific thing that you want to know, please ask. Otherwise, in terms of our action plans on the west part of it, the rigor et cetera will continue to be as it is and it is going to help us in terms of our overall growth.

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Sumit Gupta: So, just want to understand on the geographic mix over the next, let's say, next 2 to 3 years, so if you plan to expand into like M&A into southern market, so just want to understand will the

geographic mix remain more or less the same or will it change over the near future?

Surendran Chemmenkotil: The mix could, of course, today, west and south being our strong markets, the mix is largely skewed towards west and south at this point of time and as we keep expanding to the north and east and the next 300 more towns like we said, of course the mix will start changing a little bit as we go forward, right? But also, to some extent, things will also depend upon where we will go and do some of the acquisitions etc.

But organically if you ask me, the mix will definitely keep changing and you will keep seeing north and east part of the country getting stronger and we talked in the past that Uttar Pradesh, Madhya Pradesh, Assam are the 3 states where we are going to put lot of expansion plans and also Andhra and Telangana, right, which, of course, falls in south. So, I mean, you will definitely see some amount of change of the mix going forward on the market which today we may not be as strong as what we should be.

Sumit Gupta: And sir, just one thing on this point. What is the rationale that you look forward to when you target a new market? Just like you said Uttar Pradesh, Assam, Madhya Pradesh, so I just want to understand on that part.

Surendran Chemmenkotil: What is the rationale for?

Sumit Gupta: Like how do you target the markets? Like what do you see in the particular market that you focus on?

Surendran Chemmenkotil: when you enter into a new market you are asking, right?

Sumit Gupta: Yes.

Surendran Chemmenkotil: See, what we do, there has been a science behind entering into a new market, right? See, what we look at it is, first, of course, we start with what is the population of the city, what are the medical facilities in the city and how many hospitals, how many physicians and how many doctors in that city, what is the paying power of the city, what is the competition. So, all this put together, we have a science, we figure out okay, what is the right time to and right market to enter, what is the right time to enter.

We first process that and then we start with a B2B presence in that market. We go and appoint some of the B2B clients who will take samples for us and send it to us, the particular specialty sample. And when the volumes reached a certain level, then we go there and start our own centers and we start the B2C. And then the volume picks up to a certain more level, then we will go and put up a lab there. That's largely the broader sense of the science that we use in putting up any new cities.

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So, we mentioned that we moved from 300 to 650 cities. Some of them we put up labs and rest all we have put up centers, either through B2C or B2B, and we need to keep expanding these 650 cities first and grow the volumes. That's the first opportunity available. And at the same time, we will move the number from 650 to 1,000 cities using the science that I talked about it.

Sumit Gupta: And sir, lastly, on the, like, present scenario in these test menus, so this will be specialized routine tests. How is the present scenario going in these? And if possible, can you provide the split between all these, like you used to provide previously, specialized and specialized routine and wellness also?

Surendran Chemmenkotil: I think we talked, we have mentioned that 37% of the total revenue is coming from the specialized testing, and TruHealth, which is the wellness and illness bundle packaging, has moved up to 17%. So, 54% of the total business is actually coming from Specialty and TruHealth, and the rest is coming from the routine and semi-special.

Sumit Gupta: So, how is the pricing scenario, like what kind of pricing power do you have ? And we just want to understand this pricing also . So,

what is exactly driving the pricing down? Is it only the routine test that is there, or are you facing competition in some other category as well?

Ameera Shah: I will just add something to that. See, if you have to remember one thing, pricing is not a competitive on a specific test. It depends on the customer. I will tell you what I mean by that. People think that routine tests or pricing is commoditized, and specialty tests are advanced. That's not actually true.

The way it works is that if you are a patient who is fine, you are not a patient at all, you are a healthy customer, then you may start looking at discounts, depending on your mindset, as something that drives your decision -making. But even in wellness, we find there are many customers who are more, they are saying, look, we are doing this once a year. I don't want to compromise, and I will do it only from a trusted player, and I will pay whatever I need to pay. But when it comes to illness and you are sick, for example, a patient who has got heart disease, even a cholesterol, which is a very routine test, is a very important test for them . So, you can't say that the cholesterol test is therefore commoditized, and therefore the pricing is low . So, actually what we find is that pricing is completely impacted by the criticality of the patient. And the more critical the patient, the less chances they are willing to take on something going wrong, and the more chances that they will go to a branded player that they trust , and then price actually has very little meaning because you are doing testing once a year and you are average spending some Rs. 700, Rs. 800. It's not a very expensive or a deal for most people . So, we actually have found that the pricing is not a challenge. The only thing which is increasing every year has actually been the distributor margin . So, it's not that consumer prices are coming down but distributor margins are going up and now that depends on your model. If you are Metropolis Healthcare Limited
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largely led by a sort of you have built a business which is discount led and very heavily dependent on distributors , then the last few years have been challenging for you. But as an organization , as we mentioned , that has not been our model. Our model has been more direct to consumer , direct to doctor and therefore , frankly , our discount percentages are not increasing . In fact, they have been very stable when it comes to margins for distributors. Suren, if you want to add anything.

Surendran Chemmenkotil: Yes, that's fine I think you mentioned that.

Moderator: Thank you. The next question will be from the line of Pranav Chawla from Antique Broking . Please go ahead.

Pranav Chawla: So, on the gross margin front , if I see our gross margins has only marginally improved whereas our B2C mix has continued to improve . Our focus in Tier-1 cities has also improved . So, on those sides , it has always been tick-tick-tick, but our gross margins are not meaningfully improved . So, is there a particular reason why that is the case?

Rakesh Agarwal: : So, I will just add on one or two things . While you are right that this whole B2C growth and total growth should add the gross margin , there are certain activities which we are doing , is also taking away like new labs which we are creating . So, that actually has a higher material cost in the initial six to eight months period because there are very low volumes there and the cost of material is directly proportional to the volume. So, that is one reason.

Second is that we are also expanding our test menu in lot of new geography . So, we have added lot of new tests in new locations and in RRL . So, that also in the initial stage we face a bit of gross margin lower in that sense and definitely as we are expanding our test menu every

month ,

so we are adding lot of new tests and when we add new tests , especially in the Specialty segment , though this test comes with a lower volume in the initial stage and a higher cost. So, these are the three factors which actually nullifies , but obviously from a long run point of view , all these are worth investing .

Pranav Chawla: Sir, just to follow up on that . So, when if you are saying the new expansion , I fail to understand how new lab additions is impacting gross margin because I am assuming these may be franchisee centers majorly and you may be processing tests at your main center . So, your other expense, the traveling, the test movement cost will be captured in other expenses and the processing may be happening in your main processing center.

Surendran Chemmenkotil: Please understand this, we are expanding the centers to the franchisees and labs, all labs are our own labs . The one which is coming up in Tier-2, Tier-3 towns is all our own labs . So, when a new lab comes up, the test volumes in the initial months will be lower, but at the end of the day, once you open up a consumable kit, you need to consume in a certain amount of period and hence, there is a relatively higher material cost from a new lab and also all varieties of new test s that we start, right? So, that is what is impacting the gross margin like Rakesh said. And it is not

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anything to do with the franchisee when it comes to the new labs . Only the centers are being run by the franchisees and the labs run by us if that clarifies your question.

Pranav Chawla: Sir, in Delhi, if I am not mistaken, the previous couple of quarters , we were taking write-offs and making provisions for the receivables that we had from the Government . So, is there any update on that?

Surendran Chemmenkotil: So, we continue to do that . I mean, whatever revenues come from that account, we continue to provide for at this point of time and while our efforts on collecting the monies are also on.

Pranav Chawla: Sir, but if I am not mistaken, we had decided that we were planning to stop supplies until we start receiving from the Government .

Rakesh Agarwal: : So, there are two accounts in the Government . Obviously , one is this Aam Aadmi , which we spoke last year and there are discussions that how should we continue the relationship, the volumes has come down particularly this year. And there are contractual obligations on Aam Aadmi which we are looking at and accordingly we will act upon . We cannot just abruptly stop any services . These are publicly distributed and there is no other contract as such as of now where we are not receiving payment, and we are continuing to supply services.

Moderator: Thank you. The next question is from the line of Aashita Jain from Nu vama Institutional Equities . Please go ahead.

Aashita Jain: A couple of questions . So, firstly on your revenue growth guidance, would it be possible that the 13% to 15% growth to seem sustainable even for the next two to three years in the absence of price hikes? As things turn today, we are expanding, and the focus is more on the product mix. How do you see this coming out in the next two to three years?

Surendran Chemmenkotil: So, Aashita , we have guided for 13 % to 15% for the full year and also , I mean , once we do that , and I think going forward also , we should be talking about a similar levels of growth . And on the pricing part , like Ameera also mentioned this , we are not saying that price increase is not on the cards , so we are not going to take the prices up . I mean , we said it 's appropriate time we will do that and we need to earlier , I mean , as of now the industry does in 2 years , 2.5 years once , but we will keep looking at the opportunity to increase the price as we go , right ? And till then we will

I focus on the volume growth , the text mix between TruHealth and the specialty and also the volumes coming from the new accounts and the new labs that we launch . It's a combination of all that . So, we are optimistic that you know the guidance that we have given for this year can even be stretched to the years beyond the next one year .

Aashita Jain: My second question is on TruHealth. So, it's already contributing to about , I think , 17% to our revenues . Where do you see it settling in the next two to four years ?

Surendran Chemmenkotil: Well , I think I don 't want to put a number to it because there is a great opportunity . As and when we keep exploring , we keep seeing a lot of opportunities in this segment , bundling in the illness

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packages and also driving the wellness . We are clearly seeing good opportunities because one , of course , is giving a lot of good information about health to the consumer and also commercially good for the consumer , right ? And we are delivering it through technology-enabled platforms and also 360 degree , like, it's going through all kind of touch points whether it is digital , whether it is our service centers or from the call center . So, every piece is helping us to grow this business . So, as and when the consumer adoption and the awareness go up, this only keep getting quarter-on-quarter . I mean , every quarter we see that this last quarter to this quarter last , year to this year , we are seeing growth , right ? So, we can be sure that this will keep going . I don 't want to put a number to it for a year or two at this point of time . We would definitely like this to be a very big part of our portfolio .

Aashita Jain: No, I just wanted to understand directionally how you are looking at this?

Surendran Chemmenkotil: Yes, of course , actually , so you would see that 17 , 20, 25. So, I don 't want to put a time band to that, but definitely there is a huge opportunity . As we keep exploring , we can clearly see that there is a good you know portfolio that you are working upon .

Aashita Jain: And just lastly on your expansion plan , so, I see that one of your key geographies that you plan to expand is UP and other players too are directing their expansion plans towards this region .

So, what gives us confidence to grow there and build a successful brand giving us a non-core market for us ?

Surendran Chemmenkotil: So, I have already , yes, sometime back I spoke about the science that we use for geography expansion , right ? The population, the medical setup, the opportunity, the competition , etc. We do all those kind of science and we arrive, these are the key cities , then we should go and operate. And also , some of the cities are not, supply side is definitely we find efficiencies in terms of

good quality diagnostics . So, on the base of tha t, we are identifying these terms and putting up labs and so far, our success rates have been pretty good, right? And of course, like you know, UP, MP are all very highly populated places and medical facilities can still get better . So, that's the reason w hy we are focusing on these cities, and we are seeing wherever we are putting up labs and centers, we are finding things getting better sooner.

Moderator: Thank you very much. Ladies and gentlemen, that was the last question for today. I now hand the conference over to the management for closing comments.

Ameera Shah: Thank you everybody for joining us today for the Quarter 1 review of Results. As we have mentioned and just wanted to summarize, it's been a good quarter for us. We have been very encouraged by the very positive volume growth, the ARPP growth and we believe that we are very much on the right track and obviously the margin holding up as well. Usually, Q2 and Q4 are the best quarters of the year for th

e organization. And we have alrea dy gone into Q2 with that optimism of being able to have this quarter be better than obviously Q1.

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We are very excited also about the other robust things that we are building in the organization, whether it 's the systems, processes, technology and really good quality of team. And all of these things together really make us very excited about the future and really diversifying into new markets. Like we said, they are present in about 630 other markets which contribute low revenues today and how we can real ly make these centers much more productive and expand further into 1,000 markets of India .

So, the future and the runway for growth for this industry and for Metropolis specifically is very high. And we w ould definitely continue to see these engines of growth keep churning quarter by quarter.

Additionally, of course, we will be looking at acquisitions as well, things that are strategically aligned to us and look forward to accelerating growth through that aspect as well. But we are very excited and pos itive and look forward to continuing to chat with you on the next quarter. Thank you everybody.

Moderator: Thank you. On behalf of Centrum Broking Limite d, that concludes the Conference Call. Thank you for joining us , and you may now disconnect your lines.

Call: Nov 2024

www.metropolisindia.com | support@metropolisindia.com

Ref: MHL/Sec&Legal/2024 -25/68 Date: November 16, 2024

To,
BSE Limited National Stock Exchange of India Ltd.
Scrip Code: 542650 Scrip Symbol: METROPOLIS

Dear Sir/Madam,

Sub: Earnings call transcript for Q 2FY25

Pursuant to Regulation 30 of the SEBI (Listing Obligations and Disclosure Requirements) Regulations, 2015, please find enclosed herewith the transcript of Q 2FY25 earnings conference call held on November 11, 2024. The transcript is also available on the Company 's website i.e. www.metropolisindia.com

We request you to kindly take the above information on record.

Thanking you,
Yours faithfully,

For Metropolis Healthcare Limited

Kamlesh C Kulkarni
Head – Legal & Secretarial

Encl: A/a

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"Metropolis Healthcare Limited
Q2 FY '25 Earnings Conference Call "
November 11, 2024

E&OE - This transcript is edited for factual errors. In case of discrepancy, the audio recordings uploaded on the stock exchange on 11th November 2024 will prevail

MANAGEMENT : MS. AMEERA SHAH – CHAIRPERSON AND EXECUTIVE
DIRECTOR – METROPOLIS HEALTHCARE LIMITED
MR. SURENDRAN CHEMMENKOTIL – CHIEF EXECUTIVE
OFFICER – METROPOLIS HEALTHCARE LIMITED
MR. AVADHUT JOSHI – CHIEF BUSINESS DEVELOPMENT
OFFICER – METROPOLIS HEALTHCARE LIMITED

MODERATOR : MR. TAUSIF SHAIKH – BNP PARIBAS EXANE RESEARCH

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Moderator: Ladies and gentlemen, good day, and welcome to the Metropolis Healthcare Q2 FY '25 Earnings Conference Call. As a reminder, all participant lines will be in the listen -only mode and there will be an opportunity for you to ask questions after the presentation concludes. Should you need assistance during the conference call, please signal an operator by pressing star, then zero on your touch -tone phone. Please note that this conference is being recorded.

This conference call may contain forward -looking statements about the company, which are based on the beliefs, opinions and expectations of the company as on date of this call. These statements do not guarantee the future performance of the company, and it may involve risks and uncertainties that are difficult to predict.

I now hand the conference over to Mr. Tausif Shaikh from BNP Paribas Exane Research. Thank you, and over to you, sir.

Tausif Shaikh : Thanks, Segal , and good morning, everyone. On behalf of BNP Paribas Exane Research, I, Tausif Shaikh, India analyst for pharma and healthcare services, welcomes you all for Metropolis Q2 FY '25 earnings con -call. Today, we have the senior management of the company represented by Ameera Shah, Chairperson and Executive Director; Surendran Chemmenkotil , Chief Executive Officer; and Avadhut Joshi, Chief Business Development Officer.

I will now hand the call to Ameera ma'am for opening remarks. Over to you, ma'am.

Ameera Shah: Hi. Good morning, everyone, and thank you for joining us on the Q2 FY '25 earnings call today.

As mentioned, I'm joined by Surendran, Avadhut and Aditya Shinde, who is the Interim CFO; and SGA, our IR advisors. We've uploaded our updated results documents in the exchanges and the company's website, and I hope everyone's had an opportunity to go through the same.

Let me begin with the industry scenario, so we can set the context for today's call. The Indian diagnostics industry continues to be highly fragmented with over 85% of the market dominated by unorganized and independent diagnostic centers. These centers have succeeded in building a hyper local customer base with repeat clients, but they struggle to attract new customers and are experiencing a decline in volumes as more organized and trusted diagnostic chain entered these micro markets.

Independent centers typically offer a limited range of tests with an average of around 100 tests compared to over the 4,000 varieties of tests that are available at Metropolis. The limited test offerings, the inability to serve specialized doctors, the lack of reach that the local labs have and the inadequate digital and physical capabilities that makes it difficult for them to increase volume and scale.

We continue to see a larger trend of unorganized moving to organized, especially in metro markets. Recognizing this as a significant opportunity, we have been focusing on selective micro markets across India, working to establish trust and credibility with doctors through a comprehensive test menu, accurate results, best -in-class experts for test interpretation and high -touch doctor engagement.

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As unwell customers prioritize quality and experience over price, our brand reputation, in -house expertise on every test leading to huge trust and our physical and digital infrastructure have made us the preferred choice. In smaller towns, the local lab is still relevant due to strong local relationships and large chains like Metropolis create relevance for ourselves through specialized testing, which the local labs cannot do.

Speaking of competition from organized players and tech -enabled e -health companies, we have not observed any significant changes compared to the previous quarter. The new age companies continue to face challenges in gen

erating illness -driven volumes. And due to competitive pressures and low conversion rates from wellness to illness services, they have been compelled to raise prices to improve their unit economics.

Additionally, we have witnessed that they have been forced to enter the traditional physical brick -and-mortar model, a process that is both long and challenging, especially when it comes to scaling and achieving profitable growth. There has also been talk about competition from hospital players, but we believe this remains confined to a few geographies like Delhi and Hyderabad, where major hospital chains have had a presence for decades.

Pricing for these hospital chains remains a real challenge as their retail price for a patient outside the hospital is one-third what they charge inside the hospital, leading to huge conflicts with their customer and cannibalization of their own business. This is one of the reasons we believe it will be challenging for hospitals to truly scale their pathology ventures in a profitable manner.

Let me provide some insights on regional competition. A strong regional lab in one city with B2C business would try and expand into other markets close by via the B2B route as they would not have a brand in another city. While capital is available to expand, the ability to build brand, attract talent and management expertise to provide quality and quick results across regions consistently and build a complex test menu that are needed to scale the business are actually the moles of our industry when you're building a chain, which is why most city-focused businesses have not been able to scale and definitely not scale profitably.

We have been leaders in the West and South regions and continue to experience consistent growth in our core geographies, often exceeding the company's average growth rate. For instance, in Mumbai, our B2C revenues have increased by 17% to 20% over the past 8 quarters, and we remain the fastest-growing player in these regions, well positioned to capture market share even if we see further competition.

This is largely due to the market composition with a significant portion still unorganized, providing ample room for growth if you have execution excellence. We are confident that we will outperform industry growth, gain more market share on the back of the trust and credibility that has been built over the years with doctors and consumers. In noncore geographies, like North and East, where our consumer brand is relatively less established, which often we call them seeding cities, we have been strengthening our presence through our B2B business.

We have built a strong medical reputation amongst recognized grade E hospitals and labs, which choose our services due to our testing capabilities and high-quality offerings with specialized and quality testing at the core of our value proposition. We are also seeing significant growth

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opportunities beyond the core geographies, especially in Tier 3 and Tier 4 cities, which are becoming increasingly important focal point for us.

We are deepening our presence in emerging markets like UP, MP, Punjab and Assam, where the demand for reliable diagnostics is rapidly rising. With a highly fragmented diagnostics industry, we are strategically focusing on micro markets with minimal organized competition with stand-alone centers and unorganized players are having majority of the market share.

In Tier 3 and Tier 4 cities, we are experiencing strong growth, and we anticipate continued acceleration as health care access and awareness expand. By tailoring our approach to meet the unique needs of these communities, we are ensuring that our reach extends beyond the traditional urban centers, establishing a robust network that serves diverse populations.

In Q2 FY '25, revenue from these towns accounted for over 25% of our

contribution with a year-over-

year growth rate of 23%. Currently, the revenue for Metropolis from India's top 10 most popular cities comprises nearly half of our revenues, while the top 100 cities contribute around -- close to around 75%. The balanced revenues are contributed by smaller towns and cities, which are growing at a fast pace.

This increase in revenue growth is driven by our aggressive network strategy, adding new locations across smaller cities and finally, being able to earn the trust of doctors to get patients to walk into the center. Today, we are present in over 700 towns with plans to expand to at least 1,000 in the next 12 to 18 months. These cities not only offer promising growth, but also opportunities to build enduring relationships and trust within these communities.

Moving forward, we will prioritize increasing our leadership in our core B2C markets where we have consistently outperformed industry growth. Additionally, we will focus on expanding our presence in Tier 3 and Tier 4 towns, which have limited diagnostic providers and services. By implementing targeted marketing and sales strategies, we aim to enhance our market penetration and establish a strong foothold in these regions, driving overall growth in the coming years.

Let me quickly share some insights into our inorganic expansion strategy. We've been actively pursuing inorganic growth with 2 main objectives. First, to acquire capabilities in advanced specialty testing areas such as genomics, oncology, histopathology and molecular diagnostics. The biggest growth is going to be in these areas in the next 10 years, and therefore, we definitely want to get deeper into these testing capabilities.

Second, to extend our reach in noncore geographies, particularly in North India, where Metropolis has a relatively lower brand presence. These acquisitions will serve as entry points into these specific micro markets, allowing us to then build the business organically after acquisition by implementing Metropolis standards in testing, service and delivery, along with specialized testing an extensive test menu and scientific marketing through doctors.

We are in advanced stages of evaluating potential targets, and we'll keep you updated at the

appropriate time. Aligned with the government's focus on workforce development and skill enhancement, we have launched the Metropolis Institute of Laboratory Education and Skilling or what we abbreviatedly called MiLES in partnership with DY Patil University and Metropolis Healthcare Limited

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Maharashtra State Skills University. to offer specialized fellowship and certificate programs for both MD and DMD students.

Covering fields like advanced clinical chemistry and molecular pathology, these programs blend theoretical learning with hands-on training on advanced diagnostic technologies. MiLES reaffirms our commitment to bridging the health care skills gap and preferring professionals to meet the sector's evolving demands. Before I hand over to Suren to take you through the quarterly highlights, I would like to reiterate the key growth strategies for Metropolis, we will continue to focus on.

Number one, expanding lab and collection center networks in new cities, largely in Tier 3 and Tier 4 cities in India to expand our addressable market. Number two, focus on being the pioneer in new test advancement and amplify our engagement with specialty doctors to grow our specialty volumes. Number three, focus on our TruHealth packages with curated packages and increase our contribution, which is currently 16% of revenue to keep increasing this as we go further.

Fourth, fostering the inorganic growth and foray into adjacencies. And fifth, heighten our service standards and digital marketing initiatives to improve productivity and customer experience.

That's all for now, and I'll be happy to chat with you guys

So more over the question and answers.

Suren, over to you.

Surendran Chemmenkotil: Thank you, Ameera, and good morning, everyone, on the call. We have been consistently able to deliver industry-leading growth over the last 8-plus quarters now. Our revenue for quarter 2 financial year '25 grew by 13.4 percentage on a year-over-year basis well within our guided range of 13 to 15 percentage band. Our EBITDA for quarter 2 grew by 22% with an EBITDA margin inching up to 26.2%, an increase of 190 bps year-over-year. Our PAT for the quarter stood at INR46.7 crores, an increase of 31% year-over-year. PAT margin also witnessed a growth of 180 bps year-over-year.

On the broader KPIs, our patient volume growth for quarter 2 stood at 7% and our test volume growth stood at 8% on a year-over-year basis, which stands to be one of the highest in the industry. Our patient and test volume growth for H1 stood at 7% and 9%, respectively. Increase in volumes are on account of increase in market share among the core geographies and expansion and deepening our presence in newer geographies, especially in Tier 3 and smaller towns across the country.

Now moving to a few of the operational KPIs. Our B2C revenue for quarter 2 grew by 21% year-over-year. Patient volumes in B2C segment has grown by 12%, signaling an increased market share driven by extensive test menu, trust and credibility Metropolis brand holds with both doctors and consumers. We have achieved faster revenue growth in our core geographies within the B2C segment. B2B revenue for quarter 2 grew by 13% year-over-year.

Our focused efforts and enhanced engagement programs with the key B2B clients have enabled us to increase our wallet share in this segment. As we have managed to keep discounts at bay and enhance our productivity per client, we believe our graded approach to customers can help us growing well and profitably as they look for quality and good services with Metropolis.

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The B2C and B2B revenue make up to almost 90% of our business and the combined growth of this segment is around 17% year-over-year. As shared in earlier occasions, we have strategically chosen to disengage with government and institutional clients, resulting in a revenue decline in this segment and impacting overall growth in the last 2 quarters, which may continue for another couple of quarters more.

This decision aligns with our focus on quality over price-sensitive business. Excluding this segment, our combined B2C and B2B patient volume have grown by 8% year-over-year. As a part of our larger strategy, we have been prioritizing the specialty and bundled product segment as dual growth drivers. Our TruHealth portfolio achieved strong year-over-year growth of 23% in quarter 2.

Carefully curated wellness and illness packages and focused marketing efforts have driven the growth in this segment. TruHealth now contributes to around 16% of our total revenue, and we would like to move this towards 20% in the coming few quarters. By upselling to our existing customer base, we have increased the number of tests per patient, leading to higher revenue.

This approach also boosts our margins as customer acquisition and servicing costs remain steady.

Metropolis has been a pioneer in the specialized diagnostic services, driving a 16% year-over-year growth in our specialty revenues. And this portfolio growth is bettering quarter after quarter. By introducing new tests and strengthening our relationship with specialized doctors nationwide through education and engagement on the latest testing capabilities, we continue to expand our specialty business.

We have launched, for example, in the last 6 months, next-gen pre-implantation genetic testing for pre-IVF, next-gen panels for oncology to guide targeted therapies and optimize cancer treatment and the fecal immunochemi

cal test, which is a very important test for early screening of issues in the gastrointestinal system.

We have also introduced comprehensive panels for testing in transplant patients and advanced profiles that provide an in-depth analysis of multiple steroid hormones in a single test, enhancing diagnostic accuracy. See, these advancements reflect our commitment to precision and progress across critical health care specialties.

Our scientific approach to selling, combined with omnichannel strategy has enabled us to reach a larger audience and drive significant traffic for specialized testing. In terms of network expansion, we added 7 labs in quarter 2 and 17 labs in H1 with the goal of reaching 25 new labs by the close of financial year '25.

We have also closed down a few labs, which is reflecting in smaller net addition number for this period. This has been done to optimize cost synergies in certain regions. We also added net of 186 collection centers in the H1, bringing our total centers to 4,336 and we are targeting about 500 adjacent centers by the end of this year.

This growth strategy is driven by the increasing presence in tertiary care outside metro cities, extending into smaller cities and towns nationwide. In line with this trend, we have entered over Metropolis Healthcare Limited

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400-plus new towns in Tier 1 and Tier 2 regions over the past 24 months, focusing on these areas as key growth opportunities.

Our reach has now expanded to nearly 700 towns, and we aim to reach 1,000 towns over the next 12 to 18 months, like Ameera mentioned. This expansion lays a strong foundation for Metropolis' next growth phase. We have made significant progress in boosting our operational efficiency and elevating customer service by heavily investing in our digital and IT infrastructure.

Our focus has been on developing a seamless intuitive platform where customers can easily explore and select tests, schedule appointments, make payments through integrated gateways and receive their reports digitally, enhancing convenience and accessibility at every touch point. In parallel, we have fostered and consolidated strategic partnerships to establish a center of enablement dedicated to optimizing our mission-critical IT applications. This collaborative efforts enhances efficiency across the organization and position us for long-term sustainable growth. This end-to-end transformation not only enhances operational efficiency, but also strengthens customer loyalty as we provide a faster, more reliable and customer-centric experience.

Lastly, if I speak of our outlook for full year '25 -- financial year '25 and beyond, we are maintaining our revenue growth guidance in the range of 13% to 15% driven by high single-digit patient volume growth. We expect margins to improve as we move forward, benefiting from operating leverage within the business.

As our new labs reach maturity, we expect margin to further enhance through increased throughput and reduced lab expansion-related expenditures. With this, now I hand over to Aditya Shinde for financial updates.

Aditya Shinde : Thank you, Suren. Good morning, everyone. I am Aditya Shinde, and let me share some of the key financial performance for the quarter. Revenue for quarter 2 FY '25 stood at INR349.8 crores, a growth of 13.4% Y-o-Y with 7% patient volume growth. Our test volume growth stood at 8% for this quarter. Revenue per test has increased by 5% on a Y-o-Y basis. Our B2C revenue stood at INR194 crores as compared to INR161 crores in the quarter 2 of last year, which is an increase of 21%.

Our B2C for wellness and specialty grew by 26% and 24%, respectively, on a Y-o-Y basis. Patient volume growth for B2C business stood at 12% Y-o-Y. Our B2B grew by 12.6% for this quarter compared to same period last year, with B2B wellness and specialty growing by 21% and 10%, respec

tively. B2B patient volume grew by 4% on a Y-o-Y basis. Revenue share of TruHealth segment stands at 15%, indicating a growth of 23% Y-o-Y. Our specialty segment

revenue stood at INR129 crores, which is a growth of 16% Y-o-Y.

Reported EBITDA for the quarter stood at INR91.5 crores as compared to INR74.9 crores, a growth of 22.2%. Reported EBITDA margins for this quarter stood at 26.2%, which is a delta of 190 bps. PAT for the quarter stood at INR46.7 crores, a growth of 31%. PAT margins for this quarter stood at 13.4%, up from 11.6% for the same period last year, which is a delta of 180 bps.

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Moving on to the balance sheet. We have a net cash surplus of INR185 crores as of 30th September 2024. That's all from my side. With this, I open the floor for questions and answers.

Thank you.

Moderator: Thank you very much. We will now begin the question and answer session. The first question is from the line of Shaleen Kumar: from UBS Securities India.

Shaleen Kumar: Congrats on a good set of numbers. Sorry, it's a bit of a long question. So there was a time when your margins were a little higher than the industry-leading peer. But obviously, you got into investment modes and expansion mode, which is great, and we could see the results coming in. But your margins are now tad lower, in fact, reasonably lower than the peer, while your revenue per test or revenue per patient are still higher.

So what I want to understand is you're still in a kind of this expansion mode where you called out your Tier 3 strategy. But how should we think about the margin and the growth balancing effect? Because your growth in Tier 1, Tier 2 city is not that great. A large part of it is coming from Tier 3 city where you need to invest and expand. But at the same time, we look forward for a margin expansion because you still have a reasonable critical mass. So any thoughts on that?

Ameera Shah: So I'll add a couple of thoughts and Suren can add to it as well. So the 2 areas we are seeing good growth is actually in our Tier 1 cities, metros, where we are continuing to -- like we gave you the example of Bombay and Pune and other markets, Bangalore, where we are continuing to see a very positive revenue growth, especially on the B2C side.

So we are finding the growth is very high in the top markets where we have a brand and the growth is high in the Tier 3, Tier 4 markets just because there is that much opportunity, right? And we believe that these 2 areas, geographies will continue giving us a good revenue growth as we go forward.

To your question around margins, see, if you look at pre-COVID, the margins actually for the industry hovered around 26% approximately. And it was during COVID that obviously they went up closer to 28%, 29%. We all knew that was not sustainable. The choice we had post COVID was, as some of the peers have done is not really invest in lab expansion or other areas, but to actually let the operating leverage flow through to the profit.

But that very clearly then reflects in a much smaller growth in volume, patient volume, test volume as well as in revenue growth. The choice that we chose to make is that considering the large opportunity in India at hand, we felt that rather than maximizing margins, that we believe that the opportunity really stood in continuing to increase access and to continue to grow across the country and we will stabilize and build the brand even further.

And therefore, we continued our process from FY '21 till '25 of adding approximately 90 labs, which was increasing our lab capacity by 50% plus and obviously adding centers. Now everybody knows that when you add a new lab, it doesn't become mature in 1, 2 or 3 years. It takes longer than that. And as we've been busy in building labs, while we have built collection

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centers, we have not built collection centers to feed these labs at the pace that we will be doing going forward.

Now we have to remember the collection center model is an asset-light model. You have -- obviously, we are building some centers at our own fixed rents, etcetera, which are patient service centers, but majority, 90% of our centers are through the franchise route, which is an asset-light model. So from FY '26, you will actually see the lab addition come down to 4 to 5 a year probably. And we will see actually the center expansion move up further, which is on the asset-light model.

We believe that as this continues to mature and our network of the last few years matures, we will continue to see operating leverage and which is why we indicated that we will see margin expansion happening. Finally, business is all about choices, and we believe that margin expansion, frankly, 100 to 200 bps can be done pretty much within our control at any point that we decide to stop expanding.

And therefore, we made the fact to really focus on the revenue expansion. And I think the next

3, 4 years, we will hopefully continue to see revenue expansion still be at our targeted numbers, but see the margins expand as well. I hope that answers your question.

Shaleen Kumar: Yes, yes, yes, Ameera, mostly, so I agree with you, right? That's the way I also look at it that a lot of margin is in your hand because you can take a call of how much you want to invest and claw back and growth. So is there a thumb rule which you play by certain amount -- certain growth you need to target. And beyond that, anything whatever you make that will spill through the margin? And if that's the case, any long-term aspiration on both growth and margin where you're seeing that's the steady state?

Ameera Shah: I mean I think we would like to -- I mean, you can't comment honestly, beyond 2, 3 years for any industry because things change. But at least I can comment for the next 2, 3 years, we are continuing to, like we said, target to 13% to 15% revenue growth and margins at about between 26% to 27% and hopefully, we'll see -- if there is an ability to be able to see further margin expansion, we'll obviously do that.

But at this point of time, we believe that today, we are already between 26% and 27% if I remove the dilution of the lab addition that we have done, adjust for that. And going forward, we also have obviously a potential to expand it further. So I don't want to give any very specific numbers because then that keeps it too tight, but we certainly see an opportunity there.

Shaleen Kumar: All right. Can I ask one last question? Let me go ahead.

Ameera Shah: Yes, please go ahead.

Shaleen Kumar: Any size in mind when you're thinking about acquisition and the payback?

Ameera Shah: Look, I think acquisitions, we are seeing lots of assets available across the country. But as we've always maintained, we find 90% of them, 95% of them, not investments that we would like to do because of poor governance or poor models, etcetera. So we will continue to be very selective.

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And the ones we will do will be high-quality businesses, which we believe can make fair profits in time.

I think sizes can be bolt-on acquisitions, which are smaller, but leaders in their cities. They could also be larger regional players or they could be pan-India players. But I think our goal will be to, one, to make sure that we are pricing the acquisition very fairly and correctly from a ROCE perspective. And like I said, second, high-quality businesses, if they are bolt-on, then they have to have majority business coming from B2C, which yields high margins.

If they are B2B businesses, then they have to be non-commoditized B2B businesses, high specialty B2B businesses, which can bring some very tangible

quality of customers to the table

and specialty volumes. So I don't have any specific numbers for you, but we will certainly keep you guys updated as we move forward.

Moderator: The next question is from the line of Anshul Agrawal from Emkay Global.

Anshul Agrawal: My first question is on the B2B portfolio that seems to be lagging our B2C portfolio over the last 4, 5 quarters, predominantly in terms of volumes. While you have alluded in your opening remarks, the reasons for the same, do you expect this divergence to continue as online players sort of target this portfolio?

Surendran Chemmenkotil: All right. Anshul, B2B portfolio, if you look at it, the overall growth in the last 2 quarters, we are growing at 13% now. I mean -- so in the past quarters, if you look at beyond that, we were at single-digit numbers on B2B. And B2B growth has come on the back of a very, very focused approach towards improving the client experience and our relationship with them, etcetera, getting strengthened and also by controlling the discount.

We have really that sort of long tail that we have in terms of which were giving us a little bit of volumes, but were not giving us enough margins and revenues. So in this transition phase where we are actually trying to get focused in the head part of the B2B client and also hold the discounts and increase revenues, there could be a small phase of time where you will not find volumes going up because those volumes which was coming was not very good margin volume.

So otherwise, if you look at the overall portfolio, it looks good. The revenue per patient is looking good. The overall revenue is looking good. And as we go forward, I think you'll get to see the volumes also stabilizing.

Anshul Agrawal: Okay. So are we saying that B2B volume growth would be lagging B2C volume growth for the next, say, couple of quarters or longer than that?

Surendran Chemmenkotil: Well, I mean, we may take a couple of more quarters to get over with the long tail that story that I was talking about it. We can't equate or I can't make a comment about whether B2B and B2C volume will be going in the same trajectory or not. But I think we are aiming for an overall volume growth of coming close to 2-digit number. That's a high single-digit number as we go forward, and we can clearly see that happening.

Anshul Agrawal: Sure. This is the patient volume, not the sample volume that you're alluding to, correct?

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Surendran Chemmenkotil: Exactly. We are talking about the patient volume growth, yes.

Anshul Agrawal: Great. Just one clarification on the number of labs that we have added in H1. Our presentation says a net lab addition of 3. And if I heard you correctly, you said they added 17 labs in H1. So is there a rationalization of almost 14 -odd labs on the Hitech portfolio?

Surendran Chemmenkotil: Yes. Actually, that's exactly what I mentioned in my speech as well. We have just -- wherever there are synergies possible between Metropolis and Hitech, we just combine those labs and with the numbers will then be lesser in the total count. And there are also a few other geographies where we could find an opportunity of lab consolidation or the lab was not making sense to us. So some of those -- in these last 6 months, we just took a call to consolidate that without really impacting whether the revenues or customer experience at all.

Anshul Agrawal: So we'll just add about 8 labs in H2?

Surendran Chemmenkotil: Well, I mean that's the plan. 25 is the number for the year, and I think we are on course.

Anshul Agrawal: Okay. Great. Just one last question. Now our top line while it has grown because of volume as well, but because of RPT improvements, which would be on the back of price hikes that we have taken probably at the start of this year, which would sort of be in the base b

y, say, Q4. Do you

see volume growth to sort of expand materially to keep our top line growing by the 14%, 15% number say, after FY '25 as well? Just broad understanding what it would be useful.

Surendran Chemmenkotil: Yes, that's right. I think our top line growth -- like we mentioned in the speech, it's going to be in the range of 13% to 15%. Maybe a couple of quarters later, the impact of the price increase that we took last year may a little bit fade away. But then I think the volumes should start substituting for that.

Ameera Shah: Just want to add one thing to that, that see the ARPP growth is I think 5% to 6%. Of that, the price increase is only -- the net price benefit is only about 2.3% to 2.4%. So all of it is not coming from price increase. It is actually coming from product mix largely.

Anshul Agrawal: So -- and obviously, considering that we are trying to enhance/seek higher contribution from specialty, wellness portfolio, this should -- RPT should continue to increase sans the price hike numbers.

Ameera Shah: That's right. So the product mix changes will continue to drive the revenue per patient increases as they always have. Obviously, the volume growth, we would like to continue to see it to move on an upswing. And a combination of these 2 is where we believe that the targeted guidance should land between 13% to 15%.

Anshul Agrawal: Got it. Just one small follow-up on this. Our sample per patient number is hovering around 2.07, 2.06 since 3, 4 quarters. Would this significantly improve now that we are focusing more on wellness portfolio?

Surendran Chemmenkotil: In fact, this was in the range of 2.2, 2.3 - 3 to 4 quarters back. On the -- as we keep progressing on the TruHealth and the specialty getting better, now we have come closer to 2.7%, 2.8%. As

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we keep focusing on this portfolio mix of TruHealth and Wellness, this should start getting better, and we're really driving the bundled products in the last 2, 3 quarters.

Moderator: The next question is from the line of Nikhil Mathur from HDFC Mutual Fund.

Nikhil Mathur: So my first question is around the geographic mix of revenues. So I'm on Slide #13 of the investor presentation, which says that Tier 1 and Tier 2 growth was around 8% and 7% in this particular quarter. So just wanted to understand what's happening on the Tier 1 and Tier 2 side. I don't know if you have talked about this on the opening remarks, but the growth seems to be lagging in these important clusters for the company. Is it because of B2B defocus or competitive pressures? If you can just elaborate a bit more on Tier 1 and Tier 2.

Surendran Chemmenkotil: Yes. In fact, if you remember, I just mentioned that we just a little bit defocused on the institutional business and government business, which were normally giving us the lower margins. And this business were largely coming from Tier 1 cities, right? So I mean that's one of the reasons you will find a temporary drop in the Tier 1 revenue growth. And then -- and also the Tier 3 growth is definitely fueled by the increase in number of labs and centers that (Inaudible 36:06) Tier 2, Tier 3 towns.

So I think we -- like I said, 1 or 2 more quarters, we will have a little bit of very focused view on the corporate and institutional business. And post that, I think you will see that the Tier 1 cities also coming to a double-digit kind of a growth.

Nikhil Mathur: So I'm just a bit curious to understand...

Ameera Shah: Yes. Nikhil, if you even in this quarter adjust for the institutional degrowth, like, for example, we had this government contract in Delhi, which was the Mohalla clinics contract, right, which we sort of have been -- which has come down, etcetera, and we are exiting. But for example,

these businesses like this, there are 2, 3 contracts, which are all part of the government

institutional.

So if you adjust for those and you actually look at the B2C and B2B, the Tier 1 cities are actually doing really well. But on the reported revenue side, it will not show up because like we said, of the degrowth on the institutional, which happened to be in these Tier 1 cities. I hope that clarifies.

Nikhil Mathur: Yes, sure. Also, just curious to understand your B2B customers predominantly would be hospitals, right, if I'm not wrong. And hospitals today are in the pink of health. I mean, if you look at how hospitals are reporting their numbers, they are at top end of the margins that the industry has ever reported.

So is the B2B nongovernment private market now getting more lucrative for diagnostic place for you? Or because they are so doing well, they would rather invest themselves and boost their own diagnostic requirements rather than outsourcing to a company like Metropolis?

Surendran Chemmenkotil: Okay. If I speak -- I mean, just I'll clarify on the B2B segment split. And if you look at the B2B segment split, a large part of B2B, Nikhil, is on the B2B smaller labs. The hospital on the B2B portfolio is relatively much smaller. right? And we keep engaging with the bigger hospitals as well as smaller labs, which is across the country, the wider geographies. So our dependence on Metropolis Healthcare Limited

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the big hospitals is not as high that tomorrow, if they start in-sourcing the test, that will have a huge impact on our performance.

So I mean, we -- at the same time, we keep building volumes from the hospitals where we are already doing business with, and we also start -- keep getting more and more B2B hospitals at the same time. But our focus on B2B clients, which is basically the doctor run pathology labs or the technician run pathology labs, that's the larger in terms of size, and we keep engaging with them and keep giving them better services, as I said. Ameera, please.

Ameera Shah: Yes. I just wanted to give you a little larger context, the hospital business as well, if you have to remember that 85%, 90% of it continues to be smaller hospitals. the guys you see on these listed platforms, listed exchanges are only in the top 5% or 10% of organized players, right, which are a handful. 85%, 90% of the market is unorganized, smaller hospitals like it is in diagnostics. And really, the -- like to Suren's point, the larger customers come from that 85% to 90%, not from the top 10%, which are the organized listed companies. So the organized listed companies will obviously do their best effort to try and do more and more testing in-house and especially chains will try to consolidate their own volumes.

Now despite all their efforts, still a fair amount gets outsourced from their hospitals because doctors sometimes don't want a hospital a pathology. They want another expert pathology player like us or somebody else. And therefore, still some amount of revenue will get outsourced -- testing will get outsourced.

But that is not our key focus anyway. Our key focus is 85% to 90% of the hospitals and other labs that will outsource the specialized test to us who do not have the capability to do this themselves.

Nikhil Mathur: Understood. And one final question on the Tier 3 market. I mean the growth is phenomenally strong in that market. The strategy of venturing and expanding in these cities is clearly playing out as we can see in numbers. Just wanted to understand we are also seeing on the wider health care penetration side in Tier 2 and Tier 3, especially when I look at hospitals, not just the top 10, but beyond that as well, they're also generating decent ROCEs now in these markets, which was the biggest pain point for the overall I sector until now.

So do you see a correlation between more hospitals being set up in Tier 2, Tier 3 that driving your pickup as well in these markets as you are

venting in many of these cities? Any case studies,

any correlation that you are able to track or understand in these markets?

Ameera Shah: I'll just comment on the broader pattern and Suren can comment on the specifics. Just on the broader pattern, what we've seen for the last 5, 7 years or maybe a little bit more than that, actually 8, 9 years, is that doctors who used to come to metro markets to come and study, let's say, oncology or gastroenterology and would stay back in metros and jobs in the corporate hospitals, started actually migrating back to their hometowns and setting up their own hospitals or their own practices in specialty care in their small cities.

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And we started seeing this sort of reverse movement, which is why Metropolis then proceeded to say, look, we need to expand our labs into these cities because the specialist doctors are going

back into small towns, it's only a matter of time that those diagnostics will be required there. And that's the reason we went on an aggressive last 4 years lab expansion spree in these markets. It's like you're saying it's paying off for us.

We are now seeing hospitals coming up in these smaller markets as well, and that is increasing volume because the minute you have treatment and therapy be done in a local place, automatically, the diagnostics for that will increase because more of those diseases and symptoms are getting diagnosed and then treated. So we believe this trend will continue for the next at least 2 decades where you'll continue to see sort of strong access to health care in Tier 3, Tier 4 markets, and that will bring about good volumes for diagnostics.

Moderator: Mr. Nikhil, does that answer your question?

Nikhil Mathur: Yes. Can I just squeeze in one last, please?

Surendran Chemmenkotil: Please go ahead.

Nikhil Mathur: Yes. So the bundled testing TruHealth and Tier 3 markets, at a very broad level, now quite a few quarters have passed since you have been seeing traction in both these segments. Now if things have stabilized, what is the impact of these 2 segments on gross margin and EBITDA margin for the company? Are they accretive at both these line items, accretive at dilutive at gross margin, accretive at EBITDA margin? Any broad thought you can share on this?

Surendran Chemmenkotil: Largely, the TruHealth packages, the wellness packages largely in line with the company's margins, right? It's not any dilutive at all, right? And as we -- if we are able to add more tests per patient, then it will be good in terms of revenue -- I mean EBITDA, it will be improving because you don't have to go back and take the samples one more time and you do n't have to service it one more time.

So we'll keep focused on increasing the number of tests per patient. That will really help us in terms of improving the margins. But otherwise, neither specialty nor the TruHealth packages are diluting the margins. So in a steady state, it is only going to help us with the overall margin for the company.

Nikhil Mathur: And sir, Tier 3 market?

Ameera Shah: Yes, I just want to make one comment on the Tier 3 market. See, frankly, if you look at Metropolis' strategy current ly and in the past, we focused a lot on the Tier 1 markets and then we started expanding our labs, like we said in the last 4 years in Tier 3, Tier 4. And while that is giving us revenues, actually, it's giving us revenue, but without having a very robust sort of franchise network partnership, which operates very well.

And that continues to be an opportunity for us, which I see will play out in the next few years.

So I think the kind of franchise model structure that we are building now, the kind of engagement, the kind of marketing support, all of that, we believe will start boosting the revenues from the franchise network. So not only will the productivity of the

existing franchise

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network increase, but the numbers of the network should also increase quite significantly in the time to come.

And all of this will add benefit at the bottom line. So if we are able to get that model really running very well, streamlined and tight like we're intending to do, we believe that it should be over now. .

Moderator: The next question is from the line of Rishi Mody from Marcellus Investment Manager.

Rishi Mody: So Ameera, my first question is on the 3 out of the 5 initiatives that you mentioned that you're going to focus on. First is the wellness package. Second is the foray into adjacencies. And third is the digital initiatives to drive productivity and customer engagement. So if you could just give some more details on what exactly are you all doing for each of these initiatives, that would be helpful.

Ameera Shah: Sorry, could you repeat which 3? You said one was the customer.

Rishi Mody: The wellness one, the adjacencies one and then the digital initiatives one.

Ameera Shah: Okay. I'll ask Suren to take the one around the wellness and the digital, and I'll come in fo r the adjacencies.

Surendran Chemmenkotil: Yes. So see, what we are doing is on the wellness side, we have actually curated all the packages, which is going to give good benefits in terms of science as well as commercial for all the consumers. We're now do ing it for the last 3 quarters, all the wellness package has been absolutely upgraded and it become very, very specific to the consumer segments. And now we are also driving it through multiple channels, omni -channels. One is, of course, on all the centers -- all the centers that we have.

We are actually trained all our phlebotomists in the center to provide good advice to the consumer who walk in about the best package that suit him. We've also recently launched something called recommendation engine, whic h is when the patient walks into a center with a proper algorithm that's been used on the back end, the phlebotomist will be able to recommend

to the patient about what is the next best test that he or she can actually do.

So one on the center -- the physical centers that we have. And second one is on all the digital platform, whether it's an app or website, everywhere when the consumer goes, he will be able to really navigate himself and figure out what is the best solution that he'll be able to get in. But we have actually powered all these digital engines, which is website and app, etcetera, in the last few quarters. So we are able to see much better traction.

And the third one is about the customer data platform that we have, the consumer base that we have. So we are able to engage with our existing customer in a much more programmed way using much better science, and we are able to tell them and remind them at the appropriate time about what kind of the next action that they will be wanting to take.

So the omnichannel presence, which is helping us to engage with the consumer, and we are able to see a very good traction on the customer life cycle management. So these are the initiatives

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we have taken to get the wellness portfolio right. I mean that's the -- what was the next point you said? One is on the wellness and...?

Rishi Mody: On the digital initiatives to drive productivity

Surendran Chemmenkotil: Yes, not only about digital, the website, the website and our app and also our customer data platform, all this is part of the digital initiative that we have. And we also recently launched something called the recommendation engine, which will be able to advise the customer about the next best action that he can take. So a combination of all this with a good execution focus, we are able to get the traction to the tune of 23%, 25% on this

portfolio.

Rishi Mody: Sure. That's helpful on the front-end piece. On the back end, anything for cost optimization where digital is helping you all?

Surendran Chemmenkotil: Yes. See, today, 20% of the revenues are coming on the digital -- from the digital channels that we have. And digital channels, of course, doesn't carry any what you call franchisee margins or anything other than the overall IT operational cost. So that's definitely we are getting much better margin on the as the digital -- digital portfolio grows, we'll get much better margins. And also the consumer experience get that much far superior.

Rishi Mody: Okay. All right. Got it. And where do you think your wellness as a contribution will settle in?

Surendran Chemmenkotil: I said that wellness, I will put it at 2 health together, which is illness and wellness bundles together. Today is at 16% of my total revenue. So we are driving this towards the first milestone we are wanting to cross is 20% in the next few quarters.

Rishi Mody: Okay. Got it. Ameera, if you could just give on the adjacencies, please.

Ameera Shah: See, out of probably all the diagnostic chains that are in India, there's something very unique about Metropolis, which is that majority of the other guys are running all their operations through franchise network. Metropolis also does that in terms of collection centers, but we have about 500-odd collection centers that we actually own and operate ourselves. And one of the goals we've been thinking about is how do you make yourselves more and more relevant to the patient. Pathology is one part of their entire care paradigm, but they also need other services. So in line with that, we are saying how can we make these 500-odd centers more and more holistic in the experience they provide. So initially, we added basic radiology, talking about the adjacencies in all of these -- so many of these centers. Second of all, we're also experimenting with consultations and other models that we can provide to consumers as well.

So I think our idea will be how holistic care we can provide in these 500 centers. And at this point, we're just piloting, trying different models that could work. And as these work -- if they work well for us, then obviously, we'll continue to expand them. So the inorganic growth will obviously be very focused on pathology and acquiring pathology. And organically, we will then continue to foray into these adjacencies in our own centers, which are owned and operated by us.

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Rishi Mody: All right. Second, Ameera, you guided for 26% to 27% EBITDA margin over the next 2, 3 quarters. Is this pre-Ind AS 116 or post-Ind AS 116? Like does it include lease cost? Or does it exclude lease cost?

Aditya Shinde: Yes, I'll take that.

Ameera Shah: Yes, go ahead.

Aditya Shinde: Sorry. So this is blended. This is after the Ind AS lease liability has been pushed below the EBITDA margin. So that's what the guidance is. Whatever you're telling is after the Ind AS, the proper reported audited EBITDA margin guidance.

Rishi Mody: Okay. So Ameera, if I just look at the Ind AS impact, right, it's typically about 250 bps difference

that gets flown into the post EBITDA line items. And if I look at your numbers pre -COVID, right, you all were doing about 27%, 28% EBITDA margins, which included the rental expense. So ideally, if you all revert back to that, you all should be 30% plus reported EBITDA margins. So I'm just trying to understand, is there something which has changed, which is not allowing us to go to those levels? Or if you could just give some understanding there?

Ameera Shah: So firstly, pre -COVID, the numbers were not 27%, 28%, they were 26%. So that's the first thing. The second thing is that we have to remember that while Ind AS changes happened, there are also many other changes that

have happened. For example, GST came in. Now while we are GST exempt as an industry, which means we cannot collect GST from the customer, we don't get any of the reverse benefit, which means all our costs, we have a GST impact. So finally, if you look at the GST cost as a percentage of revenue, it's actually closer to 4%. That's number one. Number two, you have to remember that our materials and all the sort of chemicals that we use for testing are largely imported which are usually from the U.S., Europe, Japan, and there has been a tremendous rupee depreciation in the last 7, 8 years in the country. And that is the other aspect to it.

But still, if you look overall, our margins have remained despite the GST impact, despite the dollar fluctuation, despite that, the margins have actually remained stable and will continue to expand, primarily because of 2 things. People talk a lot about economies of scale in our industry. But what they forget is that it's not only about economies of scale, but it's about the right quality of business.

It's about making sure you're actually getting business from good quality customers who are willing to pay. And I think the metrics of our business have been very strong from that angle, and which is why we believe the unit economics are strong and it will keep growing.

Rishi Mody: Okay. I'm just trying to understand here, right? So all these dollar depreciation issues used to be there pre FY '19. I understand the volatility is higher and all of that. But over the longer term, right, do you think we'll be able to get to those levels of, say, 28%, 29% reported EBITDA margins?

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Ameera Shah: See, I mean, honestly, it's always a question of strategy for every business, right? I mean if you're asking me inherently, does the business have the capability to keep expanding margins? The answer is yes. Does Metropolis have the ability to keep moving towards 28%, 29%? Yes. But the question is about choices and what we're wanting to do. I mean, do we want really as shareholders to keep maximizing margins?

Or should we keep reinvesting back in the business of growth? So I think till the time that we believe that, look, margins are there and they can keep expanding, but we want to keep reinvesting in the business. I think we'll -- it will be safer to say that the margins will continue around 26%, 27%. They will be there inherently any time we want to pull, turn a knob and release further margin we can.

But I believe that the next few years will continue to need investments from the company more than anything because we want to be also one of the fastest -growing players from a revenue growth perspective with good quality business. So I think it's about choices, and I think it will be a good balance for us to maximize revenue growth and still maintain good margins.

Rishi Mody: All right. So 26%, 27% EBITDA margin and mid -teens growth is what I think we are comfortable with.

Surendran Chemmenkotil: Yes, that's right.

Moderator: The next question is from the line of Pranav Chawla from Antique Stockbroking.

Pranav Chawla: Congratulations on a good set of results. Ma'am, what would be our regional split between North, South, East, West for the quarter?

Surendran Chemmenkotil: Sorry, what is the regional?

Pranav Chawla: Regional mix between North, South, East, West for the quarter?

Surendran Chemmenkotil: So exact numbers on regional split, I think we will come back to you on the exact split of the region -wise, but the trends are in line with the post -- just a minute, I think I can get some number.

Pranav Chawla: Sure. And have we done -- have we added a focus city in this current quarter?

Surendran Chemmenkotil: Yes.

Pranav Chawla: Because I think that number has inched up significantly.

Surendran Chemmenkotil: Okay. Let me come to your first question.

I think our -- West is around half of our total business and then about 25%, 26% is coming from the South and the remaining is from the North and East. That's the contribution from the region -wise, if you ask me. And what you said focus cities, we will not change the focus cities. Focus cities remain the same, Pune, Bangalore, Mumbai,

Chennai and Surat, the 5 focus cities. And we are growing 10% on the focus cities in the last quarter year -on-year basis.

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Pranav Chawla: Okay. So sir, in the South region, I think our growth has been pretty muted. If I see our base numbers, we have broadly seen a flattish sort of a number. Anything particular that you would like to highlight over this muted growth?

Surendran Chemmenkotil: So in South also, we have had a couple of institutional businesses, which we have to take a call and we have to disengage with. That's one of the reasons. And going forward, we'll keep seeing South also coming and joining the party in terms of the overall growth.

Pranav Chawla: So can you highlight what is the quantum of institutional business in the base year or the quarter so that we get a clearer idea?

Surendran Chemmenkotil: I think when we started the year, it was close to around 10%. And then at the end of the quarter 2, I think we are about 7%, 8%.

Pranav Chawla: Okay. And lastly, what would be the impact from these new centers on our margins as of now?

Surendran Chemmenkotil: So I think we said about 1.2% EBITDA is what it can be rolled as a result of all the new centers and new labs, right? And we -- like we mentioned, we will -- our mega lab expansion will largely end this year by the time we end this year. And next year onwards, we'll only have a handful of labs getting added. But the center addition will continue to happen, but we largely focus on out of the 500 centers get added, roughly 400 centers gets added through the franchisee route, about 100 centers gets added our own centers.

So not too much of a dilution on the center specifically, but a little bit less. So 1.2% is the total impact of the new labs and the new centers put together. And in the next couple of years' time, this will start settling.

Pranav Chawla: So sir, if I see correctly, our major lab addition has happened in FY '22, that is around we have added 20, 25 -odd centers in FY '22 and the next big tranche was in FY '24. Has the FY '22 tranche is margin positive as of now?

Surendran Chemmenkotil: Yes. So typically, it takes about 2 years plus to get to margin positive. In year 1, normally, we find a minus 10% kind of an EBITDA in the new labs. And the year 2, we normally see it coming to a little bit positive single -digit positive. And the year 3 onwards, 30th month onwards, we start seeing coming closer to the company margins. But this also has got different split.

Geography -wise, this could be different.

And West and South, this could be much lesser time and North and East could be a little longer time. So on an average basis, what I'm telling you. So yes, '23, we added around 14 labs and '24, we added about 24 labs. And this year, so far, we added 17 labs and a little bit of lab consolidation at the same time.

Pranav Chawla: So currently, only probably our lab addition in FY '24 and '25 would be weighing on our profitability, most likely?

Surendran Chemmenkotil: So, 2 things actually get tied up because we said we want to move from 700 to 1,000 towns. So we have just created a plan that how many labs we require to service the 1,000 towns. And that

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by the end of this year, largely we would have met those objectives and may be left with maybe a few more labs or 8 to 10 more labs, which we will take up in the coming year. So by the time we exit this year, our focus on 1,000 cities

in terms of expansion and those being

served by the labs that we have will actually be done with. That's the reason we are confidently saying that our strategy on lab addition and strategy on network expansion are going together.

Moderator: Ladies and gentlemen, we will take that as the last question. I would now like to hand the conference over to the management for closing comments.

Ameera Shah: Thank you, everybody, for joining us today. It's been a pleasure to chat with you as always. We obviously feel very positive about the business. We've seen a good growth over the last many quarters. We've seen operational efficiency run through as well. And more importantly, the brand of Metropolis is getting broader, wider, not only in the existing markets in which we are strong, we are getting much deeper.

But we are also getting -- going into many new markets and creating access to patients and health care and building our brand amongst new doctors all over. We believe that a lot of the investments that we've made in the last 3, 4 years, whether it is on technology, on labs, on people, on management will continue to play through for us in the years to come.

We believe not only will Metropolis be able to accomplish good organic growth and operating leverage with it. But our M&A strategy will also play out well for us. And just to remind everyone, we have done a large amount of M&A, about 20 -odd deals in -- over the last 20 years.

And we have been able to very successfully integrate all our acquisitions and get value from them.

The key is, like we said, is to make sure that the payment date is not -- is fair. And once we have made the fair payment and valuation for these businesses, then it is really up to our team to execute and to drive a good ROI from it. So we're excited. We believe there's a long runway for growth from both these levers.

And we, as a team, will obviously continue to work very hard and be very focused in our attempt to be able to deliver the right value for the organization and to all the shareholders. Thanks, everyone, and talk to you next quarter.

Moderator: On behalf of BNP Paribas Exane Research, that concludes this conference. Thank you for joining us, and you may now disconnect your lines.

Call: Dec 2024

www.metropolisindia.com | support@metropolisindia.com

Ref: MHL/Sec&Legal/2024 -25/76 December 16, 2024

To,
BSE Limited
Scrip Code: 542650 National Stock Exchange of India Ltd.
Scrip Symbol: METROPOLIS

Dear Sir/Madam,

Sub: Transcript of investor conference call

Pursuant to Regulation 30 read with Part A of Schedule III of the SEBI (Listing Obligations and Disclosure Requirements) Regulations, 2015, please find enclosed herewith the transcript of investor conference call held on December 10, 2024. The transcript is also available on the Company's website i.e. www.metropolisindia.com

You are requested to take the above information on record.

Thanking you,
Yours faithfully,

For Metropolis Healthcare Limited

Kamlesh C Kulkarni
Head – Legal & Secretarial

Encl : A/a

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"Metropolis Healthcare Limited
Business Update Conference Call "
December 10, 2024

E&OE - This transcript is edited for factual errors. In case of discrepancy, the audio recordings uploaded on the stock exchange s on December 10, 2024 will prevail

MANAGEMENT : MS. AMEERA SHAH – CHAIRPERSON AND EXECUTIVE
DIRECTOR – METROPOLIS HEALTHCARE LIMITED
MR. SURENDRAN CHEMMENKOTIL – CHIEF
EXECUTIVE OFFICER – METROPOLIS HEALTHCARE
LIMITED
MR. AVADHUT JOSHI – CHIEF BUSINESS
DEVELOPMENT OFFICER – METROPOLIS
HEALTHCARE LIMITED
SGA, INVESTOR RELATIONS ADVISOR – METROPOLIS

HEALTHCARE LIMITED

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Moderator: Ladies and gentlemen, good morning and welcome to the Metropolis Healthcare Limited Business Update call. This conference call may contain forward-looking statements about the company which are based on the beliefs, opinions and expectations of the company as on date of this call. These statements are not the guarantees of future performance and involve risks and uncertainties that are difficult to predict.

We request participants to discuss the acquisition announced by the company and refrain from discussion on current quarter performance. As a reminder, all participant lines will remain in the listen-only mode and there will be an opportunity for you to ask questions after the presentation concludes. Should you need assistance during the conference call, please signal the operator by pressing star then zero on a touchtone telephone.

Please note that this conference is being recorded. I will now hand the conference over to your host, Ms. Ameera Shah, Chairperson and Executive Director, Metropolis Healthcare. Please go ahead, ma'am.

Ameera Shah: Good morning, everyone, and thank you for joining us today for this important business update call. I'm joined by our CEO, Surendran Chemmenkotil, Avadhut Joshi, Chief Business Development Officer and SG A our IR Advisor. Yesterday, we announced the strategic acquisition of Core Diagnostics, a significant milestone that reinforces our commitment to leadership and specialized diagnostics.

The relevant documents have been uploaded on the exchanges and our company's website, and I hope you've had the opportunity to review them. As you know, throughout our previous discussions, we have consistently emphasized our focus on exploring inorganic growth opportunities from two primary strategies. The first strategy is that we aim to acquire local pathology labs which have strong B2C brand presence in non-core geographies.

This approach would enable us to establish a foothold in that city or region and subsequently grow the business organically by leveraging Metropolis' extensive testing, distribution, and management capabilities. The second strategy is where we have been identifying targets who have built a niche business in a highly specialized testing area. While they have built expertise and trust with doctors in a specific therapeutic area, their scalability remains limited in the absence of a national footprint and distribution network.

Under the Metropolis umbrella, these businesses can scale rapidly and become more profitable, supported by geographical reach and operational efficiency. Our acquisition of Core Diagnostics aligns seamlessly with the second strategy. This move reinforces our commitment to advancing specialized diagnostics, particularly in oncology, a market which is poised for significant growth in the coming years.

The Indian oncology market is one of the fastest-growing sectors, projected to achieve a CAGR of 17.5% over the next five years. This rapid expansion is driven by rising cancer incidences,

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advancement in treatment technologies, improved healthcare access, and increased public health awareness. There are about eight labs offering specialty cancer testing facilities.

Half of them amongst national players and half of them stand-alone. Cancer testing is one of the many things they do. These players are not just focused on cancer exclusively. Most of these stand-alone, single-specialty labs are operating at subscales, as they don't have a large distribution network, and therefore are not able to get large volumes.

The total cancer diagnostics market in India is estimated to be between INR 4,000 to INR 5,000 crores, but this includes entry-level to specialized oncology testing. With this acquisition, Metropolis is well

positioned to address the increasing demand for advanced cancer testing with Core.

Core Diagnostics is renowned for its excellence in advanced cancer diagnostics, supported by robust infrastructure and a team of world-class experts. This strategic acquisition positions Metropolis to become India's leading cancer testing chain. It also paves the way for the company to become a center of excellence in cancer diagnostics, offering a comprehensive portfolio of tests under one umbrella.

Additionally, Core Diagnostics' strong presence in North and East India enhances Metropolis'

ability to connect with leading cancer specialists and hospitals in these regions, deepening our partnership and increasing our penetration into North and Eastern geographies. As cancer continues to grow very fast globally, and in India, and with approximately 1.4 million Indian cancer cases anticipated annually, all we can do is to diagnose faster and better to manage the disease. With new developments in the treatment like targeted therapy, there is a significant need for advanced diagnostic services to support effective cancer treatment.

Growing healthcare awareness and an increasing focus on early detection and prevention are further driving demands for specialized diagnostics in this domain. This acquisition is more than a growth milestone. It represents our continued dedication to scientific excellence, operational growth, and meeting the evolving needs of our healthcare ecosystem.

I'm confident that this new phase of growth will enable Metropolis to better serve patients and strengthen our collaborations with top healthcare providers. Now I'll hand over to Surendran to take you through an overview of Core and its strategic set with Metropolis.

Surendran C.: Thank you Ameera and good morning everyone on the call. Let me give you some more information about Core Diagnostics. Core Diagnostics is a specialized diagnostic chain headquartered in Delhi, NCR, with a significant presence in Northern and Eastern India.

It is recognized as India's leading specialized cancer testing lab chain, supported by some of the most cutting-edge diagnostic technologies, best on-go pathologies, and large number of cancer biobank and data analytics. With a portfolio of over 1,300 plus high-end tests, including 150 plus tests in oncogenomics, Core Diagnostics operates through six sub-specialized divisions which contributes to 85% of the total revenue. Their collection network spans over 200 plus cities, supported by NABL and a CAP Accredited central reference lab in Gurgaon, one regional reference lab in Hyderabad, and seven satellite labs.

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51% of the revenue comes from via B2C channels with 1,600 plus oncologists writing prescriptions. Approximately 15% of the business comes from pharmaceutical companies and balance from 1,200 plus B2B labs and hospitals. The top cancer doctors and hospitals of India are their customers.

Since cancer is one of the fastest growing illness segments, cancer testing is very critical for the patient and the supporting doctor in diagnosis and therapy management. It takes several years for the lab to build the trust among oncologists. Having a larger cancer test menu and very accurate reports is fundamentally important for this segment.

However, high-end oncology and oncogenomics testing remains a challenging segment, offered by a very few players in India due to significant entry barriers. This field requires specialized expertise, strong R&D capabilities in oncology, and the ability to scale operations by earning trust from oncologists is particularly challenging and takes time to build. With Core Diagnostics under our umbrella, we believe we can make these services more available and accessible in India for patients suffering from cancer.

For Metropolis, while we work with to

top oncologists and top hospitals in the West and South, offering the mid-entry level and specialized cancer testing solutions, we currently don't offer the super specialized test menu here. While building these scientific capabilities internally could have happened in two years, but building the same trust with 1,600 oncologists and 1,200 hospitals and clinics predominantly in North and East would have taken us maybe five plus years. This acquisition accelerates our growth and positions us as a leader in end-to-end oncology testing across the country and gives us a great platform in North India to make bigger inroads in specialized testing.

Aligned with our strategy to enhance capabilities and enter niche and super specialized testing domains, this acquisition is a seamless fit within Metropolis portfolio. Through the acquisition of core diagnostics, Metropolis will be able to do the following things. One, deepen our relationship with leading oncologists across the country through 100 plus specialized technical sales team.

Two, strengthen our North and East India operation through their partnership with hospitals and labs. Three, integrate advanced cancer testing and capabilities with state-of-the-art testing solutions, reinforcing our expertise in oncology which is the fastest growing area of the next many years. Four, leverage cross-selling opportunities offering Metropolis extensive test menu to Core Diagnostics hospitals and doctor customers while adopting core super specialized cancer test portfolio for Metropolis oncology clients.

And five, achieve cost synergies through consolidation of overlapping labs, procurement efficiencies, logistic integration and overhead cost reduction which will drive operational efficiencies and improve profitability. Together, Metropolis and Core Diagnostics will emerge as the largest and the most profitable oncology testing provider in India. With 85% of revenues

derived from specialized testing, this acquisition now aligns perfectly with our strategic focus. With this, now I will hand over to Avadhut for financial merits and the integration plan.

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Avadhut Joshi: Thank you Surendran. Good morning everyone. As the leading oncology testing lab in the country, Core Diagnostics has demonstrated robust performance with a revenue CAGR of around 22% over FY '22 to FY '24, an average revenue per test of around INR 2,300 and a solid patient base of 4.5 lakhs annually.

Metropolis will acquire a 100% stake in Core Diagnostics at an equity value of INR 246.8 crores representing 2.2x their FY '24 revenues or approximately 2x of current year FY '25 revenues. The company is professionally managed with its majority stake being held by institutional investors comprising Artiman Ventures and Eight Roads.

While Core Diagnostics is marginally profitable at an operating level, we anticipate significant EBITDA growth over the next 2-3 years driven by cost synergies, operational scale and the strength of Metropolis brand. For H1 FY '24, the unaudited revenue stood at INR 59 crores. The acquisition will be financed through a combination of 45% equity swap and 55% cash consideration.

Metropolis remains a zero-debt company with cash and cash equivalents of around INR 187 crores as of September 2024. We expect the acquisition to be EPS-accretive from year one that is FY '26 and ROCE and ROE-accretive from year three that is FY '28 ensuring strong returns for the stakeholders.

While the business is marginally profitable due to subscale, our internal calculations show we are acquiring it at FY '25-'26 estimated EBITDA multiples of 13.9x to 14x showing a strong IRR on the capital deployed. It would take 3-4 years to derive full cost synergies in the P&L and to bring Core Diagnostics margin equivalent to Metropolis. Because the profit

profile has not played out fully at Core Diagnostics, it gives Metropolis a very good opportunity to buy a high-quality specialized business at a very reasonable price and turn it into a valuable profitable company. We have also planned how we will integrate Core with Metropolis over the next 18 months.

The first phase will be of 6 months. To ensure a seamless transition and unlock the acquisition's full potential, we will establish a steering committee to oversee integration and track the progress. The organizational structure will be redesigned, supported by retention plans to secure key talent. Resource optimization efforts will focus on streamlining outsourced test costs, merging overlapping labs, and consolidating resources. IT integration will involve rolling out Metropolis systems, software, and providing comprehensive training to the team.

The test portfolio will be optimized by refining existing tests, while processes will be standardized to align with Metropolis quality standards. Additionally, cross-selling strategies will be launched to promote non-oncology tests and new offerings, driving revenue growth and enhancing client engagement.

The second phase of integration will span between 6 to 18 months. In the second phase, the integration plan will focus on bringing the oncology segment of both the companies together under a common brand name and aligning marketing strategies to establish a unified brand.

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identity. Full business integration will consolidate teams, back-end operations, and support functions into Metropolis systems while implementing unified performance metrics.

International business operations will be centralized to streamline management, enhance scalability, and leverage the expanded network for global outreach. Similarly, pharma alliance operations will be integrated and centralized to maximize efficiency, strengthen collaborations, and expand service offerings with key partners.

These steps will ensure complete operational alignment, enhance efficiency and position the acquired company as a fully integrated part of Metropolis ecosystem. Fortunately, Metropolis has done 24 acquisitions in our journey, and we understand the key drivers to make this acquisition successful.

That's all with me, now I request our Chairperson Ms. Ameera Shah to provide her concluding remarks. Thank you.

Ameera Shah: In conclusion, I'd like to emphasize that the strategic new position of Metropolis as the leader in advanced cancer diagnostics, one of the fastest growing diagnostic segments. It also gives us deep access to top doctors and hospitals in northern eastern India, and allows us to create large

shareholder values, so better management and scale of a sub-scale high-quality asset. By reinforcing our leadership in oncology, we're also poised to expand into future-focused areas such as genomics, further enriching our diagnostic portfolio.

Our commitment to high-quality specialized diagnostics drives our vision of shaping the future of healthcare in India. With a strong foundation for sustainable growth, Metropolis continues to solidify its position as the most trusted and respected scientific healthcare brand in the country. We are confident that this acquisition will create long-term value for our stakeholders, enhance patient outcomes, and contribute to the advancement of healthcare for years to come.

We welcome the Core team to the Metropolis family and look forward to a strong partnership.

Thank you for your time, and that's all from my side. Now we're happy to answer any questions that you may have.

Moderator: Thank you. Ladies and gentlemen, we will now begin the question-and-answer session. The first question comes from the line of Harith Ahamed from Avendus Spark. Please go ahead.

Harith Ahamed: Hi, good morning. Thanks for the opportunity.

So when I look at the profitability of Core

Diagnostics over the years, except for maybe the COVID years, it's been loss-making. But your comment on EPA's accretion in FY '26 and the transaction multiple that you mentioned of around 14x, I presume it's on a FY '26 basis, EV EBITDA.

So that implies Core Diagnostics EBITDA margins moving up to a healthy double-digit level.

So that's a significant ramp-up. So, first question is why Core Diagnostics was struggling to be profitable over the last few years, and what are some of the drivers that you've seen for such a sharp margin improvement?

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Ameera Shah: Sure. Thanks for your question. See globally, let me tell you what a trend that's happening is that there were, in the last 10-15 years, many such companies that started which were focusing on single specialty diagnostics. Oncology was one example, neurology or genomics, etc. And what all of them landed up finding was that the cost that it takes, the operating cost that it takes to do high-score customer acquisition at the hospital at the doctor level doesn't always get justified as a single specialty player.

And therefore, you're finding that across the world, single specialty players are getting acquired by larger players like a Quest or a Lab Corp in the US, and similarly, a Metropolis in India, because you're able to manage these assets, single specialty much better as part of a larger group, rather than standalone on their own, where the unit economics don't play out completely.

So, we're actually following a similar trend line by bringing in a single specialty diagnostic into a larger benchmark. And if you actually look at '23-'24, while they have been not profitable, in '24-'25, we will be seeing a low single-digit margin profitability in the book, in the operating EBITDA. And obviously, in '25-'26, we expect this to move higher.

Plus, we will add in the cost synergies that will come out of bringing these two businesses together, some of which Avadhut mentioned, like the material procurement cost, some of the overlapping lab infrastructure. There are many areas that we can potentially synergize together and with this we believe over the next 4 years, we will be able to bring the margin of core up to the MHL margin over this period of time, but starting with 2025-2026 itself, which will be the first of operations under Metropolis, it will certainly move up.

Harith Ahamed: Okay. My second question is on one of your slides. You mentioned that in super-specialized oncology testing, especially in the therapy monitoring area, core diagnostics is very strongly positioned and in fact it's stronger than Metropolis in this particular segment. So just trying to understand what exactly or if you can give some examples for some tests in therapy monitoring that will make this capability a little bit clearer for us?

Ameera Shah: Avadhut, do you want to explain a little bit about entry-level to super-specialized.

Avadhut Joshi: Yes, I can. So if we look at the overall oncology testing portfolio, we can divide that into three parts. One is entry-level tests, which are typically prescribed by the normal doctors or the physicians when you go and visit them and they are age-appropriate, for example, PSA total and so on. These are blood-based tests. And then there is a second level of testing which happens when the consultant finds something significantly wrong which are typically the tissue-based tests.

So this is a combination of the entry-level tests and then the mid-specialty or specialty tests. The super-specialty tests typically include certain genomics-based testing which is required for the targeted therapy. If we look at the overall journey of oncology treatment over the last 10 years, we have come a long way. And today, the treatment options are

primarily based on the targeted

therapy, personalized medication and making sure that there is a companionship with the

So the super-specialized segment today actually is something which we are not fully catering into, which core diagnostics has emerged over the last 7 to 8 years. And this is how we are looking at combining our ability to perform the entry-level tests, mid-specialty tests and their ability to conduct the super-specialty tests. So there is an overlap of about 30%, but the rest is completely exclusive with both the companies. I hope that answers the question.

Harith Ahamed: Yes, that's helpful. That's all from my side. Thanks for taking my questions.

Moderator: Thank you. The next question comes from the line of Kunal Shah from ANOVA Capital Management. Please go ahead.

Kunal Shan: So my first question is what specific steps are being taken to ensure the seamless integration of core team, technology and infrastructure into Metropolis?

Ameera Shah: So, I'll talk a little bit about the people and I'll ask maybe Avadhut to talk a little bit about the technology and infrastructure. See what we have done as far as the people because at the end of the day you have corporate team, you have a back-end team and a front-end team.

Obviously, the leadership team of core which currently runs the business is aligned to us closely and will be continuing to run the business under our umbrella over the next many years to come.

We have put in the long-term incentivization in programs, the kind of cultural alignment that is needed to really get the teams to work closely together. Avadhut do you want to talk a little bit about technology and infra.

Avadhut Joshi: Yes, absolutely. So what we have observed very carefully is while they have a presence in 200 cities, that is also an additional cost for them because they are currently operating at a subscale level whereas we have overlapping collection centers or the lab infrastructure which can very well be utilized when we merge both the operations together. On the people side, what we have noticed is there are overlapping teams where we will see how we can optimize the resources and then the key talent at core diagnostics will continue functioning with us on the similar platform.

Surendran C : If I can just add one or two more things. We have also created a steering committee at teams between Metropolis and core diagnostic team and then we will work together and keep looking at the progress of the integration on a regular basis on all the domain which is technology, which is products, which is people distribution etcetera. So, this will be purely backed up by the Metropolis leadership team for a better execution.

Kunal Shan: And one more question post integration, the labs and logistics they will be overlap. So, how will it be optimized going forward?

Avadhut Joshi: So, with respect to the lab infrastructure currently they have nine odd labs out of them. One of them is the national reference lab which will continue functioning and the satellite labs is where we can see overlap and we can look at merging operations. And this is where the Steer core will come into the play and they will work out the overall seamless integration over the next 6 to 8 months time.

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Kunal Shan: Okay. And after this our presence will increase in more than 10 countries, I think. And then the question is how will Metropolis plan to centralize and also expand international operations?

Avadhut Joshi: So, let me take this. So, if you look at the Metropolis current structure of international operations, it can be divided into two parts. One is where Metropolis has made

direct investments which is

in sub-Saharan African countries. We have presence in five countries through 25 odd labs. And there are around 10 countries where we are doing in sourcing of the specialty test.

So, we have franchise partners in 10 countries who send sample back to our global reference lab in Mumbai. If you look at core infrastructure, the core has exactly the similar structure, which is the franchise operated network and they import or they in source all specialty tests. So, we see a part of overlap, close to about 40% overlap between both the countries. I mean, where the client servicing is happening currently and other 60% are the completely new countries.

And the way it will happen is today, some of the test portfolios which are currently offered by core is not really being offered by us to our customers. So, there is a lot of cross-selling opportunity we can see very clearly. So, the customers what they currently have will get an opportunity to send tests which are there with Metropolis and vice versa.

Kunal Shan: Okay. Thank you.

Moderator: Thank you. The next question is from the line of Kunal Randeria from Axis Capital. Please go ahead.

Kunal Randeria: Hello, good morning. So, while I note that this oncology market is growing - oncology testing market, I'm just wondering how much of this opportunity can be tapped by companies like Core because from what I understand a lot of hospital chains are focusing on providing end-to-end Oncology services and are also investing in diagnostics. So, just want your thoughts on this?

Ameera Shah: See, even if you look at today all the top hospitals you can think of outsource the specialized pathology, not only in oncology, but across segments. Because a company like Metropolis has 4,000 varieties of tests. The most top hospital in India will have maybe 1,000 to 1,500 varieties of tests. Now, if you are a hospital provider who's competing in diagnostics, you still don't have all the facilities and all the kinds of tests that Metropolis does or Core does.

So, even though it appears as if hospitals are investing in diagnostics, what they're investing in is the top 1,000 to 1,500 varieties of tests out of the 4,000 that we already have and obviously with this acquisition, our 4,000 also goes up because we're adding now new varieties of tests to your menu. This situation and trend is not unique to India. It's global. The hospitals actually treatment are focused players and they don't have expertise in pathology and therefore they outsource it to the pathology experts like Metropolis or the equivalent in other countries for super specialized testing.

This is a trend that we don't expect to change. Even if your top one or two hospital chains start in sourcing more tests, there will always be enough tests that they have to continue to outsource because they lack the skill and they lack the technical expertise to be able to do the entire range that we do.

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Avadhut Joshi: In addition to that, what I would like to say is in the oncology treatment a lot of these tests are dependent on the therapy and these are not necessarily required when the patient is actually hospitalized. So you would see in the oncology, you have two clear streams. One is the oncology - surgery stream and then the medical oncology stream. So medical oncologists are the ones who are clearly using the therapy monitoring test, confirmatory tests and so on. And these are largely the daycare surgeries or daycare procedures or the polyclinics where they are practicing. And this is where you will see even Core diagnostics get a lot of these patients directly from the clinicians because they are operating in the OPD environment.

Kunal Randeria: Then how much of your revenue would be coming from these kind of tests?

Avadhut Joshi: So today, if you look at the core

specialty, almost 85% of their revenue is based on the oncology, which is into these segments. And look at the overall contribution, today, Metropolis contribution of the oncology is about 4% to its total revenue. With course acquisition, it will jump to around 10%.

Kunal Randeria: Sure. Second question is on the gross margins and from the data that is available, I see that despite overgaining scale in the last decade, the gross margins are somewhere around 60% for this business. So just wondering, is this the kind of margin that these businesses typically operate at?

Ameera Shah: So specialty businesses and specialty diagnostics always operate at a lower gross margin than routine tests. If you were to break up Metropolis' P&L or competing peers' P&L, you will find the same story that the specialized businesses operate at lower gross margin, but the EBITDA per sample obviously is higher because your pricing is higher as well. So we actually find that the right balance for diagnostics is to be able to have a fair amount of routine business, which gives you the high gross margin, but to have a fair amount of specialty business, which gives you the cutting edge brand, and which also gives you the high EBITDA per sample if you're able to get the good quality business, because that's when you're relevant.

If you're only a routine player, you're not relevant in the doctor's mind, because you could be one among 300,000 labs. It's the specialty diagnostics that's actually separating you from all the other peers. I'm talking about unorganized and organized. And therefore, even though you make lesser margins at the subscale level, at a scaled level, your margins can still be good on the specialized platform. And that's one of the reasons why the stand-alone single specialty diagnostics players are not really functioning well economically, partly because of the low gross margin.

Kunal Randeria: Got it. Just one more, if I can squeeze in. You did mention of some cross-selling synergies that you foresee in future. Can you talk a bit more about how you intend to do this?

Ameera Shah: Sure. So, I mean, the synergies will come from two, three places, right? So one, as Avadhut referred to, there are some stat labs, which satellite labs all across the country, where we do believe there is an overlapping infrastructure and potentially you can have a consolidation there.

The second key area would be sort of material procurement. Obviously, the prices at the Metropolis get test and some prices at which Core gets test, there is a synergy there and that

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could be certainly leveraged to be able to overall aid the common sort of combined P&L of the businesses.

Besides that, the management costs in the diagnostics business have only increased over the last 10 years. Talent has gotten more expensive. The infrastructure, IT has gotten more expensive and therefore you're finding that sub-scale players are not able to operate on a sort of clean unit operating basis on their own. So you'll also find some consolidation in some of these infrastructures and corporate costs. So overall, we believe that as sort of Core lands up becoming a very key segment within the Metropolis umbrella, there will be a fair amount of synergy at the back end that we'll be able to leverage at the operating level and as a company.

Surendran C.: I think the question was around the cross-selling opportunities, right?

Avadhut Joshi: So when we talk about cross-selling opportunities, today Metropolis works with close to 10,000 plus hospitals. And we have close to about 1,000 plus oncologists where we touch base upon. And today, some of these super specialty tests are not in our portfolio and not really being focused, right? And as we mentioned earlier, that it may take a couple of years for us to really build those capabilities.

It will take probably four to five years to build that trust. So with Core coming in, the test menu is readily available with Metropolis, which can be easily offered from day one to its existing customer base. So that's one opportunity.

The second opportunity is taking the Metropolis test menu, which typically falls at the entry level screening and allied test, confirmatory test, and then the routine follow-ups, which typically happens after the treatment of any cancer patient can also be offered along with the Core super-specialty test to its customers. Currently, as we mentioned, Core offers their test menu to close to 1,200 plus hospitals and 1,600 plus oncologists, right? So that remains a great opportunity for us to take our test menu of 4,000 tests and actually sell it to them. So this is how the cross-selling opportunity will come in play.

Geographically, the way we are looking at is North and East is extremely strong with Core Diagnostics with great brand equity with oncologists and great hospitals, whereas West and South is very strong with Metropolis. And this is how, again, the cross-sell will play.

Kunal Randeria: Sure. This is a bit of an oversimplification, but do you kind of -- will come up with, let's say, onco packages, new bundled packages for patients with this?

Ameera Shah: See, oncology works primarily through once you've obviously got cancer and you can try to bundle some of these, but frankly, each doctor has its own unique prescription. So we will try to do certain things on the screening side where consumers can make decisions on their own or try to do some for the doctor prescription. But we'll have to see whether doctors still prefer to make their own prescriptions that are customized to each patient or whether bundling can be an option.

Moderator: The next question comes from the line of Shyam Srinivasan from Goldman Sachs.

Shyam Srinivasan: Just the first one is on the CAGR growth that has been provided, right, to 22%. I just want to kind of know if you can in terms of volume and price changes, if any, right, the average revenue Metropolis Healthcare Limited

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per test seems to be 2,300. I just want to understand how these two dynamics, like price and volume have actually changed over the years, even a longer timeframe would be helpful?

Ameera Shah: See, with specialized tests, what usually happens in the portfolio is that when you launch new tests, you're obviously pricing them a little bit higher because you don't have the volume. And over time, actually, as the volume builds, we find that the pricing starts to move downward. This trend happened over the last 10 years in oncology where you'll see that actually the pricing has moved to more affordable levels because you have to remember that in India, the OPT testing, which is not in hospitalization, is still paid out of pocket and is not covered by insurance. And therefore, the more affordable and accessible we are able to make for the specialized testing, the more volumes we're able to actually build.

So traditionally, you would have seen even with Core or with other players, the volumes would have been probably about 10% to 12%. And the rest of it would have come from sort of moving up the value chain, not necessarily from an average revenue per patient, but really moving up the value chain. It's not because the price increases, but because you're selling more complex and more advanced tests. And therefore, you're able to get sort of a higher price for that particular test. So the volume of patients obviously will continue to increase at a fair pace. But more importantly, are you able to get the patient the right test for them at the right time and keeping them moving in the value chain that's required for that. Avadhut, do you want to add anything?

Shyam Srinivasan: Yes, so Ameera, I'm just trying to get the

outlook here. So essentially, you're saying price will now stabilize after it has fallen, and then we'll get volume growth, like whatever, cancer incidence is going up. So 11 %, 12%, should that be, if you were to make a projection, I'm not asking you to, but I'm just saying, how should we look to model this one out in the path forward in terms of growth?

Ameera Shah: See, like if you take the overall CAGR, I don't want to break it up at this point into volume and price, because we would also like to continue to understand the segment deeper. But if you look at the CAGR of 22% that we cited for the last few years, is actually removing COVID, adjusting for any COVID testing, and adjusting for any non-recurring revenues.

So for example, they may have got a government contract which expired, etc., So 22% has been the CAGR from a B2B and a B2C perspective, which is a pretty fair CAGR. And our goal is obviously going to be to try and uphold that, but with a better quality of earnings and better quality of business. Often what happens in the unorganized space is that without too much governance, there is a focus on growth.

Obviously, when a company like ours comes in and puts even stronger governance parameters, you may see growth moving down 1%, 2%, 3%. But you will also see, because of better management capabilities, the growth moving up. So hopefully, I think this 22% CAGR sounds like a fairly good number to us, even for the future.

Moderator: Thank you. The next question is from the line of Abdulkader Puranwala, from ICICI Securities. Please go ahead.

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Abdulkader Puranwala: Thank you for the opportunity. Just first question, if I look at the first half revenue numbers, there seems to be a slowdown as compared to the 22% CAGR what we are talking about. So could you highlight some on it that why there was, say, the first half number was just close to INR 59 crores?

Ameera Shah: Sure. See, if you look last year, the revenue was approximately INR 110 crores. But actually, if you look at one layer beneath, it also included a hospital lab management contract, which actually came to an end before we acquired this business on April 24. And actually, therefore, the base would have reduced to INR 103 crores instead of INR 110 crores.

So if you look at the growth, actually, of what you're seeing today, from INR 103 crores to preferably where it will land at INR 120 crores, INR 122 crores, if you take the run rate of each one. So the growth from INR 103 crores onwards, you can calculate which would be around the same level. And that's why we said that the CAGR 22% is adjusting for COVID and non-recurring businesses.

Abdulkader Puranwala: That's helpful. And the second one is on, I know in your PPT, you mentioned about certain ESOPs being offered to the key team members which are going to come from Core. Could you highlight what could be the incremental hit on the EBITDA and how does that pan out in the 14x EV EBITDA multiple, what you spoke about for FY26?

Ameera Shah: So that's not really being plugged into the Core piece. It will be plugged into the overall Metropolis piece. And the idea is very simple. It's to basically create long-term alignment for the Core team to Metropolis. Because if you want to get them to not only execute the synergies, revenue and cost with Metropolis team, they need to be aligned to the performance of core, but also the performance of Metropolis. So the similar kind of programs that we have done for our Metropolis leadership team has been done for the Core leadership team.

Moderator: Thank you. The next question comes from the line of Anshul Agrawal from Emkay Global. Please go ahead.

Anshul Agrawal: Hi, thank you for the opportunity. So I suspect Core Diagnostics doctor network would be more oncology focused. So how do you plan on cross

-selling our non-oncology test bouquet over

here? I understand there are cross-selling opportunities around entry-level oncology testing. But how do you plan on sort of cross-selling a routine test menu or other category test menu over here?

Avadhut Joshi: So the cross-selling will happen on two levels. One, if you see, we have mentioned clearly that they have contracts with 1200 plus hospitals and labs, correct? So those 1200 hospitals and labs are not only oncology testing hospitals and labs, but they are multi-specialty hospitals, which also cater to oncology, right?

And what we have seen is that there is barely about 5 % to 10% overlap in the client base. That means 90% of those hospitals and labs remain open for us to take all other test menu of Metropolis because they are anyway outsourcing some of these tests to some other labs, correct? The only oncology piece is actually going back to Core. So using that existing relationship,

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which is built over last 4 years to 5 years can be utilized to sell Metropolis test menu. That's, you know, one on the hospital side.

On the consultant side or on the oncology side, if you look at the patient journey, even at the detection level, you will need to do a lot of blood markers, right? Which are blood-based entry-level tests. And that portion is quite significant. And then you move to the super-specialized test segment when the patient needs the therapy. Once the therapy is over, then the patient moves to survivorship. The survivorship is nothing but the follow-up.

And during the follow-up also, then oncology tests have a sunset. And then again, the routine tests like CBCs and the total blood markers, etc. will follow through. And those tests are typically required at least twice or thrice a year. So what we have noticed in the entire life cycle of a cancer patient, the number of tests required is close to about 3 to 4 times than a normal patient requires.

Out of which, maybe one-third will be contribution from the oncology. But the rest two-third comes in from the routine test, which is the strength of Metropolis. And currently, those patients are being offered only the oncology-based test because Core currently doesn't really deal with that kind of test menu.

Anshul Agrawal: Got it. I take your point on the B2B portfolio. But in the B2C portfolio, is there a use case wherein we can sort of cross-sell non-oncology test packages or test menu to these B2C consultants?

Ameera Shah: Yes, because if you see, for example, like Avadhut said oncologist, let's say when a patient goes to them, first the patient has to get routine tests done, right, in whichever location that oncologist is. And then they get some semi-specialized and specialized tests done to figure out whether the patient first has cancer or not and is the early part of the treatment, right.

Now, Core today doesn't have that infrastructure at all in terms of number of labs across the city to be able to do that entry level and that specialized testing for the patient. And therefore, that doctor is working with some other lab for those entry level and specialty cancer tests. And Core only comes into the picture when that doctor wants to now outsource super-specialized tests. So, we could potentially leverage that relationship with the doctor to try and capture the entry level and specialized tests, because Metropolis does have labs in 200 labs all across the country, and we are present in 750 cities.

So, by doing the entire journey from entry level to super-specialized, we are able to capture all the prescriptions of the patient via the doctor or via the hospital and continue the relationship from start to finish rather than it being a broken relationship.

Moderator: Thank you. The next question comes from the line of Rishi

Mody from Marcellus Investment

Managers. Please go ahead.

Rishi Mody: So, my first question was on, if you were to build a bear case scenario for this acquisition, what could go wrong out here? Where do you see near-term headwinds? Like, let's say 50% is B2B.

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Do you see any business that you would not want to continue because of receivable management or anything?

Ameera Shah: So, while 51% is B2C, out of the balance 49%, 15% is actually from pharma support programs, where pharmaceutical companies that are providing targeted therapies are in many cases doing the subsidized testing for those patients. And the balance, 35%, is from B2B. Look at the end of the day, the risk here is really on the business execution side, where it's up to now Metropolis and Core management teams to be able to generate the synergies at hand, whether it is the cost synergies, whether it is the revenue synergy.

So, that obviously is one risk that we obviously are on top of and want to make sure that we're able to generate the kind of profits and returns that we would like to do from this deal. I think this synergy is what we're alluding to. We don't see very large because the customer bases are quite different.

But there is some price points which will be different between Metropolis and Core. And while that does not impact in the first year, when we do start to do a full integration, we may see that some of the pricing has to be tweaked to make sure that you're putting out a common pricing in the market. So, that has been already captured into our business model as we have looked at these synergies and we see these synergies of both these businesses coming together.

Rishi Modi: Any other risks or downside cases that you think can or may play out?

Ameera Shah: I mean, theoretically, there are obviously many risks at hand in any transaction you do or any business you do. But I would say these are the ones that we are the most mindful of. While we have identified all the risks, we have tried putting mitigation plans for all of them. Now, will all

of them work 100% perfectly? Never does in real life.

So, we'll have to see and try our best to ensure that all the risk mitigation plans are completely put in place and then create the buffers for the situations which are unforeseen.

Rishi Modi: Understood. On the upside, any non-linear optionalities or something which, I heard a gentleman mention that you all have been researching some new testing, but it's just started. So, if you could just give color on what's the upside which is not there in the business today but can come through.

Ameera Shah: See, I think the capabilities in the business, the R&D capabilities and Core in terms of starting new tests and tapping new segments, you know, has been reasonably strong. And we believe that with some amount of capital, with some amount of focus, with some amount of re-energizing the system, we are wondering whether there is an ability to keep sort of doing more and more R&D and sort of really staying ahead in terms of testing.

That obviously will remain unknown at this point. Only once we deep dive and we start to put some energies there, we'll come to know whether there is an option to really create a completely new test menu, not only in oncology but potentially in other areas as well that allows us to then keep expanding.

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So, I think that remains an option at this point which could be, like you said, have a very different trajectory, but it's too early to sort of have any commitment to that at this point.

Moderator: The next question comes from the line of Gagan Thareja from A SK Investment Managers.

Gagan Thareja: Sir I have just one question and that pertains to the financing arrangement for the acquisition. In

particular, what made you choose a CCPS and a cash structure rather than perhaps an all-cash or a cash-and-debt deal here?

Ameera Shah: So, we looked at a couple of things. I mean, we felt that, we had about INR 187 crores of cash at the end of September and Metropolis has been working all year on a relief funnel of potential M&A that we would ideally like to do. And, usually in our industry, when you're buying businesses from pathologists who are on the ground.

You can't really use your stock as currency as an option to do that because they are not comfortable with stock as an option and they only prefer cash. We felt this was an opportunity to use our stock as currency as well because these are private equity players on the other side. And with them, they have the understanding of how it could work.

And therefore, we thought that instead of using all our cash or debt lines at this point, let us use partly our stock as currency as well. And since we have that availability of usage in this particular case that also leaves things more open for us to do additional M&A if the right deals and targets come about and doesn't really block us from doing anything further.

So really, it's about just using all the different channels at hand. And while we are aware that obviously doing a stock trade as part of it will slightly dilute the ROE and the ROCE and that gives us the ability to recover from that equity, may not. We felt overall doing all the pros and cons using stock as currency also sets potentially a precedent for the future to say that if there are deals that come about, is there a way for us to do it?

So that was really the thought process behind it.

Gagan Thareja: Yes. And just one more question is around the profitability of the acquired entity. Well, I think someone did ask this. I would just like to perhaps try and get more of details here if possible.

You know, given their relatively more complex test menu and working in an OPD on-go sort of environment, why have the margins of the company been relatively perhaps lower or still in the last five years being a loss-making entity?

If you could break it down, under cost heads to whatever degree possible for an explanation, it would be helpful.

Ameera Shah: So I mean, if you look at sort of the things that should be diagnostic layer, usually the gross margins, as we talked about, tend to be around 55% to 60%, while margins of let's say a Metropolis or one of our peers would be really more close to between 75% to 80%, right? So one big difference there.

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Now, this tends to be the case more because of affordability issues in India, not because of anything else. So because it's not covered by insurance, you're trying to make the test more affordable for Indians, and the minute it goes over a few thousand rupees, that becomes sometimes a challenge.

And therefore, you're not able to price it as the way you're able to price routine tests, because as you're trying to build scale, you need to make it affordable for the volumes of people who need it. So that's one part of it. The second part of it, obviously, is the corporate cost, that when you're functioning at a sub-scale level, you still need the whole shrine of your professional team. Your compliances, your IT infrastructure, all of that, which obviously then becomes a heavy cost on a sub-scale player. And third, as you're trying to sort of go and do custom acquisition, like we gave you the example of an oncologist, where you're trying to go and get business from. Now, when you go to get to an oncologist and say, doctor can you please give me the super specialized, the doctor will say, Yes.

But I have to give the basic entry and this to somebody else. Why don't you start a lab? So you can do the basic test in the city, because I need the

support fast. And then you can take the super specialty to your Gurgaon lab. So you then land up starting local infrastructure, which is sub-optimal and non-productive, to do the local test there.

But you're not able to really compete with all the other players in the market. And therefore, you land up adding infrastructure costs, which is actually not needed. And it's not a shared service that could be potentially, the weight will be between Metropolis and Core in the future. So overall, the way we see it is that if this was a fully mature, profitable lab at 25%, 26% EBITDA. The value of this business would have been closer to -- between INR 700 to INR 1,000 crores. Today, you're able to get this business at a much more reasonable price, because it has not been fully matured from a profitable perspective. And we have the opportunity to create that value internally in Metropolis as a benefit for the shareholders of Metropolis.

So we believe that mostly good quality assets do not come at this price. And obviously, if it was already mature, it wouldn't. But that's the opportunity at hand for us. And we believe that we should be able to get it to the Metropolis margin in four years time.

Moderator: The next question comes from the line of Alankar Garude from Kodak Institutional Equities.

Please go ahead.

Alankar Garude: Firstly, Ameera, is it possible to share some details on the current shareholding structure of Core?

Ameera Shah: Sure, the private equity players that own it, I think 50% is owned by Artiman Ventures. How much is Eight Roads Ventures?

Avadhut Joshi: 34%. Around 34% by the Eight Roads and balance, you know, the small shareholders.

Alankar Garude: So basically, the founder of the company doesn't have any equity right now?

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Ameera Shah: Very small, if anything, maybe one or two percent. But not...

Avadhut Joshi: At least 2% to 3%.

Alankar Garude: Understood. Second question, possible to share some basic P&L balance sheet details, particularly lease rentals, net debt and gross block as of first half FY25?

Ameera Shah: If you're okay with it, we'll come back to you offline with the details.

Alankar Garude: Fine. And the final one, what is the payback period you're looking at?

Ameera Shah: So we believe that from a payback period, we think from a EBITDA perspective, it should be about six to eight years and from a PAT basis should be about nine to ten.

Moderator: Thank you. The next question comes from the line of Rishi Maheshwari from Aksa Capital.

Rishi Maheshwari: Congratulations to the team on culminating this. Ameera, I wish to understand basic tests for oncology would be PET scan, CA markers, which are in the vicinity of about INR 9,000, INR 10,000 to about INR 20,000 depending where you're doing it from. So the per unit test at about INR 2,300 that you've suggested or indicated does not necessarily resonate. So if you can bridge that gap, what are we exactly doing over here in terms of the test? So general oncology tests, as we understand, are these. So please help me understand.

Ameera Shah: So I'll add a couple of thoughts and Avadhut will add to it. A PET scan is not an only screening technique firstly. Usually people will do and that's on the radiology side, but people will usually do a sonography or you might find something to a clinical examination which you're then going to get done. On the pathology side, the early entry markers, which are the CA markers, like you rightly said, are priced closer to INR 600-INR 800 per test and not at INR 10,000, which is more for radiology on the PET scan side.

Once you have figured that, yes, you do have some sort of a tumor and some sort of a lump, then you'll get a biopsy done, which is the basic histopathology screening that happens, which is again priced at anything between

INR 400 to INR 1,000 approximately and not at higher than that. It's

only once you have figured that, yes, I have cancer and now I'm in treatment. Before that, before you're doing surgery and you need to find the targeted area, that's when you do a PET scan and

to figure out the next step.

And on the pathology side, and when you're getting treated, you have all of these tests which need to figure out from a surgery and from a treatment perspective how your body is sort of reacting to them and genetically and through molecular genetics, which is where we are finding all these super specialized tests relevant. And the price point for those will be anything between sort of INR 2,000 and INR 5,000, INR 6,000, INR 7,000 per test. So when you're saying INR 2,300, you also have to remember that there is a post-discount, which is passed on from a B2B perspective to hospitals or to labs and therefore it's a metric. Avadhut, is there anything you want to add?

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Avadhut Joshi: No, you fairly covered and that's absolutely correct. In fact, what I would like to just add here is these are not the only entry-level tests, but as the patient progresses towards more confirmatory tests, the doctors typically prescribe even more complex tests which are blood-based, right? And it could also include the common tests like HIV, hepatitis B, hepatitis C, vitamin B12, D3. So these are a set of tests which are definitely required for any patient while it goes on to the confirmatory therapy, and this is before the therapy.

During the therapy, also the patient would be required to accompany certain tests every time they go for either radiotherapy or the chemotherapy. And once the patient is into the survivorship, as I mentioned that in, again, you need to conduct at least seven or eight varieties of tests in the bundled packages, minimum twice a year. So there is ample of opportunity and just we mentioned earlier as well somewhere, the use case could be, we can look at how Core has also built over the last two years.

I mean, until two years, they were only into the super specialized segment, but when they realized that there is an increasing demand from the oncologists for certain entry-level tests, they decided to actually do a pilot and they set up six, seven routine labs. And today, almost the routine lab revenue has reached close to about 8% to 10%, which is in and around the specialty tests.

So that can be used as a kind of a use case scenario. Plus with us also, we have typically observed that when we get one patient for cancer markers and we look at the historical data of the patient, there are at least three to four X varieties of tests which are prescribed for the same patient. So we have looked at both the scenarios and actually made the plans around that.

Moderator: The next question comes from the line of Surya Narayan Patra from PhillipCapital. Please go ahead.

Surya Narayan Patra: Congrats for this transaction. My first question is about your entry into the radiology side also. See, because I wanted to have a sense running a diagnostic business targeted for oncology without radiology, how advantages or disadvantages this would be as a strategy? And in the past also you have mentioned about entering into some adjacencies, having some kind of radiology service offering. So should we consider Core is a first step towards entering into the radiology way also?

Ameera Shah: In the past, when we talked about radiology, we talked about basic radiology with the X-ray, ECG and maximum sonography. We, in fact, emphasize that we are not planning to get into advanced radiology, which is PET scans, CTs and MRIs. And that continues to be the standard at this point of time. Core is not a stepping stone into advanced radiology. Core is very much a stepping stone, like you mentioned, for specialized

ed pathology on the cancer side and for the
North Indian side.

The radiology business is not one that we favor heavily for many reasons, but primarily one we find the financial metrics on the business to be poorer than pathology. We also find the governance in the sector to be very poor. And frankly, in the last few years, we have seen an overabundance of supply when it comes to things like CTs, especially across the country when

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COVID came in. And that has actually landed up in some ways commoditizing some of the industries. And in healthcare, we have to remember that there is an expertise mindset.

So, people often ask the question, oh, hospitals have come in and they can become very good at this business. That's not how it works in healthcare because everybody has to do what they are really good at. And you try to get into somebody else's market and they are really good at it. It doesn't always work. So hospitals are good at certain specialties in treatment and they do really well at that. They may or may not do well in pathology. The same way a radiology player who

expands into pathology may not necessarily do well in pathology. And the same way a pathology expert like us going into radiology may not really succeed in that. So it's all about the expertise in each segment and trying to actually go deeper in your segment to build value.

Surya Narayan Patra: Second question is about the kind of volume share what Core is adding to Metropolis. So let's say in terms of the patient volume or the test volume, if you consider, it is low -single digit percentage of now, although it is bringing in the quality super specialty test to our portfolio. But number wise, it is really insignificant at this juncture. So how do you see this addition or the core acquisition will be influential to our overall growth, let's say, over the next two to three years?

Ameera Shah: Yes, I think acquisitions, the way we look at them are more strategic. I don't think they are really looking to go and buy revenue and even their gesture. They're looking at them more strategically as an entry point into something which we can then organically create. So if you look at these 24 acquisitions that we did in the past, we may have acquired a business worth only INR 5 crores annually at some point. But that business today is INR 100 crores in value.

And frankly, for the shareholders of Metropolis, that is a far more advantageous strategy than going in just buying businesses which are fully built out and which already have maximized their revenue and therefore adding a lot of significant revenue and profit to the mix. But you're not then getting a chance to create that on your own, which is frankly where the return on capital employed will be the highest.

So we like this model of going and buying what we may feel as not very significant pieces, but strategically give us a great platform to leap from. And then we are able to create the rest of the journey organically, which is really where, like I said, the value will come in for all of our shareholders. So today, it may not seem very large, but we believe it's a great step for the future.

Moderator: Thank you. Ladies and gentlemen, we take the last question from the line of Naman Bagrecha from IIFL Securities Limited. Please go ahead.

Naman Bagrecha: Ma'am, just a few clarifications. I'm not sure if you highlighted the cancer or oncology testing market. I heard some INR 4,500 crores, INR 5,000 crores kind of market. Is that correct?

Ameera Shah: That's right. It's about when I'm talking about pure oncology. We are talking about a INR 5,000 - crores -odd market not counting the routine tests which come along with the prescription, but just the pure oncology test. We can't compare this to the overall revenues that you see of the diagnostic players, but you have to

compare it only to the specific cancer tests that are being done.

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Naman Bagrecha: And what would be, let's say, this market expected to grow?

Ameera Shah: The CAGR for the next five years is expected to be about 70%-odd.

Moderator: Thank you. Ladies and gentlemen, this concludes our question -and-answer session. I now hand the conference over to Ms. Ameera Shah for her closing comments.

Ameera Shah: Thank you everybody for joining. It's been a very exciting start of the week for us, something we've been working on for a year. And I think it continues in line with the strategy that we highlighted. Metropolis will continue to build a specialty portfolio in a profitable way as we move forward and Core is a clear addition to that. It allows us to become the leading cancer testing facility and chain of labs in the country and really positions us to participate in a segment that is the fastest growing in the country. And also at the end of it, we take care of people who are suffering from this deadly disease.

Along with that, it also positions us very well to go deeper into the North and East of the country, really building specialty business with the top hospitals, with the top doctors in a region where we would like to increase our market share and really go deeper and closer to doctors. The financing for this and the value that will be created from the deal we feel quite strongly and comfortable with. And we believe that going forward, this acquisition will add very well to the return to shareholders. So thank you all of you for joining and look forward to chatting again.

Moderator: Thank you. On behalf of Metropolis Healthcare Limited, that concludes this conference. Thank you for joining us. You may now disconnect your lines.

Call: Feb 2025

www.metropolisindia.com | support@metropolisindia.com

Ref: MHL/Sec&Legal/2024 -25/99 February 11 , 202 5

To,
BSE Limited
Scrip Code: 542650
National Stock Exchange of India Ltd.
Scrip Symbol: METROPOLIS

Dear Sir/Madam,

Sub: Earnings call transcript for Q 3 FY25

Pursuant to Regulation 30 read with Schedule III of the SEBI (Listing Obligations and Disclosure Requirements) Regulations, 2015, please find enclosed herewith the transcript of Q 3 FY25 earnings conference call held on Wednesday, February 05, 202 5. The transcript is also available on the Company 's website i.e. www.metropolisindia.com

You are requested to take the above information on record.

Thanking you,
Yours faithfully,

For Metropolis Healthcare Limited

Kamlesh C Kulkarni
Head – Legal & Secretarial

Encl: A/a

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"Metropolis Healthcare Limited
Q3 FY '25 Earnings Conference Call "
February 05, 202 5

E&OE - This transcript is edited for factual errors. In case of discrepancy, the audio recordings uploaded on the stock exchange s on February 05, 2025 will prevail .

MANAGEMENT : MS. AMEERA SHAH – CHAIRPERSON AND EXECUTIVE DIRECTOR –

METROPOLIS HEALTHCARE LIMITED
MR. SURENDRAN CHEMMENKOTIL – CHIEF EXECUTIVE OFFICER –
METROPOLIS HEALTHCARE LIMITED
MR. ADITYA SHINDE – INTERIM CHIEF FINANCIAL OFFICER–
METROPOLIS HEALTHCARE LIMITED

MODERATOR : MR. RAHUL JEEWANI – IIFL CAPITAL

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Moderator: Ladies and gentlemen, good day, and welcome to Metropolis Healthcare Q3 FY '25 Earnings Conference Call hosted by IIFL Capital. As a reminder, all participant lines will be in listen-only mode, and there will be an opportunity for you to ask questions after the presentation concludes. Should you need assistance during the conference call, please signal an operator by pressing star, then zero on your touchtone phone. Please note that this conference is being recorded.

I now hand the conference over to Mr. Rahul Jeewani from IIFL Capital. Thank you, and over to you, sir.

Rahul Jeewani : Yes. Hi. Good morning, everyone. I welcome you all to the third quarter earnings conference call of Metropolis Healthcare being hosted by IIFL Capital. From Metropolis, we have with us today Ms. Ameera Shah, Chairperson and Executive Director; Mr. Surendran Chemmenkotil, Chief Executive Officer; and Mr. Aditya Shinde, Interim CFO. Over to you, ma'am, for your opening comments.

Ameera Shah: Thank you, and good morning, everyone. We have uploaded our updated results documents on the exchange and company's website, and I hope everyone has had an opportunity to go through the same. Industry consolidation continues to be a key growth driver, and Metropolis is well positioned to capitalize on evolving landscape. Our recent acquisition of Core Diagnostics is a major milestone, reinforcing our leadership in high-end specialty diagnostics and strengthening our commitment to innovation in advanced testing.

To ensure a smooth integration process, we have set up a steering committee that will work closely with teams from both organizations to maximize synergies and optimize operations. This committee will focus on restructuring the organization while retaining key talent, driving synergies to improve operational efficiencies, integrating IT systems and processes to streamline workflows, and identifying cross-selling opportunities to expand our market presence.

We aim to finalize the acquisition by the end of this month with Core expected to contribute to MHL's revenue starting in March of this financial year. Once Core is fully integrated into Metropolis systems and processes, we anticipate a fair number of operational efficiencies with synergies driving an expansion in margin profile of Core from Q1 FY '26 itself.

Additionally, we are focused on expanding Core's capabilities and enhancing our test offerings across the network, which will provide opportunities to strengthen our specialty testing portfolio. With our proven track record of successful acquisitions and integrations, we are confident that this move will create substantial long-term value for all stakeholders.

We are actively pursuing additional inorganic growth opportunities with several potential targets currently under consideration, especially in northern markets. We are optimistic about achieving early closure on these opportunities. Our strategy is focused on acquiring local labs that have strong brand recognition and deep consumer connections within their regions.

Many of these companies are market leaders at the local city level, but face challenges in scaling due to limited resources. Once integrated into the Metropolis network, we will partner with these

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local stakeholders to expand their test offerings, improve service quality, shorten turnaround times and drive greater market penetration.

We are in advanced stages of evaluating a few such acquisitions that have successfully built strong B2C brands in North Indian cities. These align well with our goal of expanding our footprint in the region through acquisitions focused on creating a string of pearls that enhance our presence and capability.

While Core Diagnostics remains a specialty-focused

business, bringing scale, the other

acquisitions we are considering are city-leading B2C businesses in North India with high EBITDA, high ROCE and strong OCF to EBITDA ratios. We see significant value in these acquisitions, not just from a geographical expansion perspective, but also because we are

confident that leveraging Metropolis' brand leadership and capabilities will enable us to scale their operations and drive sustainable growth.

While we anticipate Core revenue growth to be strong and margins of Core to move up fairly in year 1 of acquisition, the margin profile will be lower than Metropolis for the first 12 months and will dilute the margin temporarily. It will take up to 2 to 3 years to reach the margin -- Metropolis margin profile. However, it will be EPS accretive from year 1 and the ROI on the Core acquisition should be strong considering the above numbers mentioned.

The second area we continue to focus on is strengthening our specialty diagnostics leadership in the country. Metropolis continues to lead in specialty diagnostics and now with the new focus on genomics, molecular diagnostics and high-end oncology testing, which will be built on the back of existing Metropolis and new core capabilities, we are continuously investing to strengthen our specialty segment by bringing in more senior expertise, expanding product lines and fostering deep collaboration, and advancing next-gen sequencing capabilities to drive scientific progress.

Additionally, we are creating centers of excellence and launching multiple industry-first specialized tests in oncology, genomics and molecular diagnostics. While it's easy to add machines, the secret sauce is really the therapeutic expertise a lab has that becomes a differentiator for doctors to decide to recommend one lab over the other, which is what Core Diagnostics has with oncology. With the integration of Core, we aim to further enhance our oncology portfolio and expand our molecular diagnostics offering, ensuring that Metropolis remains at the forefront of precision diagnostics.

We are also focusing on strengthening our management team and Board, and we'll be making a few additions and changes to add the right skill sets at both levels. Our new CFO, Sameer Patel, will be joining February 10, as already announced. His background is audit services at Deloitte and then business and finance roles in retail chains. His last role being 8 years at a large public listed QSR chain, where he has been CFO for many of those 8 years.

Looking forward at the business performance, Q3 has historically been a challenging quarter across the industry, especially in markets where Metropolis has a significant presence in West and South India. This trend is primarily influenced by festival and holiday season, which impacts Metropolis Healthcare Limited

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patient footfall and diagnostic volume. While revenue growth for Metropolis in Q3 has been similar to peers, we are surprised by the low volume of acute patients. When we try to understand if this is a competitive or seasonal issue, our understanding from the ground and data points of the pharma industry reflects that in Q3, chronic diseases did quite well, but acute illnesses were lower.

Since Metropolis business is majority acute in nature, therefore, the volume impact in Q3 has been surprising for us. However, we consider this a temporary seasonal fluctuation, and early indicators in Q4 suggest a rebound in volumes as we saw in H1. We remain on track to achieve our guidance of approximately 13% revenue growth in Q4 and EBITDA margins in the range of 25% plus for Q4 '25 and going forward, reaffirming the strength of our strategy and execution. Looking ahead, our key areas of focus will be: number one, accelerating the inorganic growth with a strong pipeline of acquisition opportunities, as mentioned; second, expanding our

Specialty diagnostics segment, particularly in genomics and molecular oncology; third, implementing region-specific micro marketing strategies to optimize offerings and enhance productivity. We will also be entering health care verticals such as basic radiology, vital checkups and consultation, Suren will talk a little bit more about this, and enhancing our customer service and analytics.

With this, I would like to hand over the call to Suren to speak about the strategies in detail the key operational and financial performance. Thank you so much. Suren, over to you.

Surendran Chemmenkotil: Thank you, Ameera, and good morning, everyone. Let me begin by sharing key highlights of our quarter 3 performance. In quarter 3, we delivered a strong 11% year-on-year revenue growth, largely driven by 4.1% increase in the patient volumes. The remaining 6.5% of the growth came from strategic adjustments in our product mix and pricing. Excluding institutional business, which is in our focus areas, as I've mentioned in the past, our revenues grew by approximately 13% year-on-year for quarter 3.

For the 9 months ended in December, revenue increased by 12.5% with EBITDA growth surpassing revenue, driven by the advantages of high operating leverage. Our ongoing investments in strategic areas are delivering results, contributing meaningfully to both EBITDA and PAT. Our B2C portfolio continues to gain strong momentum, achieving 8 consecutive quarters of steady growth. In quarter 3, the B2C revenue grew by 15% year-on-year with TruHealth and Specialty testing leading the charge, growing 25% and 20%, respectively. The B2C segment now accounts to 55% of total revenue, up from 53% in the same quarter last

year, reflecting 200 basis point improvement. Maharashtra continues to be a key market for us with B2C segment in the region growing 16% year -on-year. These figures highlight the strength of our brand presence and the loyalty of our customers.

We have seen strong double -digit growth from our existing centers and high single -digit contribution from our newly established centers. As these new centers mature, we expect to see a higher revenue and volume, further boosting the overall contribution and profitability of our B2B segment.

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A major contributor to our B2C growth has been our robust clinician engagement strategy. By reinforcing scientific credibility and trust in our offerings, we continue to strengthen our partnerships with the medical community, ensuring that Metropolis remains the preferred diagnostic partner for both specialized and routine testing. Our B2B revenues have increased by 10% for quarter 3 on year -on-year basis. We are focused on deepening engagement with top -tier clients while streamlining the long tail, ensuring improved profitability and efficiency.

As a part of this effort, we have launched a dedicated partner portal, which provides a comprehensive, transparent and efficient digital platform for our B2B partners. This portal enables real -time sample tracking, offering end -to-end visibility across the diagnostic process, along with improving communication and streamlining workflows. By leveraging technology to enhance transparency, operational efficiency and partner engagement, we are strengthening our B2B relationships, improving client retention and increasing our wallet share with key partners. To address the market challenges and opportunities, we have taken a significant step to customize our approach based on the unique needs of different geographies. To optimize market presence and drive growth, we have introduced the following micro marketing strategies, which will help us grow our market share in Tier 2, 3 and 4 towns.

One, customized market relevant price rationalization strategies to enhance competitiveness; second, revamp partner model and empower partn

ers with great autonomy and resources to

increase partner engagement and productivity; three, efficiency improvements, enhance logistic operations and reduce turnaround time to boost service levels; and fourth, market -specific test menus, expanded offerings tailored to the unique demands of each region.

We have launched a series of customer -centric initiatives as well designed to elevate the overall experience, including industry -first offering like integrated testing packages, home -based ECG services, vital parameter checks at our centers, doctor consultation, enabling a seamless end -to-end health care journey. MedAdvisor, our AI -powered recommendation engine that leverages historical data and test combination to improve the accuracy of cross -selling and upselling. Infrastructure improvements in our centers, enhancing the brand experience and reinforcing customer trust.

While most of our peers use franchisee networks as their collection centers, Metropolis gets a large part of the B2C revenue from the centers which are owned and managed by the company. This allows for better customer experience and better influence over the customer choices, demonstrated by high customer NPS scores of 86%, and higher revenue per patient compared to the rest of the industry. While this adds more fixed costs in the short run on the P&L, but as centers mature, it allows more operating leverage and better profitability.

The above micro marketing and customer -focused strategies have already delivered positive results, reflected in a consistent footfall growth every quarter. The net impact of this strategy is expected to deliver a positive 2% increase in overall revenues starting quarter 4, setting the stage for sustainable growth and market leadership.

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We have also made significant strides in the final phase of our aggressive network expansion strategy, adding 7 new labs and 125 centers this quarter, bringing our presence to over 750 towns across the country now. With this expanding footprint and continued maturation of our new centers, we anticipate higher throughput, driving further volumes and revenue growth in the coming quarters.

Beyond our core markets, we see major growth opportunities in Tier 3 and Tier 4 cities, where demand for organized diagnostic services is increasing rapidly. Currently, these regions are dominated by stand -alone centers and unorganized players, creating a significant opportunity for Metropolis to introduce high -quality structured services. With our final lap of lab expansion in quarter 4, it will end our lab expansion phase from financial year '25, '26. The focus will be more on accelerated collection centers and allowing gradual EBITDA and margin accretion in the year '26.

We have consistently invested in technology upgrades and digital transformation, and our efforts continue to enhance both operational efficiency and customer experience. One of our most exciting developments in this is the launch of our new mobile app, which now includes personalized health dashboards, offering users insights based on their test history and trends, real-time sample tracking, allowing customers to monitor their sample journey from the collection to result, simplified appointment booking, making scheduling easier and more intuitive.

AI-driven recommendations powered by our advanced recommendation engine will suggest next best action tests based on diagnostic history, enhanced security features, ensuring safe access to test results and easy sharing with health care providers.

Beyond the app, quarter 4 will see significant acceleration in other key technology platforms, including laboratory information management system, enhancements to improve accuracy and efficiency, website upgrades for smoother user experience, advanced AI-powered recommendation engine, further

refining test suggestions for personalized health care. Our commitment to leveraging cutting-edge technology will not only enhance patient engagement but also drive higher operational efficiencies, enabling us to scale faster and provide an industry-leading health care experience.

So looking ahead, as we move forward, our focus will remain on accelerating TruHealth portfolio by integrating value-added services like home-based ECGs, health consultations and AI-driven test recommendations, expanding our network presence with a focus on high-growth Tier 3, Tier 4 markets, scaling digital insights to personalize patient experience and optimize clinician engagement.

I now hand over the call to Aditya, who will take you through the financial highlights. Thank you, and over to you, Aditya.

Aditya Shinde: : Yes. Thank you, Suren. Good morning, everyone. Let me share some of the key financial performance for the quarter and 9 months ended FY '25. Revenue for Q3 stood at INR 323 crores, a growth of 10.9% Y-o-Y with 4% patient volume growth. Our test volume growth stood

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at 6% for the quarter. Revenue per test has increased by 4% on a Y-o-Y basis. In Q3, our B2C revenues stood at INR 178 crores as compared to INR 155 crores in the same quarter last year. That is an increase of 15% Y-o-Y. Our B2C revenues for Wellness grew by 25% and Specialty grew by 20% on a Y-o-Y basis. Patient volume growth for B2C business stood at 5% and test volume growth for B2C stood at 5% on a Y-o-Y basis as well.

Our B2B revenues grew by 10% for the quarter compared to the same quarter last year with B2B Wellness growing by 21% and Specialty growing by 9%. B2B patient volume grew by 4% and test volume grew by 7% Y-o-Y. Revenue share of TruHealth stands at 17%, that is a growth of 25% Y-o-Y for the quarter. Our Specialty segment revenue contribution stood at 37% with a growth of 13% Y-o-Y for the quarter.

Reported EBITDA for the quarter stood at INR 71.6 crores as compared to INR 65.5 crores. That is a growth of 9.4%. Reported EBITDA margin for the quarter stood at 22.2%. PAT for the quarter stood at INR 31.5 crores, that is a growth of 15% Y-o-Y. PAT margin for the quarter stood at 9.8%. Revenue for 9 months grew by 12.5% with patient volume growth of 6% and test volume growth of 8% on a Y-o-Y basis. Reported EBITDA for 9 months stood at INR 241.3 crores at 24.5% margin, a growth of 17.8% Y-o-Y and a growth of 110 basis points Y-o-Y. Reported PAT for 9 months stood at INR 116.3 crores, indicating a growth of 26.5% and a PAT margin of 11.8%. On to the balance sheet. We have a net cash surplus of INR 205 crores as of 31st of December 2024. We also have a working capital cycle of 13 days. We will be closing the Core Diagnostics transaction by February end and MSL March consolidated numbers should include Core Diagnostics if all goes according to the timeline planned. We will be using approximately INR 135 crores of internal cash for this acquisition, and the balance will be a stock swap.

That's all from my side. With this, I open the floor for questions and answers.

Moderator: Thank you very much. The first question is from the line of Prakash Kapadia from Avendus Spark PMS. Please go ahead.

Prakash Kapadia: Two questions from my end. You were contemplating price increases. Could you give us some color? Is it across the board? Is it specific geographies, routine tests, specialized tests? That's the first question. And if you could also highlight some pricing discount competition, what is happening in the industry? How are smaller players doing? And from here on, what kind of volume growth we expect?

Surendran Chemmenkotil: All right, on the pricing, let me just clarify a few things. I just talked about the micro marketing strategy that we have rolled out already for the last 2, 3 months

ow. What it means is we just

look at the -- we adjust the prices according to each of the markets. In the last 2, 3 months, what we have done is, we looked at the pricing versus the competition in each of the markets. And wherever there's an opportunity for us to increase the prices, we have taken the prices a little up. And wherever we have the need for us to correct the prices downwards, we have done that. So I mean, the net results of this would be upside of about 2%, which is coming in from this quarter. So we did market-specific pricing, not necessarily product specific across the board. So only a

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few markets, prices have gone up, and a few markets, we corrected the prices downwards. The net impact, like I said, is about 2%.

Coming back to your second question on the discounts. So on the B2B segment, we definitely see continued efforts by some of the players to bring down the discounts, particularly on the tail end of the B2B segment, who really keep pushing the operators for the price discounts. But as far as Metropolis is concerned, we just stayed put with our discounting structure. We have not really increased any discounting to the B2B clients in the last 2 to 3 quarters at least now or maybe more.

Prakash Kapadia: Okay. And what should be the time line for completion of merger of Core Diagnostics?

Surendran Chemmenkotil: Well, I think we are almost in the final stages of the merger. I think maybe another month or a bit more than that, I think 4 to 6 weeks' time.

Moderator: The next question is from the line of Aniket Kulkarni from BMSPL Capital.

Aniket Kulkarni: So since we are actively looking at multiple acquisition opportunities, could you please let us know how much importance you pay to the acquisition being EPS and ROCE accretive to our overall financials, right? So because a lot of companies do acquisitions which compress their ROE, ROCEs for many years to come. So how do we plan to protect ourselves from this situation?

Ameera Shah: So whatever acquisitions we are doing, for example, even Core Diagnostics, will be EPS accretive from year 1. And the reason for this is because Core Diagnostics on its own did not make any significant profit. And obviously, under the Metropolis umbrella, there's a different plan, not only to ramp up the revenue, but to also create operational synergies and, therefore, over time, bring it to a similar Metropolis margin profile.

But obviously, this doesn't happen overnight in a services business, and it does take 2 to 3 years for this to get there. So we do believe that in the first year itself, it will be EPS accretive. And overall, the ROI on this investment will be good, because we have paid a very fair price for it, and it is not an overvalued acquisition.

As far as the other ones which I mentioned about that we are evaluating and in advanced stages, these are independent labs, which are more bolt-on acquisitions. These are not chains, where they are the leaders in their markets or in their city, but they are all independent stand-alone centers with maybe a few collection centers attached to it.

And this is the model that Metropolis has done many times where we obviously buy majority in these businesses, or in some cases, 100%, but usually majority. And the promoter pathologist continues to own a minority in the business and operate it with us over some years to come.

These obviously get purchased at a lower multiple of EBITDA than obviously where the public companies are trading at and tend to be high B2C businesses, which means high sort of operating cash flows to EBITDA, high ROCE and high EBITDA margins. And what we do, why we buy these is because these labs, which are stand-alone, while they are successful in their own cities,

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they're

only able to offer a limited test menu of maybe 300, 400 varieties of tests and cater to certain kinds of doctors.

And when they come under the umbrella of Metropolis, we're able to create a larger base and a bigger scale for these particular independent labs, and it becomes a win-win for both parties. So we are hopeful that some of the talks that we are in will result into concluding transactions in the near term.

Aniket Kulkarni: Got it. Got it. And secondly, so in FY '25, we should approximately be at about FY '22 EBITDA levels on a whole. So the company has now reached the EBITDA level similar to what the company was talking during the COVID peak. So just wanted to understand where do we go from here? And in the next 3 years, where do you envision Metropolis to be in terms of revenue and operating profit? And how do you see the competition panning out also over the next 3 years?

Ameera Shah: See, obviously, COVID was a black swan event, I think for many businesses. For some businesses, obviously, revenue and EBITDA completely crashed. For some businesses, it went

artificially high, like it did for health care at the time. So very difficult to keep comparing it to that base. But if you look at our CAGR over the years, obviously, the company has continued to perform well, if you look at it over a 5 -, 6-year period or even a 10 -year period. I think going forward, as we've mentioned, we have a very positive outlook for continuing to outpace the peers as far as revenue growth and also continue to expand margins.

The last 4 years, as we knew that we were creating more profits, at least during the COVID period we were creating more profits in the business, we decided to use it as a time that we could actually reinvest. And therefore, the last 4 years, we've added almost 90 labs into the system, which was actually almost increasing our lab capacity by 50%, which was a very significant leap forward in terms of lab capacity.

Now as we have said that by the end of FY '24, '25, we will be at the end of this accelerated lab expansion project. And from '25, '26, it will be just a normal lab expansion, which will be few labs a year, probably 5 or 6, but not more significant than that. And that will allow us, therefore, our margin expansion to continue.

Moderator: Mr. Aniket, does that answer your question?

Aniket Kulkarni: Yes, yes.

Moderator: The next question is from the line of Kunal from Axis Capital. Please go ahead.

Kunal: Firstly, can you help me understand this micro market strategy that you spoke of in the presentation? I'm still not completely understanding what this is and whether it's a one-off kind of a growth bump or you expect it to continue in the outer years?

Surendran Chemmenkotil: Micro -marketing strategy has basically 4 objectives, right, basically. So what we have done is all the markets in the country, we have divided to 100, 120 markets. And we looked at 4 specific aspects on each of these markets. So one is what are the prices in these markets versus the Metropolis Healthcare Limited

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competition? And if there's an opportunity for us to correct upwards or downwards, do it. So that's one aspect. And second one is we just revamped our partner model in all these markets to ensure that the partners are more empowered. They have more autonomy. They can work with some or the other clinicians, etcetera, in the market. That's the second one that we have done. Third one, we looked at the logistics arrangement in each of these towns to make sure that we can provide a much better turnaround time. So every market will have different strategy required for ensuring a better turnaround time. And the fourth one is we looked at the specific test menus in each of these markets. So different parts of the country would require different type of test menu so that they could serve their market better.

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So I mean, each of these markets, very carefully, we have just looked at it and whatever correction needs to be done, we started initiating those corrections. So which is, like I said, with respect to price, with respect to distribution, with respect to logistics, with respect to test menu. Now we looked at each of these markets separately and we've done the required course correction, or we are in the process of completing the course correction. So as a result of all this, we will be able to gain a couple of percentage more revenues going forward.

Kunal: I'm sure this must be something that you would have been doing over the years also, right? So I'm just wondering what is driving that additional growth now?

Surendran Chemmenkotil: So, let's say, if I were to take an example, for example, wellness packages of, let's say, INR 5,000, may work in Mumbai, but may not work in a small town in U.P., for example. So we have tailored a product which is suiting that particular market when it comes to wellness, okay? And the price for a particular test in that market may not be as our standard Metropolis pricing may be slightly on the higher side in that particular market, and hence, we are not able to drive volumes.

So we just looked at all the operators in that market, and we adjusted the prices accordingly. And some of the markets, let's say, Mumbai or Pune or parts of Maharashtra, where we have much bigger strength, we have an opportunity to take the prices up, right? So we have taken the prices up in the market. So a combination of all these things put together, the net result is an upside on the overall revenue performance.

Kunal: Right. Got it. Second question is on TruHealth. Now how much of this revenue is coming from which channel? Is it largely B2C?

Surendran Chemmenkotil: Well, I think most part of it is coming from the B2C channel. When I say B2C channel, one, of course, you have a digital channel, and then second, you have the brick-and-mortar channel. Both put together is where majority of this B2C revenue is coming from. And then now even the B2B markets, B2B partners also started participating in the TruHealth journey.

So I mean, we start seeing acceleration. You would have seen the number that we just called out, around 20% increase in the TruHealth revenue from the B2B partners as well. So largely contributed by B2C, but I think B2B also started participating in this journey.

Kunal: Sure. And how would the realization be different for the same test across both channels?

Surendran Chemmenkotil: As far as the TruHealth is concerned, I mean, of course, B2C channel will have a relatively better realization. And B2B channel, we operate on a discount basis, right? So I mean, B2B channels will have relatively lesser realization.

Kunal: No. I mean I get that gist qualitatively, but from a quantitative perspective, is it about 15%, 20% lower?

Surendran Chemmenkotil: No, no, not 15%, 20%, maybe 3% to 4% difference.

Kunal: Right. Got it. That's very helpful. And lastly, I believe in your opening remarks, you called out the growth in Maharashtra. Can you specify what your growth in Mumbai was in Q4 and your expectation for the next 2 to 3 years?

Surendran Chemmenkotil: Q3, Mumbai was also 16% growth we had.

Kunal: 16%. Okay. And how do you expect this Mumbai region, MMR region to continue for the next couple of years?

Surendran Chemmenkotil: Yes. I think we are putting all our efforts in terms of further expanding the distribution even in Mumbai. Like I said in some of the earlier calls, our endeavor is to make sure in Mumbai, maybe every 2, 2.5 kilometers, you find a Metropolis collection center. And also, we find opportunities in expanding the peripheries of Mumbai, where the core Mumbai would have been largely down the distribution, but there is a lot of opportunity sitting in the peripheries of Mumbai. So Mumbai will continue to be our focus markets and we'll keep making these investments and keep growing in Mumbai.

Kunal: So just to kind of finish this point, then if you want to, let's say, open a Metropolis every 2.5 kilometers, how many pickup points or collection centers or labs do you intend to kind of go to in the next few years?

Surendran Chemmenkotil: I mean labs, of course, we don't require additional labs in Mumbai. We have 7 labs in Mumbai, and that is good enough to serve the geographies of Mumbai. And we have close to 500 collection centers in Mumbai. So every year, we keep adding around 30, 40, 50 collection centers. And I think that trajectory should continue maybe in the next couple of years as well.

Moderator: The next question is from the line of Nikhil Mathur from HDFC Mutual Fund.

Nikhil Mathur: My first question is on the collection centers to lab count ratio. If I look at 9 months, we had around 22 collection centers per lab, and the number was quite similar in FY '20 as well at around 21, 22. So just wanted to understand, over the course of the next 3, 4 years, do you expect the collection center to lab count ratio to go up meaningfully? And if that were to materialize, would that drive the margin improvement going forward? Would that be the only major driver for margin improvement?

Surendran Chemmenkotil: Yes. Thanks, Nikhil, for asking this. So you're absolutely right. So in the last 2, 3 years, what we have seen is, we were accelerating on both the counts. Number of labs are also going up, number of collection centers are also going up. And as a result, the ratio between the collection

centers and the lab remain largely around the range of 20%. Now the larger expansion of the labs is almost over.

We are almost good to serve about top 1,000 towns in the country. So now going forward, in the next couple of years, our focus will be to increase the number of collection centers, not so many labs. You may require 1 or 2 labs here and there, but not many labs, as we did in the last couple of years. So the next 2 years, we'll definitely see the ratio getting better. Maybe at the end of 2 years from now, this could be 30 collection centers for a lab. So that's actually the endeavor. That's the plan that we have in mind.

Nikhil Mathur: And can you quantify some bit of drag on margins, let's say, 9 months from the increase of lab infra that we have done in the last 12, 15 months or so?

Surendran Chemmenkotil: Yes. So I think the standard what we communicated in the past as well, the first year of the lab launch, we normally have a negative single-digit EBITDA. And at the end of year 2, we get into about double-digit positive EBITDA. And about 2.5 years to 3 years, we get normal to the company levels. Now this is at an overall company level. But if you look at market-wise, it could be different.

In a strong market like Maharashtra, Central, South, et cetera, the breakeven normally happens even at 2 years. I mean, the company-wide EBITDA happens even in 2 years. But some markets in East and North, we may take a longer period of time, maybe 3 years. So that's the split of it, but largely, at a company level, in 2 years, we get into the double-digit EBITDA, and about 2.5 to 3 years, definitely get into the company level.

Nikhil Mathur: Okay. The second question is on the growth split across different class of cities. So focus growth

ex of institutional business is at around 12%. And B2C Maharashtra is at around 16%. So I suspect that a predominant part of Maharashtra would be part of focus. So where is the growth drag coming from? I mean, is it within Maharashtra cluster or beyond Maharashtra? So if you can help us understand within the focus cities, where is the growth drag coming from?

Ameera Shah: If

you look at the B2C growth, it's about 15%, like we said in Q3, and B2B growth is almost 11%. So if you were to just take these 2, we would have been closer to 13%, right? But the drag has obviously come, like we said, from the institutional business, which is not a priority for us. Now within this B2C growth of different markets, like we said, Maharashtra, Gujarat, some markets in South are doing well for us and growing fast.

And obviously, we don't do much of B2C business in the North and very little in the East as well. The business there is more B2B or institutional. So I think these are the markets where we would try to increase the B2C, in the North and the East.

And also once we have these institutional things behind us, which was the loss of some government contracts, going into FY '25, '26, hopefully, the B2C and B2B growth will be really the real growth of the business with no drag from the institutional business. So we are finding generally, if you look at the focus markets out of Maharashtra, Bombay and Pune are the only 2 cities which are included in the focus markets. Everything else in Maharashtra is either in seeding or in others.

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Same way every state is broken up into these 3 sort of categories, where you're finding other markets, which is obviously the smaller markets where we have created the new labs are obviously the highest growth, because they are the smallest base, and that's where we are making the investment. But you would also see the focus markets actually have done quite well. It's only the seeding markets which we actually believe that need to be improved and that growth can actually move up.

The reason the seeding markets are not performing as well is because these are markets like where we don't have a large amount of B2C, and the B2B discounting or the B2B price or the B2B competition is impacting some of these markets for the last few years. So we believe that's where, by building better infrastructure, by building deeper networks, better execution, we believe we can pull that up as well. Surendran, do you want to add anything?

Surendran Chemmenkotil: No, that's it actually. Just to clarify to you, the focus cities are just 5 cities, Pune, Chennai, Mumbai, Surat and Bangalore. It's not all the towns in Maharashtra. So rest of the details actually Ameera has already shared.

Nikhil Mathur: Understood. Another question I had was on, obviously, great headway has been made in the last 4, 5 years in terms of increasing the share of B2C in the mix. So currently, the company is at 55%. Over the course of next 3 years, where do you think this number can settle at? Can it cross 60%, settle at around 65%, 66%. So any particular range will be quite helpful, if you can give?

Surendran Chemmenkotil: Yes. So sometime back, I was talking to you telling that the next 2 years is the time for us to further expand the centers, right? So you will find a lot of work happening on that and further accelerated pace. And a lot of it has been specifically concentrated on the B2C markets as well. So we should definitely get into around 8% to 10% upside in the next 2 to 3 years' time on the B2C segment.

Nikhil Mathur: So that would mean your gross margin profile will further improve from 80%. Obviously, some headwinds can be there from dollar appreciation, but notwithstanding that, can the gross margin continue to improve over the next 3, 4 years?

Surendran Chemmenkotil: Ideally yes. Yes.

Ameera Shah: The only clarification I want to give there for a little bit more nuance is that the B2C is of 2 varieties. One is obviously the B2C which you do for routine tests, where you are getting self-referrals, you're doing wellness, all of that, which obviously gross margin increases. But we also build the B2C Specialty business, which is the specialized doctors who are referring

patients to

directly walk into our centers, where you're getting a mix of routine tests as well as specialized tests.

And those specialized tests are very critical for a business, because that's really what creates the stickiness, it creates the brand value, it creates the differentiation in the market. But obviously, the gross margin of specialized tests is slightly lower than it is of routine tests. So we will hope to see a gross margin improvement, but just wanted to give that little nuance.

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Surendran Chemmenkotil: Yes. And largely on the organic side of the business. And when we get inorganic acquisition happening, at a group level, it all will be a mix of what you bring on to the side, yes.

Moderator: The next question is from the line of Shyam Srinivasan from Goldman Sachs.

Shyam Srinivasan: Just the first one on the EBITDA margins for the quarter, right, 22.2%. I remember last year quarter, we had the PDD. I think we also had 120 bps of network expansion cost. So I think the number that was quoted in Q3 presentation last year was 24.5%, excluding these 2 things, right? So from 24.5% to 22.2%, if you could explain. I know there could be -- so is there an additional layer of network expansion on top of the 120 bps already there and the PDD is not there. So if you could just help us do the margin walk there, please?

Surendran Chemmenkotil: Yes, sure. So see, first of all, let me put it this way. I mean, the margin of the last 3 quarters put together, we are at 24.5%, which is largely on course as we have been guiding so far this year, very close to our guidance number. So that 3 quarter combined performance, I'm talking about it.

Now coming particularly to quarter 3, 2 things have happened differently. One is between quarter 3 and quarter 4, we really got accelerated on our lab expansion. We've just got in about 13, 14 new labs only in these 2 quarters which has come in. So which has come with an additional cost for the time being and not necessarily the corresponding revenues immediately, right? So that has definitely added some cost on to us in quarter 3.

And secondly, like Ameera mentioned in her opening speech, we have seen some decline in the illness patterns in quarter 3, particularly on the acute diseases. And that used to be relatively higher margin business for us. So that was a surprise to us. Because of these volumes have come down because of seasonality or specific trends that we have observed in the market. So that has actually put in a little bit of dent on the quarter 3 margins. So these are largely 2 reasons why the quarter 3 margins are what it is. But I mean, we're already seeing the trend has been changed in the quarter 4, and we are back to the YTD trends, and we only should get better from there.

Shyam Srinivasan: Helpful. Just the second question is on the test per patient number. I know different companies could be calculating it differently. But any thoughts on how this can likely move? Or can we catch up with some of our peer numbers, let's assume maybe TruHealth is lower than -- say, bundling is lower for us. So what could be some of the drivers to take test volume growth up relative to, say, patient growth? And if I got Aditya's numbers right, for B2C, patient volume growth and test volume growth was the same number, 4% or 5%.

Aditya Shinde: Yes.

Surendran Chemmenkotil: Yes. So I think the patient volume growth, of course, is one of the focus areas for us. We've just seen a temporary decline in the last quarter, particularly because of the declining illness trends in the last quarter. And on a YTD basis, we are at 6%, right? And also this year, we talked about it in the past, we have also defocused on the institutional business, which is largely high volume and lower revenues and lower margin business.

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We just had some government contract, which is coming down, and 1 or 2 other institutional businesses, we tried to defocus. So that is what is leading to the volume number into 6%. Otherwise, we used to be on 7%, 8% range in the previous year. So that should come back by the time we close this year.

And 2 things which definitely will help us to increase the patient volume growth. One, of course, our increased collection center expansion strategy. Last quarter, we did 125 new centers, and this quarter a similar number or better number should happen. So as and when we penetrate into larger and newer markets, we should get higher volumes of patients.

That is one thing. And secondly, I just talked about something that we have rolled out in this quarter, which is the recommendation engine, which is basically when a patient walks into a center, he has been advised to do any additional tests, which is the next best action that can be done in his health care journey.

So that will definitely add the number of tests into a patient, right? That will get you more, what you call, revenue per patient. So that's the second initiative that we have done on the test count as well as increasing revenue per patient. So these are some of the initiatives over and above that what we have been doing otherwise.

Moderator: The next question is from the line of Anshul Agrawal from Emkay Global.

Anshul Agrawal: First question is on margins. So while I understand post our network expansion, lab expansion, our margins are expected to sort of expand by 100, 150 basis points in FY '26. But with this acquisition of Core, what would be the margin trajectory going ahead?

Surendran Chemmenkotil: Like Ameera mentioned, the year 1, we will definitely have a little bit of dilution because of the Core acquisition for the group level. And by year 3, it comes down to the company levels of margin -- current levels of margin in the company. So for a couple of years, I think relatively, there will be dilution coming off the Core acquisition, but it comes with a higher upside.

definitely on the revenue side, right? So that will be the first 2, 3 years. But I think on the third year, you will see that the margins from Core Diagnostics is equal to that of Metropolis.

Anshul Agrawal: So on reported basis, our margins should not expand meaningfully because of these 2 factors. Would that be a correct assumption to make?

Surendran Chemmenkotil: I mean one is our organic business, like we said, we have an opportunity to get the margins better going forward. And also our lab expansion will be slowing down in this coming year. And the year after that, you will get a much better operational leverage by reducing this year's -- when you expand the labs in 1 year, the next year also a little bit of impact will be there. The year 2 will be much lesser impact. So on an organic basis, I think you will get a better margin expansion going forward.

And also, we will also be adding more acquisitions, some of which could be coming at a much better EBITDA level. So I mean, exact number is very difficult to predict at this point of time, but some of the targets that we are talking about is, of course, a higher B2C business and better Metropolis Healthcare Limited

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EBITDA margins if they come, and there will be a different combination and composition in terms of the overall EBITDA in the company.

Anshul Agrawal: Okay. And have our employee costs have shot up by about 17% Y-o-Y in this quarter. Again, is that trajectory expected to continue given that even by adding many more collection centers, franchise-owned centers as well, the employee costs should be at a run rate of around INR 80 crores, INR 80-odd crores per quarter?

Surendran Chemmenkotil: I think if I'm right, my employment cost on a year-on-year basis have only go

ne up by about

9%, 9.5%, not 17%. And yes, 9% is normally because of the yearly inflation and increment cycle, et cetera. Nothing extraordinary has happened in terms of the employee cost. On the contrary, we have been driving a lot of productivity enhancements in the company, which is what, despite adding so many labs and so many centers, we are not seeing a corresponding increase in the employment cost as such. I'm not sure about the 17% number, I may have to see that.

Anshul Agrawal: It's just the quarter 3 number. But yes, I'll check up on the 9-month number. The second question that I had was on these micro-market strategies where we have taken price hikes in certain markets. Shouldn't we be prioritizing on volume trajectory in markets where we are already lower in pricing versus competition? Wouldn't that help us grow our -- strengthen our market leadership position in those markets over a longer term period?

Ameera Shah: See, we are assuming that price and volume have a direct linkage. Actually, in health care, we find that, that linkage is not as obvious. So just because you lower price doesn't mean that you're able to pick up larger volumes. So often, we have found in health care, actually where your brand is stronger, you actually are able to make a better mark and you're able to generate better profit. So yes, while we will continue to expand into newer markets and build the brand in newer territories, because that must be done for future growth engines. But just trying to build volume quickly through price may not necessarily be the best strategy that gives you the best results. But obviously, we are trying to create additional profits out of the existing markets as we make inroads into new markets.

Moderator: The next question is from the line of Rishi Mody from Marcellus Investment Managers.

Rishi Mody: Yes. So on the gross margin front, if I heard you all correctly, you are expecting gross margins to expand on the back of increasing B2C. So from a near-term perspective, are we expecting a hit on gross margins because of the dollar depreciation and hence, repricing? Or we already have gotten that repricing done and it's there in this quarter? Secondly, on the cost front, going forward, like now that the lab expansion is largely done, what is the trajectory that you all are expecting that the absolute cost should grow at?

Surendran Chemmenkotil: All right. So we definitely have some impact of the dollar. There's no doubt about it. But we, at the same time, start figuring out how better negotiation and better efficiencies can chip in. So as a result, largely, we try to kind of take care of the increased pressure because of dollar through our efficiencies and our productivity, et cetera. So I'm not expecting an immediate escalation on the cost because of the dollar as a reason. That's on the first question.

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And on your second question on cost, when the next 2 years, we'll have more of center expansion, less of lab expansion, should definitely should have a better productivity on the cost side. So currently, I think at a 12.5% revenue increase, we are roughly 10.8% cost increase. And the gap is roughly 2%. I think going forward, that should further get better. So we have not put the right math around it at this point of time, but I think the cost increase should be relatively

lower than the current one as we go forward.

Rishi Mody: Second, on the top line front, right, we've seen 16% top line growth in Bombay and rest of Maharashtra. How much would you reckon is the clawback of, say, market share from these online players? And secondly, is the competitive intensity from any incumbents increasing?

Ameera Shah: See, I think we have to remember that the online players actually took market share not really from the large incumbents like us, they actually took it from

the unorganized sector, because

really people who are going to the local lab were sort of saying that, look, if I'm able to get the same price from an online brand, which I consider as reputable, then that is a good alternative. Having said that, so therefore, really, it was about the unorganized moving to the incumbents or the unorganized moving to online players. It was really not online players taking business from incumbents, at least what we have seen in our experience.

So I think that pace of taking business from unorganized players has significantly come down for the online guys, because we saw that pace really grow during COVID, but not really sustained post. And really, the market has gone back to where the online players are growing actually at the same pace now as the rest of the industry and not significantly faster. And obviously, they still continue to not make any profit on it.

So I think from our perspective, our focus continues to be on illness patients. And while we have a mix of chronic and acute, we have majority acute patients. We believe those are the ones who genuinely care about the quality of the result and, therefore, are willing to pay also the fair price for the quality that they get.

So not facing really that severe competition at this point. And from new players, I mean, honestly, since '22, we haven't seen any new players enter the market. The last we saw was a couple of hospital chains and pharma chains who came in. And they obviously have their own challenges in the industry. So we feel quite comfortable and confident on our own journey.

Rishi Mody: Right. And from the incumbents like Suburban, Neuberg?

Ameera Shah: I mean, honestly, again, we are outpacing all the peers, including the acquisitions done in Bombay. You'll see from any public data available that Metropolis continues to be the fastest-growing player in Bombay and in Maharashtra.

Rishi Mody: And nothing changes qualitatively versus, say, over the last year for competition from these 2 guys at least?

Ameera Shah: Nothing that is obstructing us, frankly.

Moderator: The next question is from the line of Abdul from ICICI Securities.

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Abdul: Just firstly on your institutional business. So I understand that the share has gone down considerably as what we were last year. But going ahead, say, in FY '26 or so, is there any portion where we would be at risk of getting discontinued, whether it's some PPP contract or some short-term contracts which would still be into the base of the first 9 months of fiscal '25?

Surendran Chemmenkotil: So see, at this point of time, there is no such thing that we are seeing in the coming months to happen. I think none of the tenders that we have participated or -- we have been very selective in this segment, places where we can really make enough margin and the whole receivable process is relatively smooth. That's only kind of PPP that we are getting into. Very selectively we are doing it. And we are not actively considering any new contracts as we speak, which can translate into revenue in the next 6 months to 8 months' time, as you're asking.

Abdul: Okay. And secondly, on the lab network addition what you have done. So what is the gross number that you have done in the first 9 months of fiscal '25?

Surendran Chemmenkotil: Gross lab numbers?

Abdul: Yes, lab addition numbers?

Surendran Chemmenkotil: Yes. So we've done 23 numbers -- 23 labs we have done in the first 9 months.

Abdul: Okay. Understood. And lastly, on the consolidation, what we are talking about. So within the peer groups also, we hear there is a sizable level of consolidation happening in the Western and the Southern region. So going ahead, how should we see the landscape? Would it happen that the organized guys will try to bring in more efficiencies by acquiring the smaller ones who are

not able to scale up, or at some point of time, you will think that the market is getting saturated

and consolidation is the only way to move ahead for?

Ameera Shah: See, I think each player will have their own thoughts, but I think we can tell you from an industry perspective, there is no such saturation, because still, if you look at the top 5 players in the country, the chains, they are less than 5%, 7% of the whole industry, right? So there is a huge opportunity to continue to grow. And the unorganized space is still 85% of the entire industry.

So certainly, no cap, no end of runway, no saturation for sure.

Really, your question around is consolidation the only way to grow? Absolutely not. I think

organic growth is really, frankly, the mainstay for us. And if you see, in the last many, many years, organic growth has been the primary contributor to our growth. And inorganic is really not about growing the revenue faster, but it's really about strategic entry points into either new testing capabilities or new geographies and really using that as a base to then catapult from there. And we will continue to do it from a very strategic perspective. But the mainstay will be organic growth, which we believe continues to be a big opportunity going forward. And therefore, as you've seen, our YTD revenue growth is 12.5%, and we expected the Q4 as well to be higher than that.

Moderator: Ladies and gentlemen, that was the last question for today. I now hand the conference over to the management for closing comments.

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Ameera Shah: Thank you, everybody, for joining us today. As just mentioned, we continue to believe there's a very long runway for growth for the diagnostics industry in India. And while we've had obviously a fair number of roller coaster rides from 2020 till 2023, '24 because of obviously COVID and then coming off COVID, we do believe there is a certain normalcy now. And while, of course, there has been increased competition in the last few years, but the good quality players continue to be differentiated and stand apart from the rest.

As we've often mentioned, entering this industry is easy, but truly scaling and staying in the industry and building a profitable business in this industry continues to remain a Herculean task. And there are very few of us who are creating good profitable companies with high ROCEs and a real differentiator in the market. And Metropolis is certainly one of the leaders in this space. Going forward, our expansion will be mostly collection center led and less about lab expansion, which will help us increase our margins. And we are obviously excited about closing the Core Diagnostics acquisition and welcoming them into the family effective March 1, and also closing the other advanced discussions of bolt-on acquisitions that we're having, which will increase our B2C and our presence and relevance and visibility in Northern India.

So look forward to continuing to work with all of you on this journey and look forward to chatting with you at the end of Q4. Thank you.

Moderator: Thank you. On behalf of IIFL Capital, that concludes this conference. Thank you for joining us, and you may now disconnect your lines.

Call: May 2025

(no extractable text — likely a scanned image PDF)

Call: Aug 2025

Ref: MHL/Sec&Legal/2025 -26/54 August 14, 2025

To,
BSE Limited
Scrip Code: 542650 National Stock Exchange of India Ltd.
Scrip Symbol: METROPOLIS

Dear Sir/Madam,

Sub: Earnings call transcript for Q1 FY26

Pursuant to Regulation 30 read with Schedule III of the SEBI (Listing Obligations and Disclosure Requirements) Regulations, 2015, please find enclosed herewith the transcript of Q1 FY26 earnings conference call held on Friday, August 08, 2025 . The transcript is also available on the Company 's website i.e. www.metropolisindia.com

You are requested to take the above information on record.

Thanking you,
Yours faithfully,

For Metropolis Healthcare Limited

Kamlesh C Kulkarni
Head – Legal & Secretarial

Encl: A/a

“Metropolis Healthcare Limited Q1 FY'26 Earnings Conference Call”

August 08 , 2025

E&OE - This transcript is edited for factual errors. In case of discrepancy, the audio recording uploaded on the stock exchanges on August 08, 2025 will prevail

MANAGEMENT : MS. AMEERA SHAH – CHAIR PERSON & WHOLE -TIME
DIRECTOR , METROPOLIS HEALTHCARE LIMITED
MR. SURENDRAN CHEMMENKOTIL – MANAGING
DIRECTOR , METROPOLIS HEALTHCARE LIMITED
MR. SAMEER PATEL – CHIEF FINANCIAL OFFICER ,
METROPOLIS HEALTHCARE LIMITED
MR. AVADHUT JOSHI – CHIEF BUSINESS DEVELOPMENT
OFFICER , METROPOLIS HEALTHCARE LIMITED
MODERATOR : MR. AMEY CHALKE – JM FINANCIAL INSTITUTIONAL
SECURITIES LIMITED

Metropolis Healthcare Limited
August 08 , 2025

Moderator: Ladies and Gentlemen, Good Day and Welcome to the Q1 FY26 Earnings Conference Call of Metropolis Healthcare Limited hosted by JM Financial.

This conference call may contain forward -looking statements about the Company which are based on the beliefs, opinions and expectations of the Company as on date of this call. These statements do not guarantee the future performance of the Company and it may involve risks and uncertainties that are difficult to predict.

As a reminder, all participant lines will be in the listen -only mode and there will be an opportunity for you to ask questions after the presentation concludes. Should you need assistance during this conference call, please signal an operator by pressing “*” then “0” on your touchtone phone. Please note that this conference is being recorded.

With this, I now hand the conference over to Mr. Amey Chalke from JM Financial. Thank you and over to you, sir.

Ameey Chalke: Yes. Thank you Shruti. Good morning, everyone. I, Amey Chalke, on behalf of JM Financial, welcome you all to the 1Q FY26 Earnings Conference Call of Metropolis Healthcare.

From Metropolis, we have with us today, Ms. Ameera Shah – Chair person and Whole -Time Director, Mr. Surendran – Managing Director, Mr. Sameer Patel – Chief Financial Officer, and Mr. Avadhut Joshi – Chief Business Development Officer.

Now, I will hand over the call line to you, ma'am. Over to you for your “Opening Remarks.”

Ameera Shah: Thank you, Amey and good morning, everyone, and thank you for joining us today for the Q 1 FY26 earnings conference call.

We have uploaded our investor presentation and the related documents on the stock exchanges and the company's website, and I hope everyone has had the opportunity to go through the same.

India's healthcare landscape is gradually shifting, shaped by a range of developments in how care is delivered, accessed, and monitored. We are keeping an eye on a few broad global trends that stand out. AI -led diagnostics, decentralized point -of-care testing, and the growing relevance of GLP -1 therapies in metabolic health.

Artificial intelligence is increasingly being embedded into diagnostic workflows, particularly in radiology, enabling faster, more accurate reporting and personalized recommendations. In the pathology space, globally or in India, it has started entering the workspace in customer service, back -end processes, and a few technical tests. However, it is far from being used en masse at this time.

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At Metropolis, we are applying AI across multiple areas, from clinical reporting in a few tests and customer chatbots to test recommendation engines and AI -assisted testing in prostate cancer, allergy testing, and Karyotyping. No one in India at this time is using AI in a very scaled or differentiated way that we believe is going to be severely disruptive to the existing industry.

The second trend when we review, which is of POC diagnostics, we are seeing it help improve access, especially in semi -urban and rural areas where setting up a lab may not make economic sense. POC is not cheaper or more accurate than the current standardized platforms, and therefore usage is more for urgent reporting in metros and access to diagnostics in smaller rural markets. If a scaled use case for using Point of Care diagnostics is found, Metropolis would certainly consider using this channel to access new customers or markets.

On the GLP -1 front, the receptor agonist market in India is currently valued at USD110 million in 2024 and is projected to grow to USD 500 million by 2030. This shift is likely to drive higher demand for regular monitoring of glucose, lipids, liver, kidney, and cardiac parameters. This marks a shift from episodic testing to longitudinal health tracking, with consumers increasingly invest

ing in

proactive wellness. Together, these trends point to a gradual move towards more personalized and proactive diagnostics. Also, certain metabolic panels and other tests may increase as the use of GLP adoption increases.

We are very happy to announce that our strategy of strong organic growth plus good quality acquisitions has resulted in 23% growth in revenues for MHL in Q1 FY26. Despite Q1 typically being a soft quarter in the West and South regions, Metropolis recorded a strong 13.2% organic growth in revenues, driven by volumes and improved realizations, supported by our strong brand equity and expertise in the specialty diagnostic segment. With North region now contributing 17% to our overall revenue, we will continue to spend considerable time and effort in growing this base in the Northern region.

While quality, scale, and brand equity continue to be strong pillars, we are now doubling down on our scientific depth and innovation to further distinguish ourselves from competition, particularly in a market where pricing gaps are narrowing.

We are in the process of building a center of excellence in oncology diagnostics, which will centralize expertise in genomics, histopathology, cytogenetics, molecular oncology, and precision testing. The center of excellence will serve as a hub for clinical collaboration, research, and education, positioning Metropolis as a leader in onco -diagnostics and enabling differentiated, high -trust offerings for oncologists and hospitals.

The acquisition of Core and putting together skills and expertise Metropolis already had in oncology, along with the new relationships acquired by Core. We will create a platform for Metropolis to not

only be the largest oncology testing player in the country, but keep innovating and investing for growth for the next several years to come.

Additionally, we have been actively upgrading our labs and associated IT systems while complementing and implementing several lab automation initiatives to boost efficiency and productivity.

As we keep increasing our test menu not only in our global reference lab, but also in our regional reference lab, it allows us to deliver faster results and reduce turnaround time for our specialty tests for all markets.

But fast is not enough. The result has to be precise and accurate according to each doctor's individual experience. To achieve this, we not only have curated our own quality protocols, but we have standardized them across all our labs and continue to uphold and show our quality results in every test for every patient and every doctor.

Our rigorous internal quality control process operates in parallel to NABL accreditation and is rolled out across every lab under Metropolis. This does not mean just testing the sample on a single machine and transferring the data to the patient, but it includes multi-Tier-validation of test results using various testing platforms to cross-check results, curation of reference ranges for the Indian patient, daily quality checks, and the use of global quality controls and standards, ensuring consistency, accuracy, and reliability across our entire lab network. This system provides a significant edge over not just unorganized players where such uniformity is often lacking, but also sometimes organized players where our expertise and knowledge of pathology over 40 years proves to be a differentiator. Just to update you on the integration of our recently acquired targets, last quarter we completed three acquisitions as part of our inorganic growth strategy. The integration of these assets is progressing smoothly with no significant challenges encountered. Led by a dedicated steering committee, the integration process is already yielding synergies, particularly in lab consolidation, procurement efficiencies, and reduction of overlapping overheads. Our immediate focus is to

fully integrate these

acquisitions into the Metropolis ecosystem, prioritizing operational efficiency and margin improvement in the first year. Once integration is complete and synergies are captured, we will shift our focus to accelerating revenue growth through cross-selling opportunities and expanding the reach of co-diagnostics across Metropolis' pan-India network in year two.

While Metropolis has organically posted a 24.7% EBITDA margin and scientific pathology and DAPIC in Dehradun are of good margin profile, co-diagnostics will be turned around this year, and this year it will drag the consolidated EBITDA margin.

If we look at Core, Core has shown positive year-on-year revenue growth, with a notable improvement in profitability, achieving a low single-digit EBITDA margin compared to the break -

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even levels in Q4 FY25 before we acquired it. Our immediate goal is to integrate the team into our culture and processes while unlocking efficiency gains through integration synergies. Our goal for year one is to reach a high single digit EBITDA margin primarily through cost synergies, and we should see the progress over the coming quarters.

PAT for Q1 is negative, largely due to high depreciation and high-cost interest for Core, and it has a Rs 12 crore loan in the books, which has been now refinanced in July at a much better rate, and hence Q2 PAT for Core should be much better.

Scientific Pathology, Agra and DAPIC, Dehradun, both entities have delivered revenue growth in line with the Company average, and margins are slightly above the Company average for both. These brands are well-established in their respective local markets, and our strategy moving forward is to expand into adjacent regions by increasing the number of collection centres and offering the broader Metropolis test menu, helping us capture a larger share of the market. It is worth noting that DAPIC, Dehradun has been consolidated only from 23rd May 2025, and Scientific Pathology Agra only from 16th June 2025, and hence only proportionate revenue and margins have been consolidated in Q1. Just yesterday, we have announced our acquisition of Ambika Diagnostics in Kolhapur, founded and run by Dr. Patil, and considered the most credible lab in Kolhapur. We have been in a management contract with Ambika for the past two years, where Metropolis runs the business, with employees on our payroll and all processes and technology under the Metropolis guidelines. This lab on lease model has done very well in the last two years and has grown 60% since we started managing it. We believe this growth will continue to be strong in the future, and we thought it made sense to acquire this business and merge it with the Metropolis Kolhapur Lab that runs independently in the city. While the revenue of the business was Rs.8 crores in FY25, with Rs.1.8 crore EBITDA, when we look at the valuation of Rs.17 crores for 100% of this business, while it looks like a 9.4 multiple of FY25 EBITDA, in reality, the EBITDA will be Rs.3.4 crores once we acquire it, because it will include the synergies between the labs, and fully show the profit on its own, which would actually effectively be

a 5x multiple of EBITDA.

Together, the labs, when we merge them, would create the largest lab in Kolhapur, and in the whole region, and it would be a good hub to grow in the whole district.

Dr. Patil will continue to work with us for over 10 years, and we believe this acquisition to be highly accretive from day one. The acquisition would be done from internal accruals and closed within 30 days from signing, which was yesterday.

We have consistently emphasized our strategic intent to diversify into adjacent healthcare services, which can enhance customer engagement, and we have been piloting several initiatives, including basic radiology, like ECG and X-ray, and non-blood vital checks

checkups, and on-call doctor consultations.

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We are pleased to share that our basic radiology pilot has gained encouraging traction. We have 20 locations offering full radiology services, which include X-ray, sonography, and ECG, 36 locations offering X-ray services, and 240 locations offering ECG services across the Metropolis network. The integrated approach, which combines blood diagnostics, non-blood vital assessments, and basic radiology, enables us to offer a more comprehensive and holistic preventive health solution under the TruHealth brand. By expanding these services within the Metropolis ecosystem, we are not only enhancing customer value, but we are strengthening our ability to deliver end-to-end health screening packages, and this is helping us improve our RPP as well. Early indicators from these pilots are promising, and we believe this direction has the potential to serve as an additional growth lever, further deepening customer engagement and expanding our service footprint.

Lastly, on the competitive front, the landscape remains largely unchanged. The unorganized sector continues to lose market share to organized players, and the growth is largely through marginal volume increase from the unorganized guys. This shift in market share was initially concentrated amongst a few organized players pre-COVID, and post-COVID, the same is now distributed across a slightly larger number of organized players. Meanwhile, in the last 12 to 18 months, health tech players are facing heat on lower growth and trying to demonstrate that their unit economics work. In response, they have been consistently raising prices and are now transitioning from discount-led volume growth to a value-based growth approach, bringing their revenue growth similar to organized players.

We continue to remain excited for Metropolis' revenue and profit growth for this year. While we will spend most of our energy on growing the business organically and integrating the deals already announced, we will continue to evaluate other acquisitions that make sense for Metropolis for the future, always from the goal of buying businesses which fit our culture, long-term vision, and can create financial value for shareholders.

I will now ask "Suren to Brief You on the Progress on Key Operational and Financial Metrics."

Thanks, Suren. Over to you.

Surendran C: Thank you, Am eera, and good morning, everyone.

So, we are pleased to report that both patient volumes and the revenue growth have returned to normalized levels in line with our guidance. After a temporary dip observed in Quarter 4 last year, largely due to seasonal and regional factors, the business has rebounded as expected in Quarter 1. The recovery has been broad-based, supported by several strategic growth levers.

Let me just deep dive into some of them :

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TruHealth Preventive Portfolio or TruHealth Range of Preventive Health Checkups continues to be a key contributor to patient footfall and test volume growth. With increasing awareness around preventive diagnostics, lifestyle management, and chronic disease monitoring, and our ability to upsell, TruHealth is witnessing healthy adoption across urban and rural markets. Our integrated approach of blood diagnostics, non-blood vital checks, and doctor consultation is helping us, and we have now reached 18% contribution via TruHealth Portfolio.

Specialty :

Growth in high-end specialty tests, particularly in oncology and genomics, have added both volume and value to our overall portfolio. These tests not only come on higher RPP, but also reinforce Metropolis' positioning as a science-led, differentiated diagnostic provider.

On the network expansion side, continued geographic expansion, especially in underserved Tier-3 and peri-urban markets, has been strong tailwind. Through

a focused hub-and-spoke model, we have

added new collection centers, enhancing access and brand presence across newer micro-markets. The Tier-3 cities have shown strong growth in patient volume, driven by increasing brand preference, widening test menu availability, and improved local infrastructure.

Starting this quarter, we have reclassified our B2C and B2B portfolios to align with prevailing

industry standards to make it easier for investors and analysts to compare. We have dropped the institutional bucket completely and divided the business under only B2B and B2C. Under the revised classification, B2C now includes all revenues from company -owned centers, franchisee centers, and rural collection centers. B2B encompasses service to laboratories, hospitals, government institutions, corporates, and phar ma companies. As per the revised definition of B2C, our organic business B2C revenue contribution stood at 59% as compared to 57.8% in Q1 last year. For MHL Group, B2C now contributes 56% to overall revenues, including the acquisitions. The decrease in B2C contribution on group level is largely on account of Core Diagnostics , which has the majority of revenue contribution from B2B channels. Going forward with the cross -selling of the specialty tests across the Metropolis network and leveraging the existing channels, we believe that the contribution of B2C share from Core should increase.

Our organic B2C business continues to show robust and consistent growth, with revenues increasing by 16% on a year -on-year basis. Now, t his is driven by 9% growth in patient volumes, reflecting strong consumer engagement and reach, 6% growth from improved test mix and pricing, highlighting our ability to offer differentiated and value -addressed diagnostic services.

In Maharashtra, which is our largest B2C market, revenues also grew by 16% year -on-year, with Mumbai outperforming the state average, fueled by deeper brand equity, broader test offerings, and strong medical community trust.

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Our strategic push into Tier-2 and Tier-3 towns is further strengthening our B2C footprint, helping us reach new patients through an expanding network of collection centres and franchisee partners.

A key enabler of B2C growth has been the increased adoption and engagement of our mobile and digital platforms, including the Metropolis app.

Key enhancements that are driving stickiness and repeat usage include personali zed health journeys, tailored test suggestions, and curated wellness content based on patient experience preferences, real -time tracking of orders, seamless report access, and proactive health reminders significantly improving customer satisfaction.

End-to-end digital integration of test booking, home sample collections, payments, and customer support resulting in smooth and hassle -free experience. This omni -channel approach is not only enhancing patient convenience but also improving operational efficiency by reducing dependency on manual channels and lowering service turnaround times.

At the local level, our micro -marketing strategy is helping us drive targeted awareness and patient acquisition in high -potential clusters. This includes hyper -local campaigns based on regional health trends and demographic insights , clinician engagement programs to build strong relationships with the medical community , expansion of Met link Partner Program, which has been made the relationship between franchisee and Metropolis even stronger.

These initiatives have led to a visible increase in B2C contributions, stronger brand recall, and improved penetration in both metro and non -metro markets. We remain confident that this momentum will continue to build as we scale our reach, optimi ze our best offerings, and deepen customer relationships.

Our B2B revenue for the quarter grew by 10% and now stands at 41% as per the revised classificati on

of the B2B definition for the organic business. Patient volume in the B2B segment grew by 4% while RPP rose 6%, primarily driven by high -value outsourcing from hospitals. This reflects our continued focus on speciali zed and advanced diagnostic offerings that command better pricing. We increased our partnership with pharmaceutical companies in line with our strategic emphasis on specialty diagnostics.

Upgrades to our partner portals have improved end -to-end visibility and service experience for B2B clients. Enhanced tracking, transparent reporting, and faster resolution mechanisms have significantly improved partner satisfaction and operational efficien cy across labs, hospitals, and corporate accounts.

This quarter, we also concluded our engagement with Aam Aadmi Mohalla Clinic s, in line with our long-term strategy to move away from low -margin government or institutional contracts. Instead, we

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are focusing on science -led B2C and B2B segments which offer higher margins and greater long -term sustainability.

Turning to the "Productivity and Margin Performance" :

The EBITDA margin for Q1 stood at 24.7%, higher than Q4 last year. Our attempt will be to strengthen the margin every Q uarter for this year on an year -on-year basis. The reported group EBITDA has been marginally impacted by the consolidation of Core Diagnostics, which is currently operating at low single -digit margins. While it is an improvement from the break -even levels of Q4 last year, before we acquired the business, we remain hopeful of bringing the Core's margin profile

to high single -digit by the end of the financial year, as integration progresses and synergies begin to reflect the financials. To strengthen the EBITDA margin this year, in the next few quarters, we have rolled out multiple productivity initiatives aimed at unlocking efficiencies.

Over the last 18 -24 months, we have invested heavily in automation and digital workflows to streamline operations, reduce manual effort, and enhance service turnaround time. Many of these programs are now live and will start to add to our productivity efforts.

Also, our lab infrastructure expansion, which is in execution for the last four years, successfully got concluded last quarter.

With this foundation in place, our focus has now shifted to rapidly scaling our collection center network to feed these labs, thereby driving higher throughput and utilization, which will translate into better margins over time.

In Q1, we have added 80 new collection centers, and we are on track to add approximately 400 -plus collection centers across various regions this year, with a strong focus on tier -2 and tier -3 towns. This expansion is being supported by our recently strengthened lab infrastructure in these geographies.

As of now, we are serving customers in about 750 towns across India, and we aim to expand our footprint to 1,000 towns soon, further deepening access and enhancing our presence in high -growth, underpenetrated markets.

So, in summary :

We have entered financial year 2026 with renewed momentum, clearly shifting gears compared to the previous two quarters. The integration of recent acquisitions is well on track, and we expect this to unlock meaningful cost synergies over the remainder of the year. This includes operational consolidation, procurement efficiencies, and overhead optimization, all of which will contribute positively to our margin profile.

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On the growth front, we are seeing healthy revenue performance, supported by strong execution across core growth drivers. The TruHealth preventive portfolio, specialty diagnostics, and tier -3 market expansion continue to be the leading contributors. These segments not only add scale, but also bring higher value and margin -accretive business, aligning with our strategy of moving to

wards

quality -led growth.

We have also deepened our focus in science and quality, an important pillar of differentiation for Metropolis. This includes enhancement in quality processes, test menu expansion, steps towards building centers of excellence, and continuous upskilling through scientific platforms. These actions position us as a trusted diagnostic partner, especially in the specialty and high -end testing space.

In parallel, we have activated several productivity enablers across the organization, from digital process automation to optimized resource deployment. These efforts are already yielding improvements in operational efficiency and are expected to support sustained margin expansion going forward.

Overall, with strong execution across integration, science, digital, and operational fronts, we are entering the year with solid traction and clear line of sight to our goals. We remain committed to delivering profitable, sustainable growth with reinforcing Metropolis position as a trusted, science -led diagnostic leader.

With this, I hand over the call to "Sameer, who will take us through the Financial Highlights." Thank you and over to you.

Sameer Patel: Thank you, Suren. Good morning, everyone. Let me now "Share Some of the Key Financial Performance for Q1 FY26. "

We have bifurcated our reporting on two aspects – one, MHL Group and second, MHL Organic.

MHL Group includes recent three acquisitions of Core Diagnostic s, DAPIC, Dehradun, and Scientific Pathology , Agra , and MHL Organic excludes these three acquisitions.

Also, change in definition of B2C and B2B segments to streamline the same with the industry standards. B2C now includes all owned franchises and rural centers. And B2B now includes B2B Lab, Hospitals, Government, Corporate, and Clinical Trials.

Moving on to the "Financial Performance and Operational Performance" :

First, I would like to highlight operational performance of MHL on organic basis :

Revenue and EBITDA grew by 13.2% and 11.9% respectively and PAT grew by 21.2% on year -on-year basis.

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Patient volume stood at 3.2 million, a growth of 7%. Our test volume growth stood at 8% on year -on-year basis with increased contribution from TruHealth segment. We consider one profile as one test, which is different from peers. As on like -to-like basis as peers, this number would be significantly higher.

Our B2C revenue stood at Rs.209 crores, a growth of 16% on year -on-year basis and B2B revenue

stood at 10%. TruHealth and Specialty segment grew by 22% and 16% respectively.

As the revised classification of B2C segment, B2C contributes 59% of total revenue compared to 57.8% in Q1 FY25. Revenue for Specialty B2C segment grew by 18% and B2C TruHealth segment grew by 19% on year -on-year basis. B2B revenue stood at 14% of total revenue and B2B Specialty grew by 15%. Tru Health segment for Q1 FY26 grew by 22% and TruHealth now contributes at 18% of overall revenue. Specialty segment grew by 16% on year -on-year basis for Q1 FY26.

EBITDA for MHL on organic basis stood at Rs.87.5 crores, a growth of 11.9% on year -on-year basis. EBITDA margin for MHL organic stood at 24.7 %, an increase of 40 basis points on a sequential basis and we are optimistic of seeing an uptrend every quarter.

PAT for MHL on organic basis stood at Rs.46.2 crores as compared to Rs.38.1 crores in Q1 FY25, a growth of 21.2%. PAT margin for MHL organic stood at 13% compared to 12.2% in Q1 FY25, an increase of 80 basis points.

Now , let me "Share the Key Performance Indicators for MHL at a Group Level" :

Revenue grew by 23.2% on year -on-year basis from MHL group including the revenue of recent acquired entities. Patient volume stood at Rs.3.4 million and test volume stood at Rs.7.1 million, a growth of 11% and 12% respectively. B2C business revenue grew by 19% for Q1 FY26 and B2B revenue grew by 29% on year -on-year

basis.

On MHL group basis, revenue for the North now contributes 17% of overall revenue, largely because of integration of Core D iagnostic s which has its major presence and revenue coming from North region.

Revenue growth from Tier-1 town s stood at 27% and Tier-3 town s stood at 17% on a year -on-year basis. Revenue for Tier-3 town now contributes 17% of total revenue for Q1 FY26. Contribution from B2C revenue stood at 56% in Q1 FY26. The decrease in B2C compared to organic business of 59% is largely on account of consolidation of Core D iagnostic s which has majority of revenue contribution coming from B2B.

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EBITDA margin for MHL group stood at 23.1%. The decrease was largely attributed to lower margin profile of Core D iagnostic s. However, we are happy to report that Core has turned positive with a lower single -digit margin profile in Q1 FY26.

PAT for MHL group stood at Rs.45.2 crore s. This PAT is at 11.7%.

That is all from my side. With this, I open the floor for question -and-answer.

Moderator: We will now begin the question -and-answer session. The first question is from the line of Anshul Agrawal from Emkay . Please go ahead.

Anshul Agrawal: My first question is on integrated offerings. While our pilot projects have indicated positive traction in basic radiology, what could be our plan going forward -- would we sort of think about getting into advanced radiology or continue to expand basic radiology across our network? Secondly, would this be on an asset -light basis or would we own these equipments?

Ameera Shah: I think on the basic radiology, we will continue to spread it across our centers. And t hese equipments are not very expensive. These are smaller equipments. A lot of them are on asset -light basis. We are not procuring all of them. It is a combination, I would say. We are evaluating whether higher -end radiology is something worth getting into or not. We do not have a clear answer at this point of time.

I think over the next couple of quarters, I think we will have a clearer direction. But meantime, we will continue to roll out the basic radiology. Where we are finding this is helping, it is not helping obviously very significantly on the independent revenue front. But what we are finding is that customer frequency to our centers increases with it. And when they do add on some of these tests, either radiology or the non -blood vitals, it actually helps them do a larger screening program, wellness screening program with us. So, it is more about the completion of the services at the center, which also helps in NPS with the customer.

Anshul Agrawal: The second question is on GLP medication. From what we understand, Am eera here, would patients or would customers start becoming healthier by using this medication? Is that a risk for the healthcare delivery providers? How should we look at it? I understand this is in a very nascent stage. But why do you view this positive light versus what the product is destined to deliver?

Ameera Shah: See, the product is only talking about weight loss. The product is not necessarily saying better health.

These are not necessarily the same thing. They can be . For somebody who has got obesity as an issue , obviously, automatically, it brings about diabetes and other things. But you also have to remember anytime you are giving a dosage of a drug, there also can be side effects , there are known side effects of these drugs , and therefore, doctors who are prescribing them will also have to continue to monitor those patients to watch out for whether those side effects are taking place in the patient 's body or not. So, our sense is that while it has a potential to manage maybe diabetes better because of obesity coming down, but things like cardiovascular risk and other risk, which could be caused because of

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these GLP -1 medications, will continue to have to be monitored. And it is not just the heart, but also watching the liver, the kidney, and to see if there is any impact there. We have to remember that these drugs are not that old in the world and people have not had a chance to really see what is the impact of these for the next 10 -15 years. So, I sense that people will continue to monitor their health , because one of the things they are doing to take GLP -1 is to get healthier and that automatically means a lot that you want to do a more regular wellness screening to make sure that all parts of your body are working fine.

Anshul Agrawal : That is clear. Just one more if I can squeeze in. In terms of our margin profile, I am talking about the organic part of the business excluding the acquisitions. When do we start seeing the benefits of the hub expansion that we took over the last two to three years to sort of creep in , in our margins? Should we not expect a margin expansion of 100 -150 b ps on our organic business?

Surendran C: Anshul, we have mentioned this in the past also that the drag on the margin from the newly acquired labs actually happens for two years. First year, it normally gets into a negative margin and the year two, it gets into a positive 10 -12% margin and the year three normally gets into the Company levels of margin. So, we have anyway halted all the accelerated expansions at the end of Quarter 4. So, this year, for the labs that we expanded last year and the year prior to that, there will be some little bit of drag on that , and the next year, we will have a little bit of drag on because of the labs we expanded the previous year. So, you will start seeing benefits on EBITDA margins coming this year to start with, and by the end of next year, we will get the full benefit of the lab expansion, which has been halted. So, I think it is a couple of years' time where you fully get that 1% plus benefit, which otherwise we used to talk about.

Ameera Shah: But compared to last year, obviously, in FY2025 -26, we will certainly see a margin bump EBITDA expansion. You will start seeing it obviously from Q2 itself.

Anshul Agrawal: Great. Those were my questions. All the very best.

Ameera Shah: Thank you.

Surendran C: Thank you.

Moderator: Our next question is from the line of Tauseef Sheikh from BNP Paribas. Please go ahead.

Tauseef Sheikh: Good morning. Thanks for the opportunity. Ma'am, well, you have mentioned in the initial comments that Core Diagnostic s have turned positive. Can you throw some light on the patient volume growth and the revenue growth in which the Core Diagnostic s has seen on a YoY basis?

Ameera Shah: So, at this point, we are not providing the separate information for each of these. But like we mentioned broadly, in year one, what we traditionally see when you acquire any organization, Metropolis Healthcare Limited

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whether it is unorganized or organized, is that sometimes the practices in it may not be exactly the same as Metropolis. And therefore, sometimes you have to cut certain customer accounts , you have to let certain things go , because you want to clean things up. And therefore, the first year, the goal will be margin expansion and cleaning up the practices and integrating. And from year two, the focus will really be on the revenue acceleration. And that is the direction that we are going in. It is obviously just the 1st Quarter of the acquisition. And some of these acquisitions have only had 15 days off in the quarter. So, we felt it is just too early to start sort of giving instructions on the separate growths and the separate margins for everything. The only reason we indicated for Core is just to show the trajectory on what we committed when we did the acquisition that we will see a quarter -to-quarter improvement in the profit profile and we just wanted to indicate that , that is on track.

ate that , that is on track.

Tauseef Sheikh: That is helpful, ma'am. And second question again is on Core Diagnostic s. At the time of the acquisition, we had mentioned we had a field force of 100 people. Have we done any kind of restructuring to this field force?

Surendran C: No, we have not disturbed the field force at this point of time. Let them continue to operate the way they used to operate. Because they are all specialist salespeople and we have not done any restructuring at this stage.

Tauseef Sheikh: Thanks. That is helpful.

Moderator: Our next question is from the line of Sumit Gupta from Centrum. Please go ahead.

Sumit Gupta: Yes, hi. Yes, so thanks for taking my question. So, first is, like, what is your strategy for expanding into this Kolhapur area like will it be acting as a bigger -

Moderator: Sumit sir, sorry to interrupt. Your voice got low and audio break. Can you please repeat your question?

Sumit Gupta: So, I want to understand on basically rationale for expanding into Kolhapur.

Surendran C: So, like Ameera mentioned, this lab, which is Ambika Diagnostics, used to be on lease -owned lab with us in the past for the last two years. And where it has been run by our people, right . Now we have decided to acquire this lab. And we also have a Metropolis separate lab in that area. So, now with this acquisition, we need to have only one of the two labs in that place because it will be fully owned by us. And this will also help us to expand into the entire Kolhapur region, right. And we already have presence in most part of the Kolhapur region, but with the addition of Ambika

Diagnostics, we will have a little more stronger footprint and good coverage across that area.

Sumit Gupta: Okay, so going forward, like, can we expect more acquisitions of this particular size or we can expect a size to be bigger?

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Ameera Shah: At this point, we do not have anything else that we are expecting to announce at least in the next quarter or so. But as we keep evaluating the funnels, we are obviously looking at different sizes. We are looking at small ones, but which are very credible, like we did DAPIC, like we did Agra, like we have done Am bika and we are looking at larger ones as well. But the final goal is not about small or big, the final goal is about whether it really fits the culture, whether it fits the way of thinking that Metropolis has a strategy and most importantly, does it create value for our shareholders and we are only doing deals where we feel that we are able to create that kind of value and not having to pay a crazy price when it becomes difficult to create value.

Sumit Gupta: Understood. Okay. And like just a bookkeeping question . What was the EBITDA for this Am bika in FY25?

Ameera Shah: So, standalone, it was Rs.1.8 crores on its own if you look at FY25. But in the model of lab on lease, we were also sharing certain revenue share with the Ambika Pathology , etc. , So, when it is in our books, it will actually be Rs.3.4 crores and not Rs.1.8 crores .

Sumit Gupta: Understood. Thank you.

Moderator: Our next question is from the line of Kunal Thanvi from Banyan Tree Advisory Private Limited. Please go ahead.

Kunal Thanvi: Hey, thanks for the opportunity and good morning. So, my first question was on the competition, like in our annual report, we have talked about some easing out of competition on pricing, also from the organized and unorganized players. So, if we look at some of the data points, like, say, gross margin for most all the listed players, and the kind of acquisitions we have been doing in the last 18 months or so, there are some signs, like from an outside investor, we can see there is some ease out in terms of the competitive intensity and the valuations also seems to have kind of normalized. If you can throw some more light on how the organized

and unorganized piece in terms of competition has

behaved, and what are the factors that have led to competition kind of abating in the last 18 months, it would be really helpful ?

Ameera Shah: See, if you look at the period of 2020 to 2023, it was as we all know, a black swan event. And in any black swan event, you have lots of people who look at opportunities that are hot and try to jump in. So, that is what really happened in healthcare. And especially in diagnostics, because there was so much COVID testing that needed to be done. But you saw a bunch of new corporate players who sort of said, "Oh, this is an industry which is going to grow for a long time, let us jump in. " And you also saw obviously health tech guys coming in. The reality of our industry, however, is that these black swan events, which create large revenues in a short period is not the norm . And the norm of the industry is you really have to work on the ground, building sample -by-sample, building it through brand trust, and credibility and expertise. And I think as some of the players, some new and some old are recognizing the challenges and actually building the business and the kind of mode there is, you

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are seeing in some cases, people saying, "Okay , maybe this is not necessarily what we have cut out for." You are seeing a funding slowing down. In some cases, you are seeing a funding winter, because companies have not been able to show and prove unit economics that work profitability for them. And therefore, funding is not coming for them. So, I think there are different reasons. But I think largely, I think it all comes back to commercial. The understanding is that look, the organic growth in the industry is about 8 % to 10%. And anything you want to do above that, or even to reach there , is going to take a lot of hard work and a lot of patience and long term vision. So, generally, we are seeing that there is a little bit more rationality on the pricing. And we are seeing the kind of intensity we saw between 2020 and '23 has certainly come down. The players have not gone anywhere, they are still in the market, they continue to compete. But we are not seeing irrationality, which is a good thing for the industry.

Kunal Thanvi: Sure. And when we look at the gross margin for most of the listed players, we have seen either stability or going up. So, like, is it to do with only the pricing , how do one look at it or there is some respite on the raw material side also, because if at all these listed players are gaining market share?

Surendran C: Can you just repeat the question?

Kunal Thanvi: So, my question was, when you look at the gross margins, right , they have seemed to have stapled down or they are improving for most of the listed players. Is it entirely due to pricing, rationality coming up or because the large guys have started seeing market share gains , so there is a second order impact on the raw material procurement cost?

Surendran C: Actually, it is a combination of multiple things . One , of course, the price stability is definitely coming and become more predictable. That is really helping us. And then a lot of operational efficiencies are

coming in because of automation, digitization, etc. , And then of course, as the scale goes up, the profitability gets better. So, basically, it is a combination of all these three things put together and you will see it is playing out for everyone as we go forward.

Kunal Thanvi: Sure. And the last question that I had was on, like, when we are moving from top towns to Tier-two, Tier-three towns, how do one look at the unit economics in those smaller towns , because the scale at which we will be operating in top eight cities will be very different when you are going to those smaller towns , can one expect similar kind of unit economics in terms of throughput, margins, e

tc. ,

in those smaller towns , how do you think about it?

Surendran C: Yes. So, see, now our expansion into Tier-3 and beyond will only be with respect to centers, not with respect to labs. The labs part of the expansion is already over, right? And these expansion of the centers are also happening on Tier-3 and beyond only through the franchisee route. So, from a Metropolis point of view, largely , the investments are in terms of clinician engagement, logistic, etc., which has been properly been stitched. So, your unit economics will largely be at par with the rest of the Tier-3 towns, and we do not really find any further stress on that going forward when we expand.

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And actually, we are going deeper into it. We have already reached 750 towns and we already have currently mechanism to engage with the clinicians and the logistic arrangements. Now, your question is only about going deeper and getting more volumes. So, the unit economics only get better from these towns.

Kunal Thanvi: Okay, sure. Maybe one thing was on the basic radiology that we talked about. Are margins again similar there as well compared to what we have in our core business?

Surendran C: See, it is too early days for us . Just in the last, year or so we have expanded into as Ameera said 20 centers. Now where we have both x -ray and ultrasound, and about 35 centers, we would not have x -ray alone, and 250 centers with ECG. So , very early days to look at the margin profile of these business separately. But we are sure that it will only be adding to the overall profitability because it is happening from the same centers, we have not set up extra centers for the same people. So, there is no additional cost other than the processing cost in these cases. So, the margin should only be at par or better.

Kunal Thanvi: Okay . Sure. Thank you so much and all the very best.

Moderator: Our next question is from the line of Ishpreet Kaur from Relax Capital. Please go ahead.

Ishpreet Kaur: Hi, I just wanted to check, is it possible to share the average revenue per test in B2C and B2B?

Ameera Shah: I do not think we have it off the top of our head, but I think there would be a difference of approximately 20% or more and we will come back to you maybe with some specifics , we have written down the name in detail.

Ishpreet Kaur: Is it also possible to give a breakup of volume in patient and test in B2C and B2B?

Surendran C: It is already provided . If you want me to call it out separately, I will just do that. Let me talk about organic business. The volume growth was 9% and the realization growth was 6%, adding up to a total of 16% revenue growth. And on B2B, the volume growth is 4% and realization is 6%.

Ishpreet Kaur: Is it possible to mention the numbers in terms of volume, not the growth , absolute number?

Surendran C: Absolute number on B2C and B2B. We can get back to you on that.

Ishpreet Kaur: Maybe as a practice, if it is possible to share it as a disclosure in the presentation, just a request from our end.

Surendran C: All right.

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Ishpreet Kaur: Just wanted to understand on the B2C part, considering the current test and the setup and the geographical expansion that we have, what is the kind of level of share that we could see from the B2C part in the next three to five years?

Surendran C: See, our first target now at a group level to move into the 60% plus levels. We are at a group level and we are at 56% after all the reclassification and keeping the group together. We are at 56%. Our first target is to reach up to 60% as we go.

Ishpreet Kaur: Right. And similarly, on the radiology side, which we started, do we see it as a significant revenue contribution in the next three to five years?

Ameera Shah: The basic radiology, I do not think will be a significant

contribution from a revenue perspective. If,

of course, we choose to go into high -end radiology, that would be different. But just on the question you asked around the volume in terms of tests for B2C and B2B, for B2C, I think it is Rs.37.9 lakhs for Q1 FY26 and B2B is Rs.32.8 lakhs.

Ishpreet Kaur: Thank you so much.

Moderator: Our next question is from the line of Raman K V from Sequent Investment. Please go ahead.

Raman K V : Hello ! Thank you for allowing me to ask questions and congrats on your excellent result. Sir, I just

want to understand how much of the total revenue is from B2 G, can you give a ballpark figure?

Surendran C: I must say it is negligible ; less than a percentage, if I can say so. So, in the last few calls, I have already mentioned that we are gradually withdrawing from the non -profitable , government businesses and other contracts are also finding it difficult to get our monies. We are withdrawing. The last one was Aam Aadmi M ohalla Clinic s, which we have withdrawn from 30th of June. So, today, the government business is very, very negligible actually.

Raman K V: How much was it three years back if you can quantify that ?

Surendran C: Three years back, we had a NACO, which is a big contract, which is about 7% revenue at that point of time. And in an overall, we had about 10% government revenue. Today, it is less than a percentage.

Gradually and strategically, we have withdrawn from these businesses.

Raman K V: Okay, sir. And my second question is, the main growth for any player in diagnostics is with respect to the volumes. So, can you give a guidance with respect to the volumes of just Metropolis and upon that, how much growth are you expecting for the acquired entities? And the recently acquired Ambika Diagnostics, can we expect it to grow 60% , because as you mentioned earlier, that it grew 60% this year in FY25 , can we expect it to repeat the same or not?

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Surendran C: Okay, let me answer all the questions one after the other. The volume growth for the MHL organic business, our guidance were always 7 % to 8%. We have already reached the 7% level. So, we expect to continue at 7% level, keep bettering it going forward. And at a group level, we have done 10 % to 11%. And we expect that is the level that we will do for this year, right? Coming to Ambika and the 60% growth, we said we grew it to 60% over a period of two years, right? And initially, when we took over, there are many things that we could do it and could get immediate gains, hence that levels of growth you could see. But going forward, I think we will definitely see this business growing better than maybe the MHL business in the first year or so.

Raman K V: And s ir, with respect to Core Diagnostics , so now it is like EBITDA positive, can we expect it to move shift towards higher single digit margin by the end of this year? And can we expect the revenue led by the volume growth to be above the industry average about like 30 % or 40%?

Surendran C: Well, I think the EBITDA margins definitely , our estimate is that it will become a high single digit as we go forward. And maybe, the year one exited a very high single digit number, right? And revenue growth, like Am eera mentioned, we have to do some of cleanup in the early days and this year our focus is largely to get the synergies and margins corrected and getting the business as much cleaner as good, right , and then focus on the revenue growth from the year after that.

Raman K V: Okay . Thank you, sir.

Moderator: Our next question is from the line of Girish Bak hru from Orb iMed . Please go ahead.

Girish Bakhru: Yes, actually, I have a few questions on Core as well. So, just wanted to understand this oncology, super specialty and companion diagnostics, how big is that market , do we have that number?

Ameera Shah: We do not have the number

right now at the top of our head, but we can certainly get back to you on this.

Girish Bakhru: And Am eera, how many players let us say we know that of course in organized diagnostics, there are, I mean, largely three, four handful players. Is that the same when we talk about companion diagnostics?

Ameera Shah: Not really. I mean, I think the lab chains or the organized players who are doing better on the oncology side may not be exactly the same as the top three incumbents overall.

Girish Bakhru: Okay. And when you are talking about of course taking margins in Core gradually higher, I mean, just wanted to understand one driver you have been talking about is, of course, putting more of the mix of using Metropolis test in Core and, of course, growing onco specialty overall in Metropolis using Core, which of these two drivers will essentially drive that margin faster to the Company level in Core ?

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Ameera Shah: See, Core margin to Company level will be driven more by cost synergies. Core was a good business on its own, but the chances of it making money on its own profitably was low because it was subscale , and the kind of corporate costs that were involved and the kind of lab costs that were involved would never have allowed it to make money on its own. Now, in the 1st Quarter , if you see, we have already integrated the overlapping labs in locations where Metropolis and Core both had labs. I think we have done almost five such overlaps, so we have merged them. So, like that, as you make Core leaner and you are using shared infrastructure, we really believe that over the next three, four years, we will be able to take Core to the Company margins and obviously then the revenue acceleration will also have to kick in by then by year two, where you are able to really take this to more clients and you are able to get more tests from your existing distribution and increase your productivity for customers. So, that is how we believe that Core will be come a profitable business.

Girish Bakhru: Understood. And Core increasing margins and TruHealth, of course, doing very well. Can you give

some directional color or sense on where this RPT or RPP number should go trend-wise?

Ameera Shah: For Fore or for Metropolis ?

Girish Bakhru: For Metropolis overall. As an overall entity, I am basically discussing this.

Ameera Shah: See, I think if you look at history, I think the last few years, we have been seeing a 3-5% increase in RPP every year. And that is coming from a combination of moving up the value chain and therefore selling more specialized tests to patients who need them. And that is a journey that we believe will continue for Metropolis. So, usually, pricing as you know has got some part of it, but it is not a huge part of it. The bigger part of it is really the product mix that plays an important role for it.

Girish Bakhru: Yes, but can you like see it doubling over the next five years, is that possible?

Ameera Shah: I do not think we have seen a double over the last five years. So, I think that would be requiring a 15%, 20% kind of an increase every year. So, unlikely. But I do think that the kind of trends that we have seen in the past five years, I think can sustain as we go into the future.

Girish Bakhru: Okay. Thank you.

Moderator: Ladies and gentlemen, due to time constraints, I now hand the conference over to the management for closing comments. Over to you.

Ameera Shah: Thank you. And thanks everyone for joining us. As we iterated, we feel very excited about the year '25-26 for Metropolis. It is a big year for us that we believe we are going to take a big leap forward, not only in breaking our own record for organic growth that we have seen, but also significantly improving on the margin, but really integrating the three acquisitions and making them really part of Metropolis and se

ting the stage for them to accelerate in the years to come. This year as we have

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seen in Quarter 1, we have had a 23 % growth and obviously we expect Quarter 2 to be a much better quarter than Quarter 1. Usually, we know Quarter 2 and Quarter 4 are the best quarters in the year for our business in our part of the world. And we certainly look forward to doing all the things behind the scenes that lead to the right kind of outcomes that we have discussed with all of you as shareholders. And while we continue to really build Metropolis into an even further technology-enabled organization, we feel our team is complete. We are set with a good leadership team to really take on all the challenges of this year. And we are seeing a lot of hunger and a lot of aggression on the ground from Metropolis. We look forward to sharing with you guys more updates in the next quarter and have a good weekend, everyone.

Moderator: Thank you. On behalf of JM Financial Institutional Securities Limited, that concludes this conference. Thank you for joining us and you may now disconnect your lines.

Call: Nov 2025

Ref: MHL/Sec&Legal/2025 -26/74 November 11, 2025

To,
BSE Limited
Scrip Code: 542650 National Stock Exchange of India Ltd.
Scrip Symbol: METROPOLIS

Dear Sir/Madam,

Sub: Earnings call transcript for Q 2 FY26

Pursuant to Regulation 30 read with Schedule III of the SEBI (Listing Obligations and Disclosure Requirements) Regulations, 2015, please find enclosed herewith the transcript of Q 2 FY26 earnings conference call held on Thursday , November 06, 2025.

The transcript is also available on the Company 's website i.e. www.metropolisindia.com

You are requested to take the above information on record.

Thanking you,
Yours faithfully,

For Metropolis Healthcare Limited

Kamlesh C Kulkarni
Head – Legal & Secretarial

Encl: A/a

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E&OE - This transcript is edited for factual errors. In case of discrepancy, the audio recording uploaded on the stock exchanges on November 06, 2025 will prevail

MANAGEMENT : MS. AMEERA SHAH – CHAIRPERSON & WHOLE -TIME
DIRECTOR – METROPOLIS HEALTHCARE LIMITED
MR. SURENDRAN CHEMMENKOTIL – MANAGING
DIRECTOR – METROPOLIS HEALTHCARE LIMITED
MR. AVADHUT JOSHI – CHIEF BUSINESS
DEVELOPMENT OFFICER – METROPOLIS
HEALTHCARE LIMITED
MR. SAMEER PATEL – CHIEF FINANCIAL OFFICER–
METROPOLIS HEALTHCARE LIMITED

MODERATOR : MR. TAUSIF SHAIKH – BNP PARIBAS INDIA

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Moderator: Ladies and gentlemen, good day, and welcome to the Q2 and H1 FY '26 Earnings Conference Call of Metropolis Healthcare Limited, hosted by BNP Paribas.

This conference call may contain forward-looking statements about the company, which are based on the beliefs, opinions and expectations of the company as on the date of this call. These statements do not guarantee the future performance of the company, and it may involve risks and uncertainties that are difficult to predict.

As a reminder, all participant lines will remain in the listen-only mode and there will be an opportunity for you to ask questions after the presentation concludes. Should you need assistance during the conference call, please signal the operator by pressing star then zero on your touch-tone telephone. Please note that this conference is being recorded.

I now hand the conference over to Mr. Tausif Shaikh from BNP Paribas India. Thank you and over to you Mr. Shaikh.

Tausif Shaikh: Thanks. Good morning, everyone. I welcome you all to the Metropolis Q2 FY '26 earnings conference call. I Tausif Shaikh, India Analyst at BNP Paribas for Pharma and Healthcare Service would like to thank the management of Metropolis for giving us the opportunity to host the call.

From the Metropolis team, we have with us today Ms. Ameera Shah, Chairman and Whole-Time Director; Mr. Surendran, Managing Director; Mr. Sameer Patel, CFO; and Mr. Avadhut Joshi, Chief Business Development Officer.

Now I would like to hand the call to Ameera ma'am for opening remarks. Over to you, ma'am.

Ameera Shah: Thank you so much, and good morning, everyone, and thanks for joining us today on the Q2 and H1 FY '26 earnings conference call. I'm joined by Suren, the MD; Avadhut, the Chief Business Development Officer; Mohan, Sameer and the other team and SGA. We've uploaded our investor presentation and related documents on the exchange and on the company's website, and I hope everyone's had an opportunity to go through the same.

Let me begin with a brief overview of the broader healthcare landscape before we move into the company's performance updates. The Indian healthcare sector is undergoing major change driven by rising health awareness, better access and a focus on affordability. Patients are increasingly proactive about preventive care and early diagnosis, leading to more patients -- more tests per patient. With great emphasis on quality, accuracy and credible reports, consumers now prefer organized trusted diagnostic chains over small unorganized local labs.

The competitive environment continues to remain stable with no major new entrants. We're observing a natural consolidation in the market, where many smaller unorganized labs are finding it difficult to sustain due to rising compliance, technology and quality requirements. This trend is accelerating the shift towards organized and trusted diagnostic players where Metropolis is well positioned with its brand equity, clinician trust and superior quality standards.

Talking about our quarterly performance, it's an exciting time for us at Metropolis as we continue to deliver strong broad-based growth of 23% year-over-year, accompanied by sustained margin

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expansion. This consistent trajectory reinforces the strength of our strategy, the resilience of our business model and the disciplined execution by our teams across the country.

We had earlier emphasized automation and digitization as key strategic priorities, and I'm pleased to share that we are seeing strong progress. Over the past few quarters, we have rolled out several digital initiatives, including a new consumer app, a partner app for our partners and lead automation for them as well, along with middleware auto authorization in our labs, helping improve

our TAT, which is our turnaround time, and enhanced inventory management system, driving material efficiency and AI call quality monitoring in our contact centre for improved customer experience.

Together, these initiatives are not just strengthening our operational backbone, but also elevating customer experience, quality, consistency and scalability, all of which are critical to sustaining our leadership position in the premium diagnostics segment.

On the acquisition front, our focus during the year has been on stabilization, process integration and synergy realization. It's only been a few months, but the priority has been to standardize operating procedures and drive cost efficiencies as per plan. I'm pleased to share that we are fully on track with the expected synergy realization and timelines.

Our integration efforts are progressing smoothly, both at the technology and people level. Teams have been harmonized under a unified performance framework, while product portfolios are being aligned to strengthen cross-selling opportunities and enhance local market relevance. As planned, year 1 is about integration and efficiency, while year 2 will be focused on scaling revenue growth over the strong foundation.

For the current year, our clear focus at Metropolis remains at margin expansion and operating leverage with multiple levers in motion, better cost controls, improved test mix, automation

benefits and productivity gains. Importantly, we are not planning any new acquisitions over the next 6, 9 months as we believe the immediate opportunity lies in organically scaling the business and strengthening key strategic drivers already in motion.

Our genomics journey continues to make steady progress, leveraging Core Diagnostics as our base platform. The integration of Core has enhanced our capabilities in high -end molecular diagnostics, oncology and next -generation sequencing, and obviously improved our front -end connect with oncologists.

We are now expanding the test menu, deepening collaboration with clinicians and building a stronger R&D -driven test development pipeline. The upcoming centre of excellence in Delhi will serve as a hub for innovation, training and scientific excellence, further reinforcing our leadership in research -driven diagnostics.

On the GLP -1 therapy front, we are closely tracking developments as this class of drug expands its presence in metabolic and weight management treatments. These therapies necessitate regular and repeat diagnostic monitoring to assess their efficacy and safety. And we see a long -term structural opportunity here to leverage our deep clinician network to co -create testing protocols,

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packages and wellness plus disease management pathways aligned with GLP -1-based treatments.

Lastly, the Metropolis moat continues to strengthen. Our differentiators remain unwavering commitment to quality, wider accessibility and a deeper customer -centric approach. These core strengths have resulted in high customer and doctor retention, a top-ranking Net Promoter Score and a steady increase in revenue per patient and revenue per test.

Our focus has always been on the quality of business rather than just volume, ensuring that every customer interaction builds trust, loyalty and long -term customer value. In essence, we are building a resilient future -ready diagnostics company, one that balances growth with profitability, science with empathy and scale with precision.

With this, I hand over the call now to Suren, who will take you through the company's strategic updates and operational performance for the quarter and half year ended September 2025. Suren, over to you.

Surendran Chemmenkotil: Thank you, Ameera, and good morning, and a very warm welcome to everyone joining this call. We are pleased to report that Group revenue for qu

arter 2 and H1 of financial year '26 grew by 23% year -on-year. Metropolis revenues on an organic basis grew by 12% in quarter 2 and margins stood at 26.8% as compared to 26.2% in quarter 2 last year, an improvement of 60 bps in line with our guidance.

TruHealth and Specialty segment revenues grew by 21% and 15%, respectively, on a year -on-year basis for organic business. Group level EBITDA margin is at 25.4% in quarter 2 with Core Diagnostics EBITDA improving in line with our plans to high -single -digit, moving from breakeven in March. The same was at 23.1% last quarter.

Speaking of the second quarter, fever -related test volume did grow, but the growth was slower than the usual monsoon season and per expectations, primarily due to lower incidence of common infections such as chikungunya, malaria, dengue and viral fevers. Despite this, Metropolis remained well positioned by quickly realigning focus areas and emphasizing growth in Specialty testing and TruHealth offerings. This proactive approach allowed us to offset the decline in seasonal volumes, maintain overall business momentum and revenue growth of 23% year-on-year.

Our B2C segment contributed 59% of total revenues for the quarter, recording an 11% year -on-year growth for organic business. This double -digit growth was driven by strong performance in our TruHealth and Specialty segments, supported by targeted micro marketing initiatives and sustained clinician engagement programs.

Our Mumbai revenues for quarter 2 grew by 13% year -on-year despite lower fever -related cases, reaffirming our strong leadership in Mumbai market. B2B segment accounted 41% of total volumes, registering a 14% year -on-year growth for organic business. This growth was driven by an increase in both patient volumes and realizations.

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Our clinical trial business also delivered a strong contribution in this quarter, aligning well with our specialty focus. With continuous enhancement to our partner portals, improved transparency and greater service efficiency, we remain optimistic about achieving higher volumes in the

coming quarters.

As Metropolis enters into the next phase of growth, enhancing productivity and operational efficiency will be key to driving sustainable progress. Our focus is on maximizing output while maintaining superior service quality through the core levers of driving people productivity, process and lab operation efficiency through AI and digitization. This 6 to 12-month road map is designed to lay the groundwork for scalable and profitable growth and continue to position the company for leadership in Specialty.

As part of Metropolis 3.0 strategy, we continue to focus on the following things: one, accelerating our presence in new markets with focus on Tier 2 and Tier 3 cities. Following the completion of our lab expansion program in last year, we are now expanding our network of collection centres to feed these labs and increase throughput.

In H1, we added approximately 200 centres with a plan to add another 300 more centres in H2. We are now present across 750 towns, as we said earlier, and are going deeper in these towns by adding new clients and clinician connects and increasing our brand visibility amongst the consumers.

Secondly, strengthening our high-end specialized testing capabilities by enhancing AI-based allergy testing, AI-based diagnostics and expansion in genomics segment and many other areas. Third, enhancing the TruHealth portfolio through science-led bundling, doctor consultation and an integrated approach with basic radiology, allowing us to address the higher ends of the market as well as the lower rural packages for Tier 3 and Tier 4 towns.

Fourth, integrating digital and AI-driven tools to enhance service quality and operational efficiency, supporting sustainable growth and scalability.

In conclusion, I would like to highlight

that our H1 performance has been largely in line with our guidance, achieving an organic revenue growth of 12% to 13% and margin expansion of approximately 40 bps on a year-on-year basis so far. We remain confident in our ability to meet the stated estimates in H2 this year. With these strategies in place, combined with the disciplined execution, a strong leadership team and alignment around common vision, we are confident in our ability to further build and reinforce our leadership position nationwide.

With this, I hand over to Sameer to take you through the financial numbers for the quarter and H1.

Sameer Patel: Thank you, Suren, and good morning, everyone. Let me now share some of the key financial performance for quarter 2 FY '26. As informed in the previous quarter, we have bifurcated our performance reporting on 2 aspects for the current year. MHL Group includes 3 acquisitions of Core Diagnostics; DAPIC, Dehradun; Scientific Pathology, Agra; and second, MHL Organic excludes these 3 acquisitions.

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Also change in definition for B2C and B2B segment to streamline the same with the industry standards. B2C includes all owned franchisee and rural centres and B2B includes B2B lab, hospital, government, corporate and clinical trials.

Moving to the financial and operational performance. First, I would like to highlight operational performance for MHL on organic basis. Revenue and EBITDA grew by 12% and 14.5%, respectively, and PAT grew by 13.6% on a year-on-year basis. Patient volumes stood at 3.6 million, a growth of 6% on a year-on-year basis.

Test volumes stood at 7.4 million, a growth of 6% on a year-on-year basis with increasing contribution from TruHealth segment, we consider one profile as one test, which is different from peers. On a like-to-like basis as peers, this number would be significantly higher.

Our B2C revenue stood at INR 230 crores, a growth of 11% on a year-on-year basis and B2B revenue growth stood at 14%. TruHealth and Specialty segment grew by 21% and 15%, respectively. As per the revised classification of B2C segment, B2C contributes 59% of total revenue. Revenue for Specialty B2C segment grew by 16% and B2C TruHealth segment grew by 20% on a year-on-year basis. B2B revenue contributes 41% of total revenue and B2B Specialty grew by 15%.

EBITDA of MHL on organic basis stood at INR 104.8 crores, a growth of 14.5% on a year-on-year basis. EBITDA margin for MHL Organic stood at 26.8%, increase of 210 basis points on a sequential basis and 60 basis points on a year-on-year basis. PAT for MHL on an organic basis stood at INR 53 crores at a growth of 13.6% year-on-year basis. PAT margin of MHL Organic stood at 13.6%, an increase of 20 basis points on a year-on-year basis.

Moving to key performance indicators for MHL Group. Revenue for the Group grew by 22.7% on a year-on-year basis. Patient volume stood at 3.7 million and test volume stood at 7.9 million, a growth of 11% and 12%, respectively. B2C revenue grew by 16% and B2B revenue grew by 33% on year-on-year basis.

On MHL Group basis, revenue from North now contributes 19% of overall revenue, largely because of recent acquisitions, which has its major presence and revenue coming from North region. Revenue growth from Tier 1 towns stood at 26% and Tier 3 towns stood at 13% on a year-on-year basis. Revenue from Tier 3 town now contributes 24% of total revenue. Contribution from B2C revenue stood at 56% in Q2 FY '26. The decrease in B2C compared to organic business of 59% is largely on account of consolidation of Core Diagnostics, which has majority of revenue contribution from B2B.

EBITDA margin for MHL Group stood at 25.4%. The decrease was largely attributed to a lower margin profile of Core Diagnostics. However, we are happy to report that margin for

or Core has

increased from low -single -digit to higher -single -digit in Q2 FY '26 compared to quarter 1 FY '26. PAT for the MHL Group stood at INR 52.9 crores with a margin of 12.3%.

That's all from my side. With this, I open the floor for the question and answers.

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Moderator: Thank you very much. The first question is from line of Anshul Agrawal from Emkay Global. Please go ahead.

Anshul Agrawal: My first question is on margins. How should we look at margin trajectory going ahead? I mean, you have posted the healthy organic margin expansion. Should we expect this to continue with ramp -up in Core as well in the latter half of the year or would we stick to our margin guidance previously of almost flattish because of Core dilution?

Ameera Shah: Suren?

Surendran Chemmenkotil: Yes. So if you remember beginning of the year, when we stated the estimates, we said for this full year, the organic business will do about 70 to 100 bps margins better than the previous year, and the previous year was 24.3%. And we said we will move up by 70 to 100 bps, and we stay committed to the same number even at this point of time.

And we said for the overall Group, there will be dilution of 1% to 1.2% because of the lower margins of Core. And that also by the time we exit this year, Core will be close to 2 -digit margins. So these are the, I mean, statements we made in the beginning of the year. And all these remain same at this point of time. We are heading towards the same direction.

Anshul Agrawal: Just a follow -up to understand this better. Our organic business margins have surpassed the expectations that we would have had at the beginning of the year. So question was from that perspective.

Surendran Chemmenkotil: Yes. So Anshul, what happens in quarter 2 is relatively a high -volume quarter, and hence, your margins will be definitely better than the average margin of the year. And hence, it's not necessary that the quarter 2 our margins will be the same going forward in quarter 3 and quarter 4. So for a full year basis, from a 24.3%, you would see 70 to 100 bps upside by the time we close this year for the full year. That will be the organic business trajectory.

Anshul Agrawal: Got it. This wouldn't be dependent on our B2C and B2B mix, or would it?

Surendran Chemmenkotil: Well, I think there will be some impact of the B2C, B2B mix, but we are not envisaging any significant shift in the B2C, B2B mix going forward, which is at currently 59%, 41% (19:44 Inaudible) and the B2C contribution can be a little higher by the time we exit this year.

Anshul Agrawal: Got it. Second question was on TruHealth. In your opening remarks, you mentioned that we are adding certain radiology tests in the TruHealth packages. Would this sort of be limited to sort of filling up prescriptions or would we think of venturing into advanced radiology at this stage?

Surendran Chemmenkotil: So currently, the plan on TruHealth with respect to radiology is that the basic radiology services like the ECG and then X -ray and ultrasound, these are 3 things that we are basically adding on to the TruHealth packages. And we have a capability of adding now the ECG across the country and the X -ray and ultrasound around 5 towns in the country. So that we now got integrated into the TruHealth packages. And not high end in this scheme of things for the rest of the year, we are not including the very high -end radiology like MRI or a CT scan, etcetera.

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Anshul Agrawal: Got it. The last question that I had was on our volume or top line guidance. I understand it's a stable pricing environment as such with more tailwinds around GLP -1 drugs and specialized testing. Would we sort of retain our top line guidance as we stated at the start of the year?

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Surendran Chemmenkotil: Yes. We want to maintain top line guidance, like I mentioned, for the rest of the year as well.

Moderator: The next question is from the line of Senjoy Chakraborty from BNP Paribas Securities India.

Senjoy Chakraborty: So my first question is on the lab testing charges. Although it's a very small amount compared to your other operating expenses, on a half yearly basis, the expense has almost close to doubled. Just wanted to understand what is the reason behind this? And will we see the same quarterly run rate in that expense in the third and fourth quarter as well?

Surendran Chemmenkotil: Just give a moment. You're talking about lab testing charges, right?

Senjoy Chakraborty: Yes, that's right.

Surendran Chemmenkotil: Yes. So I think H1, 60% of the lab testing charges are because of the Core, outsourcing charges of Core. But otherwise, nothing has significantly changed. And it's only because of the integration of Core and the trends will remain same for the rest of the year.

Senjoy Chakraborty: My second question is on your revenue for this quarter. So would it be possible to quantify how much of the revenue from your acquired assets came from each of the assets? Like how much revenue came from Core and Dr. Ahuja's and Scientific Pathology, if that's possible?

Surendran Chemmenkotil: Well, I think the overall put together is 11% of the revenue growth has come from all the integrated entities. Maybe, we can talk later to give you the details of each of the entities have contributed to each.

Senjoy Chakraborty: That sounds okay. If I can just squeeze in one more. Coming back to the margin front, if I'm correct, earlier you had guided high -single -digit margin for Core. And given that you have already are at high -single -digit margin in the second quarter itself, so are you still sticking to your earlier guidance for Core or do you think there is a scope for low -double -digit margin for FY '26? And how do you see the margins.

Ameera Shah: Yes. No, for FY '26, I think we'll stick with the high -single digit, because as you know, Q1 was I think about 2%, 3% margin. Q2 is, like we said, is sort of mid -to-high single -digit. Q3, we hope to move to a higher -single -digit and then Q4 move to sort of closer to a double -digit. So the average for the year will still come out to a high -single -digit, which gives us a good foundation to make a leap into next year to obviously move into a double -digit margin on track for what we had sort of projected when we actually made the acquisition.

So I think the synergies are playing out as we expected. But usually in these acquisitions, what we've seen is whenever you buy one of these businesses, there is a certain amount of practice clean -up that has to be done, ways of working changes that has to be done, integration that has to be done.

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And in services businesses, this not only takes a little bit of time, but it also means that sometimes you may stop certain businesses, et cetera. So whatever clean -up needs to be done, we are doing it in year 1 on the revenue side so that you don't carry that on into the future.

Moderator: The next question is from the line of Shyam Srinivasan from Goldman Sachs.

Shyam Srinivasan: Ameera and Surendran, just the competitive intensity at this point of time, maybe in B2B also, if you could kind of give us some qualitative comments. Your B2B revenue growth is now starting to trend better. So I just want to understand that. And is there any impact of this reclassification on any of these numbers?

Like it looks like B2C percentage contribution has gone up to 59%. but on a like -for-like, I can see what you have moved to in terms of definition, but where were our classifications earlier? And does that have a material impact on your growth rates for both of these 2 segments?

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Ameera Shah: So I'll answer the first question. I'll let Suren take the second one. So just talking on the competitive intensity, see, obviously, during COVID, we saw a bunch of health techs jump in, we saw a bunch of pharma guys jump in, hospital guys. We haven't really seen any new players jump in, in any significant way in the last couple of years.

So on the B2B side, while some existing players are intensifying their efforts, we haven't seen any price competition escalation. We are seeing some amount of stability for that in the last 1 or 2 years now. So it's still obviously an intensely competitive environment, which it always has been, I mean, for many, many years to come. But there's nothing new or nothing disruptive that actually spoils the market.

So also the reflection of the B2B growth being higher in Q2 is not necessarily only about our execution getting better, which it is partly, but it is also about the nature of tests that are generated in the quarter.

So this quarter, we saw more specialty and more acute business in the market and less of routine, which is represented through lower B2C growth because of the lesser routine, but higher specialty growth and higher B2B growth. So some of it is just the product mix, which is specific to the quarter. But yes, our execution on B2B is also getting stronger as we go. Suren, if you want to take the question on the B2C contribution.

Surendran Chemmenkotil: Yes. Shyam, if you remember in the last year, through all these discussions, we have mentioned that we are just cutting off some of the low margin and difficult B2B corporate, government

business, et cetera. That clean -up has actually done last year and then we are working on a very cleaner base. That's point number one.

And point number two is, and after that, the institutional business has really become relatively much smaller. And hence, we are now calling the institutional business and B2B together. So the remaining changes in the percentage is only a mathematical rearrangement after that.

Otherwise, on the base, nothing fundamentally changed.

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And in quarter 2 also, specifically, I must tell you that we have got one good contract on clinical trial. That has helped us to get our B2B better than the normal run rate in quarter 2. So that was a one quarter specific project that we had.

Shyam Srinivasan: Got it. Helpful. Just the second question is on GST impact. I know there is some benefit that accrues to you from a sourcing perspective. So just want to understand, are you calling out the quantitative impact or I know maybe you have to pass it on. So just how in the second half it flows through to you?

Surendran Chemmenkotil: All right. So see, there are GST impact on 2 aspects. One is on the reagent and consumables and second one on the equipment when we purchase. Now coming to the reagent and consumables, we already had a part of the consumables and reagents at 5% GST only. And some of the consumables and reagents have moved from 12% to 5% GST.

So not a very significant, what you call, benefit as such. But whatever benefit we will get, we want to pass it on to the customer one way or the other. And whatever GST benefit that we will get out of the equipment purchases, that will actually come on to the procurement efficiencies.

Moderator: The next question is from the line of Raman K.V. from Sequent Investments.

Raman K.V.: Sir, during the quarter and during the first half, how much was the revenue from radiology segment?

Surendran Chemmenkotil: Radiology segment is very, very small for us. We are early days on radiology segment in a very, very small very low -single -digit percentage if you ask me.

Raman K.V.: Okay. Basically, it doesn't have a substantial contribution towards your entire consolidated.

Surendran Chemmenkotil: No. As of now, no

Raman K.V.: So with the TruHealth and Core Diagnostics, how are you planning to increase your radiology segmental revenue? Are there any plans?

Surendran Chemmenkotil: So Core Diagnostics, there is nothing on radiology. It's purely whatever we do. So let me give you some information. So we have about 35 centres across the country now fully equipped with all the basic radiology testing capabilities, which is the X -ray, ultrasound, 2D Echo and ECG, etcetera. That's 35 centres in about 5 cities now fully equipped and we will - start driving the radiology in these centres and also get the portfolios, including radiology in these 5 cities. So that is one opportunity for us to drive forward.

And second thing is ECG capabilities now we have almost done across the country. So that will also start playing out. These are the 2 opportunities for at least for the rest of the year. These are the 2 levers that are available on the radiology segment, and we will start building this portfolio stronger going forward from here.

Raman K.V.: Understood, sir. Sir, my second question is with respect to the clinical trial. Can you explain the business part of it? And how do you see the traction in that particular? Like how much is it contributing to your overall pathology volumes and revenue?

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Surendran Chemmenkotil: Well, I think clinical trial is a very small percentage of the business. Only in the quarter 2, we got a contract, which we delivered in quarter 2. Basically, this is about working with the pharma companies and other alliances to do some turnkey projects. So it's not one of the things which we keep, all the time depending upon, but it's a relatively small part of the business.

Raman K.V.: So what are the margins in this business? Better than the consolidated margins or it's less?

Surendran Chemmenkotil: Yes, it's a little better than the company level margins.

Raman K.V.: So are we planning to take this sort of turnkey clinical trial projects or is it like you're trying to be consolidated?

Surendran Chemmenkotil: We are one of the key players in this segment. The size of the market itself is not very big here. But we are a significant player in this market and there are some other small operators as well. So we really reach out to all the opportunities available and then we'll keep grabbing these kind of opportunities as and when it comes.

Raman K.V.: Sir, my final question is with respect to the Core Diagnostics. So by the end of this year, in the

call, you mentioned it will move towards the double -digit by the end of this year. So can we expect that by first half of FY '27, the Core Diagnostics margins to be in the similar level of Metropolis?

Surendran Chemmenkotil: No, no. So we said in the first half of the year, it's a single -digit number. We will come closer to a double -digit number by the time we exit this year. And full year basis, we will be still a very strong single -digit number. And we said in 3 years period, we will bring this into the current levels of Metropolis margins. It will take another 2, 2.5 years more to bring up to the Metropolis levels of margin. But the efforts are on, and we are in the right direction.

Raman K.V.: Okay. Basically, you mean it will come to around 23%, 24% margin by FY '28, if I'm correct?

Sameer Patel: Third year.

Surendran Chemmenkotil: Third year, yes, third year of acquisition.

Raman K.V.: And on the volume front, you had a strong revenue and volume growth in the first half. Are we planning to maintain our mid -double -digit volume growth for the entire year or are you expecting much better than that?

Surendran Chemmenkotil: At consolidated level, our volume growth will be in double -digit numbers, 10% to 11% for rest of the year as well.

Moderator: The next question is from the line of Ankeet Pandya from Baroda B

NP Paribas Mutual Fund.

Ankeet Pandya: So I have 2 questions. So firstly, like we have completed our major lab expansion last year. And this year, we are investing in collection centres. So from FY '27, FY '28 onwards, can we see some increase in the growth numbers from current 11%, 12% to around mid -teens kind of a growth over the next 1 to 2 years?

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Surendran Chemmenkotil: Well, a little too early for me to give you a projection for the coming year. Give me another couple of quarters more to give you the right projection for the coming year. But the effort is always to bring the growth level to the next level as we go forward.

Ankeet Pandya: Okay. Sir, lastly, on the GLP. I know, of course, it is too early to give any guidance or numbers. But have you seen any traction in terms of increase in test in obesity or weight management kind of a thing over the last 1, 2 quarters, given that there's one molecule that has already been launched. So has there been any increase in test in this space?

Surendran Chemmenkotil: At this point of time, not anything that's significantly visible in terms of the growth per se, but we are quite hopeful that this will help the diagnostic operators in the days to come. I mean, the full-fledged availability of this medicine and usages in India has still not happened. I think it may happen sooner than later as per our information.

But then once they start using, then the diagnostic testings will happen to keep getting the reports in between when they take the medicine. So I mean, we are hopeful that this will help us in the coming quarters.

Moderator: The next question is from the line of Surya Narayan Patra from PhillipCapital India Private Limited.

Surya Patra: My first question is on the Core Diagnostics integration. So whether the integration is fully complete by now? And the related question is that, since last 2 quarters that we are witnessing a strong double -digit growth on the revenue per patient. Anything because of the Core Diagnostics that this number is getting influenced positively?

Ameera Shah: So I'll just add something and Suren you can add on to it. See, any time you do an acquisition, what we find is, like I mentioned in India, there are various kind of practices that different organizations follow.

So what we have seen historically is that every time we do an acquisition, we have to actually spend the first many months to clean up some of the past practices that may not be cohesive going forward for Metropolis. And that usually dips the revenue of the acquired entity for the short term.

So the revenue growth in the first 6 months of the year would have been impacted Core negatively for any business that we already stopped and that we are cleaning up. So as this gets cleaned up, it sets the foundation for a faster growth in the future. So we'll hopefully see that playing out. So that obviously will impact the overall MHL growth as well.

The second part about your question about is the integration done? See, in services businesses, you're integrating largely people and cultures along with technology and processes. The technology and processes and infrastructure part is a little easier to do. The people and culture part obviously takes a little longer than 6 months.

So that will continue to be a work in progress. While the policies and guidelines and ways of working have been integrated, but culture and some of those things will take a little longer to

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get done. But we have already -- and maybe Avadhut and Suren can talk to this, we have already -- the replication in infrastructure has been already removed.

So wherever we had duplicate infrastructure, that has been shut down, wherever we could get synergies from procurement, corporate co

st, that has been done. But there are obviously some new costs that have come in like technology, audit, other things, which they may or may not have been doing in the same way as we are choosing to do today. Suren, Avadhut, if you want to add on anything?

Surendran Chemmenkotil: No, I think you mentioned that. So he asked the question of whether integration is complete or not. No, it is not. It's work in progress. I think we will take maybe a couple of more quarters to do the full integration completed. And yes, patient volume growth, there is definitely some impact of the Core Diagnostics volumes also helping us in terms of the overall patient volume growth for the company.

Surya Patra: Sure, sir. Second point, a related aspect. See, because of the Core integration, so now possibly the service offering quality and capability would have obviously seen a kind of improvement. So because of that, whether we have seen any upgrade to our TruHealth portfolio or package? And also recently, what we have added the ECG to this thing, TruHealth, so whether these 2 Core integration as well as ECG, whether that has created any uptick in the average realization for your TruHealth packages?

Ameera Shah: Just to clarify, Core Healthcare, Core Diagnostics, we acquired because it is a genomics -based platform specific in oncology. So therefore, the customers that you're selling to are mostly oncologists as well as oncologists sitting inside hospitals, right?

And now as we continue to expand our genomics portfolio to beyond oncology to neurology and obviously, to women and child, etcetera, which we are already doing, this will obviously add all of these other customers, but this will not impact your TruHealth profile because your TruHealth packages are largely more routine tests which are things where consumers are making decisions to come and sort of do a bundle of a check-up every year.

There are some people who talk about using genomic tests for packages like this, which can give you a predictive or a preventive outlook. But just to clarify on that, those things are not still scientifically proven. So by doing it from a preventive basis, doing a genetic test for a preventive basis, the data does not exist for India today that allows you to make a good correct scientific decision on the back of that. So we do not offer genomic tests as part of a preventive profile at this point of time.

So it will not affect the TruHealth packages. The TruHealth packages RPP is moving up because we are able to provide more test to the patient scientifically that is relevant for them and patients are willing to pay us a higher price for these packages for the brand premium, the quality of care and the accuracy that we bring to the table maybe compared to others.

Surya Patra: Yes. My next question was about the AI benefit. So see, obviously, this is theoretical so far that, okay, AI initiatives are benefiting us. But have you seen any tangible benefit so far? Could you say otherwise that, okay, what is the kind of investment on the AI -related aspect or upgradations?

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And what tangible benefit that you are either have started seeing or anticipating to see going ahead? And in what form that you will be seeing the true benefit here?

Ameera Shah: See, just to talk on AI, I mean, AI has been talked about from 2 different types, right? One has been on productivity and efficiency where you are able to replace manual labour on the non - medical side and be able to make your processes smarter and therefore, have more productive workforce, right? The second part is talking about using AI in the medical tests where you don't need doctors and all of that. So I want to separate the two.

When you're talking about AI in non -medical, I think there are real use cases for it. There are organizations across industries who

are using AI, let's say, for recruitment, for screening profiles, for talent onboarding, for billing, for all kinds of things, for accounting. And I think those are real life use cases which are across industries. I think those are very much doable and implementable to get a tangible benefit from.

We are early in that journey, but there are a bunch of things that we are trying to implement that will hopefully get us there over the next year or so. And that obviously will be a continual journey work in progress. So we are investing resources and time in that area.

The second part of AI, which is using AI for medical tests where you don't need doctors is a very

hypothetical scenario. In our minds, AI is a tool to empower doctors to make better decisions, not a tool replacing doctors. And therefore, again, it's about productivity within the doctor space. Today, AI, you get these softwares or tools from the U.S. or Europe, which you cannot plug and play into the Indian ecosystem because you need Indian data sets that make these AI models relevant for the Indian patient, which at this point doesn't exist yet. And I think it's very much work in progress for the whole industry. It will take a few years for those to become relevant for the market here.

From a medical standpoint, you are seeing AI used for large -scale screening. For example, TB diagnostics are being done using AI by the State of Maharashtra, where chest X -rays are being used using AI, etcetera. But this is not a confirmatory diagnosis. It 's a very screening tool. So the precision is not there today to give a 100% accurate report from. I hope that answers your question.

Surya Patra: Yes, yes, really ma'am. Just last one point with your permission. So now obviously, that you are seeing the sweating of assets after creating significant network expansion. But simultaneously, hence, in the process that you are talking about expanding the pick-up points of the collection centres .

So is there any kind of a standard equation that you think you will go up to that level in terms of the collection centre addition looking at your network expansion? So whether any equation between like collection centre to lab or collection centre to kind of your patient service centre , such equation you have in mind?

Surendran Chemmenkotil: Well, I think neither in the company nor in the industry, there is no such ratios that lab versus collection centre per se. But we are currently at about 20, 22 centres per lab that we have, if I Metropolis Healthcare Limited
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look at across the country. The first destination is we would like to take this to 30 centres per lab. So that may happen maybe in the next 12 to 18 months' time. That's the first destination. Then we will look at the lab profitability and lab profitability has to start getting better. So that's one aspect.

And second aspect is most of the service centre addition that we are talking about is through the franchisee route. And very few of this will be happening through the own centres . Only in top 5 or 6 cities we will be adding our own collection centre . Rest all will be happening through the franchisees. So not much of impact that will happen on the overall profitability of the company while we increase the collection centres .

Moderator: The next question is from the line of Aashita Jain from Nuvama.

Aashita Jain: Just couple of questions. Firstly, on the organic growth of 12%, what percentage could be attributable to the price hikes that we took last year? And do we have any plans to take any price hikes in the coming year?

Surendran Chemmenkotil: Okay. Well, I think roughly 2% of the growth must be attributed to the price hike. And we do not have any plans to increase the prices during this financial year for sure.

Aashita Jain: Sure. And secondly, on the Core Diagnostics, we did mention that we a

re doing clean -up in the

other acquisitions as well. But how should we see the revenues of the Core Diagnostics for this year? And how should we see Core Diagnostics as well as 3 the other acquisitions, these portfolios growing in the coming years? Would it grow faster than our standalone growth rate?

Surendran Chemmenkotil: I think Ameera clarified to you, I mean, in one of the earlier questions about the overall growth, like because we are just cleaning up the business. So we are not necessarily looking at the year 1 so much about the revenue growth. We're just making sure the whole portfolio is very sound, clean and stronger.

That's the year 1 focus. And year 2, of course, we will be taking the revenue growth in all these acquired entities. And definitely, it should further help the overall growth of the Group in the coming years.

Aashita Jain: Understood. But can these grow faster than our organic growth?

Surendran Chemmenkotil: Well, I think just give us a couple of more quarters to give you more specific answers to this. And I think we will do the estimation as we come closer to the end of this year.

Aashita Jain: Sure, helpful. And just lastly, a book -keeping question. When I look at this quarter, below EBITDA line item, the depreciation has gone up, and the other income has gone down. So anything to read into this?

Surendran Chemmenkotil: Yes, just a minute.

Sameer Patel: Other income has gone down because last year we had a cash which we had utilized in the month of March to acquire Core. And that's why the investment income has come down, and that's the reason for the other income coming down. And depreciation, certainly, we have added assets

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last year during the year, which has now has the annualized effect of the depreciation. So overall, that's where it's gone up.

Moderator: The next question is from the line of Yogesh Soni from InCred Capital.

Yogesh Soni: Most of the questions have been already answered. I wanted to understand one thing on the test volume front. If one has to understand on the industry volumes based on doctor consultation tests, routine monthly tests, which are being done by patient and emergency test, can you help me understand, I mean, how much each of these tests would have a contribution in the overall volume front? If not for Metropolis, then in general for the industry?

Surendran Chemmenkotil: Well, not very handy with these numbers. So maybe we'll have to connect separately so at least we can give you the best answers.

Yogesh Soni: Okay. And just to get a sense on the Core Diagnostics scale -up that you mentioned during the start of the call for FY '27. What kind of growth are we expecting for Core once the integration is completed and we realize the initial synergies in FY '26?

Surendran Chemmenkotil: I think that's exactly what I thought I have clarified in the earlier question as well. So by the end of this year, we will be able to start giving you the correct projections around each of the entity, how much they will grow. But I think I said, overall, this should help further increase the growth of the company. That's what we will be looking at.

Moderator: The next question is from the line of Pranaya Jain from Banyan Tree Advisors Private Limited.

Pranaya Jain: So I have 2 questions. Number one is, can you talk about the consolidation that is happening in the industry? In the last 1, 1.5 years, has the pace of consolidation increased? And can you throw some colour on it based on geographies, say, like is West consolidating faster than some of the other geographies? That is question one.

And the second question is, can you throw some more light on the clinical trial business of yours? Like we wanted to understand who are your customers, what kind of projects are these and so on? Like some colour on clinical trial business

ess will be very helpful.

Ameera Shah: Sure. I'll start with the industry and what we're referring to as consolidation. See, in our industry, the biggest good and bad has been that the sort of barrier to entry has been low and which is why you saw lots of small players, and we saw lots of new larger players enter.

What people realize after spending 2, 3 years in the industry is that while the barrier to enter is low, the barrier to scale is not so low. It's actually hard. And the barrier to scale profitably is extremely high.

So therefore, after a few years in the industry people who entered quickly, land at sometimes losing interest in the business because they feel that, okay, this is either going to take extremely long and it's not going to be so easy to actually make money from it. So therefore, then people slow down their aggression in the market and some of them shut down and some of them just continue to stay there, but they don't stay very aggressive and active.

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Now at the same time, what's happening in the last 5 years, you've seen 3 changes. One, you've seen great technology adoption in healthcare because of which it naturally leads to the more organized players doing better because they have more capability and capital for technology. The second, you've seen consumers beginning to choose brands because about 2,000 players became more brand visible during COVID because these were the ones doing COVID testing versus the 300,000 players in India. And therefore, you are seeing that now consumers are preferring brands even in healthcare and in diagnostics.

And that makes it harder for the small single lab owners who are non-branded and don't have chains. So I think a combination of these trends is leading to the organized sector growing a little faster and the small independent labs actually struggling to make sort of end's need.

Now even in the independent labs, there are 2 varieties. You have the technician labs, and you have the pathologist run labs. The pathologist run labs, which tend to be of better quality are still doing fine, are able to survive. The challenge they are having is more in a succession plan because often their kids don't want to pick up medicine or pathology and therefore some of them land up shutting their labs when they get older.

The technician labs, which are the very subscale labs, which don't really have a moat, or they don't really have a differentiator are the ones who are really struggling and are actually shutting down.

Now the data third-party data around this doesn't really exist in the industry. So whatever we can share is more from a pulse of the industry and our observation. So we don't really have data to tell you is it specifically adding in more markets. But I do believe that the more markets which

are more educated and aware, which is more West and South, will certainly see probably more consolidation in my mind than markets of North and East where education is lower and therefore the focus on quality is lower.

And also markets where there is little access, like UP, Bihar and other markets, you may still see mushrooming of labs versus in the bigger cities it's harder to survive financially because of now higher competition and compliances, and therefore, probably the smaller labs will have a harder time. So I hope that gives you enough insight into that area.

Pranaya Jain: Yes, that was very helpful. Can you throw some light on my second question as well.

Ameera Shah: Clinical trial, yes. The clinical trial piece is where we really work with pharmaceutical companies. And pharmaceutical companies when they want to introduce a drug into India, either a drug that exists or a new one, they need to do clinical trial studies before exposing it to humans and selling it commercially. That's Phase I to IV. We usually partici

pate in 2, 3 and 4.

We work with these companies where we do all the testing for their clinical trials. They like to partner with companies who have the ability to pick up samples from all across sites in India, have the ability to maintain the high quality that they need, to keep all the documentation and the audit trails that are required for them to get approval from Indian FDA or U.S. FDA.

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So this business tends to be more structured and more difficult to enter compared to sort of the normal lab business. And that's why there are very few of us in the market who are able to do what we do. And I think I could be wrong, but I think that we are probably the largest player from testing in clinical trials or the number 2, if Avdhut or Suren want to add anything on that. It's not a very large market, but the market is a fairly profitable market and one that we've established a brand in.

Surendran Chemmenkotil: Yes. I think that's what on the clinical trial, the larger customers of pharmaceutical companies and healthcare research institutes to the specific question that you asked.

Pranaya Jain: This will be only for domestic testing, right?

Surendran Chemmenkotil: Not necessarily.

Ameera Shah: This will be MNC pharma as well as Indian pharma companies, both. And it will also be sort of CROs who are operating globally or in India who would be our customers.

Pranaya Jain: Any colour on how big this business is for us?

Ameera Shah: We don't have that separate segmentation actually at this point. So like I said, it's not a very large market, but it's a market that's growing. And every year is different because it depends on how fast approvals come in for drugs. So some years, it's low, some years, it's high. So while we put an effort into it, it's not a business that we expect to become extremely large.

Pranaya Jain: Understood. And it won't be ROE dilutive, right?

Ameera Shah: Not at all.

Moderator: The next question is from the line of Aditya Chheda from InCred Asset Management.

Aditya Chheda: So my question is on the centre expansion. Notably, over the last 3 years, the expansion in owned service network/ centres has been faster, almost doubled versus the franchisee growth. So is it a function of the North market where we need to open more owned centres or if you can explain about the dynamics of the same?

And since you classified rural separately, would those be a mix of franchisee and owned? And the last question is your -- if you can quantify the lab network and centre network expansion in number that you would be doing for FY '26? Yes, these are the questions.

Surendran Chemmenkotil: Well, I think the number, let's say, 2 years back, the own network numbers were relatively lower. That's why last 2, 3 years, you find more number of owned centres coming up. And then most of the labs also come along with the owned centre. So we have added more than 100 labs in the last 3 years. So with every new lab addition, one more centre also get added. So that's how in all the key cities, you will find the own network has really gone up.

And the franchisee network is otherwise largely the way that we will grow the collection centres in the days to come. And in the rural areas, definitely, there is no owned centres. I mentioned it earlier; only top 5 cities or 6 cities will have our owned centres. Rest all will be the franchisee

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centres. And like I said, for the rest of the year, we are estimating another 300 more centres to come up.

Moderator: Ladies and gentlemen, due to time constraint, this was the last question for today. I now hand the conference over to the management for closing comments.

Ameera Shah: Thank you so much, everyone, for joining us today for the Q2 call. We r

emain very excited and

optimistic about the trajectory for our growth. This has been a big year for us with organic growth doing well, but plus doing these 3 or 4 acquisitions that we h ave done in the first half of this year.

It's a lot to digest, but we are happy to say that we are on track, and the team continues to plug away and deliver the results on the acquisition front as well as on the organic front. We believe the second half of the year to continue to be quite stable. And we'll continue to focus our energies on really boosting produc tivity, boosting and accelerating organic growth in the next 6 to 9 months.

And obviously, by March, April, when we come back to you after Q4, we will be very happy to share our thoughts and plans for the next year and which direction that we believe we'll go in.

But we believe we are in a very positive time at Metropolis and very excited about this year and the years to come.

So look forward to chatting with you guys more at the end of Q3. Q3 is obviously not always the best quarter in the year. It's usually the lowest because of festivals. And however, we continue to put our best efforts to do the maximum that we can. Thanks, everyone.

Moderator: Thank you very much, ma'am. On behalf of Metropolis Healthcare Limited, that concludes this conference. Thank you for joining us. And you may now disconnect your lines.

Call: Feb 2026

Ref: MHL/Sec&Legal/2025 -26/94 February 1 1, 202 6

To,
BSE Limited
Scrip Code: 542650 National Stock Exchange of India Ltd.
Scrip Symbol: METROPOLIS

Dear Sir/Madam,

Sub: Earnings call transcript for Q 3 FY26

Pursuant to Regulation 30 read with Schedule III of the SEBI (Listing Obligations and Disclosure Requirements) Regulations, 2015, please find enclosed herewith the transcript of Q3 FY26 earnings conference call held on Thursday , February 05, 202 6.

The transcript is also available on the Company 's website i.e. www.metropolisindia.com

You are requested to take the above information on record.

Thanking you,
Yours faithfully,

For Metropolis Healthcare Limited

Kamlesh C Kulkarni
Head – Legal & Secretarial

Encl: A/a

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"Metropolis Healthcare Limited
Q3 FY '2 6 Earnings Conference Call "
February 05, 202 6

E&OE - This transcript is edited for factual errors. In case of discrepancy, the audio recording uploaded on the stock exchanges on February 05, 202 6 will prevail .

MANAGEMENT : MS. AMEERA SHAH – CHAIRPERSON AND WHOLE -
TIME DIRECTOR – METROPOLIS HEALTHCARE
LIMITED
MR. SURENDRAN CHEMMENKOTIL – MANAGING
DIRECTOR – METROPOLIS HEALTHCARE LIMITED
MR. SAMEER PATEL – CHIEF FINANCIAL OFFICER –
METROPOLIS HEALTHCARE LIMITED
MR. AVADHUT JOSHI – CHIEF BUSINESS
DEVELOPMENT OFFICER – METROPOLIS
HEALTHCARE LIMITED

MODERATOR : MS. NISHA SHETTY – ICICI SECURITIES

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Moderator: Ladies and gentlemen, good day, and welcome to the Q3 FY '26 Earnings Conference Call of Metropolis Healthcare Limited, hosted by ICICI Securities. This conference call may contain forward-looking statements about the company, which are based on the beliefs, opinions and expectations of the company as on the date of this call. These statements do not guarantee the future performance of the company and it may involve risks and uncertainties that are difficult to predict.

As a reminder, all participant lines will be in the listen-only mode, and there will be an opportunity for you to ask questions after the presentation concludes. Should you need assistance during this conference call, please signal an operator by pressing star, then zero on your touch-tone phone. Please note that this conference is being recorded.

I now hand the conference over to Ms. Nisha Shetty from ICICI Securities. Thank you, and over to you, ma'am.

Nisha Shetty: Thank you, Subham. Good morning ...

Moderator: Sorry to interrupt, Ms. Nisha, you're not audible. Your line is not clear.

Nisha Shetty: Yes. On behalf of ICICI Securities, I welcome you all on Q3 FY '26 earnings conference call of Metropolis Healthcare Limited, and I thank the Metropolis Healthcare management team for giving us this opportunity to host this call. Today, on this call, we have with us Ms. Ameera Shah, Chairperson and Whole-Time Director; Mr. Surendran Chennakoti, Managing Director; Mr. Sameer Patel, Chief Financial Officer; and Mr. Avadhut Joshi, Chief Business Development Officer.

I will now hand over the call to Ms. Ameera Shah for her opening remarks. Thank you. Over to you, ma'am.

Ameera Shah: Thank you, Nisha, and good morning, everyone, and thank you for joining us today for the Q3 and 9-month FY '26 earnings conference call. As mentioned, I'm joined by our team. Our investor presentation and financial results have been uploaded on the stock exchange and our website, and I hope everyone has had an opportunity to review them.

And I'll begin with a quick, brief overview of the industry landscape and our strategic priorities, after which we'll go through the operational and financial performance for the quarter. Over the past few quarters, the diagnostics industry has continued to demonstrate stable demand with growth increasingly driven by specialty diagnostics, preventive health care and complex testing, rather than routine investigations alone.

Clinician behavior and patient awareness are steadily shifting towards greater emphasis on accuracy, depth of insight and precision-led decision-making. At a global level, there has been considerable discussion around tariff actions and trade policies, particularly by the U.S.

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However, we'd like to clarify that healthcare and diagnostics is predominantly domestic in nature with localized sourcing and service delivery and remains largely neutral to these developments. And at this time, we do not see any material impact on our operations and cost structure.

The recently announced Union Budget signals a clear priority for health care, spanning pharmaceuticals, hospitals, diagnostics and allied services. A strong focus on workforce development, including upgrading existing allied health institutions and setting up new ones across public and private sectors aims to add nearly 100,000 professionals across 10 disciplines over 5 years, strengthening diagnostic and clinical capacity. The budget also emphasizes Tier 2 and regional health care infrastructure, which should reduce patient migration to metros and improve access for underserved populations.

Overall, it reinforces a long-term policy commitment to expanding health care access, capacity and quality across the value chain. For a health ca

re leader like Metropolis, this will create

opportunities that did not exist earlier, especially in smaller markets of India.

Within India, the diagnostics sector is gradually entering a phase of consolidation. The phase of unreasonable capital and poor unit economics seems to be over, and the phase of consolidation is back in place. Financial and strategic capital is scouting for larger acquisitions with a stronger focus on long-term growth and strong economics rather than discount-led revenue growth.

In this evolving environment, Metropolis remains well positioned given our strong brand recall among doctors and consumers, scientific leadership and pan-India network of labs and collection centers. We continue to believe we have a strong moat via our trusted brand, quality processes and scaled infrastructure and continue to scout for high-quality and reasonably priced acquisitions.

A key milestone during the quarter was the launch of our Centre of Genomics established in Delhi following the integration of Core Diagnostics, which now functions as our national

reference hub for advanced molecular testing and precision diagnostics, supported by our CAP accredited laboratories in Gurgaon and Bombay.

Together, this integrated platform of Center of Genomics brings high throughput sequencing, advanced bioinformatics and strong clinical interpretation capabilities, enabling us to deliver clinically -actionable genomic insights at scale across oncology, reproductive health, inherited disorders and preventive genomics.

This category of tests are growing fast in the industry. And at Metropolis, we believe we are building the capabilities to become very significant players in this category. This will be a key strategic growth lever for the next few years, generating business via B2C and B2B, a category of tests that requires precision, experienced interpretation by experts and a broad network of centers, falling perfectly into Metropolis' right -to-win.

Alongside genomics, we continue to progress on our digital and AI journey in a measured and responsible way. While the potential of AI and pathology is significant, we believe that meaningful clinical adoption is still at an early stage. And therefore, our focus remains on selective high -impact use cases rather than broad -based deployment. Currently, AI is being

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applied in a few targeted areas at Metropolis, including enhanced test interpretation, strengthening quality monitoring, supporting scientific selling and enhancing customer conversion. At the same time, we are actively working on a pipeline of future applications across report interpretation, clinical decision support, workflow optimization and automating certain processes.

Over the medium to long term, we believe AI will play an important role in improving productivity and report turnaround time and strengthening quality governance in pathology. AI can play an enabling role to improve productivity and analytics. But at this time, it is unlikely to replace doctors or disrupt the industry cost structure overnight. As part of our science -led innovation journey, we achieved an important milestone this quarter with the grant of our first patent by the Patent Office of the Government of India for a system focused on monitoring and managing TB -related infections.

To our knowledge, this makes Metropolis one of the first listed diagnostic companies in India to receive a patent of this nature, reflecting our commitment to scientific research and clinical innovation. Adopting this tool may help us connect deeper with the prescribing doctors treating TB patients and assisting them in diagnosis and treatment.

On the inorganic front, our recent acquisitions are progressing well and are well integrated into the Metropolis system. At Core Diagnostics, the primary focus over the past quarter

has been

on operational alignment, process standardization and cost synergy. The Q3 margins for Core were expected to be slightly higher than Q2 because the plan was to launch the genomics platform in Q3 '25, which would have meant higher gross margins for these genomics tests. However, due to delay of machine arrival into India, these got launched 3 months later in January '26. And these tests were therefore outsourced at lower gross margins earlier, resulting in a lower EBITDA. However, now these tests have started in -house as of January '26. And in Q4, we expect a meaningful improvement in gross margin, supporting a high single -digit EBITDA margin for Core in Q4 of this year.

Our smaller acquisitions, including Dehradun, Agra and Kolhapur continue to perform as expected, delivering margins better than the group average. Overall, by year -end, we expect our acquisition portfolio to demonstrate visible synergy benefits across revenue growth and test mix improvement and channel diversification.

Looking ahead, Q4 is typically a seasonally stronger quarter for the diagnostics industry. While seasonality patterns have become less predictable in the post -COVID environment, we are seeing encouraging momentum entering the quarter. With sustained traction across channels, improving productivity and better mix, we remain confident of delivering on our guidance of 12% to 13% organic growth rate and expanded margins for FY '26, along gradual uptick in margins on the back of scale and operational efficiencies, material productivity and prudent cost management.

To conclude, Metropolis remains firmly committed to building a high -quality science -led diagnostics platform anchored in accuracy, consistency and clinical trust. With disciplined

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strategy execution and strong operational leadership, we believe we are well positioned to deliver sustained long -term value for all stakeholders.

Lastly, the company is pleased to inform its shareholders that the Board of Directors has

approved the issue of bonus shares in the ratio of 3:1, that is 3 fully paid-up equity shares of face value of INR 2 each for every 1 existing equity share, subject to the necessary statutory and shareholder approvals.

This bonus issue made by capitalizing a part of the company's reserves reflects the company's strong financial position and continued confidence in its long-term growth prospects. This initiative aims to reward shareholders for their continued support while improving the liquidity of the company's equity shares in the market.

With this, I'll now ask Suren to walk you through the operational performance and the business drivers for the quarter.

Surendran Chemmenkotil: Thank you, Ameera, and good morning, everyone. We are pleased to report a strong performance for quarter 3 with group revenues growing 26% year-on-year and the 9-month revenues increasing by 24% on a year-on-year basis. Organic revenue growth remained healthy at 15% for the quarter and 13% for the 9 months, supported by balanced volume expansion, improving test mix and disciplined execution across businesses.

What is particularly encouraging this quarter is the all-round performance across segments, geographies, patient volumes and margins. Q3 is typically a seasonally softer quarter for the diagnostic industry, driven by festive holidays, reduced outpatient footfalls and lower elective procedures. In addition, this year, we also witnessed a typical seasonal behavior in quarter 2, with the usual infection-led diagnostic demand not materializing as expected.

Recognizing these early trends, we proactively recalibrated our operating focus, accelerating our efforts to build high-quality B2B and institutional business in order to create a more stable and predictable demand base entering quarter 3. This strategic pivot helped us

sustain growth

momentum despite the absence of seasonal tailwinds and deliver balanced performance across B2C and B2B segments.

Over the last year, I have spoken about our efforts towards the structured cleanup of lower-quality institutional accounts, exiting businesses that were margin dilutive, operationally inefficient or misaligned with our service and quality standards. This year, we have systematically built a pipeline of better quality institutional and corporate partnerships with sharper focus on clinical engagement, service excellence and sustainable unit economics. This changed approach to institutional business has now started yielding tangible results, contributing meaningfully to our quarter 3 performance and improving the overall quality of revenue mix.

On the organic side, revenue grew by 15% year-on-year, supported by healthy patient volume growth of 9% year-on-year. In the B2C segment, which contributed approximately 60% of our revenues, the revenue grew by 15% driven by strong performance in TruHealth and Specialty. Metropolis Healthcare Limited

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TruHealth continued to scale well, supported by curated wellness packages, increasing preventive health adoption and deeper digital engagements. Specialty testing delivered robust growth through focused clinician engagement, scientific upselling and wider deployment of advanced tests across centers.

Importantly, the integration of core diagnostics has begun to meaningfully enhance our specialty portfolio, enabling us to cross-sell super specialty and advanced oncology tests into the broader Metropolis customer base and clinician network. This cross-selling leveraging capabilities is strengthening our leadership in complex diagnostics and creating a sustainable pipeline for higher value test growth.

In the B2B segment, which contributed to 40% of revenues, 15% year-on-year growth was driven by both volume expansion and improved realization. We deepened our relationship with key institutional clients, laboratories and hospitals through dedicated account management, faster turnaround times and enhanced service quality. Specialty testing within B2B demonstrated strong traction, reflecting growing clinician confidence in our advanced diagnostic capabilities. Our corporate business played a particularly important role in stabilizing performance during this quarter by delivering predictable volumes, healthier realizations and more resilient revenue base. Geographically, our sustained focus on Tier 2 and Tier 3 markets continue to yield strong results. Over the past 2 years, we have systematically expanded our footprints across cities and towns, building hub-and-spoke network that enable efficient scaling.

During this quarter, we added 110 new centers, most of it through the franchisee, taking total addition to more than 300 centers in 9 months and strengthened our presence in 750 towns now. This network expansion is now translating into consistent volume growth, improved asset utilization and operating leverage.

On the margins, for the organic business, we delivered a strong performance this year with a healthy 29.3% year-on-year expansion of EBITDA margins, remaining robust at about 25%. The improvement was driven by a combination of material cost efficiencies, operating leverage

on fixed costs and sustained productivity initiatives across the network.

Our focus on disciplined cost management, procurement optimization and workflow efficiency continue to translate into structural margin improvements. The quarter also included a onetime impact on account of Labor Code implementation of about INR 9 crores at group level, which would have effect on profitability. Excluding this, the underlying performance remained very strong, reinforcing the quality and sustainability of our earnings.

Quality continues to remain the cornerstone of our operating

philosophy. At the beginning of

this year, we launched the Internal Quality Control Index, a scientific performance framework designed to establish uniform, measurable and real-time quality benchmarks across the entire laboratory network. This framework tracks the full diagnostic journey from the pre-analytics and logistics to laboratory examination and post-analytical reporting using rigorous clinical and operational parameters.

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It enables early identification of deviations, rapid corrective actions and consistent diagnostic accuracy across geographies. In parallel, we continue to monitor our pre-analytical compliance in test across all collection centers and logistic touch points, strengthening sample integrity and chain of custody even before samples enter the laboratory. Together, this initiative significantly enhance governance, transparency and clinical reliability across the expanding network.

Looking ahead into quarter 4, we are encouraged by early momentum across channels, the seasonality turning favorable, continued traction in specialty and wellness, expanding network reach and sustained productivity improvements, we remain confident of closing the year on a very strong note in line with our growth and margin objectives.

To conclude, quarter 3 reflects the strength of our operating model, combining strategic agility, scientific leadership, disciplined execution and quality governance. We remain well positioned to sustain momentum and deliver long-term value creation.

With this now, I hand over to Sameer Patel, our Chief Financial Officer, to walk you through the financial performance. Thank you.

Sameer Patel: Thank you, Suren, and good morning, everyone. Let me now share some of the key financial performance for quarter 3 FY '26. As informed in the previous quarter, we have bifurcated our performance reporting on 2 aspects for the current year. MHL Group includes 4 acquisitions of Core Diagnostics; DAPIC, Dehradun; Scientific Pathology, Agra; and Ambika Diagnostics, Kolhapur. MHL organic excludes these 4 acquisitions.

Moving on to the financial and operating performance. First, I would like to highlight operational performance for MHL on an organic basis. Revenue and EBITDA grew at 15% and 29%, respectively. PAT, excluding exceptional items, grew at 52% on a year-on-year basis. Patient volumes stood at 3.3 million, a growth of 9% on a year-on-year basis. Test volumes stood at 7 million, a growth of 8% on a year-on-year basis with increasing contribution from TruHealth and Specialty segment.

Our B2C and B2B revenue both grew by 15% on a year-on-year basis. TruHealth and Specialty grew by 25% and 16%, respectively, on a year-on-year basis. B2C revenue contribution, 60% of total revenue. Revenue for Specialty and TruHealth in B2C segment grew by 17% and 29%, respectively on a year-on-year basis. B2B revenue contributes to 40% of total revenue and B2B Specialty grew by 16% on a year-on-year basis.

EBITDA stood at INR93 crores, a growth of 29.3% on a year-on-year basis. EBITDA margin stood at 25%, an increase of 280 basis points on a year-on-year basis. PAT, excluding exceptional items, stood at INR 48 crores, a growth of 52% year-on-year basis. Impact of Labor Code amounting to INR8 crores is treated as exceptional item. PAT margins, excluding exceptional items, stood at 12.8 %, an increase of 300 basis points.

Speaking about key performance indicator for MHL Group. Revenue grew by 26% on a year-on-year basis. Patient volume stood at 3.5 million and test volume stood at 7.3 million, a growth of 14% and 13%, respectively on a year-on-year basis. B2C and B2B revenue grew by 19% and 35%, respectively, on year-on-year basis.

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On MHL Group basis, revenue from North now con

tributes 17% of overall revenue, largely

because of recent integration, which has its major presence and revenue coming from North region. EBITDA margin stood at 23.4%. The decrease compared to the organic business was largely attributable to lower margin profile of Core Diagnostics. The margin for Core

Diagnostics is expected to meet the stated guidelines of high single-digit as an exit run rate for quarter 4 FY '26.

PAT, excluding exceptional items, stood at INR 51 crores, a growth of 63% year-on-year basis. Impact of Labor Code amounting to INR 9 crores is treated as exceptional item at a group level.

PAT margin, excluding exceptional items, stood at 12.6%, an increase of 280 basis points.

Estimated capex for the year is INR 55 crores to INR 60 crores. This is largely towards strengthening and expanding our pan-India network, upgrading IT systems and processes, scaling up high-end equipment to support advanced and super specialty test capabilities.

Speaking about liquidity position and cash strength, the company remains net debt-free with a current cash reserves of INR 127 crores. We will continue to accumulate cash to fund and support our future growth initiatives. That's all from my side. With this, I open the floor for question and answers.

Moderator: The first question comes from the line of Surya Narayan Patra from PhillipCapital India.

Surya Narayan Patra: Sir, my first question, is it possible to share what is the organic volume growth that we would have seen in the test side?

Surendran Chemmenkotil: Organic test volume growth is 8% for this quarter.

Surya Narayan Patra: Okay. And my second question was on the genomic initiatives. So, we have added equipments that -- and we have expanded the capability there. And this is also considered to be a kind of a value growth driver for us going ahead. But concurrently we know that genomic has never been a kind of high-volume growth area for India so far, and there is a kind of a rising competition on that front also with disruptive prices that it is talked about. So, what is your thought process here and how do you see this opportunity really contributing incrementally in terms of value terms?

Ameera Shah: Look, if you look at Metropolis' positioning, we have never really been the company who is focusing on tests which are commoditized in high volume. We've been the company who's focusing on making sure that all the places that we are really working in are non-commoditized areas, which are really focusing on giving the right report to the patient when it's required in the clinical setting.

Genomics, if you look at the therapeutic areas which are growing the fastest in the world are actually oncology and neurology over the next 10 years, and the same situation is going to be in India. And for cancer and for any brain or nervous system areas, finally, the most important tests are going to be genomics.

So, we expect this category to grow very fast over the next 10-odd years and longer. And therefore, being a critical player in that market is important. And the business that comes, there Metropolis Healthcare Limited

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you're right, there are at least 7, 8 players in the space, but it's only 7, 8 compared to all the other tests where you have 3 lakh players.

So comparatively, you're talking about still this being a cohort which has lesser competition.

And even amongst the 7, 8 or 10, 15 players who are doing genomics, we have to remember that the doctors are going to need the most quality and precise report, which is where Metropolis' positioning will stay. And we are finding that our pickup of genomics has been very rapid since we have launched the test.

Surya Narayan Patra: Next question is on the network expansion side. Generally, it is also believed that there is a [inaudible 0:24:41]. Now, clearly, Metropolis is focusing more about spl

itting of the assets after

creating a strong network expansion over the recent years. and that's why there is a kind of a marginal moderation in the modern network addition front that we have witnessed. But simultaneously, that is also considered to be a kind of a key growth driver. So, given that your thought process in the subsequent period?

Surendran Chemmenkotil: So, I mean, we have mentioned this in the past also, largely the laboratory expansions are mostly done with, to service the 750 towns that we are focusing. Now the focus is on going deeper into the 750 towns and improving and enhancing the customer service network there, and mostly through the franchised channel. So that's what we have been doing in the last 2, 3 quarters, and we continue to do that, and we find enough and more opportunity to further expand our network in the 750 towns, and we'll keep doing this and get more volumes and revenue growth from the Tier 3, Tier 4 kind of cities.

Surya Narayan Patra: Okay. Just last one point from my side, sir. See, we have seen in the opening remarks also, you mentioned ma'am that there is a kind of a volatile situation in terms of the currency, in terms of the trade issues, everything that we have witnessed. So, while this industry is immune from all that, given it is a domestic one, but any impact that one should anticipate to the reagent cost and all that?

Ameera Shah: See, at this point of time, we don't anticipate anything. But look, I mean, if the rupee runs away like crazy, there is obviously always a chance that vendors can come back and potentially talk about renegotiations. But having said that, a lot of our reagents and vendors are actually more

EU-based and maybe even some Japan -based and less, I would say, U.S. So, I don't see there to be too much of an exposure at this point of time.

Moderator: The next question comes from the line of Sudarshan Agarwal from Axis Capital.

Sudarshan Agarwal: Congrats on the good set of numbers. I was just looking at the data point wherein you kind of say that you have kind of closed down 4 -odd labs in this quarter. Can you tell me, is this to do with Core or the recent acquisitions and what drove this close down? And is there any more closures expected in the upcoming quarters?

Surendran Chemmenkotil: Well, I think all the laboratories we had closed down in the last 2 quarters are largely because of integration. Wherever there are duplicate labs available in a town, we have just decided to go with only 1 of the 2. And that's always been the stated strategy that we had. And there are a couple of more labs that we may shut down in the quarter 4 as well and the numbers could be a

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little more lower than what we are starting the quarter with by the end of this year, right? So, you can expect maybe 2, 3 more labs getting integrated.

Sudarshan Agarwal: Got it. And in terms of your volume growth, as you kind of guided, it will kind of improve. It has come at around 8 -odd percent. Q4 is near term, but in a longer term, do you expect this mid - to-high single -digit patient volume growth kind of to sustain?

Surendran Chemmenkotil: Yes, 7% to 8% volume growth has always been our aspirations in the near future, and we are working towards it, and we have started seeing the early successes on our programs, and we continue to believe that 7% to 8% is a range that we can strike up in the days to come.

Sudarshan Agarwal: Got it. And lastly, on Core Diagnostics. Now, given the nature of these testing, I would believe that there is a lower portion of seasonality related to these oncogenomic tests, right? So, I think sequentially, we would have seen a sharper drop in Core based on certain assumptions that I've made. What drove this? I understand that there was some delay in machines, et cetera, but sequential drop in revenues has happened

. Is that understanding correct?

Surendran Chemmenkotil: No, there is no sharper drop as you have been just mentioning. A slight drop because in quarter 3, normally because of festivities, et cetera, the procedures comes to a halt and it comes back in quarter 4. So, a very marginal drop in quarter 3. But I mean, we are seeing quarter 4 coming back in full form.

Moderator: The next question comes from the line of Shyam Srinivasan from Goldman Sachs.

Shyam Srinivasan: Just first one on GST impact from a pricing perspective. In Q3 or maybe in Q4, is there a change to how, given all the reagent lower GSTs, how does this pan out for us? Is it already reflective in our RPP for Q3? Or we should expect realization to change in Q4?

Surendran Chemmenkotil: See, there is a very marginal impact on the GST for us. I mean, what actually has happened, if you have observed it, the GST on materials into the diagnostic was at around 12% and it's moved up to 5%. And some of the reagents were already at 5% and some of them are at 12%, 18% has come down to now 5%.

So, there's a marginal benefit that we actually get comes into our results. But if you also know that in January, every year, we used to do a price revision. So, this year, we have delayed the price revision and it did not happen in January, basically so that at least the customers continue to get the benefits of these revisions.

Shyam Srinivasan: Got it. So just understanding the price revision again because I was going to come to that second question. Is the environment conducive for price increase from a competitive standpoint? Yes, while GST benefits need to be passed on, do you think there is a revised time line now of when you can relink at pricing for yourself?

Surendran Chemmenkotil: Yes. We see the market is definitely may be able to absorb some price increase and the environment is conducive. But we are delaying it, deferring it for the reasons I've already mentioned. I mean when exactly we will do it, that's still a decision that we have to make. We

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will keep observing the market for some more time. And then at appropriate time, we will use the price levers as well.

Shyam Srinivasan: Very helpful. Last question, just on like medium -term thought process around guidance for top line only. If you are able to do 6%, 7%, I'm just using patient volume growth, 6%, 7%, somehow test volume and patient volume for you seems to be same number, similar numbers. And you add RPP 5%, 6% and then mix change, let's assume, right? Which should come in RPP. Why are we not able to grow faster than the 12% to 13%?

Surendran Chemmenkotil: Well, I think we have delivered a 15% growth in quarter 3, as you could see it, right? And in the days to come, of course, you can expect improved performance on the top line.

Shyam Srinivasan: Okay. So, there's a -- maybe in Q4, when you tell us fiscal '27 guidance, maybe that is when

you'll outline what you're going to grow next year?

Surendran Chemmenkotil: Yes. See, the estimate for this year was 12% to 13%, and we would like to hit the higher end of the 12% to 13% bracket that we have been talking about it. And hence, a similar trend that you will see it in quarter 4 as well.

Moderator: The next question comes from the line of Lokesh Manik from Vallu m Capital.

Lokesh Manik: Just a couple of questions on my end. One is on routine testing, we've seen a growth of 12%, whereas your industry peers actually have seen a dip in that segment. So, would this be attributed to the geography that we are present in ? Or would you attribute this to our strategy of focusing more on volume, which has been our strategy for the last 3 to 4 quarters or maybe more? So, some light on that would be helpful.

Surendran Chemmenkotil: See, the routine and semi -special in fact, all the test volumes category -wise that we have

mentioned in the investor deck is at a consolidated level, right? And so, in the routine and semi -special numbers that you are seeing 15% to 16% growth in quarter 3 is including all the new entities that we added during this year.

Lokesh Manik: Yes. Sir, even in organic, we have grown 12%, if I'm not mistaken.

Ameera Shah: In organic, the test volume growth is 8%.

Lokesh Manik: No, I'm talking about the revenue growth, the segmental revenue that you give. So, I was just wondering if it was driven by geography that we are in the West of India that is driving it? Or this is more a focused strategy that we have?

Ameera Shah: Not really. I mean, as at the end of the day, it's your execution strength. So, it's not so much about the geography actually, if you see North is probably the fastest -growing geography for us. So, I think it's about the strategy and the execution of it.

Surendran Chemmenkotil: 12% is routine and 9% is semi -special.

Lokesh Manik: Understood. This was on the back of the peers underperforming. So, I thought maybe just wanted a clarity where it was coming from. That is one. Second is the GLP drugs which are

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coming off patent, so do we have a focused strategy out there to take advantage of testing requirements that will come because of this? Or do we believe that the requirements are pretty much fulfilled in the existing test menu that we are offering to customers? So, just your thoughts on that.

Ameera Shah: See, the GLP therapies that are all coming out will all require a baseline testing to find out what's happening in the body to moderate dosage, along with a continuous monitoring to see whether it's actually effective and not harming the body in any way. So, our expectation is that like any other drug, if people are going to take medicines, they are going to have to monitor their blood levels as the underlying effect.

So, of course, with GLPs coming in a big way, we do expect that our GLP packages as well as all the other products we have launched will pick up in a fairly bigger way. And we've launched all of these GLP -1 packages already.

Moderator: The next question comes from the line of Kaustav Bubna from BMSPL.

Kaustav Bubna: So, I keep seeing some sort of rumors keep floating around WhatsApp. I know that some of them are really baseless, but about Reliance entering the genomics field and charging very low rates for testing. So, how is this or how strong are these rumors? Could you speak what you're hearing on the ground and how this affects your genomics business, if at all?

Ameera Shah: Look, I think difficult to comment, obviously, on their strategy or what they're doing, but there was already a business, a genomics business, which was in distress, which they then acquired from, I think, NCLT. And I don't know what their goal with it is because obviously, they are not in the larger space of pathology.

But what we are told is that it's more of a data strategy, which is more D2C and more of a screening test and not a diagnostic test. The difference between the 2 is you can do a screening test, but you can't actually take any decision unless it's a diagnostic test because that is more confirmatory versus screening test gives you some info, but you can't really take any action on it. But of course, the time will tell sort of which direction it goes in.

I think Metropolis' positioning is very different, which is we are all in the diagnostic space, which means we work very closely with prescribing doctors for very complex diseases to say that, look, this is definitely what's happening in the genome in the body, and therefore, this is the action that you can take from it.

So, 2 different strategies, a little bit like if I were to give a comparison, there are many organizations doing wellness screening, which may give you a sort of

broad indication of what's

happening in your body, but you can't actually take a decision on it versus a Metropolis, which will give you very final results where you'll take action based on it.

Kaustav Bubna: Understood. Understood. Great. And just one more question on the Labor Code . I kind of understand what could happen over the next 5 plus, 10 years, obviously, over a very long period of time that can reduce the unorganized competition. So, obviously, government regulation is one, which I don't think much has happened on, correct me if I'm wrong. But even Labor Code
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acts as some form of government regulation. Do you see that as a possible catalyst to reducing unorganized competition in any way over time?

Ameera Shah: Look, I mean, I think, definitely, with government coming in with stronger regulatory frameworks, whether it's on the wage code, whether it's on the quality framework, certainly will help organize the unorganized sector. Obviously, but this depends on enforcement, and it depends on whether people follow the rules on the ground in the unorganized sector.

But by the way, we are also seeing, for the first time, I would say, in a very long time, we are seeing the government very intent on sort of creating deregulation and appropriate regulation in multiple sectors across the country, including health care, and an intent to want to give some compliance framework, which is focused on not only access and affordability, but also quality care. So, I'm actually quite hopeful that we might actually find the sector starting to organize in a better way compared to what I've seen in the last 20 years.

Moderator: The next question comes from the line of Tausif Shaikh from BNP Paribas.

Tausif Shaikh: Ma'am, is it possible, can you share the 9 -month organic revenue growth from Northern India? The idea is just to understand whether Metropolis' positioning has changed after acquisition in the Northern market, mainly the B2C market?

Surendran Chemmenkotil: Yes. I mean, the northern part of the country is growing faster for us than the rest of the country. And I think we have just translated in terms of the contribution, which has gone up from 9% to 17%, right, in the last 3 quarters after the acquisitions.

Tausif Shaikh: So, this would include Core Diagnostic as well, right?

Surendran Chemmenkotil: Correct.

Tausif Shaikh: I would appreciate if you can highlight organic revenue growth over there.

Surendran Chemmenkotil: Well, at this point of time, I won't be able to give you the specifics, but I think it's growing faster than the company's revenue growth, I can tell you that.

Moderator: The next question comes from the line of Anshul Agrawal from Emkay Global.

Anshul Agrawal: First question is on organic margins. I believe our organic margins for the 9 -month period have expanded by about 100 bps. If I recollect, this was a slated target. have you seen any tailwinds from GST, et cetera, which have sort of resulted into this expansion? Or do we expect this 100 bps sort of expansion in margins to continue as our lab network sort of matures?

Surendran Chemmenkotil: So, Anshul, I've already mentioned that the GST impact is very marginal. And the margin expansion is largely on the back of the material productivity improvement and other productivity initiatives we have taken in the organization. And at the beginning of the year, we have made an estimate of 70 to 100 bps improvement in the margins, and we stay put with that. You can see that coming in the coming quarter as well.

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Anshul Agrawal: Second question was on our radiology foray. Any comment on how that piece is sort of shaping up? And any plans to sort of enter into the advanced radiology space even a pilot basis?

Ameera Shah: I think

we are looking at 2 spaces. One is on the low -end radiology space and how we can potentially scale that up. I think we had mentioned earlier that we already made some progress on this in this last year, where now a fair number of our centers have got ECG for sure, but also in some cases, X -ray and sonography, and we'll continue to sort of scale that up. And there are multiple opportunities, we believe, that can be leveraged through that model.

On the high -end radiology, we are certainly exploring to see whether we can pilot a couple of centers and sort of see what kind of results we get. And the idea would be obviously to leverage our very strong brand with doctors, with consumers, along with obviously a brand for technical excellence and very good customer service. So, we will certainly try to see if we can pilot something and then see the results of it before we scale.

Anshul Agrawal: Got it. The third question I had was on preventive health care testing on our proven business.

The results have been very strong. Could you help me from what I understand, while online players sort of created this market for even organized players, still on pricing front, preventive health care packages by organized players as well as us are slightly or meaningfully higher versus the online players. What according to you is driving growth in this segment? Is it maturity of the population at large? What exactly do you believe is driving this growth in preventive health care testing for organized players as well as Metropolis?

Surendran Chemmenkoti I: See, 3 things, I think we have mentioned in the past and I reiterate that 3 things are helping on the preventive health, which is wellness and the bundled packages growth. One, of course, is a much better awareness that we are able to create. As an industry, we are able to create much better awareness. And secondly, the affordability the need for doing these preventive checkups are also getting better.

And in our case, particularly, our ability to, I mean, engage with our customer base on a regular basis on a customer life cycle management process and digital channels, et cetera, is getting better. So, a combination of all this, I think we are seeing the TruHealth portfolios expanding every quarter.

Ameera Shah: So, just to add to that, I mean, I think in health care, we'll often find that, like you said, even though the market was created by the sort of digital health tech players, finally, the most important component is trust, consumer trust. And I think that even though the market creation happened, I think the ones who probably benefited more are the incumbents where there was already a brand trust created between the consumer and the institution. And then, obviously, it depends on how you execute that and whether you're able to pick up that market creation.

Anshul Agrawal: Sorry. Just a follow-up on that. So, from what I can gather from your comments, pricing is probably not a key lever in determining what customers are choosing or the diagnostic player that they are choosing from in preventative health care. Would that be a fair assumption to make now?

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Ameera Shah: Look, I think that it would not say that price is not an important factor in preventive health care. I think in illness and curative health care, it's definitely not an important factor. But in preventive health care, I think it's still an important factor, but it's not the only factor. We have to remember that finally, if somebody is buying something, they still have to trust what comes out of it. So, I think the quality of it, the acceptance by the doctor, the convenience, the experience that you go through, these are all other very important the interpretation you give, is there a doctor doing a consultation with you at the end of it. These are all very important parts of a

So how

people choose a wellness package. Price is one of them, but definitely not the only and most important factor.

Surendran Chemmenkoti: And Anshul, we also created a lot of value into the overall health check packages of last 3, 4 quarters. We have introduced the ECG. We have introduced vitals checking, and we introduced doctor consultation within Metropolis. So, the customers are seeing value in all these things and hence, the repeatability and all that is getting better.

Anshul Agrawal: Got it. Very clear. Just one last bookkeeping question from my end. Could you help us understand what is the capex number for the 9-month period?

Surendran Chemmenkoti: The 9-month period is about close to INR 40 crores at a group level. And we think that for the full year basis, this could be something close to INR 60 crores.

Moderator: The next question comes from the line of Tanya Chowdhary from Investec.

Aakash: Aakash, here. So, my question is to Ameera ma'am. Ma'am, in one of the interviews you had told that Specialty and especially now the Genomics is a high value in tests, but it won't be margin accretive. However, it will increase the EBITDA per patient. So, I just wanted to get a sense, get light on the margin accretive nature of the same. And whether if it's not margin accretive, whether our efforts at cost and other synergies will help us drive up the margins?

Ameera Shah: Thank you for the question. Just to clarify, what I mean by margin accretive or higher EBITDA is, today, if you look at the group margin of the organization, close to about 25% on the MHL organic business, right? And if the question is that is genomics margin going to be higher than that on an individual stand-alone test, maybe at this point, no. The reason for that is because the gross margins on these super specialized tests are lower than they are on the routine tests. Having said that, but if an existing, who is already coming to you for certain tests or comes to you with a bundle of tests, some routine along with a genomics test, then that additional EBITDA that it adds to your sample is quite significant. So, I think the way we have to look at it is that are you picking up a single genomic sample on its own? Or are you picking it up as a battery of tests, which includes genomics?

And if you're picking it up as a battery of test largely, the majority, it will still give you a margin accretion versus if you're picking up only a genomics test alone because the gross tests are different. I hope that's more clear.

Moderator: The next question comes from the line of Naman Bagrecha from IIFL Capital.

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Naman Bagrecha: Just 2 specific questions. If we look at the labor force charges, there's an impact of almost around

2 to 3 bps of that which would be recurring in nature at least. So if that amount has, let's say, impact on a recurring basis?

Sameer Patel: On a recurring basis, I think the amount would be around INR 5 crores to INR 6 crores, not more than that.

Naman Bagrecha: Okay. And can you also give us, let's say, the capex guidance? What I believe was that we were targeting to improve the CC per lab ratio from, let's say, around 20, 22 collection centers per lab to around 30. So, by when do we expect that?

Surendran Chemmenkotil: Well, I mean, that's an aspirational goal. I already mentioned it that way, right? And that the number of labs are getting consolidated and the centers are going up. It's a gradual progression.

And I know it's very difficult for me to put a time line to when will you hit the 30 number. And I also believe that it just doesn't matter when you will hit it, but as long as you're progressing in the right direction.

Naman Bagrecha: Okay. Okay. So, any capex guidance that you would like to give for the next 2 years?

Surendran Chemmenkotil: At this point of time, n

o. And I think at a later stage, maybe we'll be able to share with you.

Moderator: Ladies and gentlemen, that was the last question for today. I now hand the conference over to the management for closing comments. Thank you, and over to you, sir.

Ameera Shah: Thank you, everyone, for joining us for this Q3 call. As you can hear from the team, we are very strong and confident on the direction that we are heading in and the commitments made to our investors and shareholders in the beginning of the year around margin expansion and a overall revenue guidance. I think we've seen that we've been able to show that in the first 9 months, and we continue to see that in Q4 as well.

We are very excited about the coming year, and we hope that at the end of this quarter or Q4, we'll be able to give you a strategic plan and a guidance for next year as well. And I think the market continues to be strong and Metropolis' position continues to be in the leadership position. So, thank you all for joining and look forward to speaking soon.

Moderator: Thank you. On behalf of ICICI Securities, that concludes this conference. Thank you for joining us, and you may now disconnect your lines. Thank you.

Call: May 2026

Ref: MHL/Sec&Legal/ 2026-27/15 May 19, 202 6

To,
BSE Limited
Scrip Code: 542650 National Stock Exchange of India Limited
Scrip Symbol: METROPOLIS

Dear Sir/Madam,

Sub: Earnings call transcript for Q4 FY2 6

Pursuant to Regulation 30 read with Schedule III of the SEBI (Listing Obligations and Disclosure Requirements) Regulations, 2015, please find enclosed herewith the transcript of Q4 FY2 6 earnings conference call held on Thursday , May 14, 202 6. The transcript is also available on the Company 's website i.e. www.metropolisindia.com

You are requested to take the above information on record.

Thanking you,
Yours faithfully,

For Metropolis Healthcare Limited

Kamlesh C Kulkarni
Head – Legal & Secretarial

Encl: A/a

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“Metropolis Healthcare Limited
Q4 & FY26 Earnings Conference Call ”
May 14, 202 6

E&OE - This transcript is edited for factual errors. In case of discrepancy, the audio recording uploaded on the stock exchanges on May 14, 2026 will prevail.

MANAGEMENT : MS. AMEERA SHAH – PROMOTER , CHAIRPERSON AND
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MR. MOHAN MENON – CHIEF MARKETING OFFICER –
METROPOLIS HEALTHCARE LIMITED

MODERATOR : MR. GAURAV TINANI – AMBIT CAPITAL

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Moderator: Ladies and gentlemen, good day, and welcome to the Q4 and FY26 Earnings Conference Call of Metropolis Healthcare Limited, hosted by Ambit Capital. This conference call may contain forward-looking statements about the company, which are based on the beliefs, opinions and expectations of the company as on date of this call. These statements do not guarantee the future performance of the company, and it may involve risks and uncertainties that are difficult to predict.

As a reminder, all participant lines will be in the listen-only mode. There will be an opportunity for you to ask questions after the presentation concludes. Should you need any assistance during this conference call, please signal an operator by pressing star and then zero on your touchtone telephones. Please note that this conference call is being recorded.

I now hand the conference over to Mr. Gaurav Tinani from Ambit Capital. Thank you, and over to you, Mr. Tinani.

Gaurav Tinani: Thank you, Swara, and good morning, everyone. On behalf of Ambit Capital, we welcome you all on the Q4 and FY26 Earnings Conference Call for Metropolis Healthcare Limited. Today, on the call, we are joined by Ms. Ameera Shah, the Promoter, Chairperson and Whole-Time Director; Mr. Surendran C., the Managing Director; and Mr. Sameer Patel, the Chief Financial Officer for Metropolis Healthcare Limited.

We will begin the call with opening remarks from the management, followed by a question-and-answer session. Thank you, and over to you, ma'am.

Ameera Shah : Thank you so much, and good morning, everyone, and thanks for joining us today for the Q4 and the FY26 earnings conference Call of Metropolis Healthcare. As mentioned, I'm joined by Suren, by Sameer, and also by Mohan, the CMO, and the rest of the leadership team and the IR Advisors. We've uploaded our investor presentation and related documents on the stock exchanges and the company's website, and I hope everyone has had an opportunity to go through the same.

Let me begin with a brief perspective on the broader diagnostics landscape before I move into the company-specific updates. The diagnostics industry in India continues to evolve in a very constructive direction. We are seeing a steady shift towards organized trusted players like Metropolis, as doctors and consumers are placing greater emphasis on quality standards, scientific expertise, lab compliance, and an overall superior experience. This transition reflects a maturing market where trust and reliability are becoming central to decision-making.

At the same time, the industry is also gradually moving beyond routine testing-led growth with stronger contributions now coming from specialty diagnostics, wellness, and more complex and clinically relevant testing. These areas not only enhance the depth of diagnostic insights but also align more closely with evolving health care needs.

In parallel, factors such as deeper digital engagement, improved consumer awareness, and a rising focus on longitudinal health monitoring are creating new opportunities for scaled and

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credible diagnostic platforms. The broad direction has been visible in our earlier quarterly commentary as well, where we've consistently highlighted preventive care, specialty testing, AI-led enablement, genomics, and customer engagement as key structural drivers shaping the future growth of the sector. While competition will always be there, we believe the structural runway for us is strong without any unreasonable or disruptive competitive environment.

Metropolis remains well-positioned in the industry because our moat is built on capabilities that are difficult to replicate at scale: consistent lab quality, deep and long-standing doctor engagement, strong scientific and clinical expert

ise, best-in-class tech platforms, and a highly standardized operating model across the network. Together, these elements create a foundation that goes well beyond service-level differentiation and support sustained performance over time. In diagnostics, trust is built not only through brand visibility and geographical reach, but through the accuracy, consistency, and reliability of what happens behind the scenes in the lab. Institutional knowledge built over years of experience further strengthens this backbone by ensuring that complex cases are handled with depth and accuracy. This remains one of the key reasons why Metropolis continues to earn trust with doctors and patients and sustain its

leadership in the premium diagnostics space.

On digital and AI, our approach continues to be practical, measured, and focused on high-impact use cases. We do not see AI as a near-term disruptor in pathology, but an important enabler of productivity, service quality, customer engagement, and workflow efficiency. Over the past few quarters, we have strengthened our digital and AI tools across customer and partner platforms, lead management, workflow automation, contact center quality, and selective diagnostic applications.

Together, these initiatives are helping us build a stronger operating platform, improve productivity and efficiency, enhance customer experience, and support more scalable and sustainable growth for years to come.

Our genomics journey has continued to advance steadily and remains a key strategic pillar for the future. With the integration of Core Diagnostics and the ongoing strengthening of our central genomics platform, we are deepening our capabilities across critical areas such as oncology, reproductive health, inherited disorders, and precision diagnostics.

This platform is now anchored by 2 CAP-accredited genomic labs in Gurgaon and Bombay, along with the growing team of genetic counsellors across the country, strengthening our ability to deliver high-end genomic testing with quality interpretation and scale. This integration has also enabled us to expand access to high-end genomic testing across India through both our B2C and B2B channels.

Anyone can add machines, but the real difference in results lies in the personalized interpretation needed for every patient, which rests on institutional expertise and knowledge, not on the equipment. As a result, Metropolis is increasingly positioned as a trusted and credible leader in specialty genomics testing, capable of delivering advanced solutions at scale. We see genomics

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evolving into a significant growth driver over time, particularly in disease segments where precision, nuanced interpretation, and strong scientific engagement are essential.

The participation and response we received at our recent genomics and scientific symposiums in Mumbai and Chennai have been so strong, clearly depicting that there are big gaps in the market for good clinical reports, which we can fill. We believe this is a strong testament to the clinical trust in Metropolis and to the relevance of the scientific platform we are continuing to build in genomics.

At the same time, we continue to make steady progress on productivity and platform modernization. Over the last few quarters, we have worked on lab platform upgrades, infrastructure consolidation, vendor consolidation, and more optimal use of technology across the network. These efforts are helping simplify the operating backbone, improve efficiency, and support structural cost discipline over time.

On the inorganic front as well, integration is progressing broadly in line with our expectations. For Core Diagnostics, we have committed that within 4 quarters of doing the acquisition, we would move from a negative 2% EBITDA to a high-single-digit EBITDA in Q4. We have completed this m

ission.

We are on our path for our 3-year commitment for a 20%-plus EBITDA at core. Over the past few quarters, our priority has been stabilization at the core, driving process alignment, integrating platforms, and realizing synergies across the acquired businesses so they are seamlessly embedded within the Metropolis umbrella and operating DNA.

We're now beginning to see tangible value emerge, not just from increased scale, but from enhanced capabilities, lab consolidation, better network utilization, procurement efficiencies, and a stronger foundation to support future growth. As we move ahead, our focus will shift more towards driving volumes and scaling the platform. This, in turn, will unlock efficiencies and leverage, providing a clear pathway for margin expansion over time.

Speaking of FY26, it's been a strong and well-executed year for Metropolis. We delivered organic revenue growth of 13.7%, better than our stated guidance of 12% to 13%. On a normalized basis, organic margins expanded by around 140 basis points to 25.9%, supported by sustained productivity improvements, ongoing lab platform upgrades, and consolidation initiatives.

As you know, we have also done a bonus share issue in March '26 to enhance the liquidity of our shares for shareholders. I'm pleased to share that the Board has also recommended another interim dividend of INR1 per share. This reflects the strength of our financial position and our continued commitment to creating long-term value for shareholders.

Looking at the past 3 years, we have accomplished the following: an organic CAGR revenue growth of 13% on a like-to-like basis, which is largely driven by test volume and patient volume growth; restored organic revenue margins to pre-COVID levels of around 26%; expanded lab capacity by more than 50% and increased our presence to more than 750 towns and cities across India; built a strong retail D2C mindset, resulting in self-referrals contributing 40% and B2C

business accounting for 60% of revenue, both creating a more sticky, higher -margin business model for the future. We expanded into basic radiology and vital checks in some centers on a pilot basis and are encouraged by the response.

We also, in the last 3 years, established a robust technology architecture from the ground up, enabling digital channels to grow from 0% to 25% of revenue, while making the organization significantly more automated. We also strengthened our scientific capabilities and enhanced our reputation amongst doctors and hospitals , and added the entire genomics segment.

This will lead to better price elasticity and the highest specialty mix in the industry. We also expanded our North India business, increasing the contribution from mid -single -digit contribution to revenue to 17% of revenues. And we added , enhanced, and rebuilt a stable and better -quality management team that can take the business forward.

Over the next 3 years, the vision is to grow at a CAGR of 14% to 15% in revenue . Organic revenues would come primarily via patient volumes, RPP growth , and price increases. And , an additional part of it would also come by adding strategic and value -additive acquisitions. Even though we are building a target for acquisitions, these would only happen if it's at the right deal at the right price and not just to add growth.

We are going to reimagine our processes through automation and technology enablement to drive center productivity and margin expansion with the goal of achieving a sustainable group EBITDA margin of 27% to 28% over the next 3 years, supported by profitability across every regional market and bringing core to a 20% plus margin profile.

We would like to continue to be the most respected scientific brand, not just in the West and South, but also in the North and East of India, by building relationships

and trust amongst

doctors. We're also building a network of 100 mini hubs over the next 3 years, which will encompass pathology and basic radiology to service retail and corporate clientele. 50 mini hubs would be the existing collection center locations upgraded to mini hubs , and 50 would be new centers to be set up.

We expect these centers to allow us to provide more services to our existing B2C customers , as well as corporate clients and insurance. We would expand our asset -light collection center network by adding 1,500 more centers, taking the lab -to-center ratio to 1: 35 from the current 1:24, and enhancing center productivity of existing centers by 20% over 3 years.

We would also like to build capability in Metropolis to build a tech -enabled D2C vertical, focusing on enhanced chronic business, acquired and serviced via digital and physical channels. Digital revenue in Metropolis has a higher customer lifetime value than brick -and-mortar businesses, and this, while enhancing the digital contribution, would enhance the RPP growth. Overall, the direction of the business continues to strengthen with stronger foundations, sharper execution , and better visibility ahead.

With that, I'll now ask Sur en to take you through the operational performance and key business drivers for the quarter and the full year.

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Surendran Chemmenkotil: Thank you, Ameera, and good morning, everyone. Let me take you through our performance for the quarter and the full year, and then spend some time on the key business drivers across channels, network productivity, margins , and operating outlook. For quarter 4 '26, MHL Group revenue stood at INR 425 crores, reflecting a year -on-year growth of 23 % with EBITDA margin of 25.4 % . For the full year, revenue stood at INR 1,646 crores, growing 23.6 % year-on-year with an EBITDA margin of 24.4 % .

Just as importantly, the underlying quality of growth remains strong, supported by patient growth, realization improvement, better mix and productivity gains. On an organic basis, revenue grew by 14.7 % in quarter 4, driven largely by patient volume growth of 9.3 % and realization improvement of around 5 %

For the full year, organic revenue growth stood at 13.7 % , as Ameera mentioned. It is also important to note that unlike last year, we did not take a price increase in quarter 4 due to GST reasons. This indicates that the quarterly performance was driven by healthy underlying demand, mix improvement , and better execution, leaving the price lever as an opportunity for growth later in the year.

Full year '26 was not just important for the performance we delivered, but also for the progress we made on several structural changes initiated last year. This includes strengthening the network through lab consolidation and a sharper center -to-lab ratio, driving productivity and margin improvement initiatives across the lab platform changes, building stronger and better quality B2B business, progressing our M&A integration playbook, and advancing our digital agenda. Importantly, these are not one-time actions. They are structural changes intended to

improve the quality and sustainability of growth in revenue and profitability over time.

Let me first talk about the channels. In B2C organic revenue, we continue to see strong traction with revenue growth of 14.7 % in the quarter and 14 % for the full year. This was supported by the healthy demand across wellness, specialty, and routine testing, along with better customer engagement through both physical and digital channels.

In B2B organic, revenue grew by 14.7 % in quarter 4 and 13.3 % for the year. We have been working on improving ease of doing business, enhancing service consistency, deepening institutional and corporate relationships, and building a more sustainable portfolio with better unit economics.

From

a segment standpoint, for the organic revenue, TruHealth and Specialty continue to perform very strongly. TruHealth and Specialty grew by 20 % and 17 %, respectively, in the quarter and by 21 % and 16 %, respectively, for the full year. These categories continue to improve both customer engagement and the overall mix quality of the business.

Let me now come to network and productivity because this is becoming an increasingly important part of our journey. Over the last 2 years, we have invested significantly in expanding and strengthening our lab and service center backbone. That heavy build-out phase is now largely behind us, and the focus has clearly shifted from expanded capex to throughput-led productivity.

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In other words, the priority today is not just to add more infrastructure, but to get more output, better efficiency, and stronger leverage from the infrastructure we have already created. This is a key shift in our operating model and one of the important drivers of margin improvement going forward.

During the year, we added 490 centers, taking our total network to over 5,000 collection centers across more than 750 towns and 212 labs. At the same time, the productivity of the existing network improved with the same lab growth at about 14%.

And the center-to-lab ratio, which strengthened from 20:1 to 24:1, is a clear testament to the focused execution over the last year to improve network density and drive higher throughput from the infrastructure we have already built. Over the next 18 months, we expect to strengthen the feeder center network further and move this ratio closer to 30:1 in many markets, depending on cluster maturity, and over a 3-year horizon, we want to take this to 35:1 lab-to-center ratio. Another important driver of improvement this year has been our productivity and margin agenda. During the year, we continued to work on the lab testing platform upgradation and standardization, equipment vendor consolidation, better inventory discipline through barcoding, and more optimal use of technology in labs across the country.

These interventions are improving turnaround time, material productivity, and throughput across the network, and the benefits started coming in in quarter 4. We expect them to strengthen further over the next 2 quarters as implementation progresses across the network.

Organic EBITDA margin for the quarter stood at 27.2 % compared to 18.5 % in the same period last year, while the full year EBITDA margin came in at 25.9 %. Although the prior-year margin was partly impacted by onetime acquisition-related cost then, EBITDA margin expanded by 140 basis points even after adjusting for these costs.

This improvement was driven by a combination of better operating leverage, stronger specialty and wellness mix, productivity gains, integration synergies from acquisitions, and ongoing efficiency initiatives across the network.

On acquisitions, the portfolio continued to progress well. Core Diagnostics improved to a high-single-digit margin in quarter 4 '26, while Dehradun, Agra, and Kolhapur acquisitions continue to operate at margins above the company average. Over the past few calls, we have been clear that this year one is about integration, cleanup, and efficiency, followed by revenue acceleration on a stronger platform.

We believe we are now building a repeatable playbook for M&A integration across processes, people, systems, quality standards, and commercial synergies. And that will remain an important capability for us going forward.

To summarize, financial year '26 was a year in which both our business fundamentals and our operating model became meaningfully stronger. As we exit the year, we are seeing strong momentum across multiple areas of the business. Patient volume growth remained healthy in the 7% to 8 % range, supported by

by the lab additions made over the last few years, continuous center

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expansion quarter after quarter, and the acceleration of our digital agenda. We are optimistic of

sustaining the same going forward.

At the same time, RPP growth is being driven by an improving mix, led by enhanced TruHealth offerings, a higher specialty mix, and increasing contribution from genomics. Together, these drivers are helping us deliver consistent and broad-based revenue growth. Importantly, this growth is being complemented by a sustainable margin improvement. Our lab platform upgrades, vendor consolidation initiatives, and multiple productivity improvement programs are now translated into stronger operating leverage across the network.

We are also seeing encouraging progress in improving network productivity, strengthening the quality of B2B business, and successfully integrating acquisitions through a more structured and scalable playbook. What is particularly encouraging is that these gains are not driven by one-off factors, but by structural changes we have been implementing across the business over the last year. This gives us the confidence that the business is not only growing, but growing in a more sustainable, efficient, and high-quality manner. With this, I hand over the call to Sameer, who will take us through the details of the financial highlights. Thank you, and over to you, Sameer.

Sameer Patel : Thank you, Suren, and good morning, everyone. Let me briefly walk you through the key financial highlights for quarter 4 and full year FY26 for both the organic business and MHL Group. Let's start with an organic business. Organic business has delivered strong, consistent performance during the quarter and the year, driven by healthy growth across both B2B and B2C segments.

Quarter 4 FY26 revenue stood at INR 392 crores, growing 14.7% year-on-year, supported by 9% growth in both patient and test volume. Full year FY26 revenue stood at INR 1,510 crores, reflecting 13.7% year-on-year growth with patient volume growth at 7.5% and test volume growth at 8%.

B2C and B2B revenue for the quarter grew at 15% year-on-year, while full-year FY26 B2C and B2B revenue grew by 14% and 13.3%, respectively. Our focus segments continued to perform well. The TruHealth segment contributed 19% of FY26 revenue, grew by 21% year-on-year.

And Specialty segment contributed 37% of revenue with 16% year-on-year growth.

With respect to margins, quarter 4 EBITDA stood at INR 107 crores with a margin of 27.2%, grew 69% year-on-year. Full year FY26 EBITDA stood at INR 392 crores with a margin of 25.9%, reflecting 29% year-on-year growth. Quarter 4 PAT stood at INR 55 crores with a margin of 14.1%, grew 89% year-on-year. Full-year FY26 PAT stood at INR 194 crores, with a margin of 12.8% and 33% year-on-year growth.

Now, talking about MHL Group performance. At the MHL Group level, performance remained robust across revenue, volume, and profitability. Q4FY26 revenue stood at INR 425 crores, 23% year-on-year, with a patient volume growth of 11% and test volume growth of 14%. Full year FY26 revenue stood at INR 1,646 crores, reflecting 23.6% year-on-year growth with the patient and the test volume growth of 12% and 13%, respectively.

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Across channels, quarter 4 B2C and B2B revenue grew by 20% and 28% year-on-year, respectively. Full year FY26 B2C and B2B revenue grew by 19% and 31% year-on-year, respectively. Segment performance continued to strengthen. The TruHealth segment contributed 18% for FY26 revenue, grew by 27% year-on-year. The Specialty segment contributed 39% of revenue with a strong 32% year-on-year growth.

Profitability remained strong. Quarter four EBITDA stood at INR 108 crores with a margin of 25.4%, grew 71% year-on-year. Full

Year FY26 EBITDA stood at INR 401 crores with a margin of 24.4%, reflecting 32% growth year-on-year. Quarter 4 PAT stood at INR 51 crores with a margin of 12% and grew by 75% year-on-year. Full year FY26 PAT stood at INR 191 crores with a margin of 11.6%, grew by 31% year-on-year.

Now, talking about the capex for the year. We have incurred a capex the capex stood at INR 65 crores. As highlighted earlier, our capital allocation strategy will become increasingly selective and productivity-driven. Investments are now focused on targeted network addition, specialty test expansion, technology upgrades, and digital capabilities. With most of the lab network already in place, future growth is expected to benefit from improved operating leverage and enhanced productivity as volume continues to scale.

With this, I open the floor for Q&A. Thank you.

Moderator : The first question is from the line of Tausif from BNP.

Tausif : We have talked about our revenue growth guidance of mid-teens for the next 2 to 3 years. Can you give some more color on this? Can you tell us, in the last 2 quarters, have you seen some structural shift from unorganized player to organized player, and any competition from online players?

Ameera Shah : See, there's no third-party data available that tells you the move from unorganized to organized. But we have feet on the ground, the sense, like I mentioned in the speech, is that there is a movement happening, where consumers and doctors are finding more comfort in bigger brands,

which have more predictability, and also the specialty market is increasing . We have to remember that oncology and neurology are the 2 fastest -growing therapeutic segments globally over the next 10 years.

And as more of these issues come into India as well, it means more specialty tests will happen rather than just routine, which naturally will happen with the more organized players and less with the unorganized players. So this mid -teens growth, 14% to 15% that we've guided for the next 3 years , is obviously a combination of volume, RPP increase, as well as some price increase, and that's the breakup of it.

Tausif : And ma'am, do you plan to take a price hike in this fiscal?

Surendran Chemmenkotil: Well, at this point of time, we are not looking at a price increase. But as the year progresses, if there is a need for us to do it, we would not hesitate to do so .

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Tausif : Okay. And just a follow -up question on your guidance. We have guided the EBITDA margin of 27% to 28% next 3 years, but how should one see the next fiscal? Will be the range of 26%, that's quite achievable?

Surendran Chemmenkotil: So we'll definitely be looking at about 125 to 150 bps improvement in the coming year.

Moderator : The next question is from the line of Raman KV from Sequent Investments.

Raman KV : Just a follow -up on the guidance part. You mentioned that you will be growing at 14% to 15% CAGR over the next 3 years. How much will this be from the volume growth in terms of patient volume growth, and how much will be in terms of better price realization coming from Specialty and Core Diagnostics?

Surendran Chemmenkotil: So, about 8% to 9% of patient volume growth is what we are estimating for the coming fiscal for sure, right? And the remaining 5% will be coming from the realization.

Raman KV : Understood, sir. And sir, my next question is with respect to the Specialty division. For the past 2 years, the contribution the revenue mix from Specialty division has been around -- hovering around 35% to 37%. So are you planning to increase your wallet share in the Specialty division -- like contribution from your Specialty division going forward? Or do you think this is healthy

Surendran Chemmenkotil: Yes. Definitely, I think there are 2 big levers we have beyond the existing specialty testing capabilities

s . One, of course, we talked about the genomics journey that we have already started and are accelerating. And the second one is the Core Diagnostics' product capability that we have, which we'll use across our network. I mean, these two things added to our otherwise the Specialty portfolio that we have to be able to take the Specialty contribution further higher.

Raman KV : So, are you expecting that this year, it will be around 40%?

Surendran Chemmenkotil: I think so.

Moderator : The next question is from the line of Sudarshan Agarwal from Axis Capital.

Sudarshan Agarwal: In the initial comments, you said you are adding some 100 mini hubs. Can you tell me the nature this is how this is different versus your collection centers? And how many do you have in your existing network? And what is the differentiation in terms of capex requirement for, let's say, mini hub versus your normal collection centers?

Ameera Shah : So normal collection center just obviously we collect blood samples there and then transport it to lab for testing. One of the things that we have done over the past 2 years is, we have added some ECG, x -ray, and in some limited locations, sonography as well to sort of see whether we are able to increase the RPP per patient by doing that. And we have found that, that has helped us. And therefore, we wanted to go out and do this in a bigger way.

Now, it will not be about just adding X -ray, ECG, but maybe some more basic radiology modalities, not like a CT MRI, but more basic modalities that could include, in some cases, the bone density machine, in some cases, a mammogram -- mammography machine, or in some cases, a bigger X-ray machine.

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And the idea is basically that you're creating enough services at the local level. It is not just blood, but blood plus radiology, plus , in some cases, it requires a consultation possibility, and making these into slightly bigger, more service infrastructure centers, which can be then utilized not only for retail customers, but for corporate customers as well.

Sudarshan Agarwal: Okay. And the capex would be higher for these centers?

Ameera Shah : Of course.

Sudarshan Agarwal: By how much?

Ameera Shah : It will be higher than a collection center, but it will probably be similar to sort of a satellite lab.

About INR 30 lakh, INR 40 lakh is approximately what we are estimating at this point in time.

Sudarshan Agarwal: Yes. Got it. And last one more on the bookkeeping side. So I was looking at your bridge through organic to growth. So when you mentioned that Core Diagnostics is at high -single digit, but if I subtract your MHL Group revenues and EBITDA from your MHL organic numbers, the M&A

portion margin comes to around 4% to 5%. So is there some intersegment elimination that is happening? Because I cannot reconcile the numbers to high single digits otherwise?

Sameer Patel : Certainly. So, what happens when the business is generated from MHL and the processing happens on the Specialty and the Core, at the consolidation level, there is an elimination of revenue and the margin that will be there. And therefore, you will see that.

Sudarshan Agarwal: Okay. That benefit may be sitting on the organic margin that you have?

Sameer Patel : Yes.

Sudarshan Agarwal: Got it. And one last bookkeeping. Your depreciation increased quite sharply on a Q-on-Q basis. I assume this is due to the genomic machinery that was expected. Or was there some one-off or impairment that happened during this quarter?

Sameer Patel : It's a mix of a few things. Certainly, genomic machine has come. Also the capex investment in the later part of the year, and hence, Q4 sees the full depreciation impact. Also, the network expansion was more aggressive in the second half. So that's where those elements are reflected in depreciation.

Moderator : The next question

is from the line of Surya Patra from PhillipCapital.

Surya Patra : My first question is on the growth side. What I'm seeing that, okay, the Tier 1, Tier 2, Tier 3, in all the regions, the growth looks similar because the revenue mix is also similar on a Y-o-Y basis. So the general understanding is that Tier 2 and Tier 3 should be growing faster than the cities because cities would have all possible forms of competition. So, question is that, okay, so are we not anticipating better growth or faster growth in the beyond Tier 1 market, and hence, progressively better growth with the mix improving towards that side?

Surendran Chemmenkotil: Let me help you to decode the tier-wise growth, Tier 1, Tier 2 growth. Tier 1, we have added Core Diagnostics is coming on largely on the Tier 1 setup, and you see a much higher growth in Metropolis Healthcare Limited

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Tier 1 for the last year. Similarly, on Tier 2, you have cities like Dehradun, Agra, et cetera, that have been added, and you will find a higher growth in Tier 2.

And Tier 3, we are growing at about 26% to 30% in the last few quarters. So definitely, the Tier 3 and above is showing a much higher growth than the Tier 1, Tier 2 on a like-to-like basis, and it will continue to be so.

Surya Patra : Okay. And these digital initiatives that we are talking about, should we consider them as a kind of lever for margin expansion? Or it drives even volume?

Surendran Chemmenkotil: Yes. It drives volume first. And of course, the cost of servicing is relatively lesser than that of non-inorganic channel. So, definitely, the margins get better as our digital revenue scales up.

Ameera Shah : Just to add, it depends on the way you acquire a customer, right? I mean, look, there are many health tech companies that are looking at deep discounting to acquire customers, which is not the Metropolis way. So if you do deep discounting to acquire, then obviously, that business may not have strong unit economics and may not be positive profitably.

But the kind of business that Metropolis is acquiring digitally is more on the back of already a strong brand that we have in physical channels on the ground, which allows us to, therefore, have a lower cost of acquisition, a lower cost of servicing, and a higher customer lifetime value, which could therefore result in a better margin for Metropolis. It doesn't mean it's the standard for the whole industry.

Surya Patra : Another point was about the inorganic growth. Is there any scope for that? Because it looks like that, okay, having done the Core Diagnostics now, we are focusing more on the organic part. And we also do understand that the targets in the diagnostic space within India, if you see, are more or less singular labs largely. Hence, the M&A possibility looks limited or less. So what is your view on that, on the scope of the inorganic growth?

Ameera Shah : See, actually, there are -- like you said rightly, there are some 3 lakh labs in India, and 90-odd-percent of them are single labs. There are some chain labs, which have been created regionally, and there are many, many assets available for acquisition. But we generally find that the quality of many of the assets available for acquisition may not meet our standards for what we would like to buy.

So, we will remain selective, but we continue to see opportunities at play. We continue to explore them. And obviously, when something looks like the right candidate, which is adding something strategic to us at the right price, then obviously, that's when we go ahead to try to close the deal. But broadly, I think we can continue to see inorganic action for the next few years.

Surya Patra : Okay. Just one clarification from my side. What portion of our, let's say, test volume would be led by insurance companies?

Surendran Chemmenkotil: That's very insignificant at this point in time. We don't

have high volumes from the insurance segment as of now. We are just building our portfolio at this point in time.
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Moderator : The next question is from the line of Shyam Srinivasan from Goldman Sachs.

Shyam Srinivasan: Just one on the opening remarks of trying to increase our branch productivity our network productivity by 20% for mature stores. Can you just double -click on that, please?

Surendran Chemmenkotil: So we have set up all these, let's say, last year, 500 centers, and the year before that, 500 centers.

In the initial days, once the centers are set up, we just drive to improve the walk -ins into these centers by engaging with the doctors nearby, and we're also engaging with the hospitals, clinics, et cetera, nearby, and also doing many other activities to engage with the customers in the neighborhood.

So, that helps us to improve the productivity of each of the centers that we have set up. So I mean, on a year -on-year basis, about 8% to 10% improvement on the productivity of the same center, over a 3 -year period, we are definitely able to get a 20% improvement from the same centers that we have set up in the last maybe a couple of years.

Shyam Srinivasan: And is there any absolute number that we are starting with, like this is the right numbers per center, if there is any metric like that?

Surendran Chemmenkotil: So for us, in our own centers, for example, the average productivity is around INR 3.5 lakhs to INR 4 lakhs per center. When we set up in year 1, this comes to around by the end of year 1, we do about INR 1 lakh, INR1.5 lakhs, and we bring this into above INR 3 lakhs at the end of the third year. So that's the number for our own centers, and this number is different -- differ for the franchise centers and the rural centers.

Shyam Srinivasan: Got it. Very helpful. My second question on the other comment on digital channels, right, that they now contribute -- did I get it right when you said it's contributing 25% of revenue? If you could explain that, please? And what are some of the measures? I know we've been investing in technology. But just want to understand the nature of this channel? Is it like D2C? Or am I missing something?

Surendran Chemmenkotil: So these are largely D2C. Let me talk about 3 distinct channels here. One, of course, our Metropolis app, we drive customers through the Metropolis app and engage with them. And secondly, I mean, we acquire customers through our website, where we will encourage the customer to come and place his request for a test.

And the third one is our customer data platform, which is a CLM engine that we talk about, our existing customer. We reach out to them through digital modes and give them what the next best action is, or, on the basis of their past testing trends, what's the right thing for them to do in the coming days. And these are 3 big initiatives that we do to drive the digital customer acquisition.

Moderator : The next question is from the line of Alankar Garude from Kotak Institutional Equities.

Alankar Garude: Sameer, can you elaborate further on the intersegment eliminations? And how long will these eliminations continue?

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Sameer Patel : It will be there always because when the core business is going to process genomics business and oncology business, and the revenue is generated across the network. That is the synergy benefit that we are expanding the test menu across the country to all our clients and clinicians. That is where the revenue is generated, and then the processing will happen. So that's why the intercompany transfer will happen, and the elimination will be there.

Surendran Chemmenkotil: If I can just add to what Sameer said, one of our objectives is to drive high -end oncology revenue of the core products through th

e MHL distribution network. So we will keep on stepping up this particular engine and start selling more and more core products through the MHL distribution engine. So you will -- as the revenue grows, and of course, there will be more and more intercompany adjustments. But when you look at the group level, this anyway gets utilized, and we'll report the net number at the group level, as you know.

Alankar Garude: Got it, sir. Just thinking whether, ideally, we should split the intersegment eliminations between the organic and the inorganic EBITDA when you report it in your presentation. Would that be a better approach? Or do you think organic EBITDA is what the true EBITDA is as we report in the presentation?

Ameera Shah : Honestly, we can go and split the heads, but the reality is now the acquisition is a year old. So everything is going to be organic anyway from April 1. So it really doesn't matter. It will all get consolidated into the group anyway.

Surendran Chemmenkotil: Quarter 1 onwards, you will be seeing only one number in our presentation, which is the organic or a company -level number. Last year, we provided this because acquisitions and integrations were just happening.

Alankar Garude: Fair enough. The second question is, you have been seeing a pretty good increase in revenue per patient, as well as revenue per test, despite not taking any price hikes over the last few quarters. Now, premiumization and TruHealth have been important drivers. Specifically, on TruHealth,

based on the visibility you have today, from 19% currently, to what level can the mix increase over the longer term?

Surendran Chemmenkotil: Well, I mean, this can definitely go beyond 25% as well, I mean, in the next 2 to 3 years period. And I mean, every year, I think we are moving this up by a couple of percentage. Maybe in the coming year, we will further step up our efforts to make it faster because we are now, in the TruHealth packages, we are including vital checkups, consults, basic radiology, and also going forward, maybe some of the other adjacent services we'll keep adding. So the overall RPP will keep getting better on the TruHealth packages. So we would like to see this moving faster the contribution of TruHealth moving faster in the coming years.

Moderator: The next question is from the line of Kunal Thanvi from Banyan Tree Advisors.

Kunal Thanvi: I had two questions. One was on the gross margin expansion that we have seen for Metropolis and for other players as well. So, can you double-click on this and explain what is driving the gross margin improvement? Is it the scale advantage that we're seeing from our suppliers? Or does it also have to do with the on-ground competitive intensity kind of softening in the last 12 months?

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year? Because the trend has been true for almost all the diagnostic players. That was the first question.

The second question is on our core markets, something like Mumbai. What kind of volume growth are we witnessing there? Because in a few calls, you said that you've been gaining market share in the core markets like Mumbai as well. Is the trend still continuing?

And the last one was, you talked about the brand expansion is largely behind us, and from here on, we will see operative leverage. If you can, again, double-click on the current utilization level on a consolidated basis and how that is expected to move around in FY27 and then in '28? These are the three questions.

Surendran Chemmenkotil: Okay. Let me address the first question, which is the gross margin improvement. There are two things that are really helping us to improve the gross margin. One is, like we mentioned in our speech, we have undertaken a lab platform consolidation and upgrades. We are just consolidating some of the vendors and moving into better, higher efficiency, more productive, technology

enabled, and scalable platforms. In this process, we will get a scale advantage.

In this case, we'll get some advantages coming out of the technology, and overall, our material consumption hence improves. And also during this year, we have introduced barcoding in all our 220 labs, which has also further helped us improve our material consumption percentages. And the second thing that we mentioned, last year, we did not significantly add any new labs. Our lab addition phase is over, I mean, a year back. In the coming year also, we are not planning to add a high number of labs in the network. So these 2 things will definitely help us to improve the gross margins.

And the second question was about -- what you said? The Mumbai City volume side. I think it's largely in line with the overall volume growth of the company. I mean, give or take 1% here and there, that's the number that we see across the geographies. But of course, we see a much higher patient volume growth on the northern part of the market for us, I mean, for this year, because of the new acquisitions that we have done on that side. But otherwise, largely across the country, we are getting stable and good patient volume growth across.

Kunal Thanvi: Sure. And on the utilization of the lab's overall capacity?

Surendran Chemmenkotil: See, we talked the lab, what you call, productivity has gone up by 14% during this year, right, because that's why we'll not add any more labs, and add more centers to feed them, the lab utilization goes up. See, all these labs are of different category. Some of them are big regional reference labs.

Some of them are global labs. Some of them are satellite labs, and some of them are greenfield labs. It's very difficult for us to give one unit as lab utilization per se in this business. So, I mean, we can talk more about when we set up a lab, we are just starting fresh. And then, every year, if the growth goes up by 14%, 15%, that's utilization growth that you can relate to.

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Kunal Thanvi: Got it. If I can squeeze 1 more question, that has to do with -- when we're talking about this productivity improvement, the efficiency improvement, the use of technology, et cetera, et cetera, from a longer-term perspective, meaning like 5 years, 10 years out, what kind of steady-state margins this business should have?

Because the kind of gross margin expansion that we have seen, what is -- at some stage, when the expansion -- when most of the labs that we're adding today will get kind of utilized to an optimal level, like this 27%, 28% EBITDA margin is the -- aspire to make -- or over a longer period, margins can even move further if we don't choose to invest aggressively in the way we've done in last few years?

Ameera Shah : See, margins can keep expanding till the time you have operating leverage, right? I mean, as long as you're able to keep growing the business and your cost growth is lower, you can keep having operating leverage. But finally, I think when we are building a business, you look at it not only from a short-term basis but also from a medium - and long-term basis, and keep reinventing yourself and investing in strategic initiatives that allow you to build second and third growth engines for the future .

So, maximizing the EBITDA may not, in our mind, be a smarter strategy. We believe a sustainable EBITDA of 27% to 28% over the next 3 years makes sense for us. And if we are able to generate more operating leverage, we would like to invest it back in the business, either in terms of adding services or building the brand further , or building further distribution. That's the direction we would like to go.

Moderator : The next question is from the line of Kaustav Bubna from BMSPL.

Kaustav Bubna: Previously, you mentioned that your growth in Tier 1 areas is less than in single digits. So

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wanted to understand really because -- let's take Mumbai, for example, right? So, in Mumbai , even a Tier 1 city, is broken down into different areas where consumption patterns would be different because of the type of people with different income levels staying in those areas.

So, if you take Bombay, for example, right, and you say what would your expected growth rate be in a place like Mumbai? And if you had to actually break that down between the areas of Mumbai, like North, South, East and more Eastern area like the Kalyan side, etcetera, what would you say Metropolis', how you call it, presence is in these areas? And how would the growth rate differ in these different areas of Mumbai?

Surendran Chemmenkotil: See, Mumbai is our biggest market, as you know. We have about 500 service centers across Mumbai, not only in the metropolitan area, but also spread into -- across even the outskirts of Mumbai. So, we are present in whichever -- whether it's Kalyan or Borivali, or I mean, Virar, Vasai, I mean, all areas we are spread across , and we keep increasing the network footprint in this area. So, this is a continuous process. And in Mumbai also, we are growing at 13%, 14% kind of revenue growth on a year-on-year basis.

It will be difficult for me to share further micro market segmentation of Mumbai and tell you how much is the Kalyan growth, how much is the Thane growth. That will be very difficult for Metropolis Healthcare Limited

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me to share with you. But at an overall level, most parts of the Mumbai are growing at 13%, 14% kind of growth.

Ameera Shah : And just to clarify, I think the Tier 1 growth is not single digit, but it's closer to 11%.

Moderator : Ladies and gentlemen, due to time constraints, that was the last question. I now hand the floor over to the management for closing comments.

Ameera Shah : Thank you to all for joining us today for the Q4 and the FY26 earnings call. We appreciate all the engagement and all the questions. We've had quite a tremendous year, not only a year with a strong organic growth, but a year of fundamental building, which has also helped us expand our margins quite significantly, and also a year where we did four acquisitions and have not only absorbed them but seamlessly integrated them into the business, realizing obviously a very fast growth rate of about 23.6% for the whole year.

We are very, very excited and positive about the next 3 years, and we believe that via three or four channels, or via three or four sort of pillars we will continue to build the business very strongly. We are doubling down on technology. We are doubling down on building engagement with our consumers and building the brand through experience , and doubling down on science and really building the scientific foundation even stronger.

Obviously, all of this is going to be enabled by the right people, the right talent, and finally to result in a commercially and a stronger business and a bigger business in the next few years to come.

So, we continue to remain excited, and we look forward to meeting all of you soon. And we are looking forward to actually planning a sort of day in our lab sometime in June or July. We'll come back to a lot of you, where we would be very happy to host you, show you around , and engage you even more deeply . Thank you so much.

Moderator : Thank you very much. On behalf of Ambit Capital, that concludes this conference call. Thank you all for joining us, and you may now disconnect your lines. Thank you.